Toward Neuroprotective Cooling of the Eye: A Scleral-Contact Thermoelectric Device

BY

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DISSERTATION

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To my parents and brothers, whose love and help was indispensable for this work: thank you for helping me push through.
To my friends, far and close, but particularly to those in Chicago: you were the support I needed, my second family.

Accessibility Statement

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Contribution of Authors

Chapter 1 is an original introduction by the author.

Chapter 2 includes work published as Y. Nakamura, L. Mecacci, and J. R. Hetling, "Therapeutic Levels of Hypothermia Achieved in Isolated Porcine Eyes Using a Scleral Contact Interface," *Annals of Biomedical Engineering*, vol. 54, 2026 (published online November 2025) [**1**]. Y. Nakamura and the author contributed equally to that article, as stated in its author-contributions declaration; Y. Nakamura worked with the author on the preserved- and fresh-porcine experiments during the author's final year of master's study. J. R. Hetling supervised the work. The article is published open access under a Creative Commons Attribution 4.0 license. The fresh-porcine recordings from that study were reprocessed by the author for Chapter 2 together with the human-donor data.

Chapter 2 also draws on two manuscripts in preparation of which the author is first author: an ex-vivo methods paper with Y. Nakamura, K. Fong, N. Shirdade, and J. R. Hetling, and a human–porcine comparison with J. R. Hetling. K. Fong and N. Shirdade carried out earlier work on the eye-cooling system on which the methods paper builds. As an undergraduate research assistant in 2018–2019, K. Fong designed, machined, and prototyped an earlier ocular cooling device and the instrumentation for measuring its temperature. In 2018, N. Shirdade carried out the thermal simulation of the cooling of the porcine eye.

Chapter 3 is the original work of the author.

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Chapter 5 is an original discussion by the author.

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List of Abbreviations

3D	Three-dimensional
ACA	Anterior ciliary artery
ACC	Animal Care Committee
AI	Artificial intelligence
AP	Anteroposterior
ARVO	Association for Research in Vision and Ophthalmology
ASA	American Society of Anesthesiologists
ATP	Adenosine triphosphate
AVMA	American Veterinary Medical Association
BRL	Biologic Resources Laboratory
CAD	Computer-aided design
CI	Confidence interval
CIRBP	Cold-inducible RNA-binding protein
CLEAr	Contact lens electrode array
CRA	Central retinal artery
CRAO	Central retinal artery occlusion
CV	Coefficient of variation
DAQ	Data acquisition
DC	Direct current
DSC	Differential scanning calorimetry
ERC	European Resuscitation Council
ERG	Electroretinogram
ESICM	European Society of Intensive Care Medicine
FDA	U.S. Food and Drug Administration
HACA	Hypothermia After Cardiac Arrest (trial)
LED	Light-emitting diode
Micro-CT	Micro-computed tomography
MOSFET	Metal-oxide-semiconductor field-effect transistor
MRI	Magnetic resonance imaging
NEVL	Neural Engineering Vision Laboratory
NTC	Negative temperature coefficient
OA	Ophthalmic artery
ONH	Optic nerve head
OP	Oscillatory potential
PCA	Posterior ciliary artery
PWM	Pulse-width modulation
RBM3	RNA-binding motif protein 3
RM	RetMap electrode (corneal ERG electrode)
RPE	Retinal pigment epithelium
SCEC	Scleral-contact eye cooler
SCR	Scleral contact ring
SD	Standard deviation
TEC	Thermoelectric cooler
TTM	Targeted temperature management
UIC	University of Illinois Chicago
USDA	U.S. Department of Agriculture

Summary

Ischemia of the retina and optic nerve, as in retinal artery occlusion, can cause permanent blindness within hours, and no established treatment protects the tissue. Mild cooling protects neural tissue from ischemic injury in laboratory studies, but cooling the whole body carries systemic risks. This dissertation concerns the scleral-contact eye cooler (SCEC), a thermoelectric device that holds a ring on the anterior sclera at 4 °C. It asks how much of that cooling reaches the back of the eye, whether the device can be made smaller, and whether the cooling can be detected, and tolerated, in a living eye.

In the first aim, temperature was recorded at five sites, from front to back, in isolated porcine and human donor eyes held in a warm bath. At the proximal optic nerve, the deepest site, human eyes cooled by nearly 4 °C within 20 min and porcine eyes slightly less. No difference between the species was detected there, and a difference above about 2 °C is unlikely.

In the second aim, a reduced ring of about half the original volume and a ceramic ring each cooled the proximal optic nerve of donor eyes nearly as well as the original ring. In a model of the perfused human eye, body and ring temperature set the posterior temperature far more than ring design did. An integrated control board was built and tested on the bench.

In the third aim, the device with the reduced ring was applied to one eye of anesthetized pigs for 30 to 120 min, with the fellow eye as control. Retinal cooling was sought through the electroretinogram. In the animals studied so far the ring stayed near 9.5 °C, after correction for a calibration error, or near 4 °C in the latest sessions, throughout every session. In all 15 sessions of the primary animals the b-wave of the cooled eye became slower than before cooling, by a mean of 5 to 8 ms, and in all but one wider, while that of the control eye did not slow; its oscillatory potentials were reduced. Both changes largely reversed after switch-off. The intraocular pressure rose no more in the cooled eye than in the control eye, redness and staining were worse only occasionally, and no adverse event was attributed to the device. One animal died of a complication of intubation, and the histology is still to be read.

The device therefore cools the back of an isolated eye by a few degrees without surgery and, in the living eye, changes retinal function as a cooler retina would. The temperature it reaches in a perfused eye, the safety of longer exposures, and protection against ischemia remain to be tested.

CHAPTER 1

Introduction

1.1. Clinical Motivation

Acute retinal ischemia is a cause of sudden and usually permanent monocular vision loss. Its prototype, central retinal artery occlusion (CRAO), is a form of acute ischemic stroke [**2**] with a population incidence of about 2 per 100,000 person-years [**3, 4**]. In data that also count branch occlusions, retinal artery occlusion as a whole is more than three times as common [**5**]. Few affected eyes recover useful vision. Among eyes with permanent non-arteritic CRAO without cilioretinal sparing presenting at counting fingers or worse, acuity improved at all in only 22% of those seen within a week of onset [**6**], and pooled natural-history data give a 17.7% rate of recovery to 20/100 or better [**7**].

No treatment has proven effective, and the 2026 guideline of the American Heart Association and American Stroke Association for acute ischemic stroke rates the usefulness of intravenous thrombolysis within 4.5 h for disabling non-arteritic CRAO as uncertain [**8**]. In pooled observational data, conservative measures such as ocular massage and anterior-chamber paracentesis were associated with worse recovery than natural history [**7**]. Intra-arterial fibrinolysis matched conservative care at a higher complication rate, and its randomized trial was stopped early [**9**]. The first two randomized trials of intravenous thrombolysis within 4.5 h, THEIA and TenCRAOS, found no significant visual benefit over aspirin [**10, 11**]; one fatal intracranial hemorrhage occurred with tenecteplase [**11**]. A third, placebo-controlled trial, REVISION, was stopped for futility at a prespecified interim analysis after 127 participants had been enrolled, and its results have not been published [**12, 13**]. The TenCRAOS investigators concluded that effective therapy remains elusive and that

alternative approaches warrant investigation [**11**], and the THEIA investigators that the low recovery rates raise questions about how long the retina tolerates ischemia [**10**]. Time is a further barrier. Vision loss is painless, and patients present late. Median symptom-to-door times were about 13 h in a Swiss national stroke registry and a US comprehensive stroke center [**14, 15**], and 17 h to first medical contact in an Australian audit [**16**]. In the registry and stroke-center cohorts, only about a quarter of patients arrived within 4–4.5 h.

Retinal and optic-nerve ischemia also complicate nonocular surgery. Perioperative visual loss follows about 8.6 per 10,000 cardiac operations, where retinal vascular occlusion accounts for most cases, and 3.1 per 10,000 spinal fusions [**17**]. Retinal artery occlusion alone complicates 7.8 per 10,000 cardiac procedures [**18**], and ischemic optic neuropathy accounts for most cases reported to the American Society of Anesthesiologists (ASA) Postoperative Visual Loss Registry after prone spine surgery [**19, 20**]. No treatment has been shown to be effective, and guidance for visual loss after spine surgery is directed at prevention [**20, 21**]. Unlike spontaneous occlusion, the period of surgical risk is known in advance, and a protective intervention could be applied before and during it. External pressure on the globe is itself a recognized cause of perioperative retinal artery occlusion [**19, 21**]. A device that rests on the eye must therefore not raise the intraocular pressure. That pressure is a safety endpoint of Aim 3, measured before and after each application to detect any lasting change, although a reading after removal does not capture a rise confined to the period of contact [**22**].

These settings share a need for an intervention that extends the retina's tolerance of ischemia, in addition to reperfusion. Preclinical evidence motivates localized cooling for that purpose (Section 1.3). Delivering the cooling to the posterior eye through an accessible surface is the engineering problem addressed in this dissertation.

1.2. The Eye

A device that cools the retina from outside the eye works through the layered wall of the globe and against its two circulations. This section describes these structures and the temperature field they sustain.

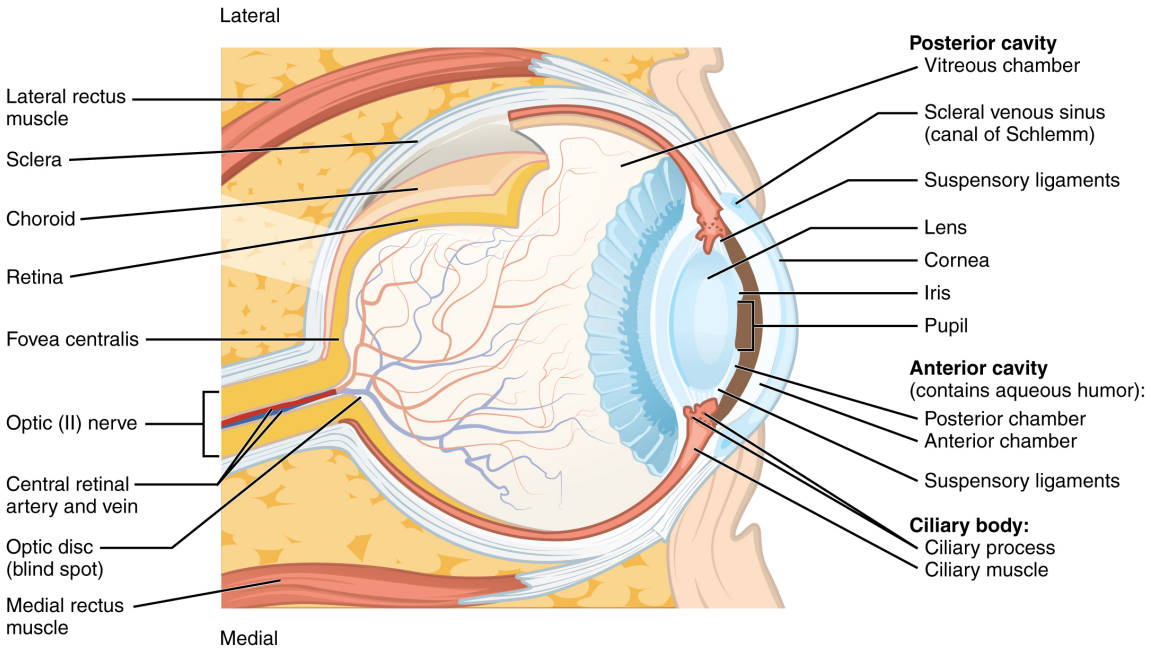

FIGURE 1.1. Cross-sectional anatomy of the human eye, showing the optical media, the anterior structures, and the sclera, choroid, and retina that form the wall around the vitreous cavity, with the extraocular muscles alongside the globe. From OpenStax College, *Anatomy & Physiology* (Houston: OpenStax, 2013), Fig. 14.13, "Structure of the Eye," via Wikimedia Commons, CC BY 3.0; reproduced unaltered.

1.2.1. Gross Anatomy. The human eye is a roughly spherical globe about 24 mm across [**23, 24**], organized as three concentric coats that together transform incident light into neural signals. The outer *fibrous* coat, comprising the transparent cornea continuous with the opaque sclera, provides mechanical integrity and the first refracting surface. The middle *vascular* coat, or uvea, comprises the iris, ciliary body, and choroid; it carries the greater part of ocular blood flow, although the inner retina is served by a separate retinal circulation (Section 1.2.2). The inner *neural* coat is the retina, the light-sensitive tissue that this work aims to protect. Figure 1.1 shows these

structures in cross section. The scleral-contact cooler described in this dissertation acts from the outer surface of the fibrous coat. The anatomy most relevant here is the layered wall between the point of thermal contact and the retina, together with the optical path.

The transparent cornea, aqueous humor, lens, and vitreous form the optical path to the retina. The avascular cornea supplies most of the refractive power; the lens focuses light through the vitreous, a transparent gel occupying the posterior cavity [**25**]. These tissues also participate in heat transfer between an anterior cooling interface and the posterior eye.

At the posterior pole the eye wall is a laminated structure comprising, from the vitreous outward, the retina, the retinal pigment epithelium (RPE), Bruch's membrane, the choroid, and the sclera [**25**]. Within the retina the photoreceptors lie outermost, apposed to the RPE, while the inner neurons and the ganglion-cell axons that form the optic nerve lie toward the vitreous, so that visual information is progressively processed as it passes inward. The retina is among the most metabolically active tissues in the body [**26**], and its outer, photoreceptor layers are avascular, depending on the underlying choroid across Bruch's membrane and the RPE (Figure 1.2).

The sclera forms five-sixths of the outer coat and provides the contact surface for the cooler. Its collagenous stroma is normally avascular, although vessels and nerves traverse it; nutrition is supplied by adjacent episcleral and choroidal networks [**28**].

Scleral thickness is markedly non-uniform. It is greatest at the posterior pole (1.0–1.35 mm), decreasing to 0.4–0.6 mm at the equator, reaching about 0.3 mm under the rectus muscles, and increasing again to roughly 0.8 mm at the limbus, where it blends with the cornea [**28**]. Histological measurements give lower values, 0.50 ± 0.11 mm at the limbus (Table 1.3).

1.2.2. Blood Supply. Blood flow matters here as a source of heat. The ocular circulation delivers blood at core temperature, and a cooling device must remove that heat as well as the heat stored in the tissue. The eye's high metabolic demand

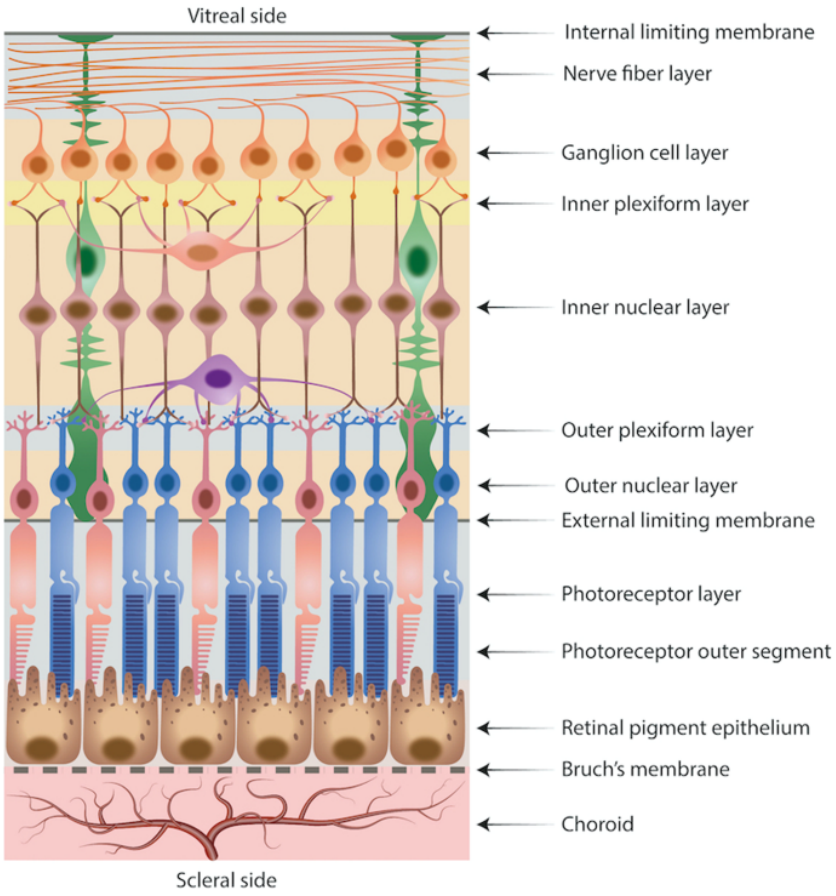

FIGURE 1.2. Retinal layers and their relationship to the retinal pigment epithelium, Bruch's membrane, and choroid. Reproduced without modification from Ferrara et al., Fig. 3 [**27**], CC BY 4.0.

is met by two largely separate vascular beds fed from a common source and drained into a common sink. Arterial inflow arises almost entirely from the ophthalmic artery (OA), usually the first major branch of the internal carotid artery, and venous outflow returns through the superior and inferior ophthalmic veins, which drain primarily into the cavernous sinus [**25**]. Three arterial pathways are relevant here. The central retinal artery (CRA), one of the first and smallest branches of the OA, supplies the inner retina. The posterior ciliary arteries (PCAs) are also direct branches of the OA. The anterior ciliary arteries (ACAs) arise not from the OA itself but from the muscular arteries supplying the rectus muscles, and they enter the globe after the muscles [**25**].

Figure 1.3 shows the major ocular arterial pathways and venous drainage around the globe, and Figure 1.4 traces the path of the blood from the internal carotid artery to the venous outflow of the orbit.

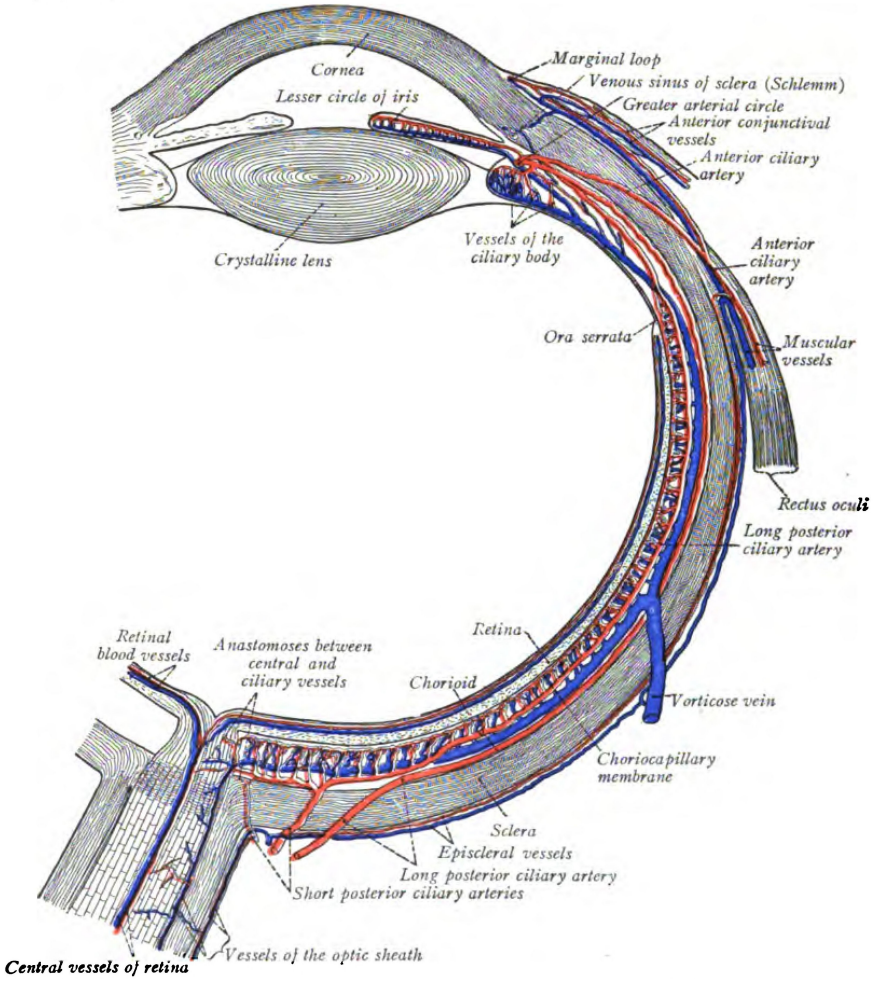

FIGURE 1.3. Blood vessels of the eyeball, diagrammatic section; arteries red, veins blue. From J. Sobotta and J. P. McMurrich, *Atlas and Text-book of Human Anatomy*, vol. 3 (Philadelphia: W. B. Saunders, 1914), Fig. 743, after Th. Leber; artist K. Hajek. Public domain (published 1914); scan from AnatomyTOOL. The printed caption below the drawing has been cropped; the drawing is otherwise unaltered.

The retinal circulation is end-arterial. The CRA enters the globe through the optic nerve head (ONH) and divides into branches that perfuse the inner retinal layers, with little collateral supply; the outer, photoreceptor layers are avascular and depend on the choroid beneath them. Venous blood from the retina collects into a single central retinal vein, which leaves the optic nerve some 10–12 mm behind the

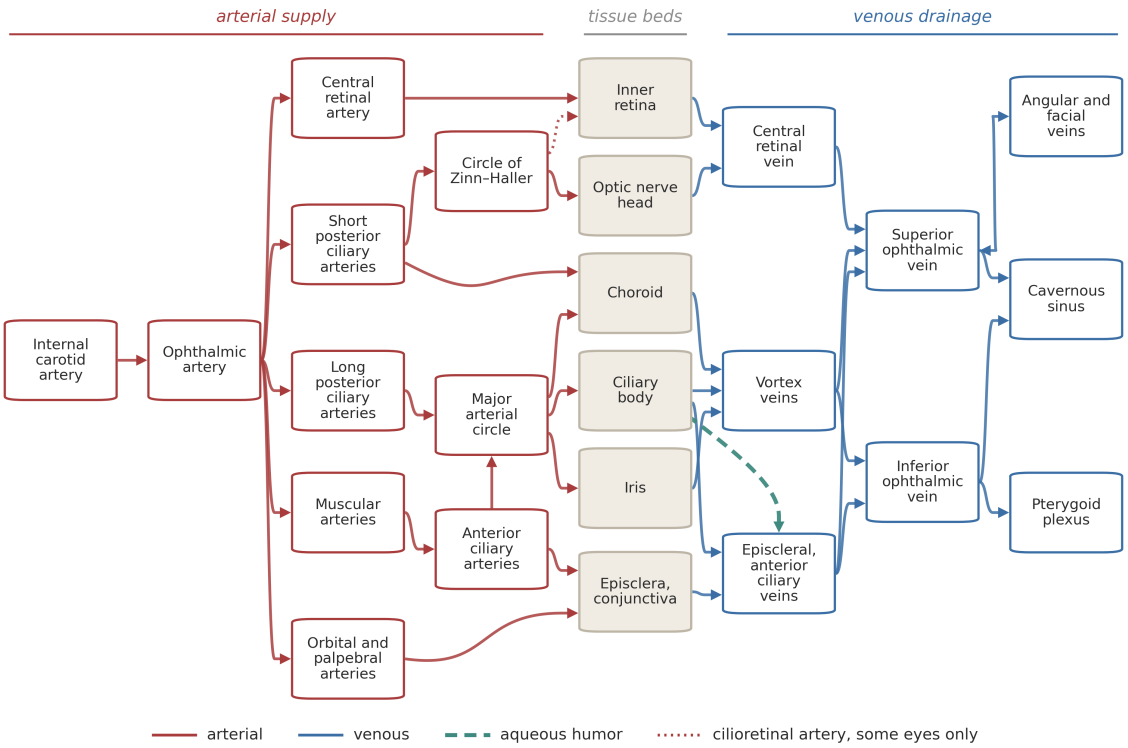

FIGURE 1.4. Flow path of the ocular circulation, from the internal carotid artery to the venous outflow of the orbit. Arrows give the direction of flow, and the two-headed arrow marks the valveless connection with the angular and facial veins; tissue beds are shaded. The central retinal vein may also drain directly into the cavernous sinus [**25**]. Drawn for this thesis.

lamina cribrosa alongside the CRA and then either joins the superior ophthalmic vein or exits the orbit to drain directly into the cavernous sinus [**25**].

Published estimates of total retinal blood flow in healthy human eyes span roughly 33–80 μL min^{-1}, using laser Doppler and Doppler optical coherence tomography methods [**29, 30, 31, 32, 33, 34**].

The uveal circulation, dominated by the choroid, carries the large majority of ocular blood flow and is supplied by the PCAs. The short PCAs, some ten to twenty in number, enter the sclera in a ring around the optic nerve and form the arterial network of the choroidal stroma; other branches of the same vessels anastomose to form the circle of Zinn–Haller, which encircles the optic nerve at the level of the choroid and supplies the optic nerve head rather than the choroid at large (Figure 1.5).

The two long PCAs run forward between sclera and choroid to the ciliary body and, with the ACAs, form the major arterial circle of the iris [**25**]. The choroid's outflow is distinctive. Its venous return does not follow its arteries but converges, essentially in its entirety, into the vortex veins. These are usually four or five, with one in each quadrant piercing the sclera about 6 mm behind the equator [**25**]. Anatomical series of human eyes report three to eight per eye, about 65% having four or five [**35**]. They empty into the ophthalmic veins and thence the cavernous sinus.

Choroidal blood flow greatly exceeds retinal flow. Per gram of tissue the choroid has the highest blood flow of any tissue in the body [**35, 36**]. Representative figures are about 2000 $\mathrm{mL\,min^{-1}}$ per 100 g for the choroid, a value the same text attributes to the choriocapillaris, and 60 $\mathrm{mL\,min^{-1}}$ per 100 g for the retina [**25**]. Choroidal flow per gram has also been put at roughly four times that of the renal cortex [**37**]. The commonly quoted statement that the choroid carries about 85% of ocular blood flow comes from the same family of references [**37, 38**]. In the monkey measurements behind it, the choroid received 607 of the 738 $\mathrm{mg\,min^{-1}}$ reaching the retina and uvea, or 82% [**39**]. All of these figures come from microsphere studies in monkeys and cats. The model of Chapter 3 likewise takes the perfusion of its choroid from microsphere measurements in monkeys: the mean total choroidal flow of four eyes, 917.5 $\mu\mathrm{L\,min^{-1}}$ [**40**], distributed over the 165 $\mathrm{mm^3}$ choroid of the model (Appendix B). Magnetic-resonance perfusion imaging of the human posterior pole gives a combined retinal and choroidal tissue perfusion of 149 ± 48 mL per 100 mL of tissue per minute in four healthy volunteers [**41**]. These tissue-normalized values come from a voxel containing both beds, and they confirm the magnitude of posterior-segment perfusion without separating its two sources.

The two beds are also regulated very differently, which matters for how each clears heat. Retinal blood flow is strongly autoregulated: myogenic and metabolic mechanisms adjust arteriolar tone both to hold perfusion nearly constant across changes in

perfusion pressure and to match flow to metabolic demand [**42**], a control also central to optic-nerve-head perfusion [**43**].

The choroid lacks the classical pressure autoregulation of the retinal bed, and its flow is governed largely by autonomic tone, sympathetic stimulation producing vasoconstriction [**25, 36**]. Some regulation is nonetheless present. In healthy subjects performing isometric exercise, a 67% rise in mean ocular perfusion pressure produced only a 12% rise in subfoveal choroidal blood flow, and the compensation broke down at larger pressure increases [**44**]. Thermally, its flow is very high, only partially responsive to perfusion pressure [**44**], and essentially uncoupled from local retinal metabolic demand [**36**].

The end-arterial retinal supply helps explain vulnerability to arterial occlusion [**45, 46**]. The high-flow choroidal circulation also exchanges heat with the posterior tissues, as discussed next (Section 1.2.3).

1.2.3. Ocular Thermal Environment. In patients, temperature rises from the front of the eye to the back In patients, temperature rises from the front of the eye to the back (Table 1.1). Shinoda and colleagues measured it at four sites with a needle thermoprobe at the start of vitrectomy, before the infusion was opened, in an operating room set at 25 °C. They attribute the gradient partly to the cooler surroundings and limit their values to these conditions [**47**]. Two later intraoperative series give different absolute values [**48, 49**]. In the second, the vitreous was warmer in front of the optic disc than behind the lens, a rise the authors attribute to the nearby choroidal circulation [**49**].

At the anterior surface, convection, radiation, and tear evaporation remove heat and help explain the cooler cornea and aqueous [**50, 51**]. Their contributions depend on ambient and surface conditions: convection and radiation can supply heat in sufficiently warm surroundings, while evaporation removes it. Within the globe, heat is conducted through the ocular wall and contents, including the vitreous, and exchanged with the circulation.

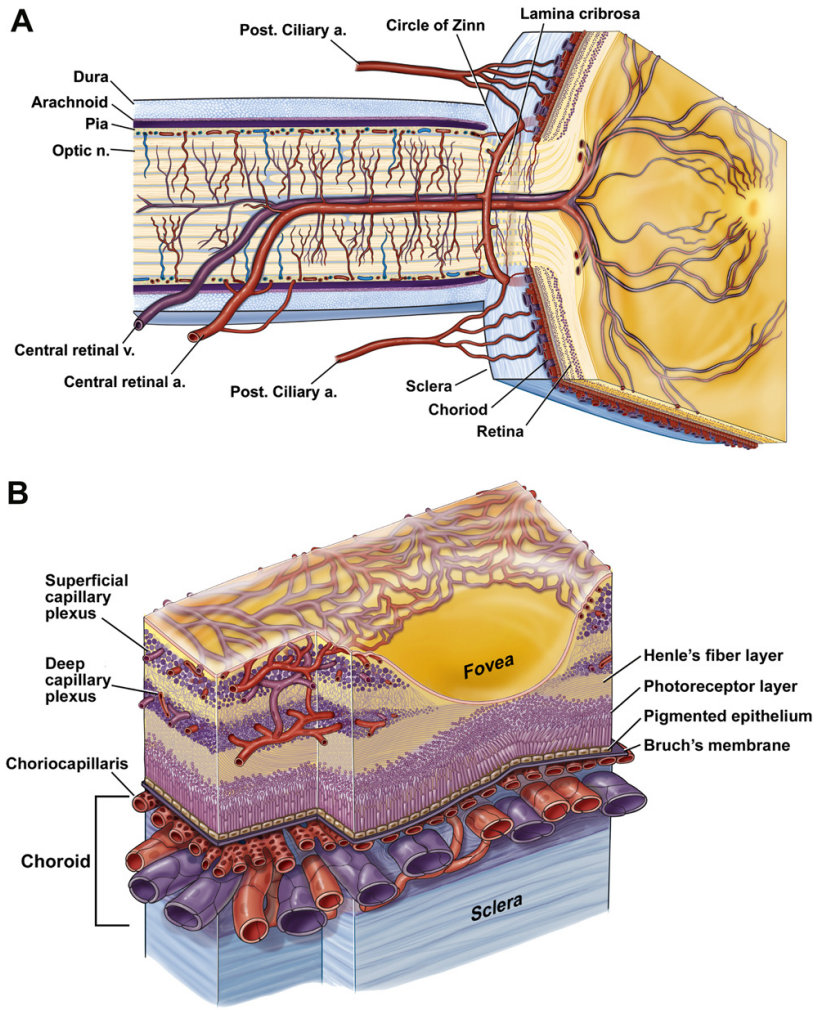

FIGURE 1.5. Retinal and choroidal circulation. (A) Vascular supply at the optic nerve and posterior globe. (B) Retinal capillary plexuses and underlying choroidal vessels. Reprinted from *Progress in Retinal and Eye Research*, vol. 31, J. Kur, E. A. Newman, and T. Chan-Ling, "Cellular and physiological mechanisms underlying blood flow regulation in the retina and choroid in health and disease," pp. 377–406, Fig. 1, Crown Copyright (2012), with permission from Elsevier [**26**]. The drawings, by Dave Schumick, are reprinted from *Encyclopedia of the Eye*, B. Anand-Apte and J. G. Hollyfield, "Developmental anatomy of the retinal and choroidal vasculature," pp. 9–15, Copyright (2010), with permission from Elsevier.

The choroid can supply or remove heat according to the blood–tissue temperature difference. In anesthetized monkeys under dim illumination, reducing choroidal flow lowered macular retinal–choroidal temperature. Under a focused microscope lamp, tissue temperature exceeded blood temperature and reducing flow raised it further

TABLE 1.1. Intraocular temperatures measured in patients during vitreoretinal surgery, before vitrectomy. The posterior values are vitreous, not retinal, temperatures. —: not measured.

Study	**Anterior chamber**	**Behind the lens**	**Mid-vitreous**	**In front of the optic disc**
Shinoda et al. [**47**]: 30 eyes with intact vitreous	30.0 ± 1.7 °C	32.8 ± 1.2 °C	34.0 ± 0.9 °C	35.0 ± 0.9 °C
Kobylianskyi et al. [**48**]: 23 eyes with proliferative diabetic retinopathy	—	—	35.6 ± 0.7 °C	—
Ríos-Nequis et al. [**49**]: 12 eyes, combined cataract and vitreous surgery[a]	28.7 ± 2.8 °C	29.6 ± 0.7 °C	—	31.9 ± 1.1 °C

[a]Anterior chamber at the start of surgery; vitreous before vitrectomy, after the cataract step.

[**37, 38**]. Thus, the same circulation that removes absorbed radiant heat can oppose externally imposed cooling. This opposition is expected to act continuously during cooling, because choroidal flow is high and only partly pressure-regulated [**44**]. The ex-vivo experiments therefore use a 37 °C circulating bath as a boundary-driven stand-in for this load (Chapter 2), and the device was also tested in vivo (Chapter 4).

Thermal models differ mainly in how they represent heat delivery: a fixed-temperature boundary, a heat-transfer coefficient, a Pennes perfusion term, discrete vessels, or multiphase blood [**52, 53, 54**]. In the model of Geiser and colleagues, choroidal and ciliary perfusion dominated retinal flow as a heat source, and a change of ambient temperature from −20 to +40 °C changed the predicted retinal temperature by about 1.9 °C [**55**]. Flyckt and colleagues supported the bioheat approach for first-order predictions and considered discrete vasculature necessary for finer spatial detail [**53**].

The measurements of this dissertation give temperatures inside the eye during contact cooling, for comparison with related models from this research program that directly address that cooling configuration. Gongal and colleagues modeled a scleral ring as a 4 °C boundary on the anterior sclera; with Pennes perfusion the optic nerve head settled at 34.0 °C [**56**], whereas a multiphase formulation, in which cooled blood

returning from the front of the eye carries the cold backward, predicted 29.6 °C [**54**]. These models motivated the device.

Temperatures inside the eye were measured directly, ex vivo, before the device was tested in the living eye, because the temperature applied at the front does not set the temperature at the back. For device design, cooling requires net heat removal, and holding a steady temperature requires extraction that balances the remaining net heat inputs. An anterior contact temperature thus defines an applied boundary, while the posterior temperature depends on transport through the eye and on physiological heat delivery.

1.3. Therapeutic Hypothermia and Neuroprotection

Therapeutic hypothermia is the deliberate lowering of tissue temperature to slow metabolism and limit ischemic and hypoxic injury. Cooling is graded by target temperature, and the clinical evidence concerns mild-to-moderate cooling. Polderman describes mild-to-moderate hypothermia (32–35 °C) as the first treatment with proven efficacy for postischemic neurological injury [**57**], a judgement resting principally on the post-cardiac-arrest literature of that time. Later trials found no benefit of cooling to 33 °C over 36 °C or over controlled normothermia [**58, 59**], and current post-arrest guidelines prescribe temperature control at a constant target between 32 and 37.5 °C with active prevention of fever, rather than mandatory hypothermia [**60**].

1.3.1. Mechanisms. Hypothermia acts on many targets at once. In Yenari and Han's review of the experimental literature, it affects nearly every investigated cell-death pathway [**61**]. In ischemia, the failure of oxidative metabolism depletes adenosine triphosphate (ATP), disrupts transmembrane ion gradients, and promotes glutamate accumulation and calcium overload, followed on reperfusion by free-radical production and inflammation [**57, 61**]. The basic effect of cooling is metabolic. Brain oxygen consumption and glucose metabolism fall by roughly 5% for every degree Celsius of cooling, and ATP stores are preserved, which may limit calcium overload

and excitotoxic glutamate [**61**]. Hypothermia also curbs apoptosis, the post-ischemic inflammatory response, free-radical production, and edema [**57, 61**].

In the retina, protection grew as cooling deepened. In isolated rabbit retina after 1 h of simulated ischemia, recovery of light-evoked compound action potentials was 15% at 36 °C, 36% at 33 °C, and 53% at 30 °C [**62**]. Cooling to 30 °C protected porcine retinal explants during a three-hour hydrogen-peroxide exposure [**63**]. Cooling for 20 min at a retro-ocular temperature of about 32 °C, 24 h before the insult, protected the rat retina from ischemia–reperfusion damage [**64**].

Metabolic suppression is the mechanism most likely to matter in the retina, which is among the most metabolically active tissues in the body (Section 1.2.1). Rewarming also matters. Its rate affects the protection conferred by cooling in experimental models, and the systemic hypothermia literature treats controlled rewarming as part of the protocol [**57**].

1.3.2. Evidence in the Central Nervous System. Clinical outcomes depend on the indication and protocol [**61**]. Therapeutic cooling has an established role in neonatal encephalopathy. After cardiac arrest, early trials reported benefits [**65, 66**], but the TTM and TTM2 trials found no improvement with 33 °C over a controlled 36 °C or over targeted normothermia [**58, 59**]. The 2025 ERC–ESICM guideline emphasizes fever prevention [**67**]. In acute ischemic stroke, the class to which CRAO belongs, the 2026 guideline recommends against induced hypothermia in normothermic patients and lists the benefit of selective intra-arterial brain cooling as unknown [**8**].

These systemic trials do not establish the efficacy of localized retinal cooling, but they document a reason to investigate local delivery. Systemic cooling brings coagulopathy and an infection risk that rises with treatment duration [**57**], and in TTM2 more arrhythmia with hemodynamic compromise (24% against 17%) [**59**]. It is also slow: in eleven randomized trials after cardiac arrest the median time to 32–34 °C ranged from 38 to 360 min [**68**]. During stroke thrombectomy, systemic

intravascular cooling below 34 °C was followed by more serious adverse events than standard care, a difference the trial was not powered to test [**69**]. Selective cooling of the brain remains investigational [**60**]. Limiting the cooled region is an engineering strategy to limit this exposure.

1.3.3. Evidence in the Retina. How long the retina survives complete arterial occlusion is uncertain, and depends on the model and the perfusion conditions. In Hayreh's experimental model, transient central retinal artery occlusion in elderly atherosclerotic rhesus monkeys produced no detectable damage at 97 min, with progressively more extensive irreversible damage at longer occlusion times up to 240 min [**45**]. Tobalem and colleagues, in a critical review, argue that this figure is an overestimate, attributing it to barbiturate anesthesia, uncontrolled hypothermia, and residual collateral perfusion in the clamp preparation, and propose that infarction of retinal ganglion cells is likely after only 12–15 min of complete occlusion [**46**]. Loss of function and irreversible injury can also diverge. Ex vivo, light responses of ON-bipolar cells recovered after overnight incubation in oxygenated medium at room temperature in human donor retinas collected 50 to 60 min after circulatory arrest, and in two of five pairs of eyes enucleated 3 to 4 h post mortem. Mouse retinas recovered fully after 1 h of hypoxia but only in part after 3 h. These preparations lack the vascular and inflammatory components of ischemia in vivo [**70**].

In each of these accounts the window is short relative to the delays with which patients present (Section 1.1) [**14, 15**]. The uncertainty in the deadline argues for an intervention that can be applied promptly and can extend tolerance (Section 1.9).

Hypothermia has protected the retina in cell-culture, ex-vivo, and animal models at temperatures from about 34 °C down to 30 °C and below (Section 1.3.1). Cooling to 30 and 20 °C protected retinal ganglion cells in hypoxic organ culture [**71**], and cooling to 34 °C reduced the secretion of vascular endothelial growth factor by cultured retinal pigment epithelial cells by 38% [**72**].

In those injury studies, electrophysiological recovery is evidence of preserved function. In the present work the electroretinogram (ERG) is used for a different purpose, to detect temperature-sensitive retinal responses and their recovery after rewarming.

Temperature also affects the ERG response itself: cooling the eye in vivo reduces ERG amplitudes and prolongs their timing [**73, 74**]. Pitkänen and colleagues used temperature-dependent photoresponse features to estimate temperature in isolated mouse retinas [**75**]. Kaikkonen and colleagues demonstrated temperature-dependent photopic ERG kinetics in isolated porcine retinas and anesthetized pigs [**76**], and the same group has since used focal ERG kinetics to determine retinal temperature during laser heating of the living pig retina, with a standard error of 0.6 °C [**77**]. That calibration applies to focal, light-adapted responses heated above body temperature. For the full-field flash ERG recorded during cooling in this work, ERG changes are indirect evidence of cooling. Tools developed in this laboratory that could show where, and how widely, the retina cools are discussed in Section 5.5.

1.4. Ocular Cooling: Prior Approaches and Their Limitations

Attempts to cool the eye span six decades, from research techniques never intended for clinical use to devices now in early clinical trials (Table 1.2). They can be organized by invasiveness, and each falls short of the goal of this dissertation, sustained, controlled hypothermia at the posterior pole.

Invasive routes reach the target but need surgical access. Cooling the cat eye by intraocular perfusion, with body and brain held normothermic, reduced the amplitude and increased the latency of the electroretinogram, while the evoked potentials of the optic chiasm and visual cortex were little affected [**73**]. In anesthetized rabbits, perfusing the anterior chamber with balanced salt solution cooled to 11 °C by a feedback-controlled thermoelectric heat pump lowered the temperature at the retinal surface by 2 to 4 °C, and it rose back within minutes of stopping [**78**]. Irrigation at 8 or 22 °C during ischemic vitrectomy in rabbits preserved tissue relative to 38 °C

[**79**]. Trans-nasal cooling of the injured optic nerve in goats reduced injury only when combined with a protease-inhibitor cold-protectant, did not restore vision, and does not establish retinal cooling [**80**]. Cyclocryotherapy shows that cold can be delivered through the sclera, but its freezing exposure is destructive by design [**81**].

Research preparations in living animals showed that cold applied at the surface can reach the back of the eye. A plastic cap circulating cold saline over the exposed cat eye could not bring the retina below 32 °C without some fall in body and brain temperature [**73**]. In rabbits whose eyelids and extraocular tissues had been removed, water at about 15 °C in tubing glued to the sclera brought the retina within 1 °C of the sclera in less than 10 min, measured in one animal [**74**]. In rats, an ice pack during retinal ischemia preserved the inner retina, with no temperature measured [**82**], and a flow of 13 °C gel over one eye brought the tissue behind the globe to about 32 °C in a pilot group of four [**64**].

Apart from those probe readings [**73, 64**], no non-invasive cooling method has reported a temperature behind the lens. Cold compresses before intravitreal injection reduce pain but offer no temperature control [**83**]. A circulating-water contact lens reduced laser-induced corneal damage in rabbits [**84**]. A thermoelectric eyelid dressing with feedback control, specified for +5 to +40 °C for up to 48 h [**85**], shows that thermoelectric cooling can be packaged for ocular contact. Its posterior response has only been modeled, a fall of 1 to 2 °C in the vitreous and retina with the cornea at 20 °C [**86**]. Handheld thermoelectric devices cool the ocular surface for seconds to anesthetize it before injection [**87, 88**]. One of them, OcuCool, holds U.S. Food and Drug Administration (FDA) De Novo authorization for that use in patients allergic to deep-penetrating pharmaceutical anesthetics, with no posterior-segment indication [**89**].

Scleral-contact cooling interfaces are clinically tractable, as devices now in early clinical testing for chronic ocular surface pain confirm independently of the present work. The clinical ETX-4143 device draws its cooling from an internal solution frozen

before use [**90**]. Single 10-min applications produced only minor, resolving findings in rabbits [**91**]. In patients, 4-min treatments left surface signs and corneal nerve morphology stable over 12 weeks in five [**92**], and in a sham-controlled study of 31 caused no serious adverse events, with larger falls in pain significant only in a post hoc subgroup [**90**]. They are designed to treat surface pain by targeting the long ciliary nerves.

Representative device designs are shown in Figure 1.6.

Table 1.2: Selected ocular cooling approaches and limitations for sustained posterior cooling. Temperatures refer to the cooling medium or application site, not the retina, except where stated.

Approach	**Route**	**Demonstrated effect**	**Limitation**
Intraocular perfusion [**73**]	Direct intraocular perfusion (cat)	Retina cooled to 22 °C; reduced ERG amplitude and increased latency; central evoked potentials little affected	Physiological study, not injury rescue; requires intraocular access
Anterior-chamber perfusion [**78**]	Corneal incisions; balanced salt solution at 11 °C for 10 min (rabbit)	Retinal-surface temperature lowered by 2–4 °C (thermocouple)	Requires intraocular access; cooling lasts only while the perfusion runs
Scleral cooling coil [**74**]	Tubing carrying 15 °C water, coiled around the globe and glued to the sclera (rabbit)	Retina cooled to an estimated 15–18 °C; ERG changed with cooling	Research preparation; eyelids removed and tubing glued to the sclera
Hypothermic irrigation [**79**]	Intraoperative intraocular irrigation, 8 °C (rabbit)	Better ERG recovery and histology than 38 °C; ERG not different from 22 °C	Confined to the operating room; requires active irrigation
Trans-nasal optic-nerve cooling [**80**]	Endoscopic access to the pre-chiasmatic optic nerve, 4 °C (goat)	Optic-nerve protection, but only with a protease-inhibitor cold-protectant; vision not restored	Nerve, not retina; surgical access impractical for emergency presentation
Cyclocryotherapy [**81**]	Trans-scleral cryoprobe, −80 °C, 60 s	Ablates the ciliary body (refractory glaucoma)	Destructive by design; extreme, localized freezing

Approach	**Route**	**Demonstrated effect**	**Limitation**
Ice/cold compress [**83**]	Non-invasive, closed eyelid, 2 min before injection	Reduced pain measured after intravitreal injection	No temperature control or targeting
Ice pack [**82**]	Non-invasive, over the eye during 120 min of ischemia (rat)	Inner retina preserved histologically	No temperature measured or controlled
Cold gel [**64**]	Non-invasive, flow of 13 °C gel over one eye for 20 min (rat)	About 32 °C behind the globe (pilot, 4 rats); preconditioning protected the retina	Temperature measured only in the pilot and not controlled; rat eye
Circulating-water contact lens [**84**]	Non-invasive, corneal contact (rabbit)	Reduced laser-induced corneal damage	Demonstrated anteriorly; posterior control not tested
Peltier eyelid dressing [**85**]	Non-invasive, periocular (closed eyelid)	Controlled-temperature prototype, +5 to +40 °C	Clinical results deferred by the authors; posterior response modeled only (1–2 °C at the retina for a 20 °C cornea) [**86**]
Handheld cryoanesthesia applicator [**87, 88, 89**]	Non-invasive, conjunctival/scleral contact	Phase-1 feasibility (22 patients); open-label dose-ranging trial (43 patients); De Novo authorization 2024 for injection pain in patients allergic to deep-penetrating pharmaceutical anesthetics	OcuCool: 10 s cooling to −15 °C at the conjunctiva (FDA device description); no posterior indication
Scleral-contact surface cooling [**91, 90, 92, 93**]	Non-invasive, scleral contact over the long ciliary nerves	Rabbit safety; surface signs and corneal nerve morphology stable over 12 weeks in 5 patients; sham-controlled trial in 31 patients with no serious adverse events	Intended target is surface pain, not sustained deep hypothermia at the retina/ONH

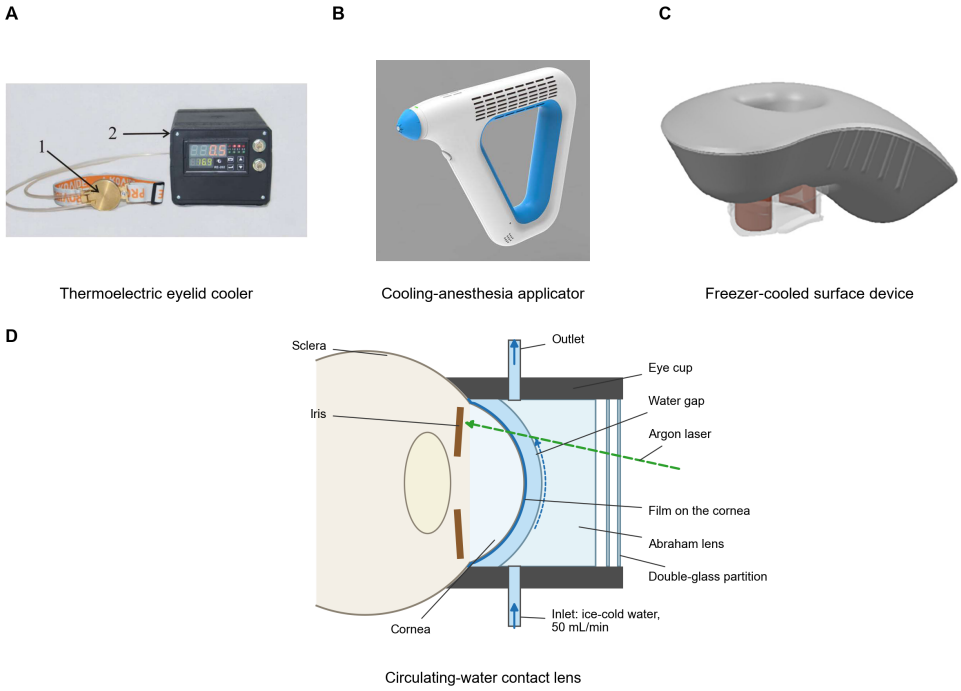

FIGURE 1.6. Representative ocular cooling devices. (A) Thermoelectric eyelid dressing and control unit (1: cooling plate; 2: control and power unit), from Anatychuk et al., Fig. 1 [**85**] (CC BY 4.0). (B) Handheld thermoelectric applicator for injection anesthesia, made by RecensMedical, the sponsor of OcuCool [**89**]; from Chao et al., Fig. 2A, cropped to the device [**88**] (CC BY-NC 3.0). (C) Freezer-cooled ocular-surface device, from Galor et al., Fig. 1 [**91**] (CC BY-NC-ND 4.0); the rabbit-study version used a single conductor. (D) Circulating-water contact lens used during argon laser iridotomy in rabbits: ice-cold water (50 mL min^{-1}) flows between an Abraham lens held in an eye cup and a film resting on the cornea; drawn for this dissertation from the description by Kataoka et al. [**84**].

1.5. Thermoelectric Cooling

Thermoelectric cooling is the means by which the device described in this dissertation moves heat. The relevant physics is the Peltier effect. When current is driven across a junction between dissimilar conductors, heat is absorbed at that junction and released at the other junction of the circuit, and the pair works as a heat pump rather than as a power source [**94**]. A conventional thermoelectric cooler (TEC) module exploits this at scale by connecting many n-type and p-type semiconductor legs electrically in series and thermally in parallel between two thin ceramic plates,

which insulate electrically while conducting heat (Figure 1.7) [**95**]. Current in one direction pumps heat from one ceramic face, the cold side, to the other. Reversing the polarity reverses which face is cold, and a single module can therefore cool or heat with no change in hardware. The Peltier contribution is reversible in that sense, but the module as a whole is not thermodynamically reversible. Joule heating (I^2R) and conduction back along the temperature gradient ($K\,\Delta T$) [**94**] are irreversible losses that act in both modes, where I is the drive current, R the electrical resistance of the module, K its thermal conductance, and ΔT the temperature difference between the hot and cold faces.

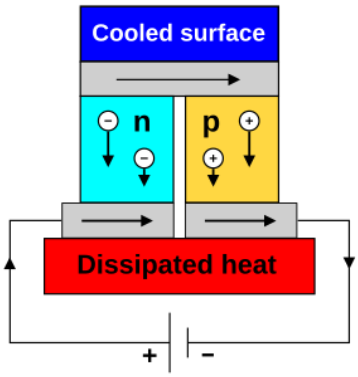

FIGURE 1.7. Operating principle of a thermoelectric couple. An applied current drives heat transfer from the cold side to the hot side through n-type and p-type semiconductor legs. Illustration by Ken Brazier, Wikimedia Commons, CC BY-SA 4.0.

The first consequence for design is that cooling capacity and temperature difference trade off against one another, and a module's headline ratings are not simultaneously available. The rated maximum heat-pumping capacity applies at zero temperature difference across the module, and the maximum temperature difference is reached only at zero cooling load. At any intermediate condition the available capacity is correspondingly reduced [**96**]. The Peltier term is proportional to current, whereas Joule heating goes as its square. Thus the coldest attainable temperature is reached at an optimal current rather than at maximum current [**94**]. A module must

therefore be selected against the required cooling load at the actual temperature difference, hot-side temperature, and electrical operating point, and not against no-load ratings. This check is made for the module of the device in Chapter 3 (Section 3.4).

The second consequence is that all the heat delivered to the hot side must ultimately be rejected to the ambient. This heat is $Q_h = Q_c + P_{\text{in}}$, where Q_c is the heat pumped from the cold side, P_{in} the electrical power supplied to the module, and Q_h the heat released at the hot side. Hot-side heat rejection is a principal constraint on realizing rated performance in a physical device [**96, 97**]. The thermal resistance of the whole path, from the target tissue through the module to the ambient heat sink, governs how much of a module's capability is delivered at the point of contact. A perfused eye adds a load that is continuously renewed.

Several properties of thermoelectric cooling nonetheless suit a scleral-contact device. The module is solid-state, without moving parts or a working fluid, and makes no vibration or acoustic noise [**97, 98, 96**]. Heat rejection still needs a system around it, and the cooler developed here (Section 1.7), like the eyelid device of Section 1.4 [**85**], uses a liquid loop. The annular module adopted here is 26.0 mm in outside diameter with a 14.0 mm central hole and a thickness of 4.7 mm [**96**], which leaves a central aperture for the cornea and the optical path. Temperature is set electronically by the magnitude and direction of the drive current, and holding it precisely requires closed-loop feedback. Thermoelectric modules are also compatible with battery operation, and portable thermoelectric cooling has been demonstrated for personal thermal management [**99**].

1.6. The Porcine Model

Among nonprimate species, the pig is a widely used large-animal model of the human eye, and its use in retinal research continues to grow [**100**]. The porcine globe approaches human scale: in enucleated eyes from domestic pigs the anteroposterior

length was 23.9 ± 0.08 mm [**101**], against a human globe of roughly 24 mm (Section 1.2.1). This supports the testing of device-interface tools intended for human use [**100**].

Porcine scleral geometry and water content support its use for interface development, but important differences from humans remain. Table 1.3 collects dimensions and tissue comparisons with their measurement conditions [**102, 103**].

For a scleral-contact device, the first difference that matters is where the wall is thinnest. Porcine sclera is thinnest only a few millimeters behind the limbus, whereas the human sclera, away from the muscle insertions, is thinnest near the equator (Table 1.3). Olsen and colleagues note that placing a device 5 to 6 mm from the limbus in the pig is therefore analogous to placing it near the equator in a human eye [**102**]. The second difference is practical. The porcine orbit is small and the extraocular musculature is unusually strong, and extraocular devices are difficult to place posterior to the muscle insertions [**102**]. This limits where a scleral ring can sit in this species.

Porcine measurements show a cooler aqueous compartment and warmer posterior vitreous, qualitatively resembling the human gradient [**47, 104**]. Thermal similarity must be assessed experimentally, as in Aim 1.

The retina itself is comparably organized. Porcine retinal circulation is holangiotic, as in humans, but the pig has no central retinal artery, the retina being supplied by several cilioretinal arteries, and the orbital supply arises from the external carotid [**105, 106**]. The distribution of retinal layers is comparable between the two species [**101, 107, 108**]. The pig lacks a true fovea and has instead an elongated visual streak of high cone density (Table 1.3) [**100, 109**]. Foveal function is not the experimental endpoint of this work, which targets cooling of the posterior eye, measured at the proximal optic nerve.

The living pig is expected to be a conservative test of posterior cooling, because perfusion is the principal heat load that a cooling device must overcome in vivo (Section 1.2.3) and porcine choroidal flow is high. It exceeds the per-eye totals measured in monkeys by the same microsphere method and the per-area values measured in humans by magnetic resonance imaging (MRI) (Table 1.3) [**110, 40, 111**], and human choroidal flow declines further with age [**112**]. The methods differ, however.

Porcine models of ocular hypertension and retinal ischemia are available for future efficacy studies [**114, 115**], but the present work induces neither. The same species serves both experimental phases: eyes from a commercial supplier for ex-vivo thermal characterization, and the live animal for functional and safety assessment in the presence of perfusion.

1.7. The Scleral-Contact Eye Cooler

The scleral-contact eye cooler (SCEC) applies a controlled thermal boundary at the accessible anterior sclera to remove heat from the globe. Its architecture combines the tissue-contact interface, a thermoelectric heat pump, heat rejection, and feedback control.

At a high level SCEC Gen 3 comprises three thermally coupled subsystems together with dedicated control electronics (Figure 1.8) [**1**]. A scleral contact ring (SCR), shaped to conform to the anterior sclera, forms the cooled boundary against the eye while leaving a central aperture over the cornea and pupil. The device therefore does not touch the cornea, although the cornea can still cool indirectly. The aperture also leaves the visual axis unobstructed, and assessments such as the pupillary light reflex remain accessible. An annular thermoelectric cooler (Section 1.5) pumps heat from the SCR to a liquid-cooled heat exchanger, or water block, which rejects it to a circulating coolant loop. Control electronics provide power distribution, thermoelectric drive, and thermal sensing. A temperature sensor embedded in the SCR is the feedback signal by which the contact ring is held at a commanded set

TABLE 1.3. Human and porcine ocular parameters from separate studies. Human scleral thicknesses are, unless another source is given, histological means from non-axially elongated eyes enucleated for melanoma or end-stage glaucoma [**113**]; porcine values are means from formalin-fixed globes grouped by body weight [**102**]. Blood-flow methods are given in each cell.

Parameter	**Human**	**Porcine**	**Source**
Globe axial length	≈24 mm (postmortem AP mean 24.44 ± 1.03 mm)	23.9 ± 0.08 mm (5 eyes, 6–8 months)	[**23, 24, 101**]
Scleral thickness, limbus	0.50 ± 0.11 mm	0.83, 0.91, 1.12 mm (small, medium, large)	[**102, 113**]
Scleral thickness, equator	0.42 ± 0.15 mm	0.56, 0.73, 0.86 mm (small, medium, large)	[**102, 113**]
Thinnest point away from the muscle insertions, and its location	∼0.4 mm near the equator, ≈13 mm behind the limbus	0.31, 0.35, 0.43 mm at only 5–6 mm behind the limbus	[**102**]
Scleral thickness, under the rectus muscles	∼0.3 mm	Not reported by the cited sources	[**28**]
Scleral thickness, posterior pole	0.94 ± 0.18 mm measured; 1.0–1.35 mm quoted in review	Thickness plateaus from ≈10–12 mm behind the limbus; posterior-pole value not tabulated	[**28, 102, 113**]
Total scleral surface area	≈16–17 cm^2	7.78, 9.66, 11.92 cm^2 (small, medium, large)	[**102**]
Scleral water content (wet mass)	$71.6 \pm 0.63\%$	$69.5 \pm 1.18\%$	[**103**]
Choroidal blood flow	≈1.1 $\mu L\,min^{-1}\,mm^{-2}$ (MRI, retina + choroid, posterior pole); monkey 0.6–1.3 $mL\,min^{-1}$ per eye (microspheres)	3.2–3.7 $\mu L\,min^{-1}\,mm^{-2}$, ≈1.6–1.7 $mL\,min^{-1}$ per eye (microspheres, 19–27 kg, pentobarbital)	[**110, 40, 111**]
Choroidal response to perfusion pressure	Partial compensation for moderate rises	Falls in proportion to perfusion pressure	[**44, 110**]
Retinal vascular pattern	Holangiotic	Holangiotic; no central retinal artery; orbital supply arises from the external carotid	[**101, 105**]
Fovea	True fovea present	Absent; elongated visual streak (20,000–35,000 cones/mm^2, higher local peaks)	[**109**]
Experimentally induced disease models	—	Ocular hypertension (episcleral vein cauterization); retinal ischemia (endovascular occlusion)	[**114, 115**]

point. In Gen 3 the thermoelectric drive was adjusted at first by the operator and later switched automatically by a comparator (Section 2.2.1). The controlled variable is the ring temperature, and tissue temperatures elsewhere in the globe are monitored responses to the imposed boundary. The set point, 4 °C, is the conventional lower limit of hypothermic organ storage. Below about 2 °C the risk of cold injury to stored organs rises significantly [**116**], so a ring at 4 °C keeps a margin of about 2 °C from that limit and lies well above the freezing temperatures that cause cryogenic injury (Section 2.2.1). The 2 °C limit comes from stored, non-perfused organs and has not been established for the living eye. The tolerance of living ocular tissue to prolonged cooling at this boundary is assessed in Aim 3. Consolidation of drive and sensing onto an integrated board is part of the Gen 4 work in Aim 2 (Chapter 3).

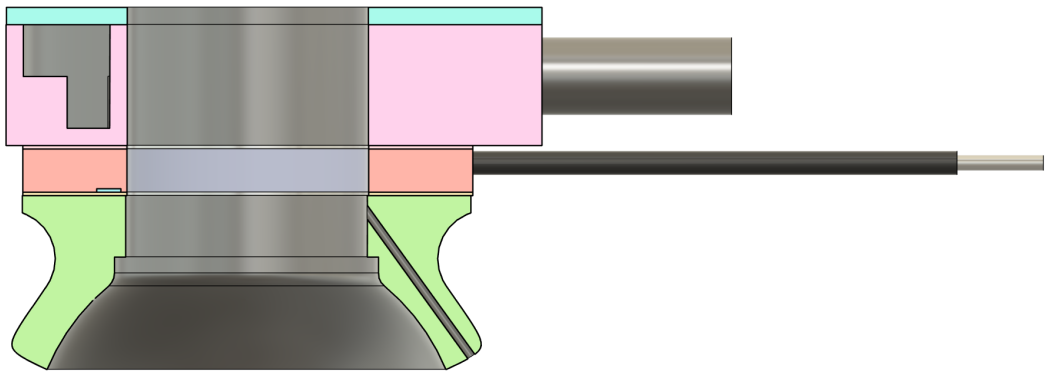

FIGURE 1.8. Cross-sectional computer-aided design (CAD) view of the scleral-contact eye cooler (SCEC), showing the contoured scleral contact ring (green), annular thermoelectric module (orange), and liquid-cooled heat exchanger, or water block (pink and blue). The central aperture provides optical access through the assembly. The coolant connection and electrical lead extend laterally, and an oblique bore through the contact ring holds the thermistor.

Potential heating and analgesic applications are discussed as future work in Section 5.5.

The liquid-cooled device of this work is the third generation of the cooler built in this laboratory, after the two prototypes described in Section 1.8. Two version names are used for the remainder of the dissertation, because the device evolves across the

three aims. *SCEC Gen 3* denotes the ex-vivo prototype validated in Aim 1 (Chapter 2). *SCEC Gen 4* denotes the enhanced device developed in Aim 2 (Chapter 3), which targets improved efficiency, portability, safety, and patient comfort. The in-vivo study of Aim 3 (Chapter 4) used a build that carried over some, but not all, of the Gen 4 enhancements. The exact configuration of that build is reconciled against the Gen 4 target in Section 4.2.4, so that "the device" refers to a definite object in every chapter.

1.8. Prior Work in This Laboratory

The scleral-contact eye cooler has been developed in the Neural Engineering Vision Laboratory over several student projects. The first prototype (Gen 1) was built by S. J. Dreyer as a portable cooler for traumatic eye injury [**117**]: an aluminum ring on the anterior sclera, with three thermoelectric modules and air-cooled finned heat sinks. It was tested on thawed porcine eyes with three scleral thermistors in a 37 °C canola-oil bath, chosen because its conductivity is close to that of orbital fat, and the same bath is used in Chapter 2. The posterior sclera cooled by about 6.6 °C on average, some tissue in contact with the ring froze, and the removal of heated air limited how cold the ring could become.

A second prototype (Gen 2) was designed and built in 2018–2019 by K. Fong, and in 2018 N. Shirdade carried out thermal simulations of the cooling of the porcine eye. The ex-vivo methods manuscript in preparation builds on both projects (Contribution of Authors).

The third generation (Gen 3), the device of this dissertation, was completed by the author during his M.S. study [**118**]. A single annular thermoelectric module sits between the contact ring and a water block, and coolant carries the heat to a copper coil in ice water. In the same work, fresh porcine eyes with thermistors were cooled in the oil bath and imaged to document the sensor positions. The next steps set out

there, a ceramic ring, automatic control of the module, a smaller system, and tests in perfused eyes and human donor eyes, are the subject of this dissertation.

Results from the porcine eyes, obtained with Y. Nakamura, were presented at two conferences [**119, 120**] and published in 2025 [**1**], and the human donor experiments were presented in 2026 [**121**]. Within the same research program, Gongal and colleagues published two finite-element models of the human eye cooled by a scleral ring [**56, 54**] (Section 1.2.3).

1.9. Knowledge Gap and Problem Statement

Acute retinal ischemia lacks a proven treatment, and most patients present beyond the window of time-limited reperfusion (Section 1.1). Preclinical evidence supports cooling as a means of extending the retina's tolerance of ischemia, but a practical delivery strategy must provide localized, active, and controlled cooling through an accessible interface, sustain it, produce a characterized response at the posterior eye, and be safe in the living eye. To the author's knowledge, no existing approach provides all of these (Table 1.2).

The obstacle is measurement as much as hardware. Cooling applied at an accessible surface must reach the posterior pole through the ocular wall and contents while the choroid continuously resupplies heat (Section 1.2.3). The temperature of a contact device therefore says nothing about the temperature at the retina or optic nerve. Before this work, no scleral-contact cooling device had been characterized with spatially verified temperatures throughout the globe. The scleral-contact devices already applied in patients target surface pain and did not measure the temperatures they produced inside the eye (Section 1.4). No such device had been characterized thermally in human eyes, and the porcine eye had not been shown to predict the human thermal response. In living animals the posterior eye had been cooled from outside only in short research preparations, none under feedback control (Section 1.4). These were a saline-filled cap over the cat eye [**73**], tubing glued to the exposed rabbit sclera

[**74**], and cold gel or ice on the rat eye [**64, 82**]. No scleral-contact device had been used for sustained, controlled cooling of the posterior segment of an intact living eye of human size.

This dissertation addresses these gaps in three Specific Aims (Section 1.10): ex-vivo characterization of the scleral-contact eye cooler in porcine and human donor eyes, with temperatures recorded at five sites verified by micro-computed tomography (micro-CT), refinement of the device, and its first test in the perfused living eye.

1.10. Specific Aims

The central hypothesis of this dissertation is that a scleral-contact thermoelectric device can produce reproducible posterior ocular cooling under controlled ex-vivo conditions, retain cooling performance through selected design refinements, and produce functional responses consistent with retinal cooling in vivo while maintaining ocular and systemic safety under the tested conditions.

These aims assess thermal delivery and preclinical tolerability. Neuroprotective efficacy after ischemia and thermal analgesia motivate the work, but establishing either requires outcome studies with an injury model or a pain measure, which are left to future work (Section 5.5). The hypothesis is addressed through three Specific Aims.

Specific Aim 1: Validate SCEC Gen 3 and characterize its thermal performance in ex-vivo eyes. The objective is to characterize, across ex-vivo models using formalin-preserved porcine, fresh porcine, and fresh human donor eyes, the cooling the scleral-contact interface produces at defined measurement sites, and how much it varies between eyes. Temperature is recorded at five ocular sampling locations (anterior, equatorial, and posterior sclera; vitreous; and the proximal optic nerve) together with the SCR feedback channel. As a secondary comparison, preserved porcine eyes are compared with fresh porcine eyes to determine whether preserved tissue can serve as a benchtop surrogate for device development.

Two hypotheses are tested. First, SCEC Gen 3 produces a reproducible reduction in temperature at the proximal optic nerve within approximately 15 min (90% of the 20-min excursion), with its size interpreted against a 3 °C benchmark. Second, the cooling excursion at this site differs between fresh human and fresh porcine eyes by an amount that is small against the excursion itself. The absence of a difference cannot be shown directly. This comparison is therefore reported as the difference between the species with its confidence interval, whose limits give the largest difference that the data allow. Cooling is measured as the decrement from each preparation's own baseline, because baseline temperature differs between preparations. Absolute temperatures are read against two reference intervals: the 32–35 °C interval of systemic therapeutic hypothermia (Section 1.3) and the roughly 30–34 °C range of preclinical retinal protection (Section 1.3.3). The size of the decrement is judged against a 3 °C benchmark, the step at which functional recovery improved in isolated retina under simulated ischemia [**62**]. It is a benchmark and not an established threshold (Section 2.4), so absolute temperatures are reported alongside the decrement. Performance is characterized by temperature–time profiles, baseline temperature, cooling excursion, time to 90% of the excursion (t_{90}, a measure of kinetics), and variability across preparations.

Variability between eyes within each preparation and the thermal difference between species are evaluated separately (Chapter 2). This aim establishes ex-vivo thermal feasibility and characterizes device performance to inform subsequent development and in-vivo testing.

Specific Aim 2: Enhance the device (SCEC Gen 4) for efficiency, portability, safety, and patient comfort. The objective is to develop and evaluate SCEC Gen 4 refinements through computational modeling, experimental assessment of selected scleral contact ring designs, and development of an integrated printed circuit board for device control. Refinements include reduced-profile ring geometries with and without an integral corneal lens, alternative ring materials, an insulating

outer layer to focus heat extraction, water-block and cooling-circuit miniaturization, TEC potting (present in every device and not evaluated separately), and consolidation of control electronics. Reduced form factor and bulk are intended to improve portability, handling, and patient comfort, while material selection and TEC potting address safety considerations at the device–tissue interface and within the thermoelectric assembly.

Computational modeling provides the primary basis for the comparisons, and experiments on selected fabricated rings provide complementary evidence. The hypothesis is that selected geometric and material refinements can reduce device bulk while preserving cooling performance at the posterior measurement sites. Patient comfort itself is not directly measured; improvements relevant to it are assessed through reductions in device dimensions and bulk.

Specific Aim 3: Validation and Safety Assessment in an In-Vivo Porcine Model. The objective is to evaluate evidence of retinal cooling and assess ocular and systemic safety in six live pigs under a protocol approved by the University of Illinois Chicago Animal Care Committee (ACC), the university's institutional animal care and use committee (Section 4.2.1). Each animal undergoes three cooling sessions, lasting 30, 60, and 120 min, followed by a terminal session for final assessments and tissue collection. Testing in the perfused living eye evaluates device performance in the presence of physiological heat delivery, including the choroidal circulation (Section 1.2.3), and allows the systemic response to its application to be assessed.

Changes in the ERG are used as indirect functional evidence of retinal cooling, which reduces ERG amplitudes and prolongs their timing [**73, 74, 122, 76**]. The b-wave implicit time and amplitude are used together, a longer implicit time and a smaller amplitude being the expected response. Amplitude is not interpreted alone, because it also varies with electrode contact and position. Changes in the cooled eye are compared with those in the fellow eye, which removes systemic and anesthetic

effects shared by both eyes. These measurements indicate cooling but do not measure retinal temperature or its spatial extent.

Safety endpoints are retinal functional recovery after rewarming, corneal integrity assessed by fluorescein staining and surface analysis, intraocular pressure measured by tonometry, tissue-level histology, and systemic stability. ERG responses during cooling and after rewarming are assessed separately to distinguish reversible functional changes from persistent abnormalities within the observation period.

The hypothesis is that device application produces ERG changes consistent with retinal cooling, that the ERG returns to its baseline values after rewarming, and that the other measured safety endpoints show no increase beyond those of the fellow control eye across the tested cooling sessions.

CHAPTER 2

Specific Aim 1: Validate SCEC Gen 3 and Characterize Its Thermal Performance in Ex-Vivo Eyes

Portions of this chapter have been published or presented [**118, 119, 120, 1, 121**]; the Contribution of Authors gives the details.

The fresh-porcine recordings, first reported in the author's M.S. thesis [**118**] and published in [**1**], were reprocessed for this chapter together with the human data, through the common quality-control pipeline described in Section 2.2.6. All fresh-porcine values reported here are the reprocessed values. They differ slightly from the published ones, and none of the published conclusions changes. The two sets are compared in Appendix C.

2.1. Background and Rationale

This chapter measures how far cooling applied at the surface reaches into the eye. An ex-vivo preparation permits temperature sensors to be placed at multiple sites throughout the globe, their positions to be verified by imaging, and the resulting temperature fields to be recorded without the variability introduced by blood flow, systemic physiology, anesthesia, and thermoregulation. The absence of perfusion is also the preparation's principal limitation. It was addressed in part by immersing the posterior globe in a continuously mixed 37 °C bath (Section 2.2.2). Like the choroidal circulation, the bath resupplies heat to the globe during cooling. It does so from outside the sclera, however, whereas the choroid delivers heat directly beneath the retina, and the heat the bath delivered was not measured (Section 2.4). The preparation thus establishes the tissue-level response to scleral-contact cooling against a physiological warm boundary.

Three preparations were used because each answers a different question. Formalin-preserved porcine eyes, which can be stored and used without the timing constraints of fresh tissue, were used to develop the platform and to test whether it resolved region-specific cooling reproducibly. Fresh porcine eyes established proof of concept [**1**] and provided the reference against which preserved tissue was assessed as a practical benchtop surrogate. Fresh human donor eyes were required because agreement between the two species cannot be assumed. Porcine and human globes have similar axial lengths. In separate histological series, however, the porcine sclera is roughly 1.3 to 2.2 times as thick as the human sclera at the limbus and equator. Its thinnest region, 0.3 to 0.4 mm, lies only 5 to 6 mm behind the limbus, close to where the ring sits (Table 1.3). The sclera is the first conductive path between the contact ring and the posterior eye. These differences could alter both the magnitude and the spatial distribution of cooling. The porcine eye can inform device development (Chapter 3) and interpretation of the in-vivo study (Chapter 4) only to the extent that its thermal response matches the human response at the site of interest.

The hypotheses are stated in Section 1.10. The cooling at the proximal optic nerve was judged against a 3 °C benchmark derived from retinal evidence [**62**], and preserved porcine eyes were compared with fresh porcine eyes as a secondary comparison (Section 2.2.6).

2.2. Methods

2.2.1. Device Description: SCEC Gen 3. SCEC Gen 3 (Figure 2.1) consisted of three thermally coupled components: an aluminum scleral contact ring (SCR), an annular thermoelectric cooler (TEC), and a liquid-cooled aluminum heat exchanger (water block). The SCR formed the interface with the anterior sclera and was machined with a curved ocular-contact surface having an intended contact area of approximately 390 mm^2. A 14-mm-diameter central aperture left the cornea exposed

and preserved access to the optical axis. A negative-temperature-coefficient thermistor embedded in the SCR provided continuous measurement of its temperature.

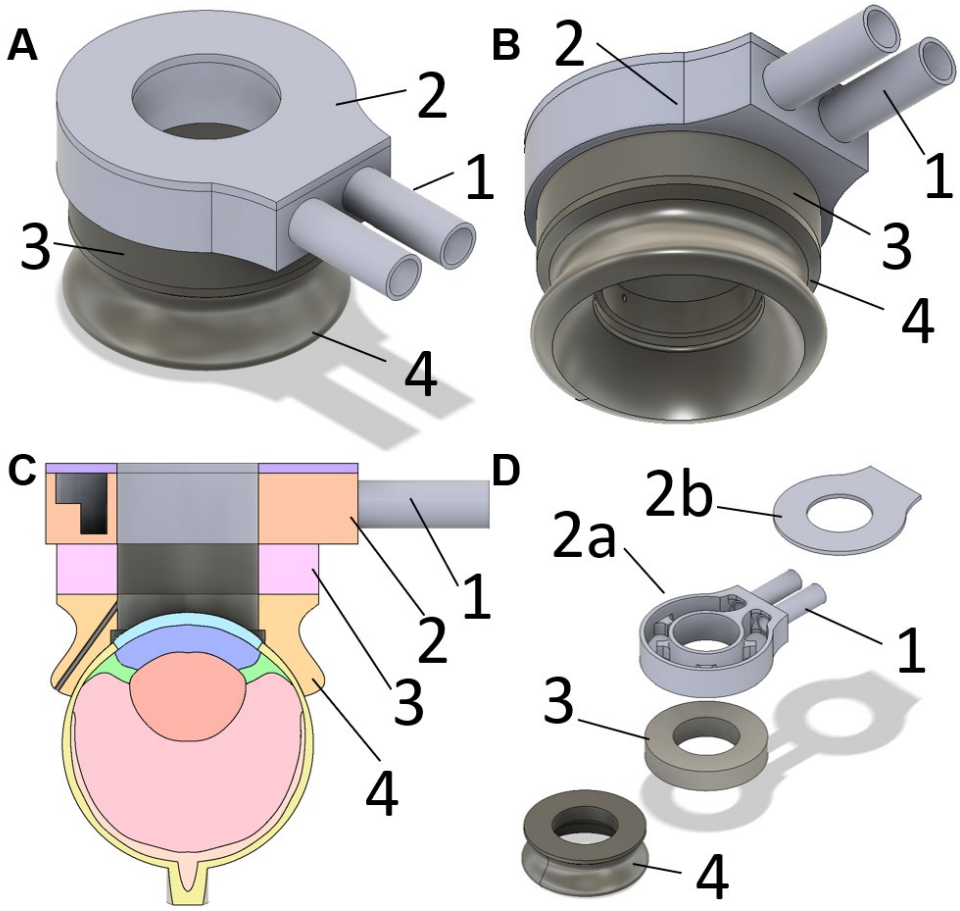

FIGURE 2.1. Components of SCEC Gen 3. (A) Solid model seen from above and (B) from below. (C) Cross-section of the device in position on a porcine eye. (D) Exploded view. 1, coolant inlet and outlet; 2, heat exchanger (2a, body; 2b, lid); 3, thermoelectric cooler (TEC); 4, scleral contact ring (SCR). Panel C is adapted from Nakamura et al., Fig. 2A [**1**], CC BY 4.0.

Heat extracted from the eye entered the aluminum SCR and was pumped across an annular TEC (RH14-14-10-L1-W4.5, Tark Thermal Solutions, Durham, NC, USA). The hot side of the TEC was coupled to the custom-machined aluminum heat exchanger, through which coolant was continuously circulated. The interfaces between the SCR, TEC, and heat exchanger were bonded with thermally conductive epoxy

(OB-101-2, Omega Engineering, USA). The assembled original prototype was approximately 23 mm in height.

The SCR temperature was the controlled variable throughout the experiments, and the cold set point was 4.0 °C in all cooling experiments described in this chapter. The TEC was driven in current-controlled mode at up to 3.9 A, the module's maximum current, about 8 W of electrical power at the supply (3.9 A at 2.1 V). In the porcine experiments, with preserved and with fresh eyes, the operator adjusted the supply to the TEC manually while monitoring the SCR thermistor, holding the SCR at the set point. For the human donor eyes, the platform had been upgraded with automatic feedback control. A comparator with hysteresis (Schmitt trigger) switched the TEC drive in response to the SCR thermistor (Figure D.2 in Appendix D). Once at the set point, the ring temperature of the human runs cycled over a band of about 0.2 °C with the original aluminum ring, 0.1 °C with the reduced-bulk ring, and 0.5 °C with the ceramic ring. Because the control method differed between the two cohorts, device performance was evaluated from the measured SCR temperature together with the simultaneously recorded tissue temperatures.

The set point was chosen at the lower limit of hypothermic organ storage. Organ preservation aims to hold tissue between 4 and 8 °C, because temperatures below about 2 °C increase the risk of cold injury [**116**], and donor corneal endothelium is preserved for up to two weeks at 4 °C [**123**]. Freezing injury requires ice formation, with lethal temperatures from about −2 to −70 °C depending on the tissue [**81, 124**], so a 4 °C boundary cannot produce it. Clinical ocular devices briefly expose the conjunctiva and sclera to much colder surfaces [**89, 92, 91**]. Tissue in contact with the ring remains warmer than the set point: in fresh porcine eyes the anterior sclera equilibrated near 11 °C (Section 2.3.2).

2.2.2. Ex-Vivo Apparatus. The ex-vivo apparatus was built for this work from custom subsystems: a positioning system for the eye and the cooling device, an

intraocular-pressure system, a warm oil bath, and a coolant circuit for the heat exchanger (Figures 2.2 and 2.3). It accepts excised porcine or human eyes instrumented with up to five temperature sensors, and it holds the induced tissue temperatures for as long as cooling is applied.

Eye and cooling-ring positioning system. An aluminum frame carried two independent vertical translation stages, one for the 3D-printed eye holder, on which the eye rested cornea-up on a perforated support ring, and one for a 3D-printed holder for the SCEC, so that the device could be lowered onto the eye.

Intraocular pressure. Intraocular pressure was maintained near the physiological range using a gravity-fed reservoir of room-temperature phosphate-buffered saline connected to the anterior chamber through a 27-Ga needle inserted through the cornea. In all three cohorts, the reservoir was set about 21 cm above the eye. A column of 20.7 cm of water corresponds to the mean intraocular pressure of normal porcine eyes, 15.2 mmHg [**114**]. In all fresh-eye experiments, fluid movement into the eye was minimal.

Warm oil bath. The posterior globe was partially submerged in a canola-oil bath maintained at approximately 37 °C. The oil was mixed continuously, at first by air bubbled through the oil and later, to reduce oil splatter, by a submersible pump. The bath stood in for the orbital tissues that surround the globe and, as a warm boundary, resupplied heat to the tissue in place of the absent perfusion. The oil level was approximately 4 mm posterior to the limbus, immediately below the scleral-contact device.

Coolant circuit. Heat rejected by the TEC was removed through the heat exchanger by coolant that a 12-V pump circulated continuously at approximately 150 $\mathrm{mL\,min^{-1}}$ through a copper coil immersed in a beaker of ice and water. The pump and its setting were the same as in the porcine runs of [**1**]. The coolant was a 50:50 mixture of propylene glycol and distilled water in the porcine experiments and distilled water in the human-donor experiments.

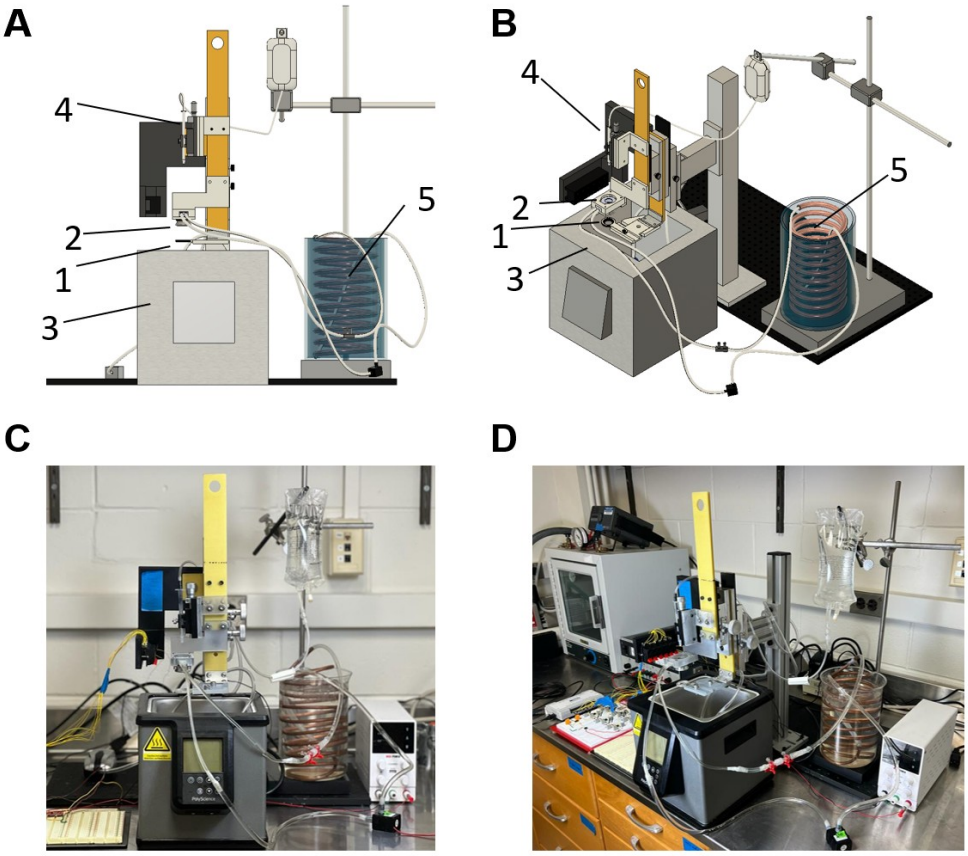

FIGURE 2.2. Ex-vivo apparatus. (A) Frontal and (B) panoramic views of the CAD model, and (C, D) the corresponding photographs of the assembled apparatus. 1, eye holder; 2, cooling-device holder; 3, oil bath; 4, intraocular-pressure syringe (fed from the elevated saline reservoir); 5, ice bath.

2.2.3. Ex-Vivo Eye Preparation. Three ex-vivo tissue cohorts were used, in the order in which they were studied: formalin-preserved porcine eyes, fresh porcine eyes, and fresh human donor eyes.

Formalin-preserved porcine eyes (Pig Eyes, Plain Preserved) were obtained from VWR International LLC (Pittsburgh, PA, USA). The eyes are sold as formalin-preserved specimens, without color staining, in a holding fluid that contains less than 0.1% formaldehyde by weight according to the supplier's safety data sheet. The preserved-eye experiments were used to assess the reproducibility and utility of the ex-vivo platform. Five preserved eyes were included in the thermal analysis.

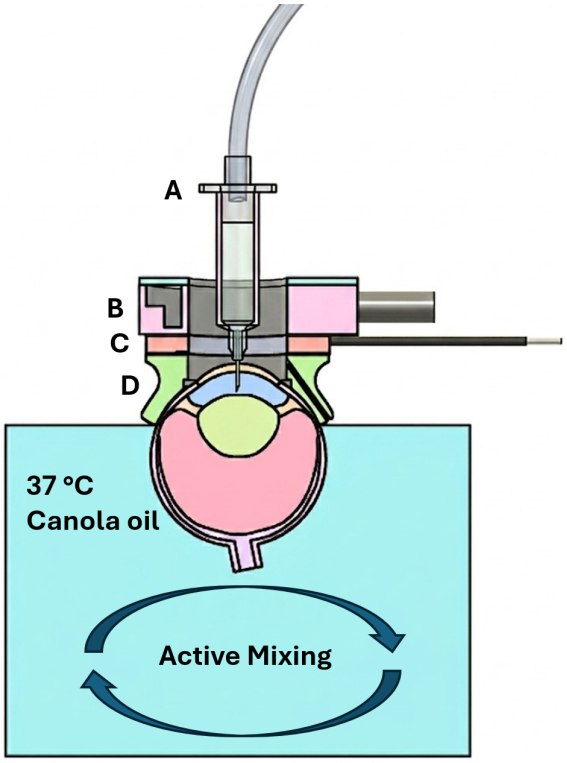

FIGURE 2.3. Schematic cross-section of the ex-vivo cooling setup. The posterior globe is partially submerged in the continuously mixed 37 °C canola-oil bath, and an infusion needle (A) in the anterior chamber connects the eye to the intraocular-pressure reservoir. SCEC Gen 3 rests on the anterior sclera: heat exchanger (B), TEC (C), and SCR (D).

Fresh porcine eyes were obtained through a commercial supplier (Animal Biotech Industries, Doylestown, PA, USA). Globes were harvested with about 13 mm (half an inch) of proximal optic nerve attached and shipped overnight in an insulated box with cold packs; they arrived the day after harvest and were tested on the day of arrival or the day after. Five fresh porcine eyes were included in the thermal analysis.

Fresh human donor eyes were obtained from VisionFirst Indiana Lions Eye Bank (Carmel, IN, USA). Eyes were eligible if they were received within 72 h of enucleation, were grossly intact, and retained at least 10 mm of proximal optic nerve. Eyes with penetrating trauma or gross ocular pathology were excluded. Nine eyes from five donors were included. Donors were male, 43–77 years of age, and the cohort supplied

for this study was exclusively Caucasian. Enucleation took place 8.5 to 19.4 h after death (median 12.4 h). The tissue was de-identified and supplied with authorization for research use.

In all three cohorts, excess extraocular tissue, including rectus muscle and orbital fat, was removed to expose the scleral surface, and each eye was then mounted cornea-up on the eye holder (Section 2.2.2) with a small quantity of cyanoacrylate adhesive.

2.2.4. Temperature Measurement and Spatial Verification. Temperature was measured with negative-temperature-coefficient thermistors (GA10K3MCD1, TE Connectivity Ltd., China) [**125**]. The sensing tip of each sensor was at most 3.3 mm long and 0.5 mm in diameter, with a nominal response time of 200 ms in liquid. Five sensors were implanted at anatomically defined locations: anterior sclera, equatorial sclera, posterior sclera, vitreous, and the proximal optic nerve, adjacent to the optic nerve head. The three scleral thermistors were introduced through small needle incisions and aligned approximately parallel to the scleral surface. The vitreous sensor was advanced toward the center of the globe. The proximal optic nerve sensor was inserted along the axis of the nerve in the porcine eyes, and parallel to the posterior scleral surface, at the junction of the nerve with the globe, in the human eyes (Figure 2.4). Cyanoacrylate adhesive was used to stabilize the leads at the insertion sites.

A sixth thermistor embedded in the SCR measured the imposed boundary temperature and provided the feedback signal used for device control. Each thermistor formed one arm of a voltage divider whose output was amplified by a non-inverting amplifier, giving approximately -8.0 V at 37 °C and $+8.0$ V at 4 °C (about $0.48\ \mathrm{V\,^{\circ}C^{-1}}$). A Hall-effect current sensor (ACS723 breakout, SparkFun Electronics, Niwot, CO, USA) in series with the TEC supply recorded the drive current. All six temperature channels were acquired simultaneously at 10 Hz using a DI-2108 data acquisition unit (DataQ Instruments, Akron, OH, USA) and recorded with WinDaq software (Figure D.1 in Appendix D).

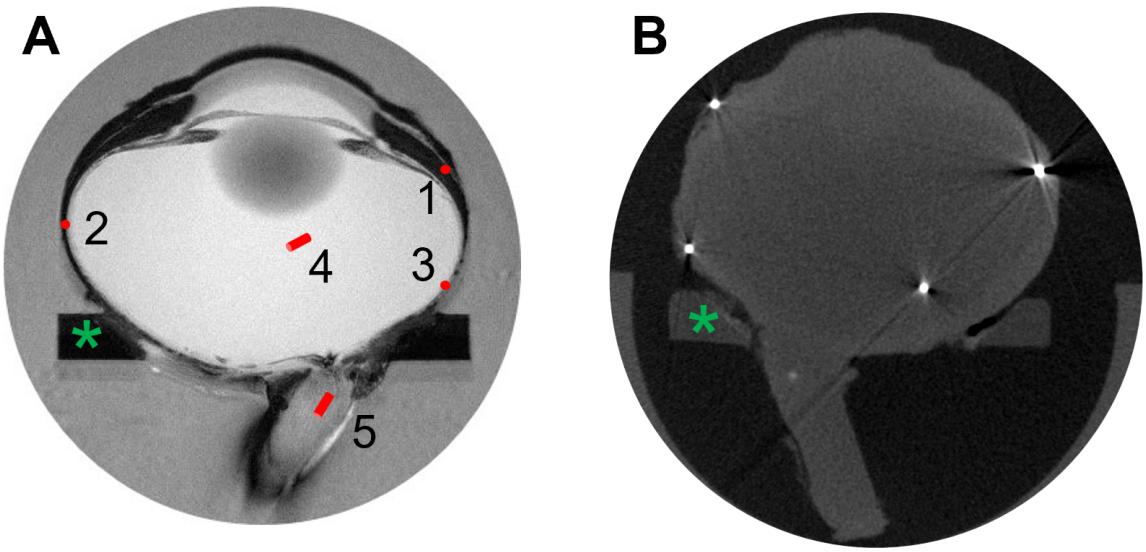

FIGURE 2.4. Target and measured positions of the implanted sensors in porcine eyes [**1**]. Left (A): MRI slice at the approximate midline of the globe, with the target position and orientation of each sensor drawn as a red cylinder: 1, anterior sclera; 2, equatorial sclera; 3, posterior sclera; 4, vitreous; 5, proximal optic nerve. The dark band between the front of the eye and the bath is not anatomy: it is the displacement of the oil bath relative to the tissue in the image (Section E.4). Right: micro-CT slice of an instrumented eye, in which the sensors appear as bright spots. The green asterisk marks the acrylic eye support. Adapted from Nakamura et al., Fig. 1 [**1**] (scale bars omitted), CC BY 4.0.

Porcine eyes were imaged by 9.4-T magnetic resonance imaging before implantation to document gross ocular anatomy, and micro-computed tomography after the cooling experiment localized the implanted thermistors in the porcine and human eyes (settings in Section E.4). Reconstructed volumes were imported into 3D Slicer, and sensor coordinates were used to assess spatial consistency and identify sensors that did not sample the intended anatomical compartment.

The calibration procedure evolved during the experimental program. In the porcine experiments, the channels were calibrated before each session in a Fluke 9102S dry-block calibrator [**126**] fitted with a custom insert. With the thermistors held at 20.5 and 37.0 °C, the two potentiometers of each channel's amplifier, which set the intercept and slope, were trimmed until the output read 0 V and −8.00 V,

respectively. These are the values of the circuit's linear voltage–temperature transfer function at those temperatures, and this single fixed function then converted voltage to temperature for all channels. For the later human-donor experiments, the channels were trimmed in the same way and three calibration plateaus, at 4.0, 20.5, and 37.0 °C, were then recorded in each session. Each plateau was read as the median of its last 3 min before the ramp to the next temperature, the last 10 s excluded; where those 3 min still drifted by more than 5 mV per minute, it was read as the median of the longest stretch whose 10-s medians stayed within ±0.01 V. The 37.0 °C plateau had settled for more than 7 min before each recording started, and where less than 3 min of it was recorded its value is the median of the whole recorded plateau. The same rule reads the in-vivo calibrations (Section 4.2.6). The voltage of each channel was converted to temperature with the least-squares straight line through its three plateaus. The conditioning circuit is linearized. On a bench recording of eleven dry-block temperatures between 2 and 39 °C, a straight line reproduced the block temperature within 0.34 °C.

The expanded uncertainty of an individual absolute temperature measurement was ±1.8 °C ($k = 2$). It is dominated by sensor self-heating, which can only make a reading too low, by at most 1.4 °C, and no correction was applied for it. Because the primary comparative endpoint was a within-run cooling excursion, static common-mode contributions largely canceled. In the human eyes, the corresponding differential uncertainty was ±0.40 °C ($k = 2$) for a small excursion, such as that of the proximal optic nerve, and ±0.85 °C for a large one, such as that of the SCR. In the porcine eyes, whose calibration was trimmed at two points, it was ±0.33 and ±1.1 °C. The uncertainty budget is given in Appendix D.

2.2.5. Experimental Protocol. For each experiment, the instrumented eye was positioned cornea-up in the 37 °C bath and the SCR was lowered onto the anterior sclera using a linear translation stage. Circumferential contact was established by directing light through the central aperture and lowering the device until light was

no longer visible between the ring and scleral surface. The sequence of steps is summarized in Figure 2.5.

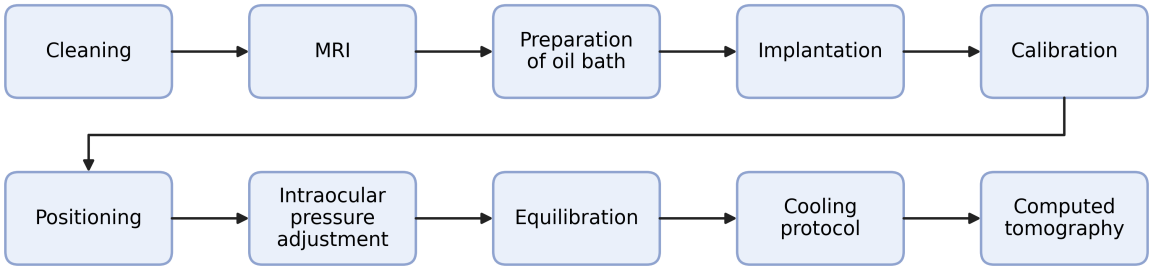

FIGURE 2.5. Experimental protocol, from receipt of the eye to post-experiment imaging, as followed for the porcine eyes. Human donor eyes followed the same sequence without MRI, and equilibrated passively in the 37 °C bath.

The porcine experiments used an active thermal-equilibration step before cooling. The SCEC was operated in reverse polarity to bring the interface toward approximately 34.5 °C, the temperature of the exposed anterior ocular surface, and the preparation was allowed to equilibrate for approximately 30 min. The temperature at each sensor at the first sample of the record defined its initial value.

At time zero, the device was switched to cooling polarity and the SCR set point was changed to 4.0 °C. Temperature at the SCR and all five implanted locations was recorded continuously for 20 min. The duration was sufficient for all measured compartments to approach a stable cooled state.

Human donor eyes were tested using the same 4.0 °C, 20-min cooling protocol, but were allowed to establish their baseline temperatures by passive equilibration in the 37 °C bath. Each human eye was cooled on the same day with three rings, the original ring reported here and the reduced-bulk and ceramic rings compared in Chapter 3, on a single set of implanted sensors and in an order that varied between eyes. Before each run the eye returned passively in the bath to about its resting state. The porcine and human experiments therefore began from different absolute baselines. For this reason, the cross-species comparison was based on the cooling excursion and not on the final absolute temperature.

2.2.6. Data Analysis. All recordings were processed with a single analysis pipeline, implemented as a Python package that runs the complete analysis from the raw files (Appendix E).

Denoising. After conversion to temperature (Section 2.2.4), each trace was median filtered (51 samples, 5.1 s) to suppress the spikes produced by the switching of the TEC relay, and then low-pass filtered with a zero-phase, second-order Butterworth filter (0.01 Hz cutoff). Before filtering, the start of each trace was extended by point reflection about its first sample, and the median filter padded the trace with its edge values to avoid underestimating the initial temperature.

Quality control. Three objective gates and a final visual review determined which traces entered the analysis. The gates do not use the size of the cooling excursion. The visual review, by its nature, was made with the traces in view. The gates are listed in order of precedence, and a trace failing more than one gate was excluded once, under the first reason in this order. First, sensor placement was verified from the co-registered micro-CT coordinates. Within each cohort, the positions of the four sensors fixed to tissue (anterior, equatorial, and posterior sclera, and proximal optic nerve) of every eye were rigidly aligned, by rotation and translation without scaling, to the mean set of the cohort (generalized Procrustes alignment, Equation E.9 in Section E.2). A tissue-fixed sensor was flagged when its aligned offset from the mean position of its site exceeded one thermistor length, 3.3 mm, the maximum length of the sensing tip given by the manufacturer. A shift of the whole eye in the scanner therefore does not flag a sensor, and the fence is the same for every cohort (Figure 2.6). The vitreous sensor is not fixed to tissue, so it did not enter the alignment and had no offset fence. It was flagged when its micro-CT position lay more than 75% of the distance from the center of the segmented globe to its wall in the same direction (Section E.4).

Second, channels whose raw voltage was flat-lined, or lay at the input limit of the acquisition unit ($\pm$10 V) for more than 75% of the record, indicating a disconnected

or saturated sensor, were excluded. One further human trace was excluded as a hardware fault although it passed this test. The anterior-sclera sensor of eye 9 was failing. It had reached the input limit in the recording made before the run, and it gave no signal in the two later runs on that eye. Third, a single-exponential decay was fitted to each remaining filtered trace, and traces with $R^2 < 0.85$ were excluded as noisy or non-monotonic. The fit served only this screen. Finally, every trace that passed the three gates was inspected against the other traces of its cohort and site. Traces showing an obvious transient artifact, a sustained oscillation or re-warming while the SCR held its set point, were excluded. Three traces were removed in this way. Every trace excluded by the hardware gate, the fit gate, or visual review is shown in Figure 2.7. Because the visual review was not blind to the traces, every comparison was repeated with the visually excluded traces retained, as a sensitivity analysis.

Endpoints. For each included trace, the primary endpoint was the cooling excursion,

$$\Delta T = T_{\text{initial}} - T_{\text{final}}, \tag{2.1}$$

where T_{initial} is the temperature at the first sample of the record ($t = 0$), at device activation, and T_{final} the mean temperature over the last 4 min of the 20-min record (minutes 16–20). In the porcine runs the SCR had already been running for a few seconds at $t = 0$. Cooling kinetics were summarized by t_{90}, the first time at which the trace had fallen by 90% of ΔT, and every trace had reached t_{90} before the final window began. The stability of the cooled plateau was summarized by the standard deviation (SD) of the median-filtered trace over the same 4-min window. Final temperature was reported descriptively.

Group comparisons. Two comparisons addressed the aim: preserved against fresh porcine eyes, to assess preserved tissue as a benchtop surrogate, and fresh human against fresh porcine eyes, both cooled with SCEC Gen 3, to test porcine–human

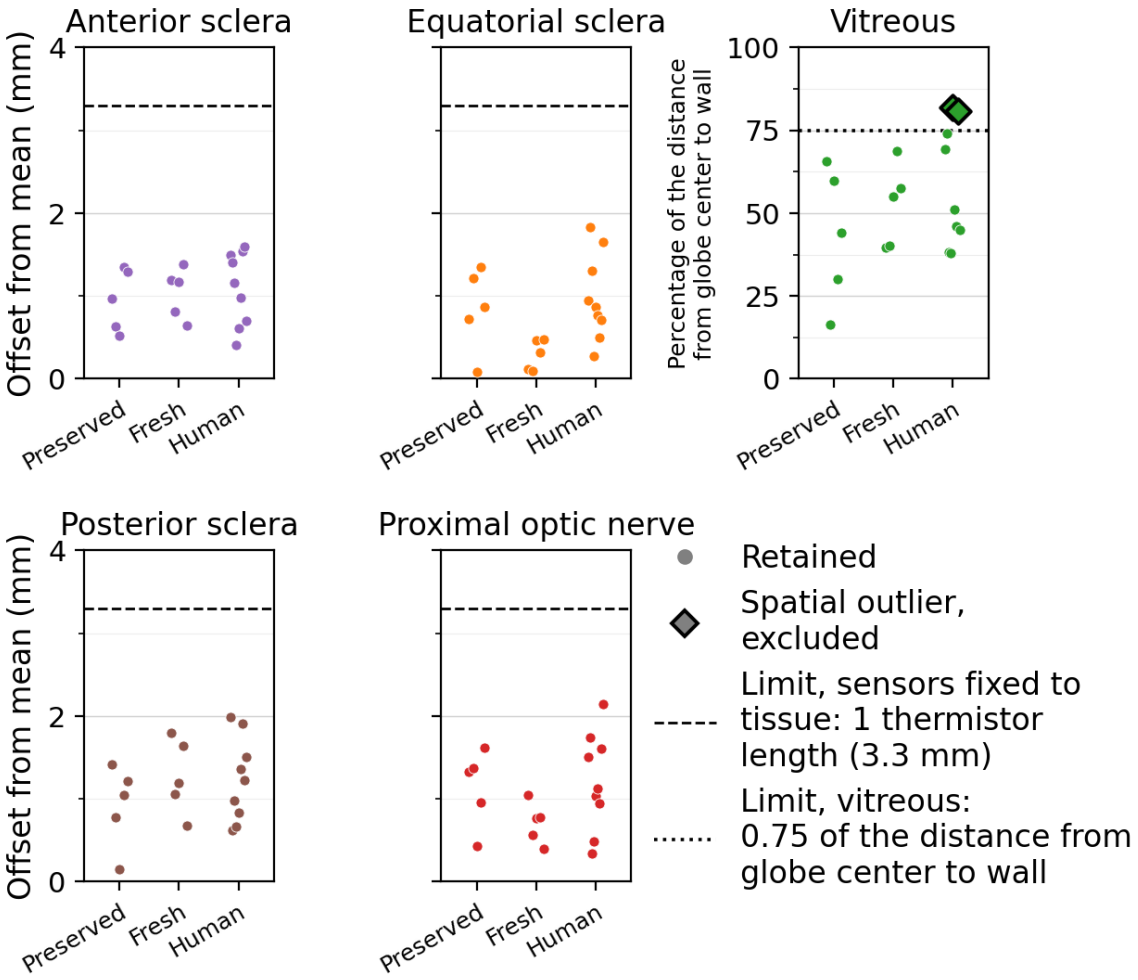

FIGURE 2.6. Spatial screen of sensor placement, in preserved porcine, fresh porcine, and human eyes. Scleral and proximal optic nerve panels: the offset $r_{e,s}$ of each sensor from the mean position of its site after the four tissue-fixed sensors of each eye were rigidly aligned to the cohort mean (Equation E.9); dashed lines mark the exclusion fence, one thermistor length (3.3 mm). Vitreous panel: the position of each vitreous sensor on micro-CT as a percentage of the distance from the center of the globe to its wall in the same direction; the dotted line marks the limit, 75%, and the two excluded sensors are drawn as diamonds (micro-CT sections in Figures E.2 and E.3).

thermal concordance. Both used the same procedure. Per-site contrasts used two-sided Welch t-tests with Holm correction across the five tissue sites. The SCR, a device endpoint, was analyzed separately and kept outside the Holm family. Effect sizes were reported as small-sample-corrected Hedges' g (the difference over the pooled SD) with 95% confidence intervals from the large-sample normal approximation, which is only

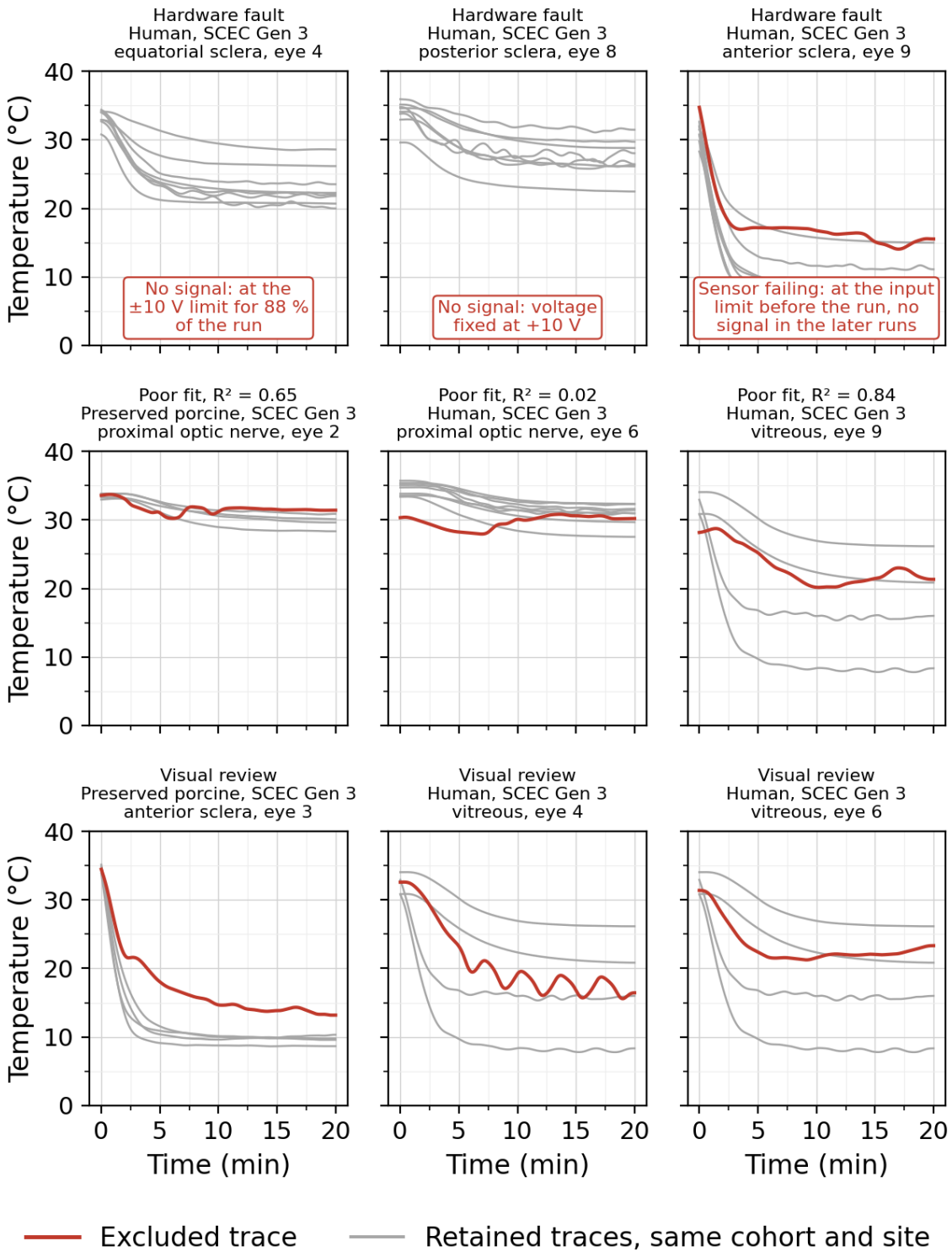

FIGURE 2.7. Traces excluded by the hardware gate, by the fit gate, and on visual review (red), each shown against the retained traces of the same cohort and site (grey). A channel with a hardware fault has no temperature signal, so its panel states the fault in the raw voltage; the failing sensor of human eye 9 had a signal in this run and is drawn with its note. For the fit gate, R^2 is that of the single-exponential fit. The two sensors excluded by the spatial screen are shown in Figure 2.6.

approximate at the sample sizes here (4 to 9 per group). Tests that treat the two eyes of a subject as independent can inflate the type I error, whereas a test on the per-subject average holds it at the nominal level [**127**]. The nine human eyes came from five donors, and each test was repeated with fellow eyes averaged to one value per donor, to verify that treating fellow eyes as independent did not drive the result. Each porcine eye was treated as its own donor, since the animal of origin was not recorded and the five fresh eyes came from one harvest. Because some sites retained only a few traces, failure to detect a difference was not interpreted as evidence that there is none.

Largest difference compatible with the data. Each comparison is also reported with the 90% confidence interval of the difference in ΔT. A difference beyond the interval limit farther from zero is called unlikely, which corresponds to a one-sided test at the 5% level. That limit is judged against the excursion at the site and the 3 °C benchmark [**62**]. Differences between groups are judged by these intervals, not by the measurement uncertainty of a single excursion (Section 2.2.4). Statistical significance was assessed at $\alpha = 0.05$.

2.3. Results

Three cohorts were cooled with SCEC Gen 3 under the original ring: five preserved porcine eyes, five fresh porcine eyes, and nine fresh human eyes. Of the 114 channel traces recorded, 103 passed quality control (Table 2.1). No scleral or proximal optic nerve sensor, in any cohort, lay beyond the placement fence. The largest offset was 2.1 mm. Two vitreous sensors, both in human eyes, lay more than 75% of the distance from the center of the globe to its wall (82 and 81%) and were excluded by the spatial screen. Three human channels had a hardware fault, three traces failed the fit gate, and three were removed on visual review. The vitreous, whose sensor is the least mechanically constrained, accounted for five of the eleven exclusions.

TABLE 2.1. Quality-control outcome by cohort. Each eye contributed six channel traces. Exclusions are listed in order of precedence: a trace that failed more than one gate is counted once, under the first reason listed.

Cohort	Traces	Spatial	Hardware	Fit	Visual	Included
Preserved porcine (5 eyes)	30	0	0	1	1	28
Fresh porcine (5 eyes)	30	0	0	0	0	30
Fresh human (9 eyes)	54	2	3	2	2	45
Total	114	2	3	3	3	103

2.3.1. Preserved Porcine Eyes. Cooling of the formalin-preserved porcine eyes was rapid and spatially ordered (Figure 2.8). All five preserved eyes were analyzed, and 28 of their 30 traces passed quality control. No sensor lay beyond the spatial fence. One proximal optic nerve trace failed the fit criterion, and one anterior-sclera trace was removed on visual review. The SCR held the commanded 4 °C set point, with a mean final temperature of 4.1 °C (Table 2.2).

Cooling varied by site (Table 2.2): the anterior sclera showed the largest excursion, 24.9 °C, and the proximal optic nerve the smallest and slowest, 3.7 ± 0.8 °C with t_{90} of 783 s (13.1 min). Over the final 4 min the deeper sites had approached but not reached a steady state. The largest dispersion between specimens was in the vitreous (final temperature SD 4.3 °C).

TABLE 2.2. Thermal response of preserved ex-vivo porcine eyes during SCEC Gen 3 cooling. Values are mean (SD) over the included traces (n). Initial temperature, final temperature, and ΔT were averaged independently, so ΔT need not equal the difference of the rounded means. Plateau SD is the within-trace standard deviation over the final 4 min of the record.

Location	n	Initial	Final	ΔT	t_{90}	Plateau SD
Units	–	(°C)	(°C)	(°C)	(s)	(°C)
SCR	5	32.1 (0.5)	4.1 (0.1)	27.9 (0.6)	111 (26)	0.06
Anterior sclera	4	34.5 (0.6)	9.6 (0.6)	24.9 (0.9)	188 (46)	0.06
Equatorial sclera	5	34.2 (0.3)	23.1 (1.9)	11.1 (2.0)	324 (123)	0.07
Vitreous	5	33.0 (0.9)	20.4 (4.3)	12.6 (3.4)	515 (72)	0.06
Posterior sclera	5	33.8 (0.4)	23.8 (1.9)	10.1 (1.8)	417 (83)	0.16
Proximal optic nerve	4	33.5 (0.4)	29.8 (1.0)	3.7 (0.8)	783 (95)	0.06

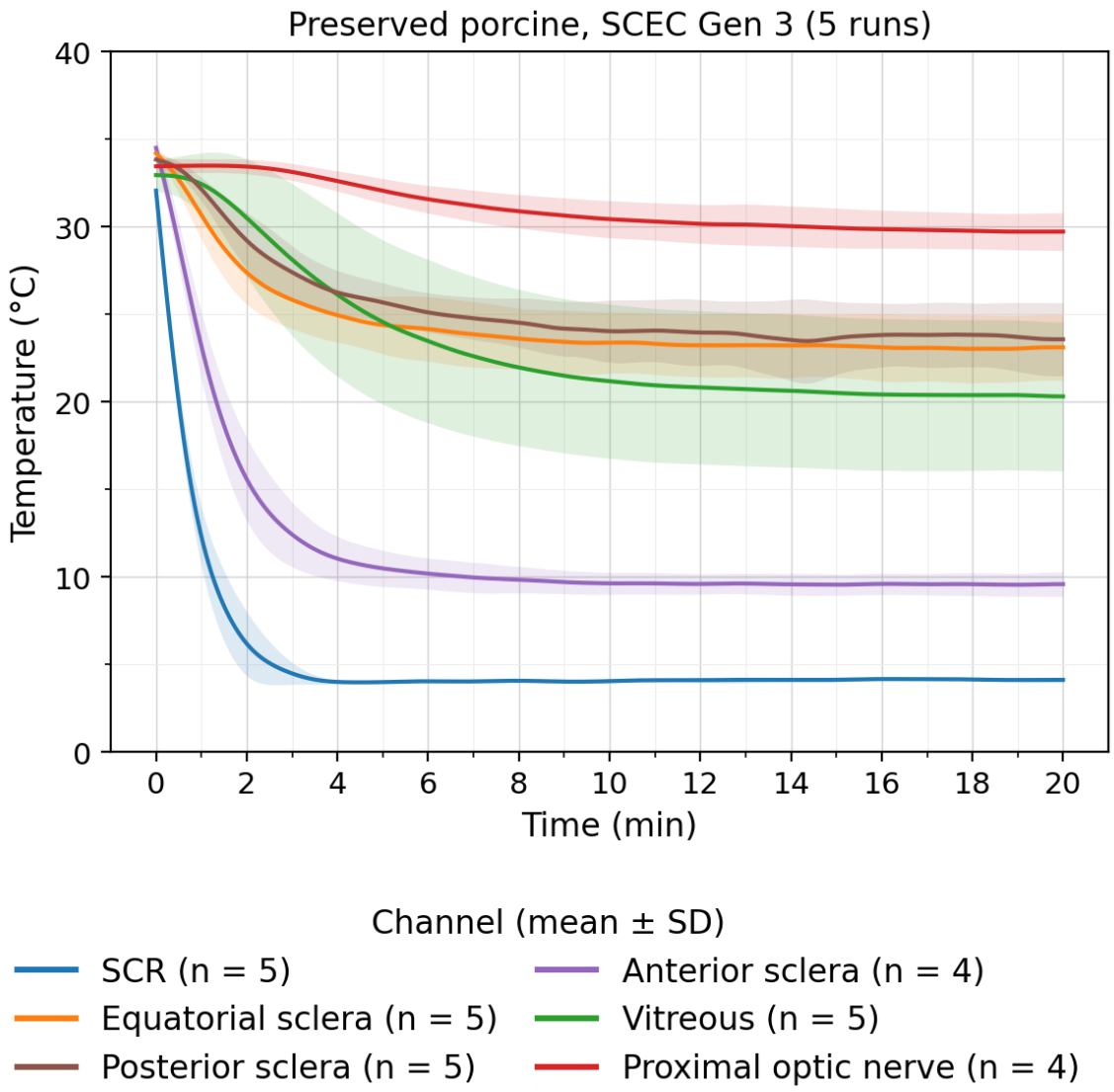

FIGURE 2.8. Temperature at each measurement site during SCEC Gen 3 cooling of five preserved porcine eyes. Lines are cohort means and shaded bands ±1 SD of the included traces; the legend gives the number of traces per site.

2.3.2. Fresh Porcine Eyes: Proof of Concept. SCEC Gen 3 produced a rapid and spatially ordered decrease in temperature throughout the isolated porcine eye (Figure 2.9). The SCR reached 90% of its excursion in 95 s and held a mean final temperature of 4.1 °C.

The anterior sclera again cooled most, by 24.3 °C, and the proximal optic nerve least, by 3.5 ± 1.5 °C, above the 3 °C benchmark, with t_{90} of 11.3 min (Table 2.3). Its

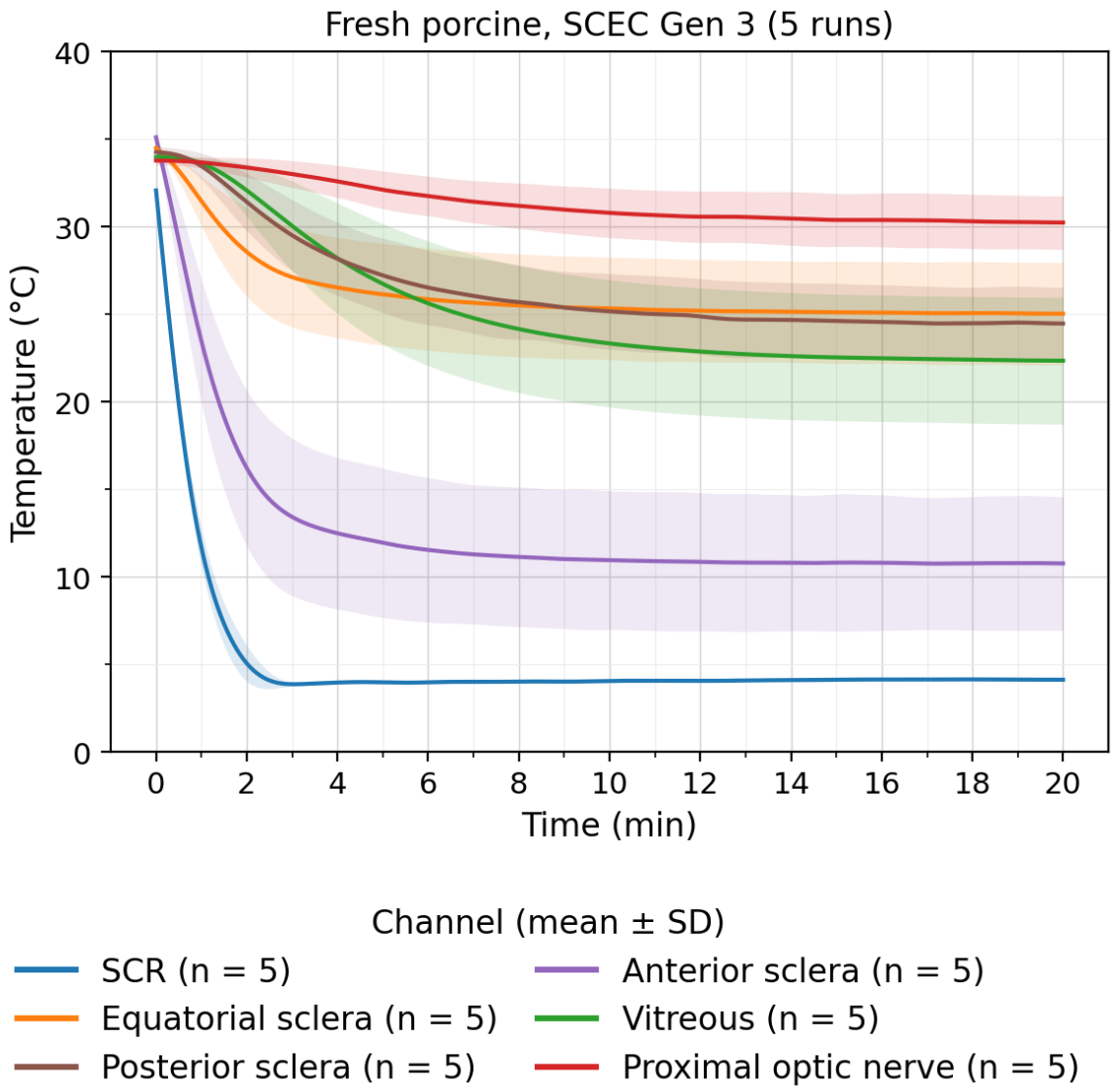

FIGURE 2.9. Temperature at each measurement site during SCEC Gen 3 cooling of five fresh porcine eyes. Lines are cohort means and shaded bands ±1 SD of the included traces; the legend gives the number of traces per site.

final temperature lay below the 32–35 °C interval of systemic hypothermia [**57**], within which the baseline already fell, and inside the roughly 30–34 °C range of preclinical retinal protection (Section 1.3.3). All 30 traces passed quality control. Relative to its small excursion the proximal optic nerve varied most (coefficient of variation 43%).

TABLE 2.3. Thermal response of fresh ex-vivo porcine eyes during SCEC Gen 3 cooling. Values are mean (SD) over the included traces (n). Initial temperature, final temperature, and ΔT were averaged independently, so ΔT need not equal the difference of the rounded means. Plateau SD is the within-trace standard deviation over the final 4 min of the record.

Location	n	**Initial**	**Final**	ΔT	t_{90}	**Plateau SD**
Units	–	(°C)	(°C)	(°C)	(s)	(°C)
SCR	5	32.1 (0.4)	4.1 (0.1)	27.9 (0.4)	95 (12)	0.03
Anterior sclera	5	35.1 (0.5)	10.8 (3.8)	24.3 (3.9)	200 (67)	0.06
Equatorial sclera	5	34.5 (0.2)	25.1 (2.9)	9.4 (3.1)	340 (92)	0.05
Vitreous	5	34.0 (0.3)	22.4 (3.6)	11.6 (3.8)	566 (63)	0.04
Posterior sclera	5	34.3 (0.3)	24.5 (2.1)	9.8 (2.1)	519 (64)	0.08
Proximal optic nerve	5	33.8 (0.3)	30.3 (1.5)	3.5 (1.5)	676 (110)	0.06

2.3.3. Fresh Human Donor Eyes. The same device was then evaluated in fresh human donor eyes (Figure 2.10). Nine eyes from five donors were tested; 45 of 54 traces passed quality control, leaving four to nine traces per site (Table 2.4).

The spatial pattern seen in porcine eyes was reproduced (Table 2.4). The anterior sclera cooled most, by 21.8 °C, and the proximal optic nerve least, by 3.8 ± 1.1 °C, to 30.8 °C, with t_{90} of 11.1 min. Plateaus were stable (mean within-plateau SD at most 0.24 °C). Human starting temperatures differed from the porcine ones because the cohorts were equilibrated differently (Section 2.2.5). Before cooling, the human eyes showed a gradient of about 4 °C from the anterior sclera to the proximal optic nerve, similar to that measured in patients [**47**].

TABLE 2.4. Thermal response of fresh human donor eyes during SCEC Gen 3 cooling. Values are mean (SD) over the included traces (n); columns as in Table 2.3.

Location	n	**Initial**	**Final**	ΔT	t_{90}	**Plateau SD**
Units	–	(°C)	(°C)	(°C)	(s)	(°C)
SCR	9	29.7 (1.4)	4.1 (0.1)	25.7 (1.4)	124 (27)	0.07
Anterior sclera	8	30.9 (1.4)	9.1 (2.9)	21.8 (2.8)	233 (82)	0.10
Equatorial sclera	8	33.2 (1.2)	23.2 (2.8)	10.0 (2.6)	420 (131)	0.14
Vitreous	4	32.2 (1.6)	17.7 (7.7)	14.4 (6.8)	437 (233)	0.15
Posterior sclera	8	33.9 (1.9)	27.4 (2.7)	6.5 (1.3)	531 (122)	0.24
Proximal optic nerve	8	34.6 (0.9)	30.8 (1.6)	3.8 (1.1)	667 (79)	0.09

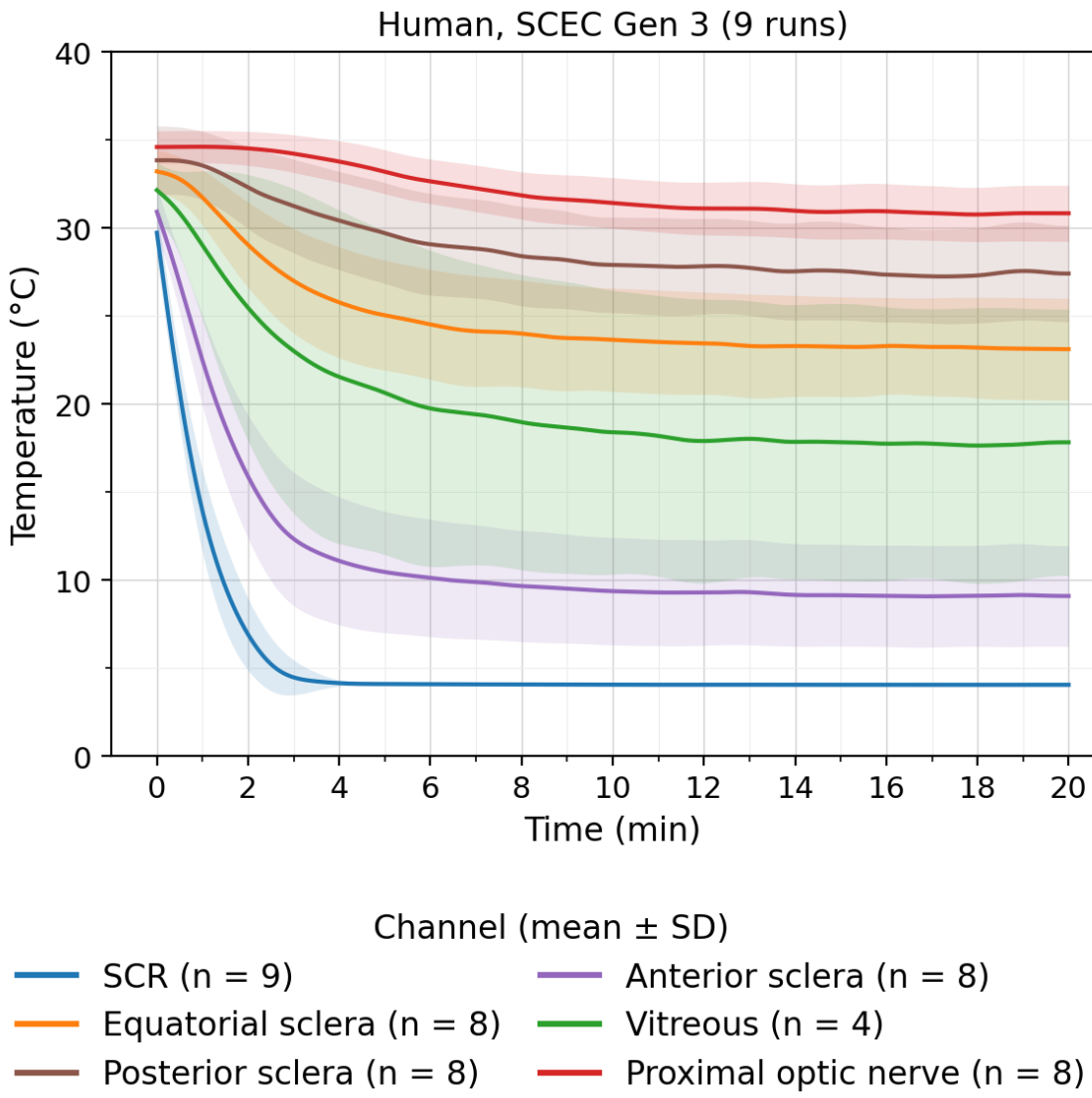

FIGURE 2.10. Temperature at each measurement site during SCEC Gen 3 cooling of nine fresh human donor eyes. Lines are cohort means and shaded bands ±1 SD of the included traces; the legend gives the number of traces per site.

2.3.4. Repeatability of Cooling at the Optic Nerve. Cooling at the posterior target was present in every eye but varied in magnitude between eyes (Table 2.5). All 17 included proximal optic nerve traces cooled, by between 2.3 and 5.9 °C. All 17 reached 90% of their excursion within 15 min, the slowest at 14.3 min. Twelve of the 17 traces reached the 3 °C benchmark, one of them at 2.999 °C. The between-eye coefficient of variation of the proximal optic nerve excursion was 22–43%, against 1–6%

at the SCR, whose temperature was controlled. The boundary was thus reproduced closely from eye to eye, and the variation at the posterior target arose within the tissue.

TABLE 2.5. Repeatability of cooling at the proximal optic nerve across eyes. Counts are of included proximal optic nerve traces. CV is the between-eye coefficient of variation of ΔT; [a]at the proximal optic nerve; [b]one trace at 2.999 °C.

Cohort (eyes)	n (traces)	**Traces reaching** $\Delta T \geq 3$ °C	$t_{90} \leq 15$ **min**	ΔT **range** (°C)	**CV of** ΔT **(%)** **Nerve**[a]	**SCR**
Preserved porcine	4	3	4	2.95–4.56	22	2
Fresh porcine	5	3	5	2.26–5.92	43	1
Fresh human	8	6[b]	8	2.51–5.79	29	6
Total	17	12[b]	17	2.26–5.92		

2.3.5. Fresh Versus Preserved Porcine Eyes. In preserved eyes, cooling closely followed that of fresh eyes (Figure 2.11). The SCR held 4.1 °C in both preparations, reaching 90% of its excursion in 111 s in preserved eyes and 95 s in fresh eyes. The spatial pattern of cooling survived chemical fixation. In both preparations the anterior sclera cooled most and fastest, and the proximal optic nerve least and slowest, with t_{90} of 783 s (13.1 min) in preserved and 676 s (11.3 min) in fresh eyes.

No difference in cooling excursion was detected between preserved and fresh tissue at any of the five implanted sites (Table 2.6; all Holm-adjusted $p = 1.00$). The largest nominal difference was at the equatorial sclera, where preserved eyes cooled 1.7 °C more. At the proximal optic nerve the difference was −0.2 °C (fresh minus preserved). The 90% confidence intervals (CI) of the difference at the SCR and the proximal optic nerve (Table 2.6) make a difference larger than 0.6 °C at the SCR, or 1.7 °C at the proximal optic nerve, unlikely. At the remaining sites the intervals were wider and allow differences of 2.6 to 5.2 °C. Retaining the visually excluded anterior-sclera trace changed the anterior difference from −0.6 to +0.2 °C and left every other value, and every conclusion, unchanged.

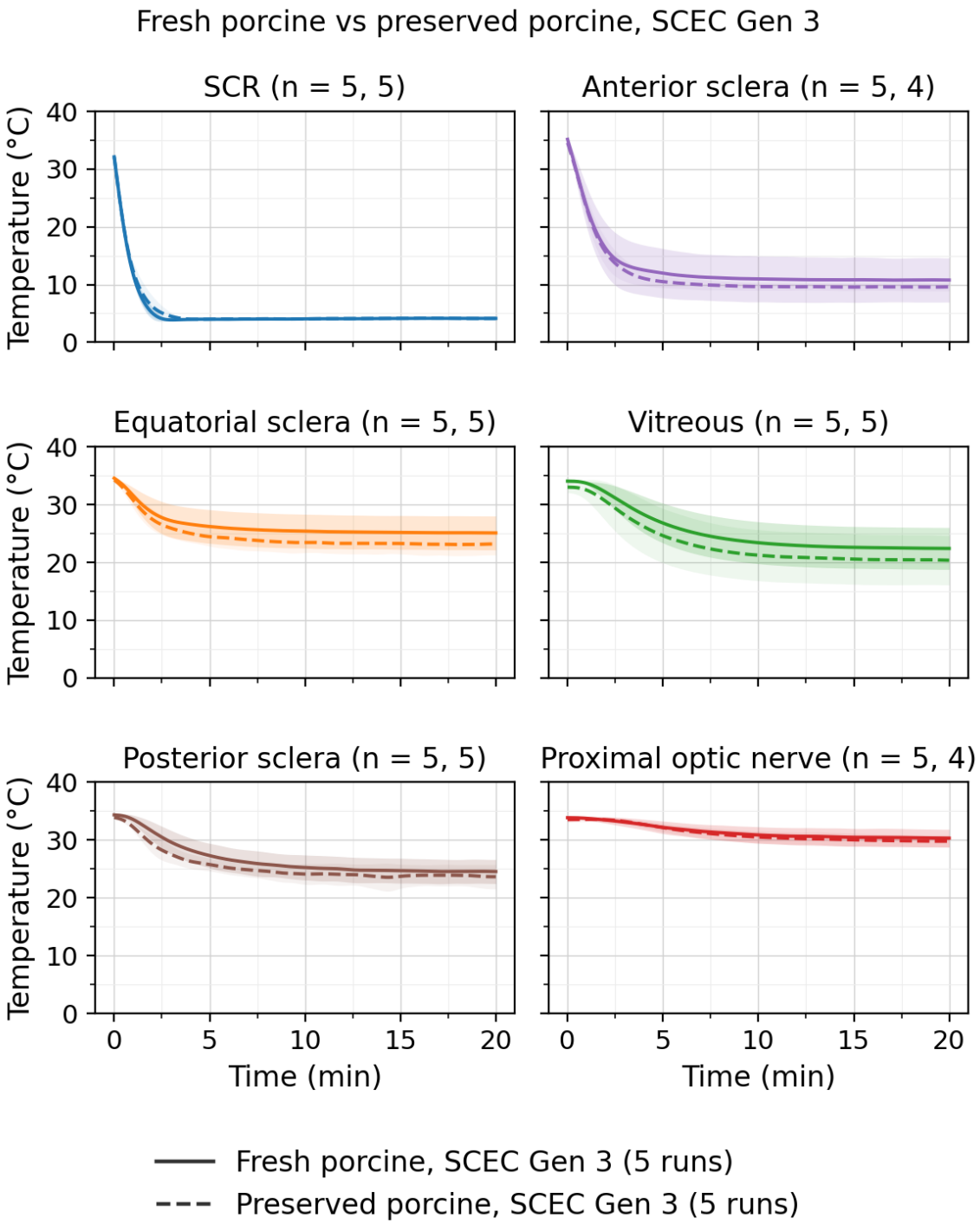

FIGURE 2.11. Temperature at each measurement site during SCEC Gen 3 cooling of fresh (solid) and preserved (dashed) porcine eyes. Lines are cohort means and shaded bands ±1 SD of the included traces; panel titles give the number of traces (fresh, preserved).

TABLE 2.6. Cooling excursion ΔT in preserved and fresh porcine eyes. Group values are mean (SD) with the number of included traces in brackets. Difference is fresh minus preserved, with its 90% confidence interval (Welch), whose limit farther from zero is the largest difference that the data allow. p_{Holm} is corrected across the five tissue sites; the SCR is tested separately, and its p (marked *) is unadjusted. g is Hedges' g with its 95% confidence interval. Anterior, equatorial, and posterior denote the scleral sites. Temperatures in °C.

Location	**Preserved**	**Fresh**	**Diff.**	**90% CI**	p_{Holm}	g **(95% CI)**
SCR	27.9 (0.6) [5]	27.9 (0.4) [5]	0.0	−0.6, 0.6	0.96*	0.03 (−1.09, 1.15)
Anterior	24.9 (0.9) [4]	24.3 (3.9) [5]	−0.6	−4.4, 3.2	1.00	−0.18 (−1.35, 1.00)
Equatorial	11.1 (2.0) [5]	9.4 (3.1) [5]	−1.7	−4.8, 1.5	1.00	−0.58 (−1.73, 0.57)
Vitreous	12.6 (3.4) [5]	11.6 (3.8) [5]	−1.0	−5.2, 3.3	1.00	−0.25 (−1.37, 0.88)
Posterior	10.1 (1.8) [5]	9.8 (2.1) [5]	−0.3	−2.6, 2.0	1.00	−0.14 (−1.26, 0.98)
Proximal optic nerve	3.7 (0.8) [4]	3.5 (1.5) [5]	−0.2	−1.7, 1.3	1.00	−0.15 (−1.33, 1.02)

2.3.6. Human–Porcine Thermal Concordance. For the cross-species analysis, human and fresh porcine eyes were compared under the same ring, so that the species effect was not confounded by device geometry (Figure 2.12, Table 2.7).

At the proximal optic nerve, the two species cooled by similar amounts. The mean excursion was 3.8 ± 1.1 °C in human eyes and 3.5 ± 1.5 °C in fresh porcine eyes. The human-minus-porcine difference was 0.30 °C (Table 2.7). No difference was detected, and one larger than 1.8 °C is unlikely, since it lies outside the 90% confidence interval and is excluded by a one-sided test at the 5% level (Section 2.2.6). With fellow human eyes averaged to one value per donor, the difference was much the same, and the result did not depend on treating fellow eyes as independent. Leaving out either of two of the five porcine eyes raised the upper limit of the interval, and pooling the fresh and preserved porcine traces narrowed it, without changing the conclusion.

Of the five implanted tissue sites, only at the proximal optic nerve was the species difference bounded below 2 °C. Away from the proximal optic nerve, the species differences varied by site. Human eyes cooled slightly more at the equatorial sclera, whereas porcine eyes cooled more at the anterior and posterior sclera. The largest consistent site-specific contrast was at the posterior sclera, where porcine tissue cooled

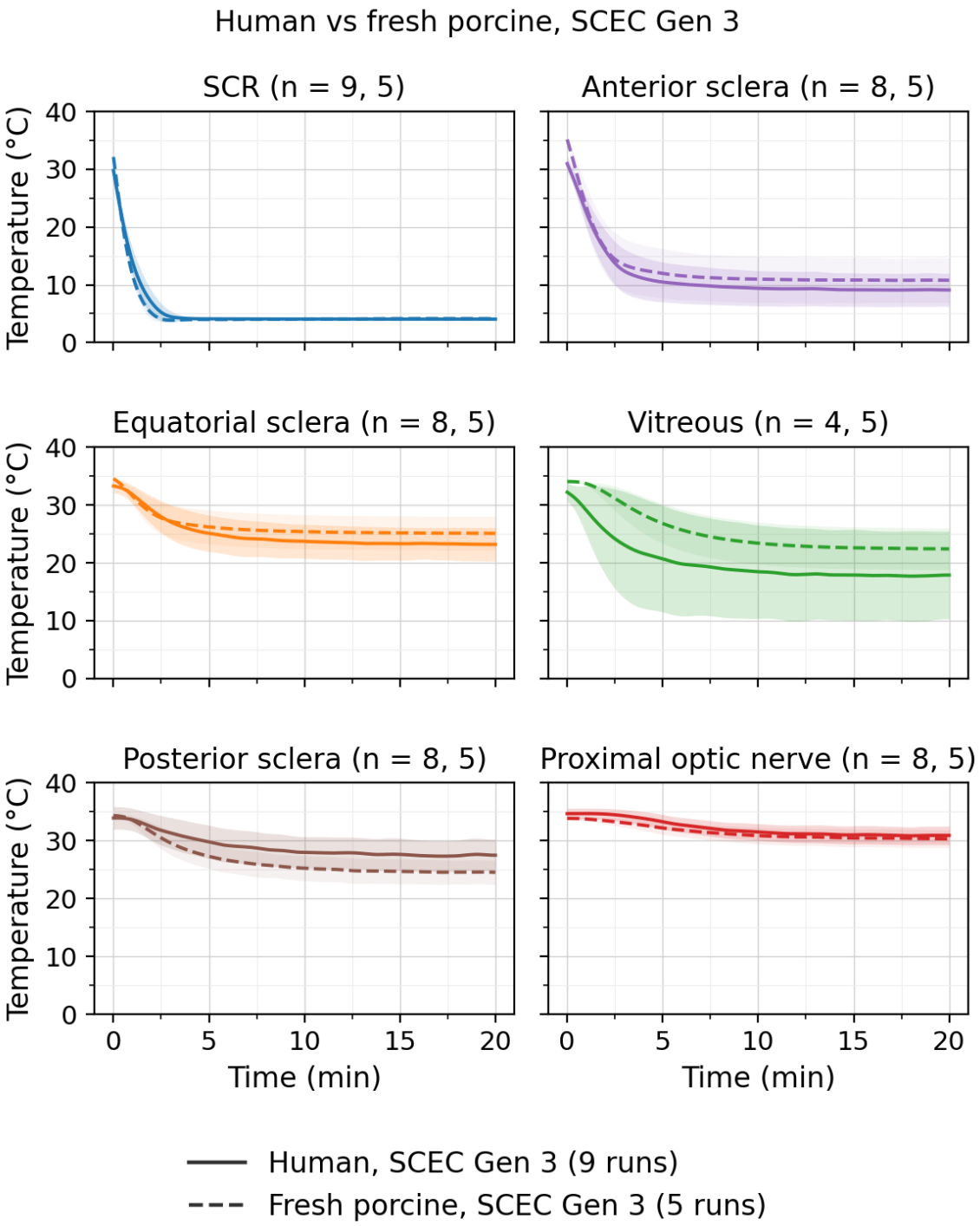

FIGURE 2.12. Temperature at each measurement site during SCEC Gen 3 cooling of fresh human (solid) and fresh porcine (dashed) eyes. Lines are cohort means and shaded bands ±1 SD of the included traces; panel titles give the number of traces (human, porcine).

TABLE 2.7. Cooling excursion ΔT in fresh human and fresh porcine eyes under SCEC Gen 3. Group values are mean (SD) with the number of included traces in brackets. Difference is human minus porcine, with its 90% confidence interval (Welch). p_{Holm} is corrected across the five tissue sites; the SCR is tested separately, and its p (marked *) is unadjusted. g is Hedges' g with its 95% confidence interval. Anterior, equatorial, and posterior denote the scleral sites. Temperatures in °C.

Location	**Human**	**Porcine**	**Diff.**	**90% CI**	p_{Holm}	g **(95% CI)**
SCR	25.7 (1.4) [9]	27.9 (0.4) [5]	−2.3	−3.2, −1.3	0.001*	−1.79 (−3.01, −0.57)
Anterior	21.8 (2.8) [8]	24.3 (3.9) [5]	−2.5	−6.4, 1.4	1.00	−0.71 (−1.79, 0.36)
Equatorial	10.0 (2.6) [8]	9.4 (3.1) [5]	0.6	−2.6, 3.7	1.00	0.19 (−0.85, 1.23)
Vitreous	14.4 (6.8) [4]	11.6 (3.8) [5]	2.8	−5.1, 10.7	1.00	0.47 (−0.71, 1.66)
Posterior	6.5 (1.3) [8]	9.8 (2.1) [5]	−3.3	−5.3, −1.2	0.10	−1.88 (−3.15, −0.62)
Proximal optic nerve	3.8 (1.1) [8]	3.5 (1.5) [5]	0.3	−1.2, 1.8	1.00	0.22 (−0.82, 1.26)

3.3 °C more than human tissue. Its effect-size interval was the only one among the tissue sites to exclude zero ($g = -1.88$). The contrast did not remain significant after correction for multiple comparisons (nominal $p = 0.02$, Holm-adjusted $p = 0.10$). The differences at the equatorial sclera and in the vitreous were small against their wide intervals (Table 2.7). With the two visually excluded human vitreous traces retained, the per-site vitreous difference fell from 2.8 to 2.1 °C. No proximal optic nerve trace was excluded visually, so the proximal optic nerve results, including the confidence interval of the species difference, were identical in both analyses. The SCR excursion was 2.3 °C smaller in human eyes ($p = 0.001$). Both cohorts ended at 4.1 °C, and the difference reflects the lower human starting temperature.

2.4. Discussion

Specific Aim 1 established that SCEC Gen 3 delivers controlled trans-scleral cooling to the deep posterior eye. The first hypothesis was supported in occurrence and timing but not in magnitude: every eye cooled at the proximal optic nerve and all 17 reached 90% of the excursion within 15 min, but only 12 of 17 reached the 3 °C benchmark (Section 2.3.4). The test of the second hypothesis found no detectable

difference between the species at that site and a bound of 1.8 °C. Preserved and fresh porcine eyes did not differ detectably at the SCR or the proximal optic nerve.

Magnitude of cooling at the posterior target. The mean proximal optic nerve excursion exceeded the 3 °C benchmark in every cohort. The benchmark is the smallest step tested in isolated retina subjected to simulated ischemia [**62**]. That step ran from 36 to 33 °C, whereas the proximal optic nerve started ex vivo at a lower temperature (Tables 2.3 and 2.4). A 3 °C decrement here is therefore comparable in size to that step but does not reproduce it. The final proximal optic nerve temperatures in fresh porcine and human eyes lie within the roughly 30–34 °C range over which preclinical retinal studies report protection (Section 1.3.3). Both are also close to the 30 °C at which retinal ganglion cells were protected in hypoxic organ culture [**71**]. These absolute values depend on each preparation's starting temperature and carry the absolute measurement uncertainty of Section 2.2.4, most of it a possible underreading from sensor self-heating.

The decrement reached in vivo is expected to be smaller than ex vivo, because perfusion resupplies heat that the bath did not (see *Absence of perfusion*, below). Delivery at this magnitude for 20 min from a boundary held at 4 °C, well above freezing, is the engineering premise on which the remaining aims depend.

The proximal optic nerve as a conservative index. Most of the retina was likely cooled further than the proximal optic nerve. The proximal optic nerve was chosen as the deep posterior target because it is the site farthest from the ring; its final temperature, 30.3 to 30.8 °C, lies at the lower end of the preclinical retinal range. It does not, however, represent the retina as a whole. The retina lines the globe from just behind the ring to the optic disc, and the cooling imposed at the ring reaches it along a gradient. The retina itself was not instrumented. It lies, however, between the vitreous and the posterior sclera, which in every cohort ended far colder than the proximal optic nerve (Tables 2.2, 2.3, and 2.4), and the peripheral retina lies closer

still to the ring. The computational model of Chapter 3 resolves this gradient along the retina.

Preserved tissue as a benchtop surrogate. Preserved and fresh porcine eyes behaved alike at the two sites where the comparison was decisive. The SCR followed the same trajectory in both, and the proximal optic nerve excursions differed by 0.2 °C, with a difference larger than 1.7 °C unlikely. The proximal optic nerve responded more slowly in preserved eyes (Section 2.3.5). At the scleral and vitreous sites, with four or five traces per group, the confidence intervals were too wide to show either a difference or its absence. Preserved eyes are therefore suitable for what the platform is most often asked to do, comparing design variants under identical conditions, where a change in region-specific cooling is the quantity of interest and tissue availability is otherwise limiting. Absolute performance, and any conclusion about sites other than the posterior target, should rest on fresh tissue.

Porcine tissue as a surrogate for the human eye. The cross-species comparison is compatible with using the porcine eye, and a porcine in-vivo model, as a surrogate for the human eye at the posterior target, and only there. At the proximal optic nerve the species differed by 0.3 °C, and by much the same with fellow eyes averaged by donor. The other tissue sites gave a mixed picture. Porcine eyes cooled 3.3 °C more at the posterior sclera, the largest consistent site-specific contrast, although it did not survive correction for multiple comparisons. At the equatorial sclera and in the vitreous the intervals were too wide to interpret. The posterior-sclera contrast is not explained by the starting temperature, which was nearly the same in the two species at this site, and it is consistent with the anatomy. The porcine sclera is thinnest only 5 to 6 mm behind the limbus, close to the ring, whereas the human sclera is thickest at the posterior pole (Table 1.3).

Differences in globe size, scleral thickness, and the depth of each site below the warm bath may contribute to these contrasts. Sensor placement also contributes. The vitreous sensor has no rigid landmark, cannot be seen during insertion, and showed

the largest variance in position in both species, and in final temperature in the human eyes.

The state of the vitreous may also differ, as the human vitreous liquefies progressively with age [**128**], and the two tissues differed in post-mortem age. Neither was measured, and both remain untested explanations.

Comparability of the cohorts. The porcine and human experiments differed in three ways that bear on the comparison at the proximal optic nerve, and two of them would both tend to enlarge the human excursion relative to the porcine one, in the direction of the observed difference. First, the control method was manual in the porcine and automatic in the human experiments. The SCR, however, reached the same 4.1 °C plateau in every cohort, so the imposed boundary was comparable. Second, the starting temperature differed. After equilibration with the unpowered ring resting on the eye, the human SCR and anterior sclera began 2.3 and 4.2 °C cooler than in the porcine experiments, and the human proximal optic nerve about 0.8 °C warmer. An excursion toward a fixed boundary grows with the distance from that boundary. Third, the proximal optic nerve sensor was oriented differently, along the nerve in the porcine eyes and parallel to the posterior sclera at the junction of the nerve with the globe in the human eyes. The human sensor thus sampled tissue closer to the cooled globe. The size of these second and third effects could not be estimated from these data.

Absence of perfusion. The ex-vivo measurements likely overestimate the cooling the device produces at depth in a perfused eye, most at the sites farthest from the ring. All experiments were performed in non-perfused tissue. The continuously mixed 37 °C bath restored, in part, the warm boundary that surrounds the globe in vivo, but it did not reproduce the choroidal circulation, which delivers heat directly beneath the retina. The perfused model of Chapter 3 cooled the deep sites less than the human eyes did ex vivo (Section 3.3.6). Blood could in principle carry cooling from the ring toward the vortex veins [**1**], but the model indicates that perfusion opposes

the cooling at depth. The main indication for the device is ischemia, in which ocular blood flow is by definition reduced, so a non-perfused preparation corresponds to part of the clinical scenario. The present measurements are a tissue-level thermal baseline and do not predict in-vivo temperatures.

Measurement limitations. Thermistor implantation was necessarily invasive, and sensor position varied between eyes, most of all in the vitreous. Micro-CT localization and the spatial screen removed sensors that lay far from the intended position of their site but could not remove smaller placement differences. Small air bubbles introduced during implantation could insulate a sensor locally, and variation in the contact pressure of the SCR could change the thermal resistance of the ring–sclera interface. Both would add to the variation between eyes. Sample sizes were modest, particularly for the vitreous after quality-control exclusions, which limited the power of every site-level comparison. The species comparison at the proximal optic nerve is limited in the same way. Its bound of 1.8 °C is about half of the excursion itself, of about 3.5–3.8 °C, and rests on five porcine eyes. The cohorts also differed in starting temperature and sensor orientation. The data therefore show that the two species differ by less than about 2 °C at this site, not that they cool alike. The human cohort was demographically narrow, five male Caucasian donors aged 43–77, and cannot show whether the ocular thermal response varies with sex, age, ancestry, or other donor characteristics.

Scope. Aim 1 evaluated thermal delivery only. An ex-vivo preparation cannot show whether the local exposure, about 10 °C at the anterior sclera, is tolerated by living tissue, or whether cooling preserves retinal function during an ischemic insult. The first is assessed in Aim 3 and the second remains future work. The remaining limitations of SCEC Gen 3, its bulk, heat-rejection hardware, control architecture, and the temperatures it imposes at the tissue interface, motivate Aim 2.

CHAPTER 3

Specific Aim 2: Enhance the Device (SCEC Gen 4) for Efficiency, Portability, Safety, and Patient Comfort

3.1. Background and Rationale

The design choices of SCEC Gen 3 were acceptable for establishing feasibility but limit any use beyond it. Specific Aim 1 showed that SCEC Gen 3 can lower the temperature of the deep posterior eye by holding a ring on the anterior sclera at 4 °C (Chapter 2). Its use outside a laboratory remained to be shown. The ring was machined from bulk aluminum, 2336 mm^3 of metal resting on the conjunctiva and under the eyelids, and it placed an electrical conductor in direct contact with the eye. Heat was carried away by a heat exchanger of similar volume and an ice-bath circuit, and the controller was assembled from separate boards.

The bulk of the ring also bears on where its cooling goes. A ring that fills the fornices contacts the eyelids over a larger area than the sclera. In the computational model of Gen 3 developed in this chapter, about 66% of the heat extracted over a 35-minute session came from the eyelids and only 33% from the eye. Heat drawn from the eyelids does not cool the target tissue, yet the module, the heat-rejection hardware, and the power supply are all sized by the total heat they must move.

Efficiency, the first of the four enhancement targets, is the share of the extracted heat drawn from the eye. Portability is the size of the ring, the heat-rejection hardware, and the control electronics. Safety covers the electrical isolation of the tissue from the powered module, surface temperatures below 43 °C, the limit for applied parts [**129**] that a clinical device will have to meet, and the behavior of the controller when a sensor or the module fails. Comfort is judged only by the dimensions and volume of the parts that contact the eye and eyelids.

No change may cost the cooling that Aim 1 established. Tissue is limited, each eye carries a single set of implanted sensors, and a new ring takes weeks to fabricate. For these reasons the refinements were explored first in a computational model, which holds every other condition fixed and evaluates each variant over a range of operating conditions. The designs that the model favored and that could be built were then tested in human donor eyes on the ex-vivo platform of Aim 1, paired within the same eyes. The control electronics were consolidated into a single board and verified on the bench.

The hypothesis of this aim is that selected geometric and material refinements can reduce device bulk while preserving cooling performance at the posterior measurement sites (Section 1.10). Whether cooling is preserved is judged at the proximal optic nerve, the one site at which the hypothesis is tested, from the difference from Gen 3 and the largest difference that its confidence interval allows, set against the excursion itself. Changes in efficiency and in the other targets are reported as measured differences from Gen 3.

3.2. Methods

3.2.1. Design Requirements and Refinements. Each refinement addresses one or more of the four targets and is judged against the Gen 3 device of Chapter 2. Table 3.1 records how far each was taken and where its evidence is reported.

Ring material. Aluminum conducts heat well but is an electrical conductor in contact with the eye. Candidate replacements were screened for thermal conductivity, electrical insulation, optical transparency, hardness, and use in medical devices, and two were carried forward. Aluminum oxide is a ceramic that combines moderate thermal conductivity with electrical insulation and can be made translucent or transparent, depending on how it is processed [**130, 131**]. Fused silica glass is transparent and insulating but conducts heat poorly. Fused silica was also retained because of its manufacturing route, which seemed of interest at first: parts of fused silica glass

TABLE 3.1. Refinements of SCEC Gen 4, the target each addresses, and how far each was taken. Refinements listed as built and tested were tested experimentally; modeled refinements were evaluated only with the computational model of Section 3.2.2.

Refinement	**Target**	**Status**	**Evidence**
Ceramic and glass rings	Safety	Ceramic ring built and tested; glass modeled	Model (three materials); paired tests in human donor eyes (aluminum against ceramic)
Reduced-bulk ring (v2)	Portability, comfort	Built and tested	Model; paired tests in human donor eyes
Integral corneal lens (v3)	Efficiency, safety, comfort	Modeled	Model
Insulating outer layer	Efficiency	Modeled	Model (0.5 and 1.0 mm silicone)
Smaller heat exchanger	Portability	Modeled	Water-block model (three designs)
Integrated control board	Portability, safety	Built and bench-tested	Bench verification
Potting of the TEC	Safety	Built	Silicone injected with a curved-tip syringe to fill the annular module between its two ceramic plates and waterproof it, in every device used in this work; not evaluated separately
Coolant circuit	Portability	Built	Replacement pump and push-to-connect tubing fittings; not evaluated separately
Higher post density and printing on the ring	Efficiency, portability	Concept only	None; future work
Contact-pressure sensor, ERG electrode, injection port	Safety	Concept only	None; future work
Patient comfort	Comfort	Not measured; implied	Dimensions and volume only

can be molded or printed from a silica composite and then sintered [**132, 133**] (Glassomer GmbH, Freiburg, Germany), which would combine transparency with ease of manufacture.

Ring geometry. The Gen 3 ring (v1) is a bulk aluminum part that conforms to the anterior sclera and leaves the cornea exposed, with a volume of 2336 mm^3 and a scleral contact area of approximately 390 mm^2. The reduced-bulk ring (v2) keeps a similar contact area, 393 mm^2, with a thinner profile that nearly halves the volume, to 1191 mm^3. The third design (v3) adds an integral lens of 105 mm^3 over the cornea to the v2 profile (1296 mm^3, 45% less than v1). The lens extends the contact area to 619 mm^2, protects the cornea, and is intended to limit corneal drying.

Outer layer, heat rejection, and control. A silicone layer, 0.5 or 1.0 mm thick, on the outer walls of the ring was evaluated as a way to direct the extracted heat toward the eye and away from the eyelids. Three heat exchangers of decreasing volume were evaluated (Section 3.2.2.3), cooled with distilled water (Appendix F). The controller must hold the ring at its set point, nominally 4 °C, and stop the module when a sensor or the module fails. A clinical device will also have to keep every patient-contacting surface, in practice the hot side of the module and the heat exchanger, below 43 °C. Here that limit was used only in the heat-exchanger model, as the safety limit for the hot side.

3.2.2. Computational Model. Two finite-element models were first built and solved in COMSOL Multiphysics 6.4 (COMSOL AB, Stockholm, Sweden): one of the eye, orbit, and eyelids with the ring in place, to compare ring materials, geometries, and the insulating layer, and one of the heat exchanger on the hot side of the module. A Python program then replaced COMSOL, and the results of this chapter are those of the Python runs. The analysis is described in Section E.6.

3.2.2.1. *From COMSOL to Python.* Both COMSOL models were exploratory, built to compare design variants under identical conditions rather than to predict the temperatures of a particular eye. The eye model coupled conduction to flow in

the humors and the air in front of the eye, represented the module as a layer on the ring, and began cooling with the ring held at ambient temperature. Its results were computed on the finest of three meshes per design, about one million elements (Table 3.2 and Figure B.3; settings in Section E.6).

Errors in how the boundary conditions were applied were then found, among them the starting state of the ring and the emissivity of the ceramic and glass rings. By then the COMSOL license had ended, so the models could not be corrected in COMSOL. A finite-element program was therefore written in Python with Claude Code (Appendix G; folder `code/ch3-heat-replica` of the code repository, Section E.7), reading the meshes, materials, and boundary conditions from the COMSOL model files and solving heat transfer only. It was validated first against exact solutions and then against the stored COMSOL solutions, with the same set-up (Section 3.3.1).

The errors were then fixed. The eye now starts at rest, with the device in place and switched off, so that the anterior sclera begins near 36 °C instead of 27 °C. The air domain and the module layer are removed, each ring takes the emissivity of its material, and the tissues take the values of the property workbook (Appendix B). The heat is computed from the solved temperature field, across each surface of the ring and from the sources of the model. The same 27 operating points, three materials, six insulating layers, and rewarming cases were solved in this set-up. COMSOL has no solution of it, so these runs were validated internally, by their energy balance, time step, and mesh. Runs beyond the COMSOL cases are called extension runs. For the heat exchanger, the Python runs give each design the same heat.

3.2.2.2. *Scleral Contact Ring and Eye.*

Geometry. The model represents a human eye in the orbit, with the ring on the anterior sclera and the eyelids closed over its outer surface (Figure 3.1). The globe is divided into cornea, tear film, aqueous humor, lens, ciliary body, vitreous humor with Cloquet's canal, retina, choroid, and sclera, and is surrounded by orbital fat, the extraocular muscles, and the eyelids. The eye has the dimensions of an adult

TABLE 3.2. Statistics of the finest mesh of the eye model for each ring, as reported by COMSOL, with the air and the module layer. Without them, as solved in the corrected runs, the meshes have 165 472, 163 912, and 171 043 vertices and 946 374, 940 447, and 979 821 tetrahedra. Element quality is the skewness measure (1 for an ideal element).

Statistic	**v1**	**v2**	**v3**
Mesh vertices	179 228	176 051	185 132
Tetrahedra	1 038 645	1 020 997	1 074 864
Boundary triangles	130 033	127 544	136 067
Minimum element quality	0.094	0.014	0.019
Average element quality	0.671	0.670	0.667
Mesh volume (cm^3)	26.08	26.11	26.11

human globe (Appendix B), and the orbit is a cylinder of radius 17.5 mm. The three rings were placed on the same eye. The model is symmetric about the vertical plane through the optical axis and only half was solved; heat flows are reported for the whole device.

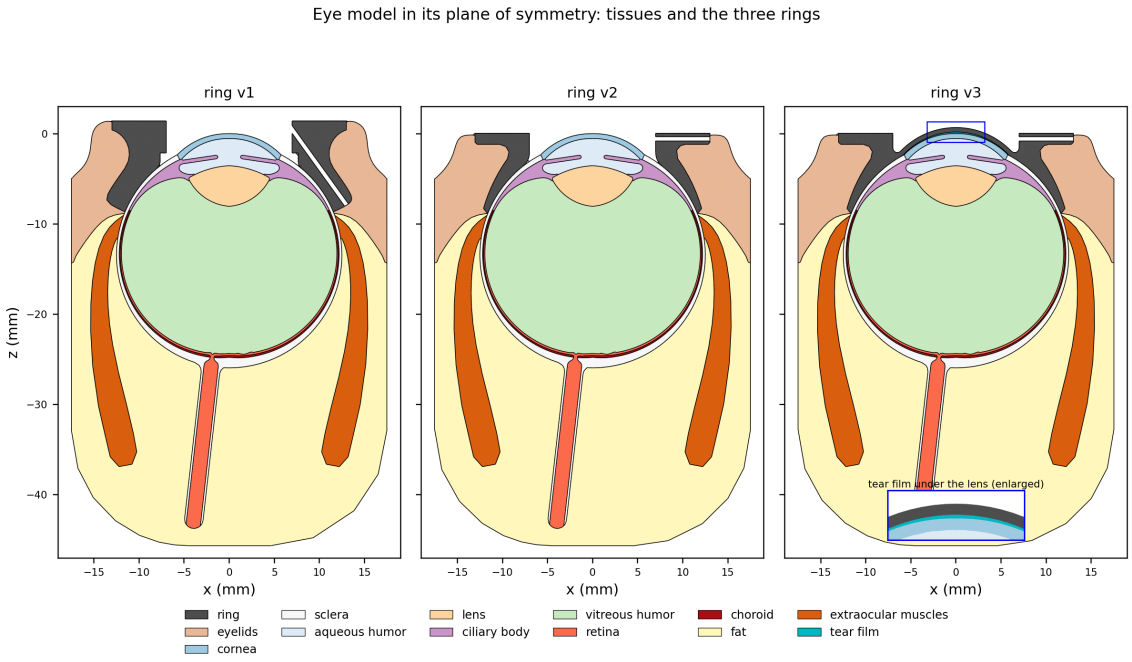

FIGURE 3.1. The eye model in its plane of symmetry with the three scleral contact rings: v1, the original ring of SCEC Gen 3; v2, the reduced-bulk ring; and v3, the reduced-bulk ring with an integral corneal lens. Half of the model was solved, and the plane shown is the cut. Colors mark the domains. The inset enlarges the tear film beneath the lens of v3.

Heat transfer and boundaries. Heat conduction was solved in the tissues and the ring, with the humors at rest. In the perfused tissues (choroid, retina, ciliary body, extraocular muscles, eyelids, and orbital fat), the heat equation carries the Pennes terms for perfusion and metabolic heat [**134**],

$$\rho c_p \frac{\partial T}{\partial t} = \nabla \cdot (k \nabla T) + \omega \, \rho_b c_b \, (T_b - T) + Q_{\mathrm{met}}, \tag{3.2}$$

where T is the temperature of the tissue at time t, ρ, c_p, and k are the density, specific heat, and thermal conductivity of the tissue, ω its perfusion rate, ρ_b and c_b those of blood, T_b the body temperature, and Q_{met} the metabolic heat (Table 3.3; sources in Appendix B). The choroid, with the highest perfusion, is the dominant perfused heat source opposing cooling. Its perfusion comes from microsphere measurements in monkey eyes [**40**].

The outer shell of the orbit is held at T_b. The exposed surfaces lose heat to the room, at the ambient temperature T_0, through a coefficient of 10 $\mathrm{W\,m^{-2}\,K^{-1}}$ and by radiation, and the cornea also by tear evaporation. In v3 the lens covers the ocular surface, so neither tear evaporation nor exposure applies to the eye (Figure B.1). The face of the ring that met the module is free in the starting state, the eye at rest with the device switched off, and is then brought from its starting temperature to the set point T_c along a step of 90 s. The transient was computed for 35 min. Further details are given in Appendix B.

Conditions. At the baseline condition (ambient 25 °C, body 37 °C, set point 4 °C) the ring material was varied among aluminum, aluminum oxide, and fused silica, and the three geometries were compared directly. Each geometry was also solved for every combination of ambient temperature (10, 25, and 35 °C), body temperature (35, 37, and 39 °C), and set point (4, 8, and 12 °C), 27 conditions per ring, to test whether the comparison depends on the operating point. A silicone layer of 0.5 or 1.0 mm was added to the outer walls, and rewarming after switch-off was simulated for a subset of conditions.

TABLE 3.3. Thermal properties, perfusion rates, and metabolic heat of the domains of the eye model as used in the corrected runs. The tear film and the ring take temperature-dependent properties from the COMSOL material library, given here at 20 °C (aluminum oxide, for example, 37.0 at 4 °C and 32.1 $\mathrm{W\,m^{-1}\,K^{-1}}$ at 37 °C). The COMSOL models and their replication used aqueous humor 0.59, 1010, 4134; sclera 0.50; eyelids 0.37, 1109, 3391; and orbital fat 0.21, 911, 2348.

Domain *Symbol (units)*	**Conductivity** k ($\mathrm{W\,m^{-1}\,K^{-1}}$)	**Density** ρ ($\mathrm{kg\,m^{-3}}$)	**Specific heat** c_p ($\mathrm{J\,kg^{-1}\,K^{-1}}$)	**Perfusion** ω ($\mathrm{s^{-1}}$)	**Metabolic heat** Q_{met} ($\mathrm{W\,m^{-3}}$)
Cornea	0.56	1070	3707	–	–
Aqueous humor	0.593	1005	4159	–	–
Lens	0.43	1126	3255	–	–
Vitreous humor	0.59	1010	4134	–	–
Ciliary body	0.49	1088	3556	0.0064	685
Retina	0.55	1014	3958	0.0039	3443
Choroid	0.51	1074	3682	0.0927	624
Sclera	0.545	1102	3431	–	–
Extraocular muscles	0.49	1090	3421	0.0045	686
Eyelids (skin)	0.34	1070	3660	0.0011	644
Orbital fat	0.2245	920	2500	0.0004	199
Tear film	0.60	998	4180	–	–
Ring, aluminum	236	2700	903	–	–
Ring, alumina	34.3	3990	764	–	–
Ring, fused quartz	1.37	2220	705	–	–
Silicone layer	0.26	1280	1781	–	–

3.2.2.3. *Heat Exchanger.* Each heat exchanger is an aluminum annulus around the module with fins on alternating walls that force the coolant along a zig-zag path (Figure 3.2 and Table 3.4). The v3 block has no lid of its own and is closed by the hot-side ceramic plate of the module, which the coolant touches directly. Each was modeled whole. The heat from the module entered as a uniform flux over its footprint, an annulus of inner radius 7 mm and outer radius 13 mm, the same in the three designs, and each received the same heat: 10 W, the operating load, and 15.5 W, a conservative upper value. In v3 most of that annulus is the coolant itself, because the model has no plate closing the block. The heat left with the coolant; the outer walls and the module plane outside the footprint lost a small amount of heat to the ambient air.

The water flow and the temperature were solved with the Python program for every combination of flow rate (0.10, 0.25, and 0.50 $\mathrm{L\,min^{-1}}$) and inlet temperature

(4, 10, and 20 °C), and the temperature was advanced in time with the flow where the flow was unsteady. The surfaces on which these conditions act are shown in Figure B.2.

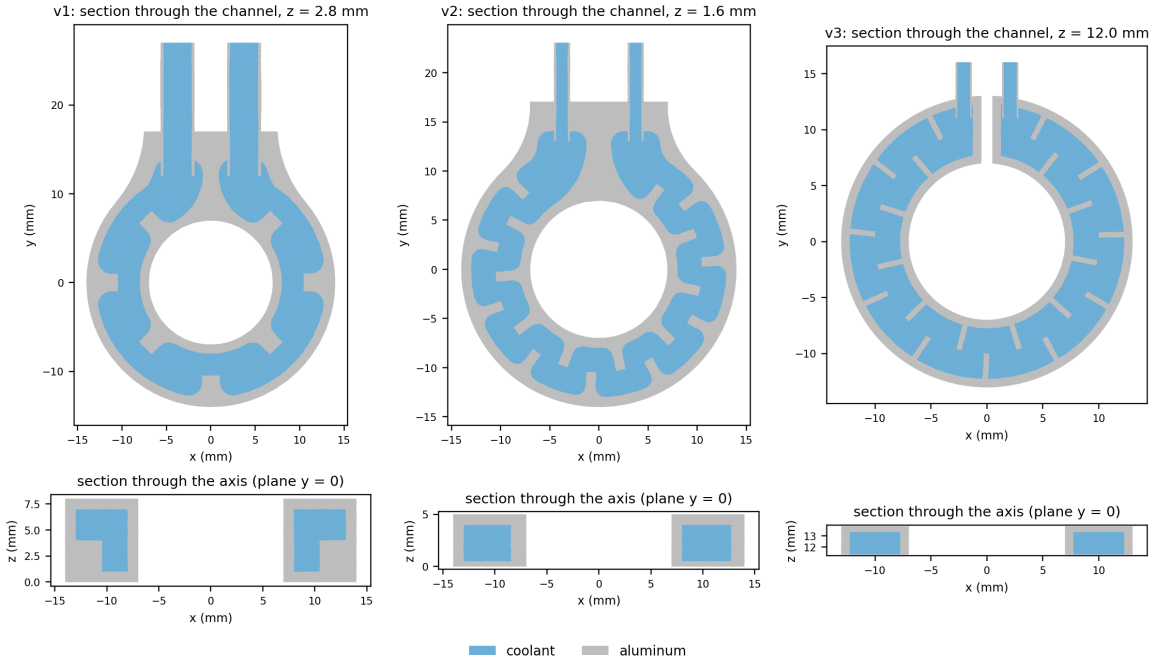

FIGURE 3.2. The three heat-exchanger designs as modeled: v1, the heat exchanger of SCEC Gen 3, with seven fins filling half the channel height; v2, with a lower channel and 18 thinner fins; and v3, with the lowest channel and 21 fins, and no lid of its own, being closed by the hot-side ceramic plate of the module. Coolant (blue) and aluminum (gray) are shown in a horizontal section through the channel (top) and in the vertical section through the axis (bottom). The horizontal sections are taken at 30% of the height of the channel, where the fins of v1 are. Dimensions are given in Table 3.4.

TABLE 3.4. Geometry of the three heat-exchanger designs. Volumes measured on the CAD models.

Feature	**v1 (Gen 3)**	**v2**	**v3**
Volume (mm^3)	2337	1619	435
Annulus, inner/outer radius (mm)	7.00/14.00	7.00/14.00	7/13
Channel height (mm)	6.0	3.5	2.0
Fins: number, thickness (mm)	7, 2.0	18, 1.0	21, 0.5
Fin height	half channel	full channel	full channel
Inlet and outlet diameter (mm)	4.00	2.25	1.41
Lid	separate	separate	module plate

3.2.2.4. *Outcome Measures.* Temperatures were read at the sites measured in Chapter 2 (Figure 3.3): the anterior, equatorial, and posterior sclera, the vitreous, and the proximal optic nerve, at its junction with the globe. A distal optic-nerve probe, 1 cm behind the globe, recorded the effect of cooling behind the eye, and domain probes the average, minimum, and maximum temperature of the retina and the whole eye. The steady temperature at each site is a weighted mean of the body and ring temperatures. The principal outcome was thus the cooling effectiveness

$$\varepsilon = \frac{T_b - T_{\text{site}}}{T_b - T_{\text{ring}}} \times 100\%, \tag{3.3}$$

where T_b is the body temperature, T_{site} the steady temperature at the site, and T_{ring} the final temperature of the ring. It is 0% for a site left at body temperature and 100% for a site cooled to the temperature of the ring. It does not depend on the operating point: a linear fit of the steady temperatures over the 27 operating points of each ring reproduced every site to within 0.003 °C, and less than 1% of a change in ambient temperature reached any site. It also gives directly the ring temperature needed to reach a target. Kinetics were summarized by t_{90}, rewarming by the time each site stayed below 35 °C after switch-off, and efficiency by the share of the heat drawn from the eye rather than the eyelids and air. For the heat exchanger, the outcomes were the rise of the average temperature of the heated face above the coolant and the peak temperature of that face against the 43 °C limit.

3.2.3. Experimental Ring Comparisons. A ceramic ring of the original geometry and the reduced-bulk ring were each compared with the original aluminum ring in human donor eyes, to test whether an electrically insulating material, or halving the volume, costs cooling.

Rings. The ceramic ring reproduced the original geometry in alumina, fired from the same design file without further machining (Ceramco, Inc., Center Conway, NH, USA). The reduced-bulk ring (v2) was machined from aluminum with the contact surface of the original ring and a thinner profile, which reduced the height of the

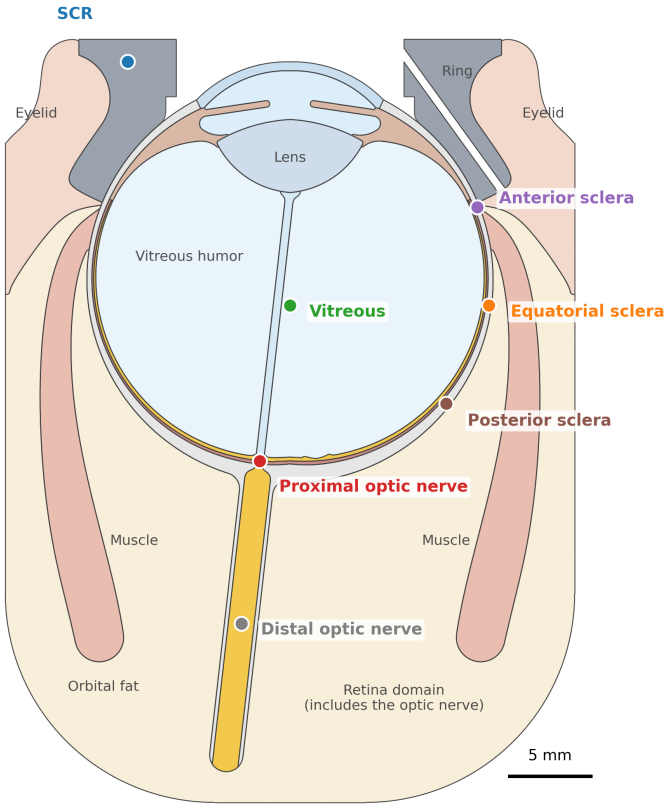

FIGURE 3.3. Probe locations in the eye model, on the section of the model geometry in its plane of symmetry (v1 ring): the anterior, equatorial, and posterior sclera, the vitreous, the proximal optic nerve, the distal optic nerve, 1 cm behind the globe, and the SCR. The SCR probe lies 10 mm from the axis on the rim of the ring, out of this plane, and is drawn turned into it. The retina domain, over which the average retina is taken, is shown in yellow and includes the optic nerve.

assembled device from 23 to 20 mm. Each ring was a complete device built as SCEC Gen 3 (Figure 3.4).

Protocol. The nine human donor eyes of Chapter 2 were each cooled with all three rings on the same day, with the same implanted sensors, under the protocol of Section 2.2.5, re-equilibrating in the bath between rings, in an order that varied between eyes. The comparisons are therefore paired within each eye, and differences in sensor placement, donor, and anatomy cancel.

Analysis. The recordings were processed with the pipeline and endpoints of Section 2.2.6. A sensor excluded by the spatial screen was excluded under all three rings, and one further trace, the vitreous sensor of eye 4 under the reduced-bulk ring, was

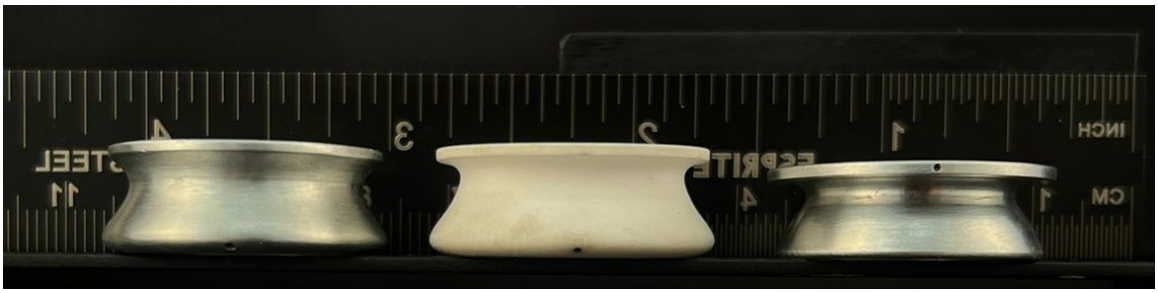

FIGURE 3.4. The three scleral contact rings tested in human donor eyes, in side view on a ruler (inches above, centimeters below): (left) the original aluminum ring of SCEC Gen 3 (v1), (center) the alumina ring of the original geometry, and (right) the reduced-bulk aluminum ring (v2). The photograph is mirrored left to right, so the markings of the ruler read in reverse.

removed on visual review for the same oscillation it showed under the original ring. In all, 23 of the 162 traces recorded with the three rings were excluded (11 for a hardware fault, 3 for a poor fit, 6 by the spatial screen, and 3 on visual review). This left 45, 47, and 47 traces for the original, ceramic, and reduced-bulk rings. One of the excluded traces is the proximal optic nerve of eye 6 under the original ring, and the comparisons with that ring at the site of the hypothesis rest on eight eyes. Each ring was compared with the original by the procedure of the species comparison in Chapter 2. Paired t tests were run at each site with Holm correction across the five tissue sites. Each difference in °C is reported with its 90% confidence interval. The paired standardized difference g_z (the mean paired difference over the SD of the differences, with the same small-sample correction) is reported with its 95% confidence interval. The limit of that interval farther from zero is the largest difference that the data allow (Section 2.2.6). The hypothesis is tested at the proximal optic nerve.

Comparison with the model. As a supplementary step, the measurements were set beside the model through the same quantity for both, the fraction of each site's own starting difference from the ring that the device removed,

$$\varepsilon_{\Delta} = \frac{T_{\text{initial}} - T_{\text{final}}}{T_{\text{initial}} - T_{\text{ring}}} \times 100\%, \tag{3.4}$$

where T_{initial} and T_{final} are the temperatures of the site when the ring was switched on and at the end of cooling, and T_{ring} the final temperature of the ring. Model and measurements were also compared through t_{90}. Both start from an eye at rest with the unpowered ring in place: in the model the eye has settled with the device off, and ex vivo the ring equilibrated with the eye in the 37 °C bath. The model is read after 35 min and the experiments over minutes 16 to 20. Each ring was matched to its model design (original ring to v1, reduced-bulk ring to v2, ceramic ring to the alumina v1 run), at a body temperature of 37 °C and a set point of 4 °C. Because the model includes perfusion and the eyelids and the experiments do not, the comparison is not a validation of the model. It was read for the order of the sites, by Spearman's rank correlation (not tested, with five sites), and for the direction of the differences, by the ratio of measured to modeled effectiveness at each site.

3.2.4. Integrated Control Board. The separate boards of the Gen 3 controller, a thermistor conditioning circuit, a comparator that switched the module through a relay, and a laboratory supply (Section 2.2.1), were consolidated for SCEC Gen 4 onto a single printed circuit board designed for this work (Figure 3.5). Its firmware, schematics, and drawings are given in Appendix H.

The microcontroller runs a digital proportional–integral–derivative loop on the ring thermistor at 5 Hz and sets the drive of a thermoelectric-module controller. Hard limits on the module current and voltage are set by trimmers, independently of the firmware, and the module is off by default. A safety routine running every 50 ms stops the module and latches the fault if the ring thermistor reads open or out of range, the 2.5 V reference leaves its band, or the module current is abnormal, and a hardware watchdog resets the microcontroller if its program stops (Section H.1).

Bench verification. The board was verified on the bench in stages, from unpowered checks and operation into dummy loads to closed-loop regulation of a ring on a preserved porcine eye in the bath, with an 18-min pull-down and hold at 4 °C (Section H.2). Three safety cutouts were triggered on purpose, by disconnecting the ring

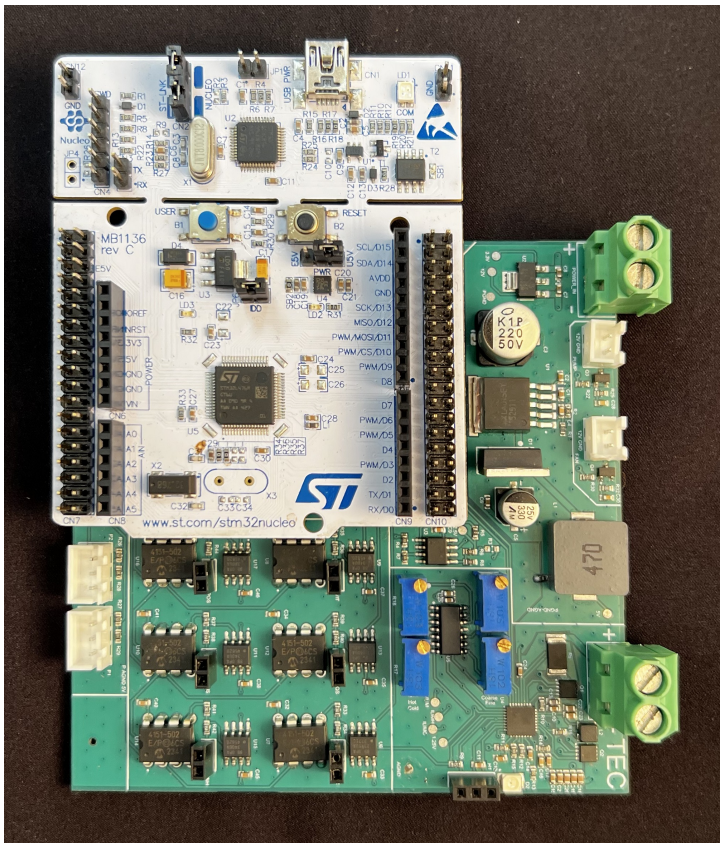

FIGURE 3.5. The integrated control board (revision 1.0) with the microcontroller board mounted on its headers (upper left). The six thermistor channels are at the lower left; the power and drive stage is on the right, with the screw terminals for the 12 V supply (upper right) and the thermoelectric module (lower right) and, between them, the connectors for the pump and the fan.

thermistor and the module and by stalling the program. The current monitor was compared with a multimeter in series with the module. Regulation was summarized by the time to reach the set point and the largest deviation while held, the current monitor by its largest deviation from the multimeter.

3.3. Results

3.3.1. Validation of the Python Program. The Python program agreed with the solutions stored by COMSOL within 0.06 °C at every probe and stored time of the 104 cooling cases, and within 0.008 °C at the end of cooling. Table 3.5 lists every check of the program. In the 22 rewarming cases it agreed within 0.054 °C at the

end, and within 0.075 °C at every tissue probe during the run with short steps after the release; the largest difference, 0.11 °C, was at the v3 ring 0.3 min after the release in the coldest case. All nine material cases agree within 0.03 °C.

TABLE 3.5. Validation of the Python program. Temperature differences are absolute values.

Check	**Compared with**	**Result**
Cooling, 104 cases, 13 probes	Stored COMSOL solutions	At most 0.058 °C at any time, 0.008 °C at the end
Rewarming, 22 cases, 13 probes	Stored COMSOL solutions	At most 0.11 °C during the run (at the ring; 0.075 °C at the tissue probes), 0.054 °C at the end
Glass ring, within the 9 material cases	Stored COMSOL solutions	At most 0.03 °C
Retina line, v1 to v3, at 0, 15, 35 min	Stored COMSOL fields	At most 0.001 °C
Plane of symmetry, 35 min	Stored COMSOL fields	At most 0.09 °C in the tissues and the ring; up to 4 °C in the air, which is at rest in the program
12 problems with exact solutions	Mathematics	All passed: largest error 0.42% of the span, order 1.91 to 2.07 in space and 2.0 in time; constant properties, no flow
Energy balance	Conservation of energy	Within 0.11% at the end (1056 runs); 456 of 457 further runs within about 0.01%; corrected runs within 0.41% over the run (1637 runs)
Time step halved	Same case	Below 0.001 °C at the end; in the first 1.5 min up to 0.02 °C at the sites and 0.05 °C at a sensor point
Nothing switched on	Starting state	No drift
Refinement, deep sites	Finer meshes	Within 0.08 °C of the mesh divided into eight, 0.14 °C of the extrapolated value (Section B.6)
Refinement, anterior sclera	Finer meshes	Not converged on the meshes of the results; finest mesh low by up to about 1.3 °C

These checks show that the program solves the COMSOL models correctly and that it is consistent with the experiment. The corrected runs use the same solver and meshes. They do not validate the model against a perfused eye. The comparisons with the eyes of patients and with the published models are in Section 3.4.

The eye model converged at the deep sites: from the intermediate to the finest mesh the final temperature changed by at most 0.04 °C at the optic nerve and the

posterior sclera and 0.11 °C at the equatorial sclera, the vitreous, and the average retina, and differences between designs smaller than this are not interpreted. The anterior sclera, next to the ring, did not converge, and refinement around the ring put it up to about 1.3 °C higher, so temperatures there are lower bounds (Section B.6).

In the COMSOL heat-exchanger model the heat carried away by the coolant matched the heat supplied to within 1.2% on average and about 5% at every operating point, and the Python runs closed within 0.2%. The COMSOL solution changed by 0.03 °C or less between the two finest meshes, but the Python program agreed with it only within 0.6 °C at the module face and 0.04 °C at the outlet. The temperature of the module face is thus known to about 1 °C, and the values on these meshes are low (Section B.6). The COMSOL sweep gave v3 about a fifth less heat than v1 and v2, so the results below come from the Python runs with the same heat on the three designs, on the intermediate mesh. The flow was steady only in v1, at 0.10 $\mathrm{L\,min^{-1}}$ and at 0.25 $\mathrm{L\,min^{-1}}$ with the 4 °C inlet, and elsewhere fluctuated by 0.6 to 7.3% of the pressure drop.

3.3.2. Ring Material. In the model, an aluminum-oxide ring cooled the eye almost as well as an aluminum ring of the same geometry, and a fused-silica ring did not (Figure 3.6A). After 35 min at the baseline condition, the aluminum-oxide ring kept 89 to 95% of the cooling effectiveness of aluminum at every site and with every geometry. With the original geometry it kept 94% at the proximal optic nerve, and its final temperatures were 0.2 °C warmer there and 2.2 °C warmer at the anterior sclera, the site next to the ring. Fused silica kept only 29 to 46% of the effectiveness of aluminum. With the original geometry its proximal optic nerve ended at 35.7 °C against 33.8 °C with aluminum, and its anterior sclera at 28.4 °C against 9.3 °C. The ring itself, held at the same cold face, ended at 7.7 °C instead of 4.1 °C. The glass conducted too little heat to cool the eye in a useful way, and on this result fused silica was dropped without a ring being built.

In the human donor eyes, the ceramic ring and the aluminum ring of the original geometry cooled the proximal optic nerve equally, by 3.7 and 3.8 °C on average. The paired difference, aluminum minus ceramic, was −0.20 °C (90% confidence interval −0.82 to +0.43 °C, eight eyes), and no difference was detected. A difference larger than 1.0 °C is unlikely with the eyes of each donor averaged, and one larger than 0.8 °C with each eye counted separately (Figure 3.7A; Section 3.3.3). The same held for the ring itself (+0.16 °C, −0.41 to +0.73), which ended at 4.1 °C with either material. The ceramic ring took longer, however, to reach 90% of its excursion, 174 s against 124 s on average. At the other four tissue sites the ceramic ring cooled more on average, by 0.9 to 1.7 °C (Table I.1). No difference was significant after Holm correction ($p \geq 0.38$). With intervals that allow differences of 2.4 to 4.3 °C, these sites say little either way.

The model and the experiment agree at the proximal optic nerve, where both find no meaningful difference between the two materials. The model predicts 0.23 °C more cooling with aluminum there, inside the experimental confidence interval. At the other tissue sites they differ in direction. The model predicts a small advantage for aluminum, at most 2.3 °C, whereas the experiment found a trend, not significant, toward more cooling with ceramic, and the model's difference lies outside the experimental interval at the anterior and posterior sclera. An electrically insulating alumina ring thus preserved cooling at the proximal optic nerve, and the glass alternative was excluded by the model alone.

3.3.3. Ring Geometry. In the model, the three geometries cooled the eye almost identically (Figure 3.8). The cooling effectiveness of the three rings differed by at most 2 percentage points at any site, and by 3 for the average over the whole eye. At the proximal optic nerve it was 9.8, 9.6, and 9.9% for v1, v2, and v3 (Figure 3.6B).

Across all 27 operating points the cooling excursion with each ring was a nearly fixed fraction of that with the original ring at every site (with the reduced-bulk ring, 97.8 to 99.5% of it at the proximal optic nerve). Thus the comparison does not

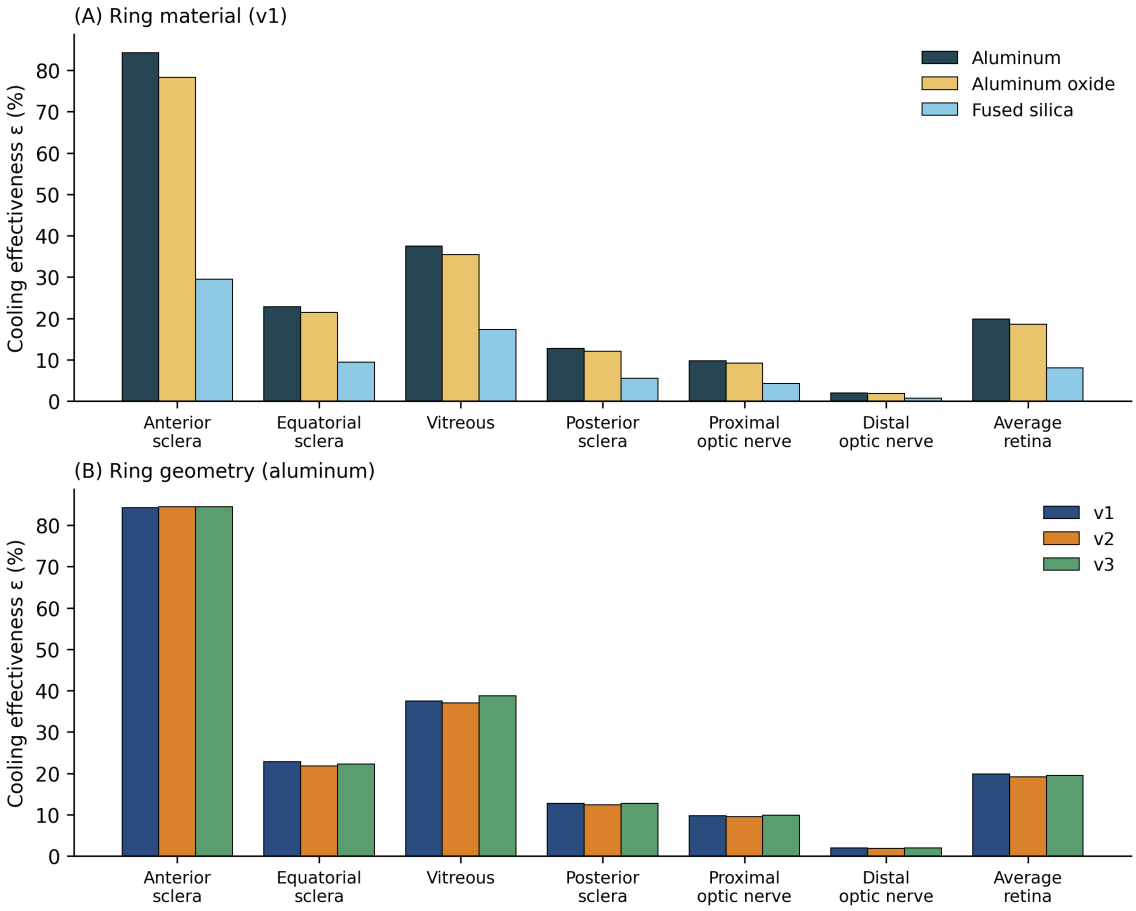

FIGURE 3.6. Cooling effectiveness ε at each site in the model at the baseline condition: (A) for v1 rings of aluminum, aluminum oxide, and fused silica; (B) for aluminum rings of the three geometries, both after 35 min of cooling.

depend on the body, ambient, or set-point temperature. At the baseline condition the reduced-bulk ring cooled the deep sites slightly less than the original: by 0.05 °C at the proximal optic nerve, 0.09 °C at the posterior sclera, 0.1 °C at the vitreous, and 0.3 °C at the equatorial sclera. These differences exceed the change of the results between the two finest meshes (Table B.2) but are small, 0.6 to 3.9% of the excursion of each of these sites (at the proximal optic nerve, 1.7% of an excursion of 3.1 °C). Adding the corneal lens (v3) restored the original ring's final temperatures at the proximal optic nerve and posterior sclera, to within 0.04 °C. It also cooled the vitreous more than either open ring, by 0.7 °C relative to v1 and 0.8 °C relative to v2, the

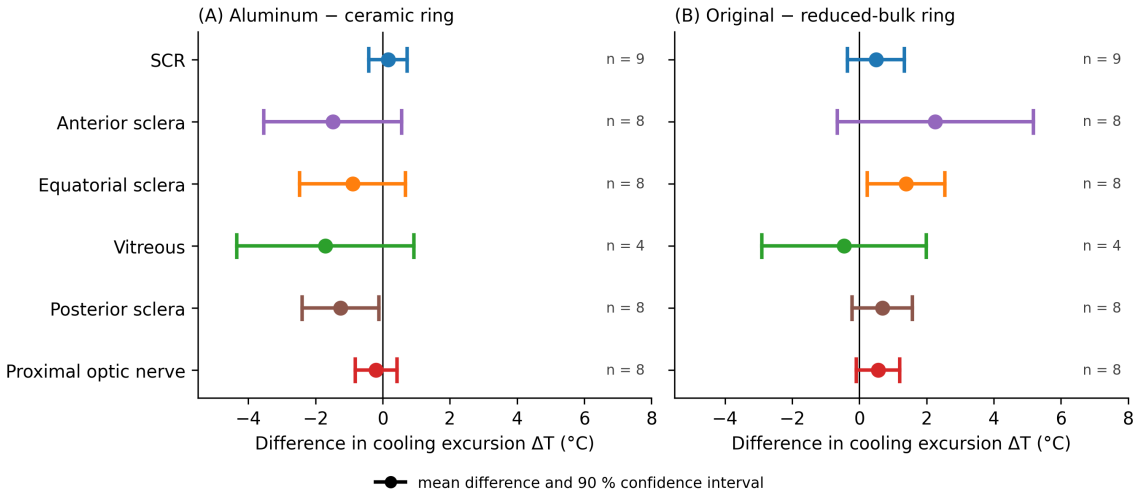

FIGURE 3.7. Paired differences in cooling excursion between the original ring and (A) the ceramic ring and (B) the reduced-bulk ring, at each site in human donor eyes: mean difference with its 90% confidence interval; the limit of the interval that lies farther from zero is the largest difference that the data allow. n, eyes with a valid pair.

largest difference between any two designs. The time to 90% of the final cooling was the same for all three rings to within 1 min at every site (Section 3.3.5).

The reduced bulk did change how much heat the ring drew (Figure 3.9). Over 35 min of cooling the reduced-bulk ring drew 14% less heat than the original, and almost all of the saving came from the eyelids. The heat drawn from the eye was nearly unchanged, and its share, the measure of efficiency, rose from 33% to 37%. The ring with the corneal lens (v3) drew about as little heat as the reduced-bulk ring, and 38% of it from the eye, because the lens adds contact with the cornea. Followed back to its sources, the 7.7 kJ drawn by the original ring came 48% from the wall of the orbit, 33% from the blood, 14% from heat stored in the tissues, and 6% from the room.

In the human donor eyes, the reduced-bulk ring cooled the proximal optic nerve by 3.1 °C on average against 3.8 °C for the original ring. The paired difference, original minus reduced-bulk, was +0.56 °C (90% confidence interval −0.09 to +1.20 °C, eight eyes). The reduced-bulk ring thus cooled the proximal optic nerve 0.6 °C less on average, not significantly, and by no more than 1.2 °C (Figure 3.7B). At the posterior

sclera the difference was +0.68 °C (−0.21 to +1.58). At the other three sites the intervals were wider, 2.3 to 5.8 °C across (Table I.1). No difference was significant after Holm correction ($p \geq 0.29$). The ring itself held its set point equally well with either geometry, ending at 4.0 to 4.1 °C, and the reduced-bulk ring reached 90% of its excursion sooner, in 82 s against 124 s on average. The comparisons were repeated with the eyes of each donor averaged (five donors) and with the vitreous traces removed on visual review put back. Neither repeat changed a conclusion.

Model and experiment agree at the proximal optic nerve, the site of the hypothesis. The model predicts that the original ring cools it 0.05 °C more than the reduced-bulk ring, inside the experimental interval, and both find that the two rings cool it almost equally. Both also find the original ring cooling the posterior and equatorial sclera slightly more, by 0.7 and 1.4 °C in the experiment and by 0.09 and 0.3 °C in the model. Compared, as for the materials, on the cooling excursion, the model's differences lie inside the experimental confidence intervals at all five sites. At the vitreous and the anterior sclera the two disagree in sign. Halving the volume of the ring kept the cooling at the proximal optic nerve, the site at which the hypothesis of this aim is tested, at a cost of about 0.6 °C, and of no more than 1.2 °C, of the 3.8 °C excursion.

3.3.4. Insulating Outer Layer. The insulating layer did not improve cooling. It was evaluated only in the model. With a layer of 0.5 or 1.0 mm on the outer walls of any of the three rings, the sites ended within −0.05 to +0.33 °C of the ring without a layer after 35 min at the baseline condition. The change was largest at the anterior sclera, 0.3 °C with the 1.0-mm layer on the three rings, the size of the change between meshes at that site. It was smallest at the deep sites: at most 0.03 °C at the proximal optic nerve, comparable to the change between the two finest meshes there, and at most 0.01 °C at the distal optic nerve. The cooling effectiveness fell by no more than 1 percentage point at any site.

The layer did change where the ring drew its heat (Figure 3.9). For the original ring the total heat drawn over 35 min fell by 5 and 7% with layers of 0.5 and 1.0 mm

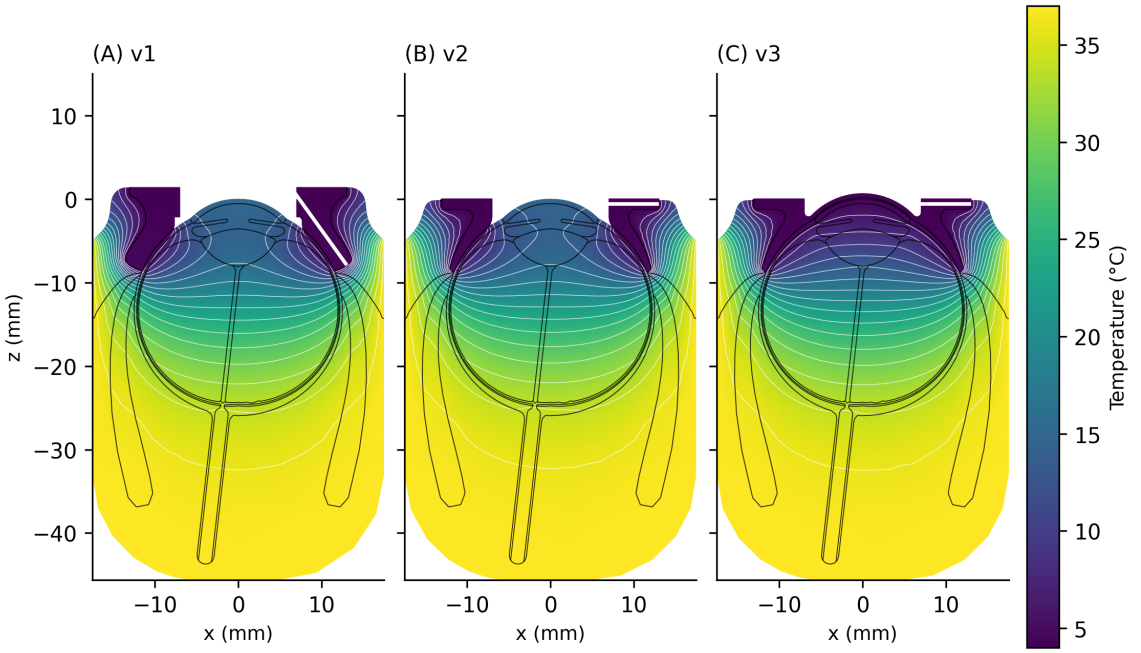

FIGURE 3.8. Temperature in the plane of symmetry after 35 min of cooling with (A) v1, (B) v2, and (C) v3, at the baseline condition, on a common color scale. White lines are isotherms every 2 °C; black lines outline the ring and the tissues.

and that drawn from the eyelids by 8 and 11%, so the share drawn from the eye rose from 33% to 35 and 36%, and the other rings behaved alike.

3.3.5. Cooling Targets and Kinetics. The steady temperature at each site depends linearly on the body and ring temperatures. Each site is thus a weighted mean of the body and ring temperatures, $T_{\text{site}} = (1 - \varepsilon)\, T_b + \varepsilon\, T_{\text{ring}}$, with ε as a fraction and the symbols as in Equation 3.3, which gives the ring temperature that brings a site to a target (Figure 3.10). The retina itself is cooled along a gradient, from its anterior edges next to the ring to the posterior pole (Figure 3.12). At a body temperature of 37 °C, the proximal optic nerve reaches 35 °C with the ring at 16.6 °C or colder, and with the ring at the 4 °C floor of the device it settles at 33.8 °C. Bringing it to 32 °C would need a ring at −14.1 °C, which the device cannot provide. The shallower sites need less: the average retina reaches 32 °C with the ring at 11.7 °C (Figure 3.10). The distal optic nerve, 1 cm behind the globe, stayed within

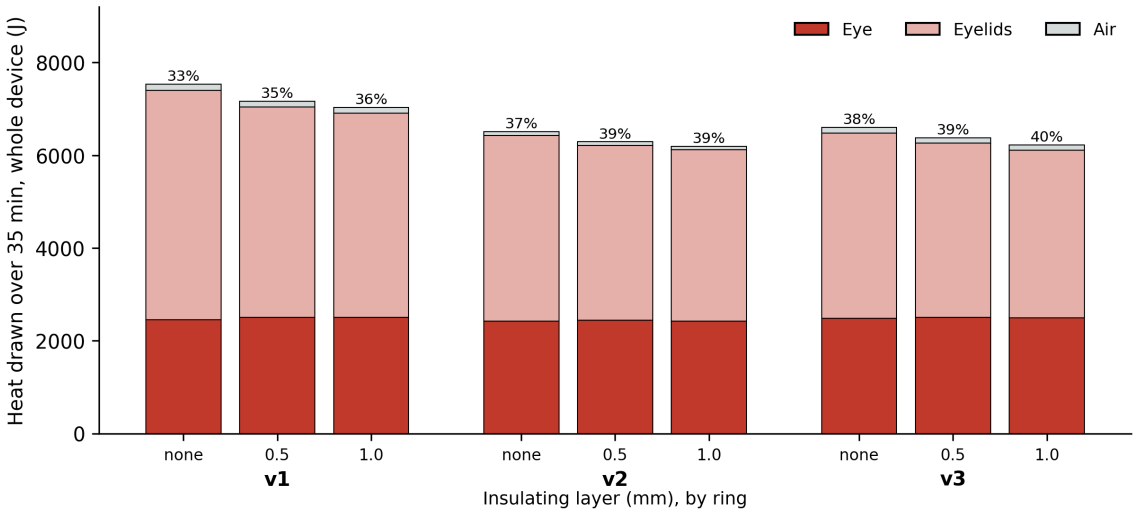

FIGURE 3.9. Heat drawn by the whole device over 35 min from the eye, the eyelids, and the air, for each ring without and with an insulating layer of 0.5 or 1.0 mm, with the share drawn from the eye above each bar. Heat reaching the lens of v3 through the tear film is counted with the eye, and heat passing through a layer with the eyelids.

1 °C of body temperature at every operating point and cannot be brought to either target.

The body temperature shifts every requirement: at 35 °C the proximal optic nerve reaches 32 °C with the ring at 4.3 °C, while at 39 °C it needs −1.9 °C to reach even 35 °C. The three ring geometries require ring temperatures within about 2.1 °C of one another (2.9 °C at a body temperature of 39 °C) for every site other than the distal optic nerve and for both targets.

Cooling was fastest next to the ring and slowest at depth (Figure 3.11). Over the operating points, the median time to 90% of the final cooling rose with depth, from the anterior sclera to the distal optic nerve (Table 3.6). These times varied by at most about 0.5 min with the operating point and the ring geometry.

After the device was switched off, the eye rewarmed over minutes (Figure 3.11). At a body temperature of 37 °C, the proximal optic nerve and the posterior sclera rose above 35 °C first and the vitreous and the anterior sclera last, the time at each

site depending on the set point and the geometry (Table 3.6). At 39 °C the proximal optic nerve never fell below 35 °C.

TABLE 3.6. Median time to 90% of the final cooling over the operating points, and time each site stayed below 35 °C after switch-off at a body temperature of 37 °C, over the set points and ring geometries, in the model. Dash: the distal optic nerve did not fall below 35 °C.

Site	**Time to 90% (min)**	**Time below 35 °C (min)**
Anterior sclera	1.6	13.5 to 15.9
Equatorial sclera	8.8	7.9 to 9.8
Vitreous	10.4	13.1 to 15.1
Average retina	10.0	7.4 to 9.5
Posterior sclera	12.4	6.7 to 9.0
Proximal optic nerve	14.3	5.8 to 8.6
Distal optic nerve	20.0	–

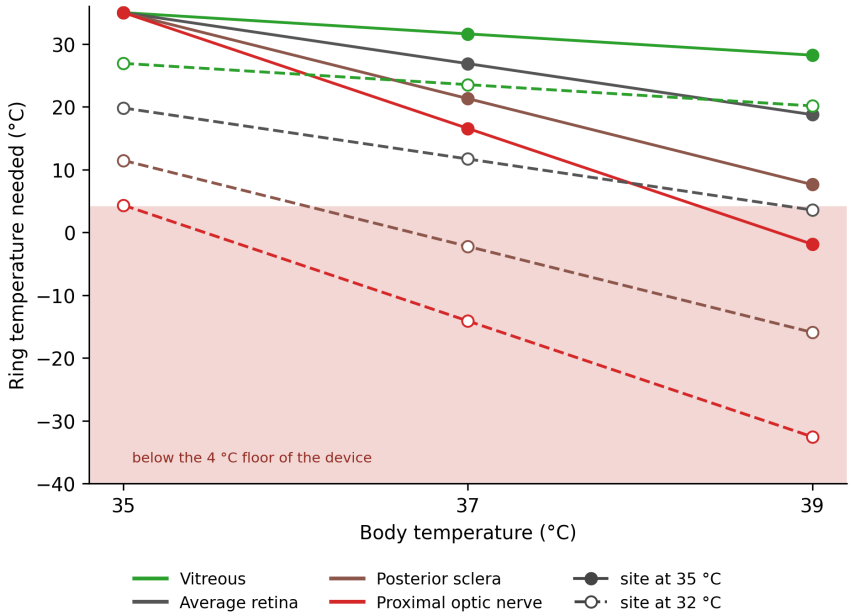

FIGURE 3.10. Ring temperature needed to bring each site to 35 °C (solid lines, filled markers) and to 32 °C (dashed lines, open markers) at steady state, from the effectiveness of v1, against body temperature. The shaded region lies below the 4 °C floor of the device.

3.3.6. Model versus Ex-Vivo Measurements. With each of the three rings, the model and the donor eyes ranked the five sites in the same order (Spearman $\rho = 1$): both cooled the anterior sclera the most and the proximal optic nerve the

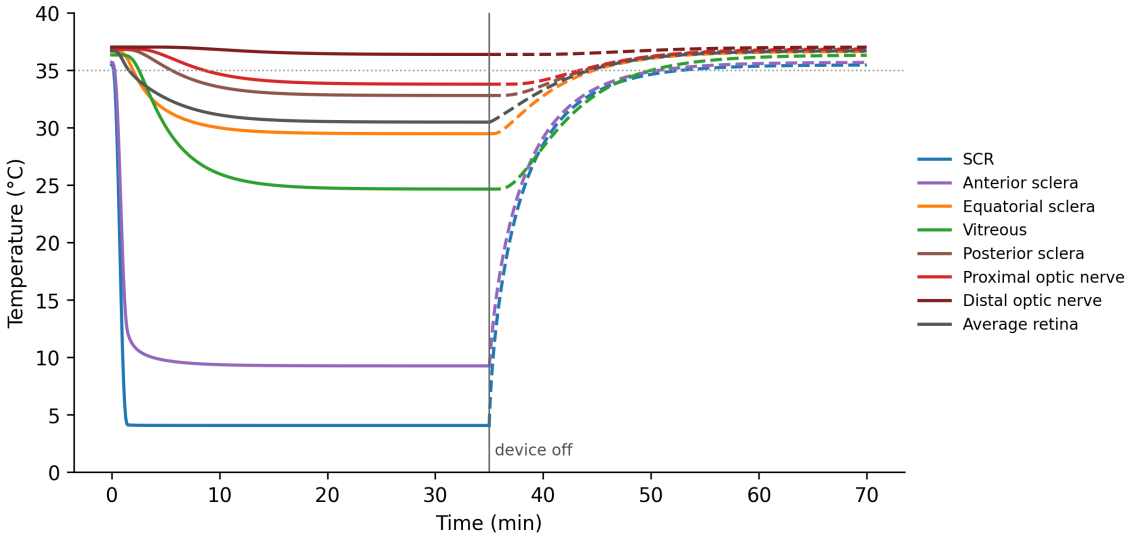

FIGURE 3.11. Temperature at each site of the model during 35 min of cooling with the original ring (v1, solid) and after the device was switched off (dashed), at the baseline condition (body 37 °C, ambient 25 °C, set point 4 °C). The dotted line marks 35 °C.

least. The amounts matched only at the anterior sclera, next to the ring, where the measured ε_Δ was 0.87 to 1.12 times the modeled value (Figure 3.13). At the deep sites the donor eyes cooled more than the model, 1.1 to 2.2 times as much, with the smallest gap at the proximal optic nerve (1.1 to 1.4 times). These factors are approximate, because the sensors at depth did not sit exactly at the points of the model (Figure 3.3). The sensors named posterior sclera lay at 66% of the axial length from the anterior pole on average, and the posterior point of the model at 81%.

Most of this difference comes from the difference between the two conditions: modeled as in the experiment, enucleated, in oil at 37 °C and without perfusion, the model cooled the equatorial and posterior sclera and the vitreous about as much as the donor eyes or more, depending on the heat-transfer coefficient of the oil, which was not measured. The donor eyes also cooled faster at depth, reaching 90% of their cooling in 11.0 to 11.4 min at the proximal optic nerve against 14.3 to 14.6 min in the model, 8.8 to 9.1 against 12.4 to 12.7 min at the posterior sclera, and 6.9 to 7.6 against 10.4 to 10.8 min at the vitreous, and more slowly at the anterior sclera, 3.6 to 3.9 against 1.5 to 2.2 min.

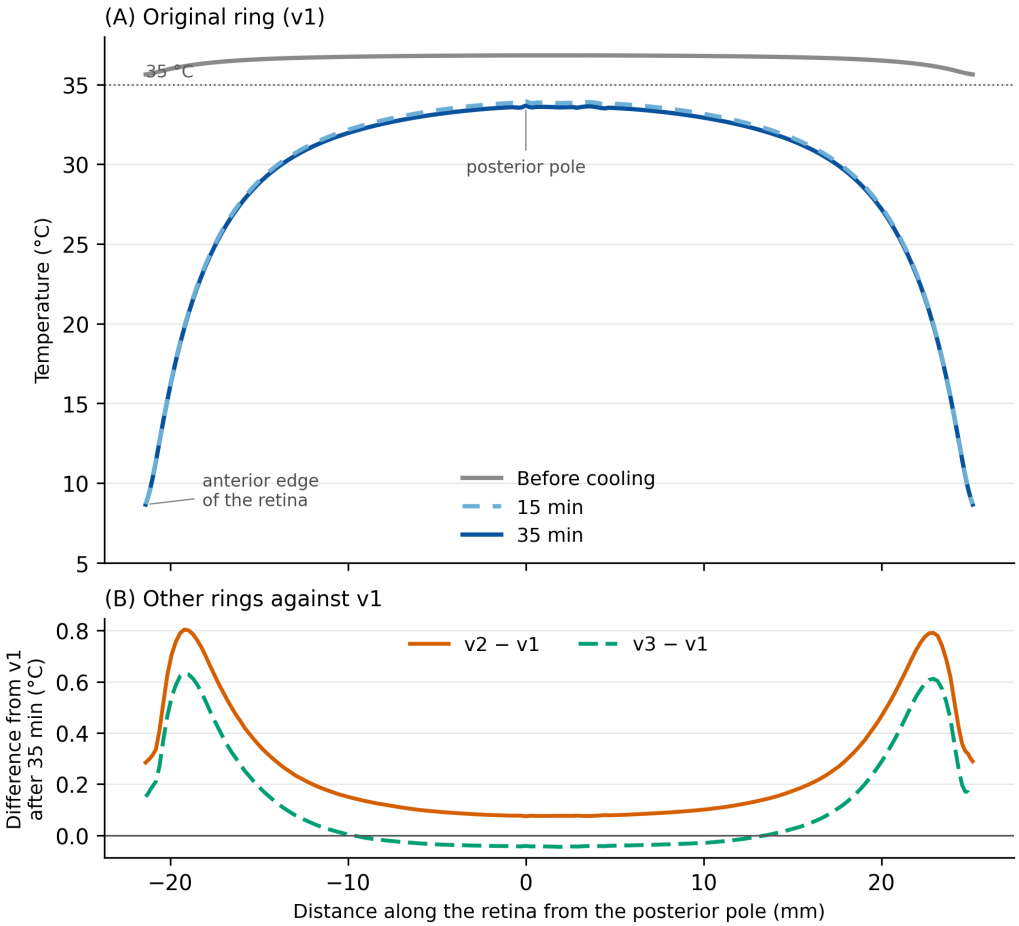

FIGURE 3.12. Temperature along the whole arc of the retina in the plane of symmetry of the model, from one anterior edge through the posterior pole to the other, at the baseline condition. (A) With the original ring (v1), before cooling and after 15 and 35 min of cooling; the lines at 15 and 35 min nearly coincide. The dotted line marks 35 °C. (B) Difference of the other two rings (v2, v3) from v1 after 35 min; in (A) their lines would lie on that of v1.

3.3.7. Heat Exchanger and Heat Rejection. With the flow fixed the temperature problem is linear, so each solved run gives the temperature of the heated face for any heat load and ambient temperature,

$$T_f - T_{in} = R\,P + c\,(T_a - T_{in}), \tag{3.5}$$

where T_f is the average temperature of the heated face in °C, T_{in} the coolant inlet temperature in °C, R the rise per watt in °C W^{-1}, P the heat in W, c the dimensionless

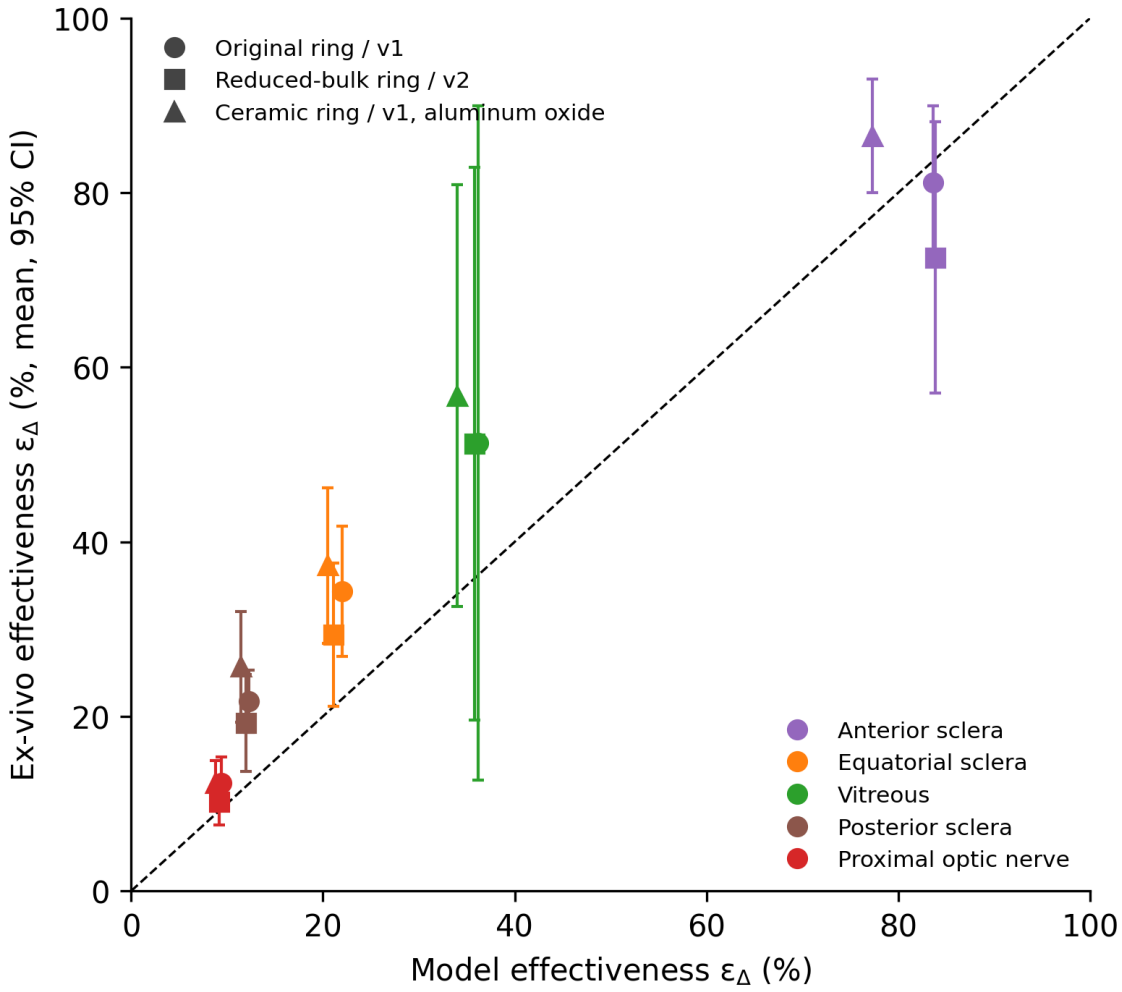

FIGURE 3.13. Cooling effectiveness ε_Δ (Equation 3.4) measured in human donor eyes (mean and 95% confidence interval, from the t distribution with $n-1$ degrees of freedom) against the model, for each site and ring, with the line of identity (dashed). Points above the line cooled more ex vivo than in the model. n, 4 or 5 eyes at the vitreous and 8 or 9 at the other sites. The model values are lower than the effectiveness ε of Figure 3.6, because ε_Δ is referred to the starting temperature of each site rather than to body temperature (at the proximal optic nerve, 9% against 10% for v1).

coupling to the air, and T_a the ambient temperature in °C. Checked against direct calculations, this relation held within 0.013 °C. With R and c of the nine runs of each design, the two heat loads and ambient temperatures of 10 and 35 °C give 36 conditions per design.

With the same heat, the smaller heat exchangers kept the hot side closer to the coolant than the original did at the same flow (Figure 3.14). Over the 36 operating points the heated face stayed 0.7 to 3.9 °C above the coolant inlet in v1, 0.4 to 2.3 °C in v2, and 0.3 to 1.9 °C in v3. At the same flow and heat, v2 rose about 60% as much as v1 and v3 about 45%. These are lower bounds, because the meshes do not resolve the wall layers (Section B.5).

In all three the temperature of the hot side was set almost entirely by the temperature of the coolant. The inlet temperature explained 97.6 to 99.3% of the variance of the heated-face temperature, and the flow rate less than 2%. The 43 °C limit applies to the hottest point, so it was checked against the peak temperature of the heated face, from runs on the mean flow. At the lowest flow, the warmest inlet, and the higher heat load the peak was 24.5 °C in v1, 23.7 °C in v2, and 28.4 °C in v3, at least 14.6 °C below the limit (Figure 3.14). The peak of v3 reached 10 °C above the inlet at 0.10 $\mathrm{L\,min^{-1}}$ and 1.9 °C at 0.50 $\mathrm{L\,min^{-1}}$. It is probably too high, because in the model the heat enters the coolant directly, with no ceramic plate to spread it. The pressure drop rose steeply as the channels narrowed (Table 3.7). The smaller channels of v2 and v3 need about 40 and 200 times the pressure of the original, so at the same pumping power the order reverses: with water at 4 °C and 10 mW the rise was 0.067, 0.105, and 0.143 °C per watt in v1, v2, and v3. The 12-V diaphragm pump of the coolant loop (CrocSee CRS-7A016) is listed at 1.3 $\mathrm{L\,min^{-1}}$ and a head of at least 5 m, about 49 kPa. It covers v1 at every flow of the table. The 42 kPa of v2 at 0.50 $\mathrm{L\,min^{-1}}$ is close to that head, and v3 at 0.25 $\mathrm{L\,min^{-1}}$ and above exceeds it, before the tubing is counted, so these blocks would run at lower flow on this pump or need a stronger one.

3.3.8. Integrated Control Board. The planned bench stages were completed, except that the reference did not stay in regulation under drive (below). The hard limits clamped the module at 3.93 A in cooling, 0.94 A in heating, and ±1.69 V. In closed loop the ring reached a cooling set point in about 30 s and held it within

TABLE 3.7. Pressure drop of the three heat exchangers at each coolant flow rate, and pumping power at the highest flow rate, computed with the Python program.

Flow rate	v1 (Gen 3)	v2	v3
Pressure drop (kPa)			
0.10 L min^{-1}	0.05	1.9	9
0.25 L min^{-1}	0.25	11	52
0.50 L min^{-1}	0.9	42	206
Pumping power (W)			
0.50 L min^{-1}	0.007	0.35	1.7

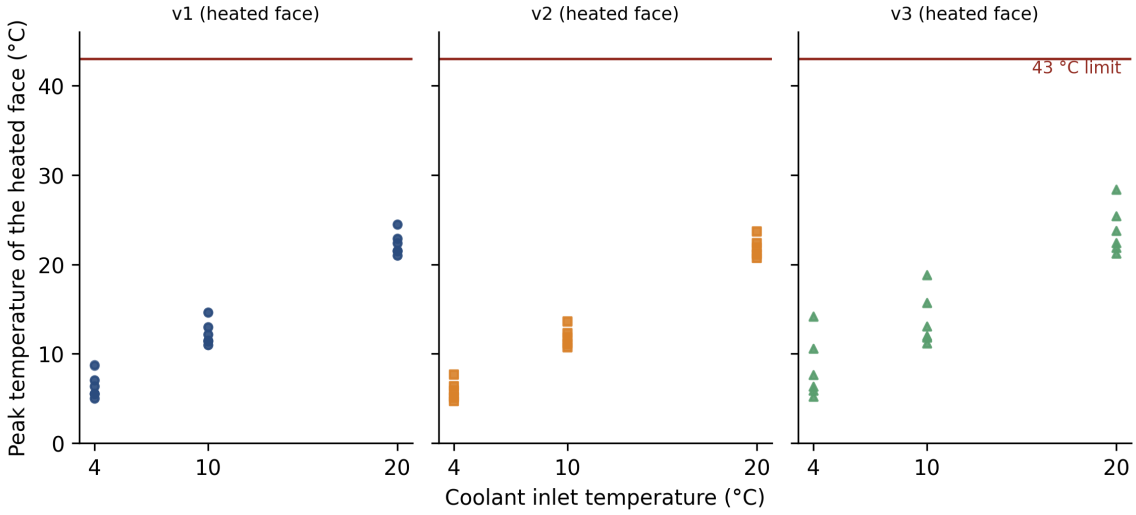

FIGURE 3.14. Peak temperature of the heated face at each of the 36 operating points of each heat exchanger, with 10 and 15.5 W on every design, against the coolant inlet temperature, with the 43 °C limit.

±0.3 °C for 8 min. With the device on a preserved porcine eye in the bath, as in the ex-vivo experiments, the ring was pulled down from 31 to 4 °C in about 5 min and undershot to 2.0 °C. It then held 4.0 °C (3.8 to 4.3 °C) for the last 7 min of a run of 18 min (Figure 3.15A). The current monitor agreed with a multimeter in series within ±0.03 A from 0.48 to 3.93 A in cooling (Figure 3.15B). The safety cutouts that were tested stopped the module when triggered on purpose (Figure 3.15C).

The other functions of the board worked: the flow, pressure, and ambient sensors read, and the board switched the pump and the fan (Section H.2).

One fault remains to be fixed. While the module is driven, the 2.5 V reference of the board at times loses regulation, at the enable step and also during steady drive. The hardware current limits are set from that reference and cannot be relied on while it is disturbed. The module current is then limited only in duration, by the firmware cutouts, which stopped the module in every logged fault. At the start of the soak in Figure 3.15A the reference dipped and the current reversed for a few seconds without a cutout, and the run continued. The cause was not found, and the board was therefore not used in the in-vivo study (Section 4.2.4).

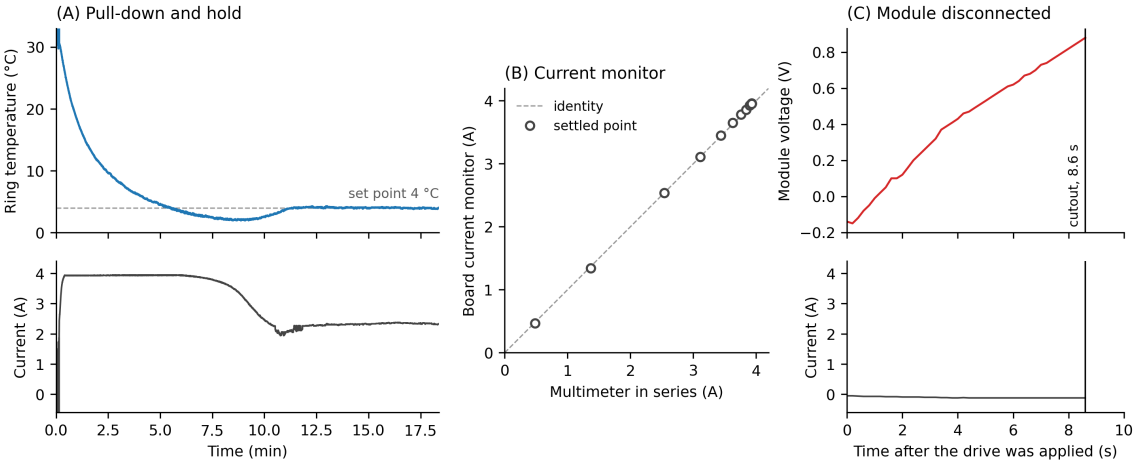

FIGURE 3.15. Bench performance of the control board. (A) Closed-loop pull-down of the ring from 31 °C to the 4 °C set point (dashed) and hold, with the module current, over 18.4 min; the current axis is clipped at −0.6 A, and in the first 9 s the current reversed to −1.5 A. (B) Current reported by the board against a multimeter in series, at thirteen settled drive levels in cooling into a resistor; dashed line, identity. (C) Open-circuit cutout with the module disconnected on purpose: the voltage applied by the control loop rose while almost no current flowed, and the cutout stopped the drive 8.6 s after it was applied.

3.4. Discussion

Both refinements that were built kept the cooling at the proximal optic nerve, the site of the hypothesis: the alumina ring within 1.0 °C of aluminum in paired human donor eyes, and the reduced-bulk ring about 0.6 °C less on average, by no more than 1.2 °C (0.05 °C in the model). The smallest heat exchanger was 81% smaller than

that of Gen 3, and the controller was consolidated onto one board. None of them cooled the proximal optic nerve more than Gen 3 did.

Alumina kept 89 to 95% of the effectiveness of aluminum, which suggests that the limiting resistances lie in the tissue, whereas fused silica became the limiting resistance itself and was excluded without being built. At the other tissue sites the donor eyes showed a trend, not significant after correction, opposite to the model.

That no refinement cooled the proximal optic nerve more than Gen 3 is consistent with the model. At every site the three ring geometries differed in cooling effectiveness ε by no more than 2 percentage points (9.6 to 9.9% at the proximal optic nerve), and the time to 90% of cooling did not change with the geometry. The temperature reached at depth is set mainly by the body temperature. At the proximal optic nerve a change of 1 °C in body temperature moves the final temperature by 0.90 °C and a change of 1 °C in ring temperature by 0.10 °C, the weight of the ring, 10%, being the effectiveness ε (Equation 3.3). At the anterior sclera, next to the ring, the weights are 16% and 84%. At steady state in the model, with its nominal perfusion, the posterior eye sits between a cold source on the anterior sclera and a warm, perfused choroid and orbit, and its temperature is a fixed weighting of the two over the operating points tested, with an effectiveness of about 10% at the proximal optic nerve. Changing the shape of the ring changes how much of the anterior globe and the eyelids it touches, but not that weighting. Deeper cooling would therefore have to come from a colder ring or a different route to the posterior eye.

The insulating layer shows the difference between efficiency and effectiveness: on the original ring it raised the share of heat drawn from the eye from 33 to 36% without improving the temperatures reached. It is worth having for the module and the heat exchanger, which carry less load, and possibly for the patient, since the eyelids are cooled less.

The central retina is cooled no more than the proximal optic nerve, and the nerve 1 cm behind the globe is barely cooled at all. At a body temperature of 39 °C the

proximal optic nerve would not reach 35 °C (35.6 °C with the ring at 4 °C), although the drop below body temperature, 3.4 °C, is no smaller than at 37 °C. The average retina reaches 32 °C with the ring at 12 °C, inside the 30 to 34 °C range of preclinical retinal protection, but the average is carried by the periphery. With the ring at 4 °C the retina ranges from about 9 °C at its anterior edge to 33.7 °C at the optic nerve head (Figure 3.12).

In the model of a perfused eye, at the one perfusion rate it assumes and a body temperature of 37 °C, a ring at the 4 °C floor of the device brings the proximal optic nerve to 33.8 °C, inside the 32 to 35 °C range of systemic therapeutic hypothermia, but 32 °C there would need a ring well below freezing. This value depends more on blood flow than on anything else in the model. Perfusion was not varied in the COMSOL cases. In extension runs of the Python program, with every perfusion rate halved the proximal optic nerve reached 32.1 °C, and with every rate doubled 35.2 °C. The Pennes term assumes that blood leaves the tissue at tissue temperature. A lumped model of retinal heat balance that uses nearly the same choroidal perfusion rate (9.63×10^{-2} s^{-1}) assumes instead an equilibration factor of 0.5 between blood and choroid, which halves the heat exchanged, as in the halved case [**135**]. The wall of the orbit, held at body temperature in the model, is the second influence. With heat allowed to leave through it (an assumed 30 $W\,m^{-2}\,K^{-1}$, range 12 to 50, not measured), at the baseline condition after 35 min, ring v1, and nominal perfusion, the proximal optic nerve reached 33.0 °C (32.6 to 33.2 over that range). The statement that 32 °C cannot be reached at the optic nerve thus holds at the nominal perfusion rates with either wall, and at half the nominal rates only with the wall held at body temperature. With half perfusion and heat leaving through the wall, the optic nerve reaches 30.5 °C.

An earlier Pennes model of a scleral ring from this research program reached a similar value at the optic nerve head, 34.0 °C against 33.7 to 33.8 °C here, with

different boundary conditions and perfusion rates [**56**]. Its multiphase successor, in which cooled blood carries the cold backward, predicts deeper cooling, 29.6 °C [**54**].

Before cooling the model is warmer than the eyes of patients measured at the start of surgery [**47**], 31.7 against 30.0 °C in the anterior chamber and 35.9 against 35.0 °C in front of the optic disc, so the cooling it predicts for a given target temperature could be conservative.

Model and donor eyes rank the sites identically and agree next to the ring, so the model reproduces the path of heat from the anterior sclera into the globe. At depth the donor eyes cooled more, mostly because of the difference between the two conditions (Section 3.3.6). The cooling measured ex vivo at depth is therefore likely to overstate that of a perfused eye, by a factor that remains to be measured.

Heat rejection did not constrain the design. The hot side ran within a few degrees of the coolant in every design, well under the 43 °C limit, which was examined in the model only because the hot side is not instrumented. With the coolant touching the module directly, the smallest heat exchanger kept the hot side closer to the coolant than the largest at the same heat and flow, so most of the volume of the Gen 3 heat exchanger could be removed without thermal penalty. The price is pressure: the narrower channels of v2 and v3 need about 40 and 200 times the pressure of v1 (Table 3.7), and at the same pumping power v1 had the smallest rise. At 0.10 $\mathrm{L\,min^{-1}}$ v3 needs about 9 kPa, within the reach of a small pump.

The module itself has less margin than the heat exchanger. After 35 min the ring of the model drew 3.42 W through the module face held at 4 °C (v1; 2.93 and 2.95 W for v2 and v3). In the heat-exchanger model the hot face of the module ran 0.3 to 20 °C above that, from the coldest to the warmest coolant. Read from the manufacturer's curves of cooling capacity against current, drawn for a hot side at 27 °C, the module pumps about 3.5 W at its maximum current of 3.9 A with no temperature difference across it, 3.0 W at 10 °C, and 2.5 W at 20 °C [**96**]. The margin is therefore small or negative. At full current the module matches the load

of the original ring only with coolant at 4 °C and falls about 0.2 to 0.4 and 0.7 to 0.9 W short of it with coolant at 10 and 20 °C, and v2 and v3 are short by about 0.2 to 0.3 W at 20 °C. A module of the same series with shorter elements, RH14-14-06-L1-W4.5, is listed at 5.4 W with no temperature difference and is 3.8 mm thick against 4.7 mm [**136**]. Scaled in the same way it would cover the load of every ring at all three coolant temperatures, but it needs 6.0 A, above the 3.9 A to which the present board is limited.

The integrated control board met its requirements on the bench, with one exception that prevented its use: the reference that sets the hardware current limits and the temperature measurements can lose regulation under drive. The firmware stopped the module each time, but a safety argument cannot rest on the firmware alone, and the next revision will set the limits from an independent reference.

The model was not validated against a perfused eye, and its tissue properties and perfusion rates come from the literature (Appendix B). Choroidal perfusion was varied only in extension runs. The points sampled in the model are not in the same places as the implanted sensors at depth, and the model and the measurements are therefore compared only approximately there. They agree quantitatively only at the anterior sclera. The lens of v3, the insulating layer, and the smaller heat exchangers were evaluated only in the model, and the ceramic ring only in the original geometry. The ex-vivo comparisons rest on nine eyes from five donors, with four to eight valid pairs per site. One ring of each kind was tested, so a difference between rings cannot be separated from the individual ring and its seating on the eye. The integrated control board was tested on the bench only. Patient comfort remains to be measured. The refinements carried into the in-vivo study were those that had been built and verified: the reduced-bulk aluminum ring on the Gen 3 heat exchanger, regulated by the Gen 3 controller (Section 4.2.4).

CHAPTER 4

Specific Aim 3: Validation and Safety Assessment in an In-Vivo Porcine Model

4.1. Background and Rationale

Neither the isolated eyes nor the computational model of Chapters 2 and 3 can answer the two questions on which any clinical use depends: whether cooling of this kind is tolerated by living ocular tissue, and whether it reaches the retina against an intact circulation. The isolated eye has no perfusion, and the bath that stood in for it resupplied less heat than a perfused eye receives, so the ex-vivo excursions are likely an upper estimate of the cooling reached at depth (Section 2.4). Only in vivo is the choroidal heat load present (Section 1.2.3), and only in vivo can retinal function be measured while the eye is cooled (Section 1.3.3).

The tolerance of living ocular tissue to non-freezing cooling lasting tens of minutes to hours has not, to the author's knowledge, been established. The storage evidence that motivated the 4 °C set point (Section 2.2.1) comes from metabolically arrested, non-perfused tissue held in preservation solution, and prolonged non-freezing cold can injure perfused tissue, with vascular and sensory stages [**137**]. Cooling also has reversible effects specific to the eye: donor corneas stored at 4 °C thin over the first hours of rewarming, as the endothelial pump resumes [**138**], and pressure applied to the globe, as by a contact device, can raise intraocular pressure and can itself occlude the retinal circulation [**6, 21**]. These effects define the safety endpoints of this aim: ocular-surface irritation and corneal integrity, intraocular pressure, retinal function during cooling and after rewarming, tissue histology, and systemic stability.

The objective of this aim was to evaluate, in a live porcine model, whether the device produces changes in the electroretinogram (ERG) consistent with retinal cooling and whether repeated cooling sessions of increasing duration are tolerated by the ocular surface, the globe, and the animal. The hypothesis is stated in Section 1.10.

4.2. Methods

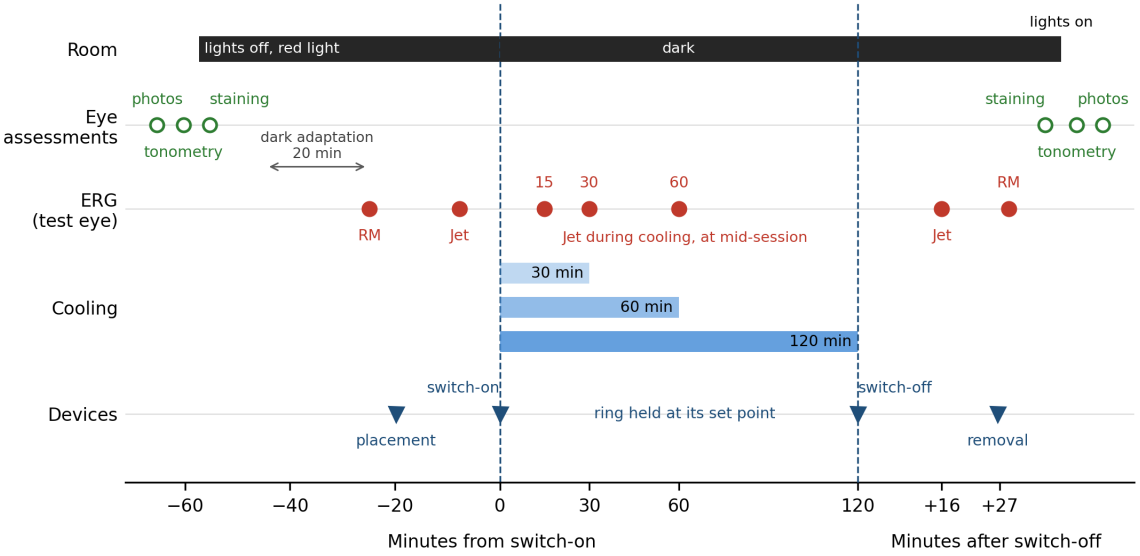

FIGURE 4.1. Sequence of a cooling session: baseline assessments, dark adaptation and baseline ERG, placement of both devices, switch-on, cooling with ERG recordings, switch-off, removal, and the post-cooling assessments. Device and ERG times are the medians of the fifteen sessions of animals 2 to 6; in animals 1 and 2 the number and times of the recordings during cooling varied. Black bar: room lights off and red light, from the corneal staining before cooling to the end of the corneal staining after cooling.

4.2.1. Study Design and Approval. All procedures were approved by the University of Illinois Chicago Animal Care Committee under protocol `IPROTO202600000055` (approved April 30, 2026), with amendments `IAMEND202600000339`, `IAMEND202600000409`, and `IAMEND202600000587`. Each animal underwent three cooling sessions of increasing duration, 30, 60, and 120 min, and a terminal session, with daily observation on the intervening days. Figure 4.1 shows the sequence of a cooling session. The sessions were planned for days 1, 4, 7,

and 10; the days performed and the weight of each animal are given in Table 4.1. In the terminal session, except in animal 1, both devices were placed and a Jet ERG recorded, without cooling, before euthanasia, enucleation, and histology. Baseline assessments were repeated before every cooling session, so that each session was referenced to the state of the eye on that day. Each session was performed under general anesthesia. In every session the right eye was the test eye, fitted with the active device, and the left eye was the control eye, fitted with an identical device that was not activated. Each animal thus served as its own control for the mechanical effects of the device, for anesthesia, and for systemic changes shared by both eyes.

FIGURE 4.2. Animals 3 and 4 in a pen of the Biologic Resources Laboratory on the day after their arrival, with wood-shaving bedding and toys for enrichment. During the study each animal was housed in its own pen.

4.2.2. Animals. The protocol provides for six domestic pigs (*Sus scrofa*; farm strain, not genetically modified), obtained from an approved vendor (Oakhill Genetics) and housed individually in the Biologic Resources Laboratory (BRL) of the University of Illinois Chicago (Figure 4.2). All six were male. At the time of writing

four had completed the study and two were in progress (Section 4.3.1); their weights are given in Table 4.1. They were less than a year old. The species was chosen for the similarity of its eye to the human eye in size, structure, and retinal vascular pattern (Section 1.6). The study was classified in U.S. Department of Agriculture (USDA) pain category D.

The number of animals was set for a paired comparison. Assuming a 30% change in the primary ERG measure (from 100 to 70, in percent of baseline), a standard deviation of 20%, a two-sided α of 0.05, and 80% power, the normal approximation gave 3.5, rounded to four animals, and two further animals were approved to allow for attrition, including equipment failure. The quantity of interest is the change of each eye from its own recording before cooling: a change in the test eye and none in the control eye over the same interval. The power obtained with the measured variability is given with the results (Section 4.3.3).

TABLE 4.1. Sex of each animal, day of each session, and weight, in kg, from the BRL record of the session. Weights marked ∼ are written as approximate on the record. Animal 3 died after session 3. Animals 5 and 6 were still in the study.

Animal	Sex	Days	Session 1	Session 2	Session 3	Session 4
1	Male	1, 4, 8, 11	26.3	26.2	28.0	28.6
2	Male	1, 4, 8, 12	32.5	32.5	33.8	36.7
3	Male	1, 4, 8, –	33.4	∼33	34.8	–
4	Male	1, 4, 8, 11	32.8	∼33	35.1	36.0
5	Male	1, 5, 9	28.9	30	32.8	–
6	Male	1, 7, 10	30.4	32.7	∼33	–

4.2.3. Anesthesia and Physiological Monitoring. Anesthesia was induced and maintained by BRL staff under the approved protocol; the drugs and doses of each session are given in Appendix J. Each pig was sedated, intubated, ventilated, and maintained on isoflurane (1–2%).

From animal 2 onward, after the protocol had been amended, rocuronium was given intravenously once adequate anesthetic depth had been verified, to abolish eye movement; no neuromuscular block was used in animal 1. Animal 2 moved once,

during session 2, and the active device came briefly out of the eye and was reinserted at once.

Flunixin was given for analgesia. Propofol, hydromorphone, and an intravenous infusion of lidocaine and ketamine were given at the discretion of the veterinarian or veterinary technician. The infusion was added after animal 2 moved, as an adjunct to isoflurane.

Electrocardiogram, pulse oximetry, capnography, indirect blood pressure, and rectal temperature were monitored continuously and recorded every 10 to 20 min. Anesthetic depth was assessed from jaw tone and the palpebral reflex together with heart rate, respiratory rate, and blood pressure. Room temperature and relative humidity were logged at the start and end of each session; in animals 1 to 6 they ranged from 19.1 to 22.8 °C and from 34 to 61%. Forced warm air was used when needed and was withdrawn when core temperature rose.

If physiological parameters left the safety range during cooling, the protocol suspended cooling for 15–20 min. A session was ended if core temperature fell below 35 °C and did not stabilize with warming, if oxygen saturation remained below 90%, or if heart rate fell below the limits set by the veterinary staff. Heart rate was watched at device placement and at the start of cooling, because of the oculocardiac reflex.

4.2.4. Device Configuration. The device used in vivo was SCEC Gen 3 fitted with the reduced-bulk ring, the geometry modeled as v2 in Chapter 3, made in aluminum and mounted on the Gen 3 heat exchanger. In the donor eyes this ring gave the proximal optic nerve 0.6 °C less cooling than the original ring on average, a difference unlikely to exceed 1.2 °C. The Gen 3 controller was set to hold the ring at a nominal 4 °C (Section 2.2.1), with the interruptions and the calibration error of Sections 4.3.2 and 4.2.6. In the first two animals the switching element was a relay, as in Aim 1. After the relay had failed in session 2 of animal 1 and in session 3 of animal 2, it was replaced, from session 1 of animal 3 onward, by a trigger switch module with two metal-oxide-semiconductor field-effect transistors (MOSFETs; ANMBEST,

rated by the seller at DC 5–36 V and 15 A), which switched reliably and silently. The heat exchanger was supplied by a pump with distilled water cooled in an ice bath (Appendix F). The control eye was fitted with an identical, inactive replica of the device, so that the two eyes received the same mechanical contact.

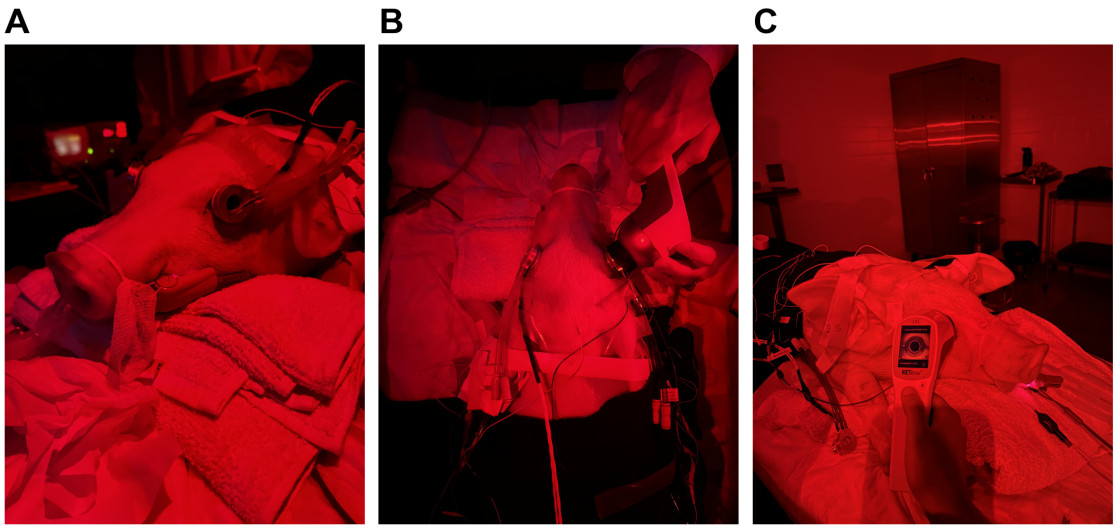

FIGURE 4.3. The cooling session in the animal laboratory, under red light. (A) The device in place on the eye of an anesthetized pig (animal 1, session 1). (B) An ERG recorded with the Jet electrode through the central opening of the device (animal 1, session 1). (C) The screen of the ERG recorder during a recording (animal 2, session 3).

4.2.5. Device Application. From animal 2 the room lights were turned off for corneal staining, and the room was kept dark (Figure 4.3), with every light source covered by a red filter screen, from the start of dark adaptation until after the last corneal staining of the post-cooling assessments. After the baseline assessments (Section 4.2.8), an ophthalmic lubricant was applied to the eye-contacting side of the ring of each device, and the ring was placed on the eye by sliding its skirt under the eyelids with the help of tissue tweezers. The active device was placed on the right eye first and the control device on the left eye 2 to 6 min later, and both were secured with tape (Figure 4.4). The coolant circuit, temperature sensor, and power of the active device were connected with the coolant pump and the thermoelectric drive off;

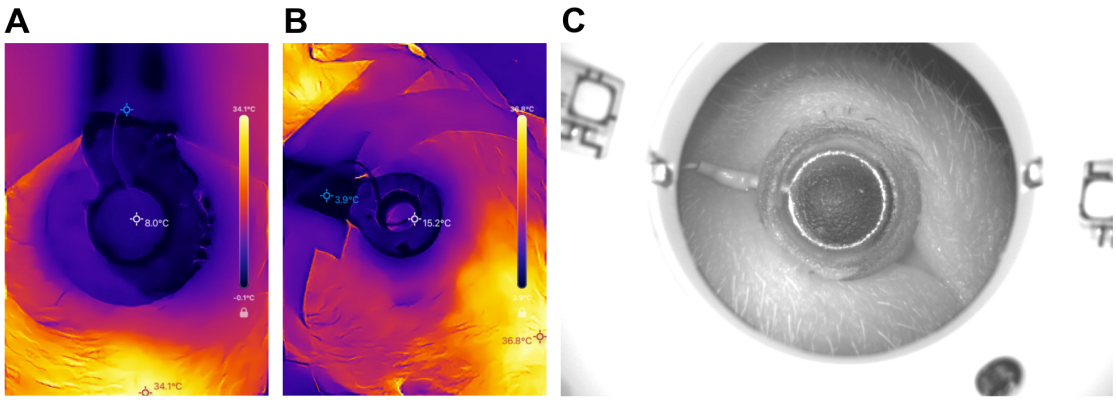

FIGURE 4.4. The device on the eye in thermal images, and the RM electrode seen through the ERG recorder. (A) Animal 6, session 1. (B) Animal 4, session 3. (C) The RM electrode on the eye, seen by the pupil camera of the ERG recorder (animal 5, session 1). The thermal images are from a phone camera with no emissivity correction; the reflective metal parts do not read true temperature, so they show the pattern of cooling only.

both were started together at switch-on. A corneal ERG electrode (Jet) was placed through the central opening of each device onto the cornea.

The ring of the active device read about 16 to 21 °C before it was placed. Each device stayed on for the duration of the session and both devices were then removed, when the ring had rewarmed to between 20 and 35 °C, followed by the post-cooling assessments.

4.2.6. Temperature Monitoring. The temperature of the test ring was measured by the thermistor embedded in it, through the signal-conditioning chain and data-acquisition unit of Aim 1 (Section 2.2.4; DI-2108 unit, WinDaq software), at 10 Hz. The drive current was recorded on a second channel.

Before each session the temperature channel was calibrated in the dry-block calibrator at 37.0, 28.7, 20.5, 12.3, and 4.0 °C. This channel is not the linearized channel of Aim 1, so the readings were converted to temperature with a model of the measuring circuit fitted to the plateaus of that session (Section D.1). Each plateau was read by the same rule as in Aim 1 (Section 2.2.4). A fit to the three original plateaus

reproduced the other two within 0.06 to 0.23 °C, and within 0.03 °C with the contact calibration described below; this is taken as the accuracy of the ring temperature.

Ring temperatures of the sessions before the well thermistor was held against the aluminum are reported with 5.5 °C added, and the set point that read 4 °C in those sessions was about 9.5 °C. Until session 1 of animal 5 the well thermistor used for this calibration did not touch the aluminum of the well, so it could not shed its self-heating as the ring thermistor does in the ring. From session 1 of animal 6 and session 2 of animal 5 a custom piece held it in contact with the aluminum, and the contact calibration is taken as correct. Before session 2 of animal 6 the channel was calibrated both ways on the same morning: against the contact calibration, the earlier arrangement read the ring 5.5 to 5.6 °C low from 4 to 25 °C and 5.2 °C low at 35 °C. Animal 5 shows the same shift between its first two sessions, 5.3 to 6.0 °C before placement and on the eye after switch-on.

No sensors were implanted in vivo. Core temperature was recorded rectally by the anesthesia team.

4.2.7. Assessment of Retinal Function. Full-field ERGs were recorded under dark adaptation. The pupils were dilated with tropicamide (1%), phenylephrine hydrochloride (2.5%), and proparacaine hydrochloride (0.5%), one to two drops each, and the eyes were dark adapted in the darkened room (Section 4.2.5). Dark adaptation was planned at 15 min in animal 1 and at 20 min from animal 2 onward, and the first recording was made at its end. In the 120-min sessions the drops were applied again during the session.

An ophthalmic lubricant gel (Celluvisc, carboxymethylcellulose sodium 1%, Allergan) was applied to the cornea as the coupling medium of the electrode. Sterile subcutaneous needle electrodes served as the ground, on top of the neck, and as the references, one for each eye between the eye and the ear, and stayed in place for the whole session. The active electrode was an RM (RetMap) corneal electrode before the device was installed and after it was removed, and a Jet electrode, through the

central opening of the device, with the device in place. Responses were recorded with a handheld full-field ERG system with an integrated Ganzfeld stimulator (RETeval, LKC Technologies, Gaithersburg, MD, USA), using its scotopic protocol for rabbit, mouse, and pig, with white flashes of three strengths: 0.06, 8, and 25 cd·s/m^2, presented at 0.5, 0.1, and 0.05 Hz. The two eyes were recorded in turn, the test eye first.

In animals 1 and 2 the number and the times of the recordings differed between sessions. In three of these sessions (session 2 of animal 1, sessions 1 and 2 of animal 2) the Jet recording after switch-off was made within 2 min of switch-off, with the ring still below 13.5 °C, and so records the end of cooling rather than recovery. Session 3 of animal 1 had no Jet recording before cooling or after switch-off. From animal 3 the times were fixed: one recording at about the middle of cooling, and the Jet recording after switch-off at about 15 min, once the ring temperature had stabilized. The last recording of each session was made with the RM electrode after the devices had been removed. The ring temperature at each recording was read from the cooling record (mean of the 2 min around the recording); it is not available for session 3 of animal 3, whose cooling was not recorded because the recording software stopped with an error.

The analysis used the raw record of each flash step, as set out in Section E.3: a 0.3 Hz high-pass filter removed drift [**139**], each sweep was referred to the 20 ms before its flash, and sweeps with artefacts were rejected before averaging. For the a-wave and b-wave the average was low-pass filtered at 75 Hz, so that an oscillatory potential could not displace the b-wave peak, a departure from the standard. The oscillatory potentials were band-pass filtered from 75 to 300 Hz, as the standard sets out, at 8 and 25 cd·s/m^2, and the measure is the sum of the first four peaks (OP1 to OP4). The a-wave amplitude was measured from the pre-stimulus baseline and the b-wave amplitude from the a-wave trough.

Dark adaptation in animals 1 and 2 lasted 10 to 23 min, against the standard's minimum of 20 min.

Recordings were grouped by electrode and phase (RM before installation; Jet before switch-on; Jet during cooling; Jet after switch-off; RM after removal).

4.2.8. Ocular Safety Assessments. Both eyes were assessed at the start of each session, before the devices were placed, and again after they were removed. At the start of the session the order was: photographs of the ocular surface, intraocular pressure, corneal staining, then the mydriatic drops, dark adaptation, and the ERG. The staining therefore preceded the baseline ERG, by 24 to 37 min in animals 1 to 4. After the session the corneal staining was done with the lights off, and tonometry and the photographs followed with the lights on.

Ocular irritation. The ocular surface of each eye was photographed, and bulbar and limbal redness were graded from the photographs in whole steps on the 0-to-4 scales of the Johnson & Johnson Clinical Assessment Guide named in the protocol [**140**], which are adapted from the Efron grading scales [**141**]. Tarsal redness, also listed in the protocol, was not graded. The grading was done by the author, who knew which eye and which time point each photograph showed. Each eye and time point was graded on one photograph, chosen by the author from the burst.

Intraocular pressure. A handheld rebound tonometer (TonoLab, Icare Finland Oy, Vantaa, Finland) was held perpendicular to the central cornea, with the lids retracted without pressure on the globe, and one to three readings per eye were averaged. The instrument is calibrated for the rodent eye and not validated for the porcine eye, so its readings might carry a systematic offset; they are used only to compare the two eyes of an animal and the same eye before and after a session. At the start of the session the pressure was measured before any topical anesthetic had been applied.

Corneal staining. The eye was flushed with sterile saline and stained with fluorescein, from a wetted sterile strip touched to the conjunctiva away from the central cornea. The excess was flushed away and the surface was photographed in the dark through a cobalt-blue filter. The staining was graded from 0 to 4 for each eye by the author on one photograph per eye and time point, chosen as for redness.

Observation. Between sessions each animal was observed daily for ocular swelling, squinting, episcleral injection, and corneal ulceration, and was weighed, scored for body condition, and checked for food and water intake.

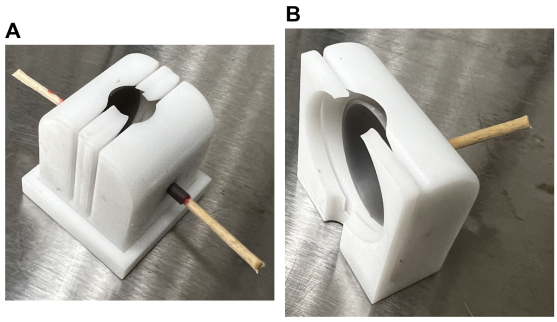

FIGURE 4.5. The 3D-printed guide used to trim the fixed globes for histology, seen from the front (A) and from the side (B). The globe sits in the spherical cavity and is kept centered by a small shell pushed in with the two sticks; the two parallel slots, about 5 mm apart, guide the blade.

Histology. In the terminal session, after the assessments, each animal was euthanized under anesthesia with an intravenous overdose of pentobarbital sodium (Fatal-Plus, 390 mg/mL; 10 mL intravenously), a method acceptable for swine under the AVMA Guidelines for the Euthanasia of Animals [**142**]. Death was confirmed by the absence of respiration, cardiac activity, corneal reflex, and muscle tone. Both eyes were enucleated immediately and fixed, in animal 1 by immersion in formalin and in animal 2 by immersion in formalin with an injection of formalin into the globe. From animal 3 onward modified Davidson's fixative was used, injected into the globe and by immersion, to limit retinal detachment. After fixation each globe was trimmed in a 3D-printed guide designed for this study (Figure 4.5), leaving a central slab containing the optic nerve head and the cooled region. The eyes were sectioned from the cornea to the optic nerve and processed by the UIC histology core for hematoxylin and eosin staining of the cornea, sclera, conjunctiva, and peripheral retina. The stained sections of all animals are imaged together with a digital microscope (VHX-6000, Keyence, Osaka, Japan) at the Robbins and Jacobs Family Biocompatibility

and Implant Pathology Laboratory (Rush University Medical Center, Chicago, IL, USA), and are read by the author.

4.2.9. Data Analysis.

Animals analyzed. Animal 1 is treated as exploratory: the procedure was still being established during its sessions (no neuromuscular block, different premedication and calibration, relay failure, sequence of assessments adjusted between sessions, corneal staining not yet done in the dark). Its results are reported for each session but are not included in the summary statistics, which are based on animals 2 onward. The sessions were grouped by the ring temperature delivered: about 9.5 °C (animals 2 to 4 and session 1 of animal 5) and about 4 °C (animal 5 from session 2, and animal 6). Summary statistics are given for the first group; the second, with one or two sessions at each duration, is reported session by session.

Evidence of retinal cooling. Cooling prolongs the implicit times and reduces the amplitudes of the ERG [**73, 74, 76**]. Because it slows the response, it is also expected to widen the b-wave, and a cooling that is not uniform across the retina would widen it further, by spreading the timing of the responses of its different parts. Each eye was compared with itself: the change in the test eye from its pre-cooling recording to the recording during cooling, made with the same Jet electrode after the device had been installed, and the change in the control eye over the same interval. The test eye shows the effect of cooling, and the control eye shows whether the same change occurred without it, through anesthesia, time, or mechanical contact. The two changes were also compared with each other. The comparison between the two eyes of one animal is the more sensitive one: the b-wave amplitudes of the two eyes of normal subjects differ by a median of 8 to 11% [**143**], in minipigs no ERG measure differs between the right and the left eye [**144**], and anesthesia and core temperature act on both eyes alike. Recordings of one eye on different days, by contrast, must differ in amplitude by 25% or more, and often 40%, before the change is significant [**145**]. A session was counted as showing cooling when the b-wave of the test eye had both a longer

implicit time and a smaller amplitude than before cooling. Every change is given as a value and as a percentage of the eye's own earlier value.

Besides the implicit time and amplitude, the shape of the b-wave was measured on the filtered trace. Its width was the time spent above 80% of its amplitude, measured from the a-wave trough, stopping at the first trough after the peak, so that a later wave is not counted.

The analysis was made separately for each cooling duration. For each duration the effect is reported for each animal and as a mean with its 95% confidence interval, and a paired t test assessed whether the mean differed from zero, for each eye's change and for the two compared. Means over all durations pool sessions of the same animals and are descriptive. The primary measures were the b-wave implicit time and amplitude at 25 $\mathrm{cd\,s\,m^{-2}}$, and the summed oscillatory potentials at 8 and 25 $\mathrm{cd\,s\,m^{-2}}$. Of these, only the b-wave was used to judge whether the retina had cooled: a slower b-wave was taken as evidence of cooling, whereas a change in amplitude without a change in implicit time was not, because amplitude also depends on electrode contact and position. The oscillatory potentials are reported alongside the b-wave but were not part of this judgment. The a-wave and the b-wave at the other two flashes, 8 $\mathrm{cd\,s\,m^{-2}}$ and 0.06 $\mathrm{cd\,s\,m^{-2}}$, whose response is driven mainly by the rods, were secondary measures.

Safety. Safety was judged by two criteria. The first was restoration: after rewarming, the ERG of the test eye was expected to return to its own pre-cooling values. This was judged primarily on the Jet recordings before cooling and after switch-off, and confirmed on the RM recordings before installation of the devices and after their removal, which span the whole session. The eye was taken as restored when the b-wave implicit time and amplitude had returned to their own baseline, by the criterion given below. The Jet recording after switch-off counted as a recovery recording only when made at least 10 min after switch-off. The three made within 2 min (session 2 of animal 1, sessions 1 and 2 of animal 2), with the ring still below 13.5 °C, record

the end of cooling and were left out of this comparison and of the spread it is judged against; in these three sessions restoration was judged on the RM pair alone. The others were made 11 to 25 min after switch-off, when the ring had recovered 73 to 97% of its fall during cooling. To show how far the ERG came back as the ring rewarmed, the change from the recording during cooling to the recovery recording was also computed, with the same paired analysis. To detect any change persisting from one session to the next, the Jet recordings at the start of sessions 2, 3, and 4 were compared with those of session 1, and the comparison was confirmed with the RM recordings before installation of the devices. Core temperature, which alters the ERG [**122**], is reported with the comparison, and within a session the control eye shows its effect.

The second criterion was the absence of a harmful increment: intraocular pressure, ocular-surface redness, and corneal staining in the test eye should rise after cooling no more than in the control eye, and no increase should persist to the baseline of the next session. Histology of the test eye was compared with that of the control eye for damage present only in the cooled eye.

Safety was not inferred from the absence of a statistically significant difference, which is expected with four to six animals even when a real effect exists. An ERG measure was considered restored when the test eye's change from its own baseline after rewarming lay within twice the standard deviation of the control eye's own change over the same interval, pooled over the primary animals and sessions, separately for each electrode, flash, and measure.

When the test eye had not returned, the control eye was examined over the same interval, because a change in both eyes points to a systemic cause rather than to cooling. Each eye's change was compared with the usual change of the control eyes, their median ± twice the scaled median absolute deviation, never narrower than ±20% or ±3 ms; when both eyes lay outside that range in the same direction, the change was attributed to a systemic cause.

Statistical considerations. The change of the test eye was compared with that of the control eye of the same animal by a paired t test. For the primary measures, the b-wave implicit time and amplitude at 25 $\mathrm{cd\,s\,m^{-2}}$, the p value of this comparison during cooling was corrected with the Holm method across the three cooling durations, separately for each measure. The ring temperature, a device endpoint, was kept outside this family, and the tests of each eye against itself were descriptive and were not corrected. With the b-wave designated as the evidence of cooling, the remaining comparisons were treated as descriptive rather than as a family of hypothesis tests. Effect sizes were reported as the small-sample-corrected (Hedges) standardized mean of the paired differences, $g = J\,\bar{x}_d/s_d$, where $\bar{x}_d$ and s_d are the mean and standard deviation of the test-minus-control differences over the animals and $J = 1 - 3/(4(n-1)-1)$ corrects the bias for n animals, with 95% confidence intervals from the large-sample normal approximation, which is only approximate with three to five animals. For restoration and for the intraocular pressure, the 90% confidence interval of the mean test-minus-control difference was reported; its limit farther from zero is the largest difference that the data allow. With so few animals, failure to detect a difference was not interpreted as evidence that there is none. The redness and staining grades are ordinal and were reported as counts by eye and session. With four animals a paired rank test cannot reach a two-sided p below 0.125, and with six not below 0.031, so no test was applied to them. The three cooling durations were always given in the same order, 30, 60, and 120 min, in the same animals, so a trend across sessions cannot be separated from the effects of repeated anesthesia and cumulative exposure, and is reported as such.

4.3. Results

4.3.1. Animals and Sessions. At the time of writing four animals had completed the study. Animals 1, 2, and 4 completed all four sessions. Animal 3 completed its three cooling sessions and died in recovery from the third (Section 4.3.6), so it had

no terminal session. Animals 5 and 6 had completed their three cooling sessions. All eighteen cooling sessions were given for their planned duration. Animal 1, which is exploratory, is reported for each session and kept out of the summary ranges, which are those of the primary animals (Section 4.2.9).

4.3.2. Device Operation and Ring Temperature. The ring temperature of the seventeen sessions with a record, corrected for the calibration error (Section 4.2.6), is shown in Figure 4.6 and summarized in Table 4.2. The cooling of session 3 of animal 3 was not recorded because the recording software stopped with an error before cooling began; the session log gives switch-on at 10:42 and switch-off at 12:42. Before switch-on the ring read 27.6 to 31.0 °C on the eye in the primary animals (27.4 to 32.3 °C in animal 1). In the sessions at about 9.5 °C it came within 0.5 °C of its regulated level in 0.8 to 2.9 min after switch-on, and the controller began to switch the drive on and off 1.0 to 3.1 min after switch-on. From then until switch-off the ring was held at a mean of 9.1 to 10.0 °C, depending on the session, with a standard deviation within a session of at most 0.14 °C (Table 4.2, which also gives animal 1). With the contact calibration the ring took 3.9 to 10.3 min to come within 0.5 °C of its level, 4.0 to 4.1 °C (sessions 2 and 3 of animal 5 and all three of animal 6). In the first two of these (sessions 1 of animal 6 and 2 of animal 5) the drive stayed on nearly all the time: the device was at its limit near a true 4 °C.

The level differed between sessions by more than the scatter within each, because the controller's switching point differed, by up to 0.4 °C from the reading at the calibration plateau.

With the relay (animals 1 and 2) the drive was interrupted in two of the six sessions. In session 2 of animal 1 cooling was restarted four times between 2.5 and 18.0 min after switch-on, and the ring rose to as much as 26.5 °C before it was cooled again. This session was extended to 69 min, and the ring reached 7.6 °C in it. In session 3 of animal 2 the drive was off for 97 s at 53.5 min, when the ring rose to 14.3 °C, and the relay failed again in the last 2 min, when the drive was switched

by hand. The lowest ring temperature of any session, after correction, was 6.7 °C, in the last 2 min of session 3 of animal 2, a reading below the calibrated range. With the contact calibration the lowest was 3.8 °C. The ring therefore stayed above 2 °C, the temperature below which the risk of cold injury to stored organs rises [**116**], the only reference available for this exposure (Section 1.7). With the MOSFET switch module (animals 3 to 6, eleven sessions with a record) the drive was not interrupted, and from 2 min after the controller began to switch until 1 min before switch-off the ring stayed between 8.9 and 10.2 °C in the sessions at about 9.5 °C.

In all, each of animals 2 to 6 had the device switched on for 210 to 211 min over its three cooling sessions, and animal 1, whose session 2 was extended, for 219 min. In the sessions of animals 3 to 6 with a record the device was switched on 13 to 33 min after placement and removed 23 to 33 min after switch-off.

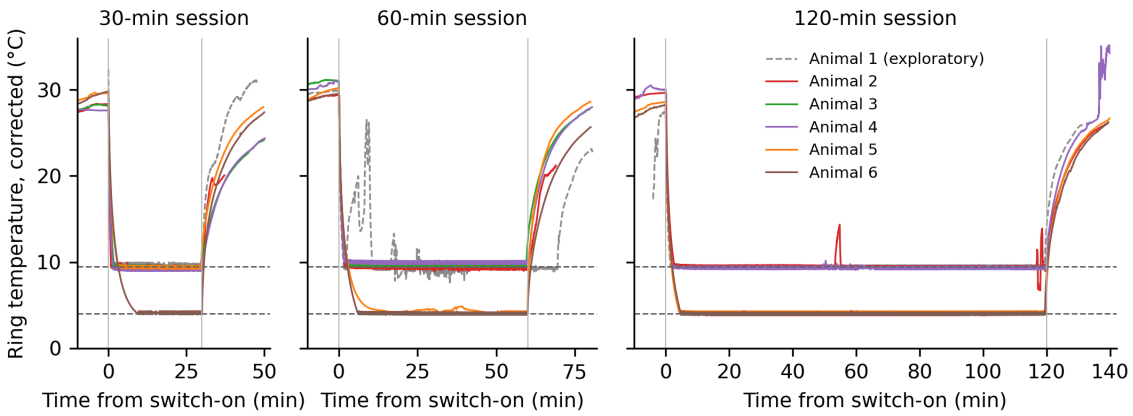

FIGURE 4.6. Temperature of the ring of the active device in each cooling session, against time from switch-on, drawn from placement on the eye or 10 min before switch-on, whichever is later, to removal or 20 min after switch-off, whichever is earlier. All traces are corrected for the calibration error. Dashed lines, the set point in the two calibrations, 9.5 and 4 °C; vertical lines, planned switch-on and switch-off. Animal 1, exploratory, dashed grey line. Session 3 of animal 3 has no record of the cooling. Each trace is the 1-s mean of the 10-Hz record. The rise of animal 4 before removal in the 120-min session coincides with the post-cooling ERG, recorded with the device still on the eye.

TABLE 4.2. Ring temperature in each cooling session. Cooling: time from switch-on to switch-off. To level: time from switch-on until the ring was within 0.5 °C of its regulated level. Level: mean (standard deviation) of the ring temperature from 2 min after the controller began to switch the drive to 1 min before switch-off, excluding each interruption of the drive and the 3 min after it, corrected for the calibration error; 5th–95th: percentiles over the same period. Interruptions: sustained stops of the drive during cooling, with their times after switch-on. Animal 1 is exploratory.

Animal, session	**Cooling (min)**	**To level (min)**	**Level (°C)**	**5th–95th (°C)**	**Interruptions**
1[b], 1	30.2	1.6	9.68 (0.13)	9.49–9.90	None
2, 1	29.7	0.8	9.39 (0.14)	9.17–9.61	None
3, 1	30.0	1.7	9.41 (0.12)	9.23–9.59	None
4, 1	30.3	1.4	9.12 (0.11)	8.96–9.29	None
5, 1	29.8	2.9	9.39 (0.10)	9.24–9.54	None
6, 1[c]	30.1	7.9	4.06 (0.09)	3.96–4.26	None
1[b], 2	69.2	1.5	9.26 (0.29)	8.81–9.59	Four (relay), 2.5–18.0 min
2, 2	60.0	1.8	9.36 (0.13)	9.15–9.55	None
3, 2	59.6	2.5	9.61 (0.11)	9.45–9.76	None
4, 2	60.5	1.7	9.96 (0.12)	9.79–10.13	None
5, 2[c]	60.0	10.3	4.09 (0.09)	3.98–4.28	None
6, 2[c]	60.0	5.6	4.04 (0.10)	3.89–4.18	None
1[b], 3	119.4	1.4	9.45 (0.12)	9.28–9.62	None
2, 3	119.8	2.6	9.44 (0.11)	9.29–9.62	Two (relay), 53.5 and 117.0 min
3, 3	120[a]	–	–	–	Cooling not recorded
4, 3	119.9	1.8	9.32 (0.11)	9.18–9.47	None
5, 3[c]	119.8	3.9	4.13 (0.11)	3.97–4.28	None
6, 3[c]	119.5	4.4	3.99 (0.11)	3.83–4.15	None

[a]From the session log; the ring record does not cover the cooling.
[b]Exploratory animal (Section 4.2.9).
[c]Contact calibration, no correction.

4.3.3. Electroretinogram. The analysis used 93 recordings of animals 1 to 6, each of both eyes at the three flash strengths (558 traces). Examples are shown in Figure 4.7. The trace checks of Section E.3 flagged 17 of the 186 traces at 25 cd·s/m^2 and 13 at 8 cd·s/m^2, and 62 of the 186 at 0.06 cd·s/m^2, where the response is small and the picks are least certain. During cooling the ring was at 9.1 to 10.0 °C at every

recording of the first group and 4.0 to 4.5 °C in the second, among recordings with a ring record.

Cooling. During cooling the b-wave of the test eye became slower than before cooling in all 15 sessions of animals 2 to 6, by 2.0 to 8.0 ms at 25 cd·s/m^2, the primary stimulus, while the control eye changed by no more than 2.5 ms in either direction (Figure 4.8, Table 4.4). With the ring at about 9.5 °C the test eye slowed by a mean of 5.4, 5.3, and 4.8 ms (16, 17, and 15%) after 30, 60, and 120 min, and over the ten sessions by 5.2 ms (16%), against 0.0 ms (0.1%) in the control eye. With the ring at about 4 °C it slowed by 7.5, 5.8, and 6.2 ms (24, 17, and 19%) after 30, 60, and 120 min, and the control eye by 1.0, 0.0, and 1.2 ms. At 0.06 cd·s/m^2, whose slower rod-driven b-wave peaks later, the change was larger and more variable. Against the control eye, over animals 2 to 6, the slowing of the test eye after 30, 60, and 120 min had p values of 0.013, 0.013, and 0.001, and 0.026, 0.026, and 0.004 after Holm correction across the three durations, with effect sizes $g = 1.53$ (95% confidence interval 0.35 to 2.70), 1.52 (0.34 to 2.69), and 2.78 (0.92 to 4.63). With the ring at about 9.5 °C alone (Table 4.4) the p values were 0.048, 0.157, and 0.005, and 0.097, 0.157, and 0.015 after correction, so in that group the slowing remained significant after 120 min only.

The b-wave also became wider in the test eye in 14 of the 15 sessions: over the ten sessions at about 9.5 °C its width at 80% of the amplitude rose by a mean of 4.4 ms (39%), against 1.6 ms (13%) in the control eye, which also widened in 13 of the 15 sessions. Sessions 1 and 2 of animal 6, the second of them the exception, had a broad response in both eyes, so their width is unreliable.

The b-wave amplitude of the test eye fell by a mean of 81 μV (20%) over the ten sessions and that of the control eye rose by 20 μV (5%), but at each duration the confidence interval of the test eye included zero. Against the control eye, over animals 2 to 6, the amplitude had p values of 0.039, 0.085, and 0.359 after 30, 60, and 120 min, and 0.116, 0.170, and 0.359 after Holm correction, so no duration remained

significant ($g = -1.08$, 95% confidence interval -2.06 to -0.11; -0.82, -1.68 to 0.05; -0.37, -1.11 to 0.37). The criterion of Section 4.2.9, a slower and smaller b-wave, was met by the test eye in 11 of the 15 sessions at 25 cd·s/m^2 and 11 at 8 cd·s/m^2, and by the control eye in 3 and 4.

The summed oscillatory potentials of the test eye fell during cooling in 12 of the 15 sessions at 25 cd·s/m^2 and 14 at 8 cd·s/m^2, and those of the control eye in 6 and 7. The fall grew with the duration of cooling, by 17, 42, and 59% at 25 cd·s/m^2, and it was 29, 60, and 59% after 30, 60, and 120 min with the ring at about 4 °C.

Ring temperature and rewarming. Across the Jet recordings before cooling, during cooling, and after switch-off, the test-minus-control implicit time of the b-wave was longer the colder the ring (Figure 4.9). Over the 47 recordings with both eyes and a ring record, it lengthened by 0.23 ms for each degree that the ring was colder (correlation coefficient -0.76). temperature less closely ($r = 0.36$ at 8 cd·s/m^2). Because the recordings during cooling were made with the ring at one of two levels, this association rests on the contrast between the cold recordings and those made with the ring warm, before cooling and after switch-off. From the recording during cooling to the recording after switch-off, the implicit time of the test eye shortened by a mean of 2.1 ms over the eight sessions with a recovery recording at about 9.5 °C (by 4.8 and 4.2 ms after 30 and 60 min), while that of the control eye lengthened by 2.1 ms, driven by session 3 of animal 3. The summed oscillatory potentials of the test eye rose by a mean of 21 μV and those of the control eye fell by 67 μV at 25 cd·s/m^2.

Restoration. The Jet recordings after switch-off counted as recovery recordings in 13 of the 15 sessions of animals 2 to 6, and in 14 of 18 with animal 1; they were made 11 to 25 min after switch-off, with the ring 73 to 97% rewarmed, and the other three, made within 2 min of switch-off, were left out (Section 4.2.9). By the criterion of Section 4.2.9, the test eye's change from its own value before cooling within twice the standard deviation of the control eyes' own change, the implicit time of the b-wave was restored in 12 of the 13 sessions at each flash strength, against a limit of $\pm$9.9 ms

at 25 cd·s/m^2. The amplitude was restored in 12 of the 13 at 25 and 8 cd·s/m^2 and in all at 0.06 cd·s/m^2, and the width in all 13 at 25 and 11 at 8 cd·s/m^2. All these exceptions were session 3 of animal 3, in which both eyes changed together, except the implicit time at 0.06 cd·s/m^2 in session 1 of animal 4, which was beyond the limit (55 against 48 ms) with the control eye also slower, and the width at 8 cd·s/m^2 in session 2 of animal 6, in which both eyes changed together. The summed oscillatory potentials were restored in 11 of the 13 at each of the two flashes: sessions 2 and 3 of animal 3 at 25 cd·s/m^2, and session 3 of animals 2 and 3 at 8 cd·s/m^2. With the RM electrode, from before placement of the devices to after their removal, the implicit time and oscillatory potentials were restored in all 15 sessions at 25 cd·s/m^2, and the amplitude and width in 14, the exception being session 3 of animal 3. In animal 1 the one recovery recording was restored, and with the RM electrode the implicit time was not in session 1 (+19.5 against a limit of 18.9 ms). The 90% confidence intervals of the test-minus-control change over animals 2 to 6 bound the lasting change that the data still allow. With the Jet pair, a lengthening of the implicit time at 25 cd·s/m^2 by more than 3.6, 3.2, and 4.3 ms after 30, 60, and 120 min is unlikely, and a fall of the amplitude by more than 92 and 190 μV after 60 and 120 min; after 30 min the amplitude of the test eye came back higher than that of the control eye (mean +89 μV, 90% confidence interval +34 to +144). With the RM pair, as confirmation, the bounds were 8.3, 4.8, and 8.2 ms for the implicit time and 179, 177, and 133 μV for the amplitude; after 60 min the amplitude of the test eye was lower than that of the control eye (mean −123 μV, 90% confidence interval −177 to −69), although each session lay within the restoration limit.

In session 3 of animal 3, the session after which the animal died (Section 4.3.6), both eyes moved beyond the usual range of the control eyes in the same direction, with both electrodes, and the eye check attributed the change to a systemic cause.

Between sessions. Over the twelve later sessions of animals 2 to 6, the Jet recording at the start of each session differed from that of session 1, test minus control, by −4.5

to +2.0 ms in the b-wave implicit time and by −110 to +148 μV in its amplitude at 25 cd·s/m^2, with the core temperature 1.3 °C lower to 0.9 °C higher than in session 1. The RM recordings before installation of the devices confirmed this, at −4.5 to +3.5 ms and −119 to +331 μV, so no slowing of the test eye persisted to the next session.

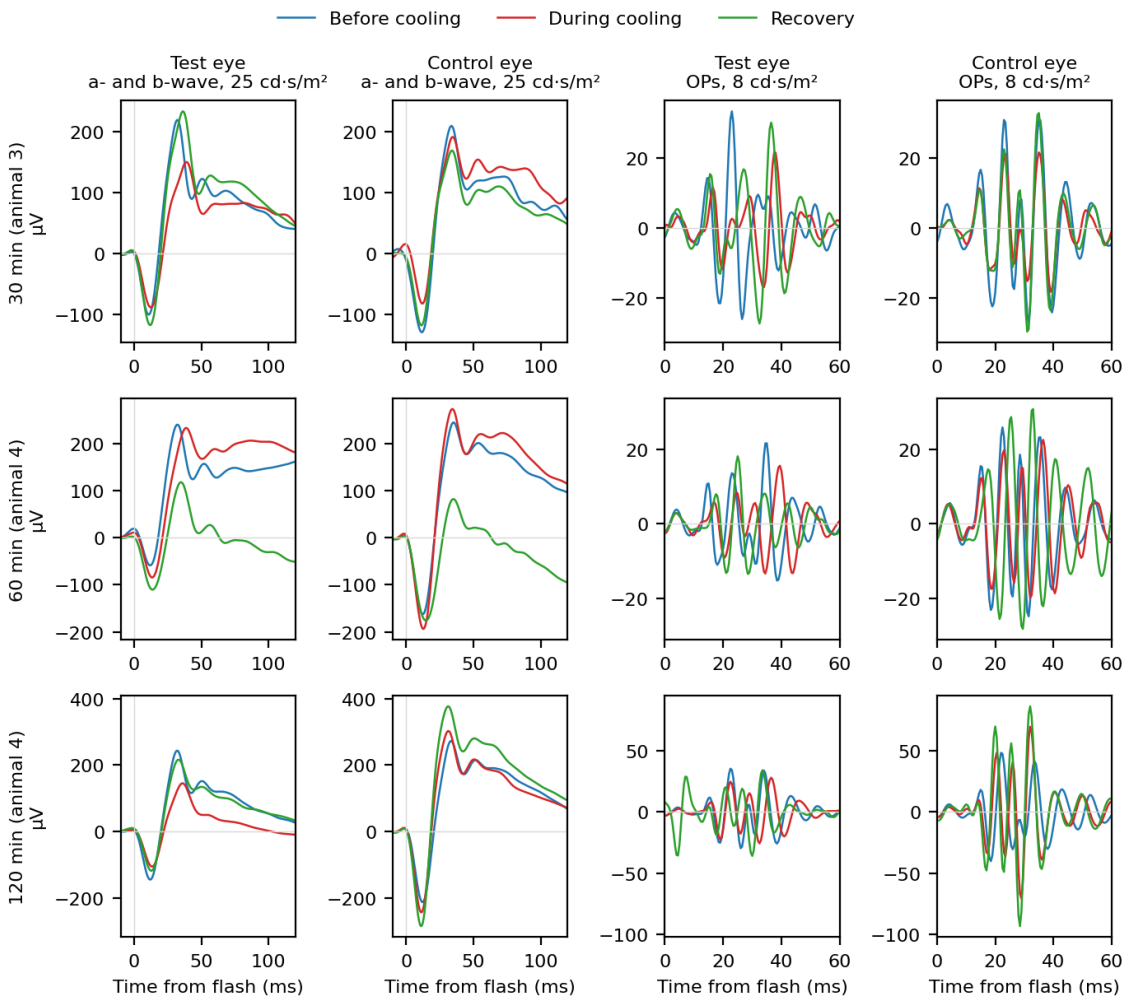

FIGURE 4.7. Electroretinograms in one session of each duration (rows; animal 3 at 30 min, animal 4 at 60 and 120 min), Jet electrode: before cooling (blue), during cooling (red), and the recovery recording 15 min after switch-off (green). First two columns, a- and b-wave of the test and control eyes at 25 cd·s/m^2; last two columns, oscillatory potentials at 8 cd·s/m^2 (75–300 Hz). These are, at each duration, the sessions with the median b-wave slowing among those with a recovery recording, at the ring of about 9.5 °C.

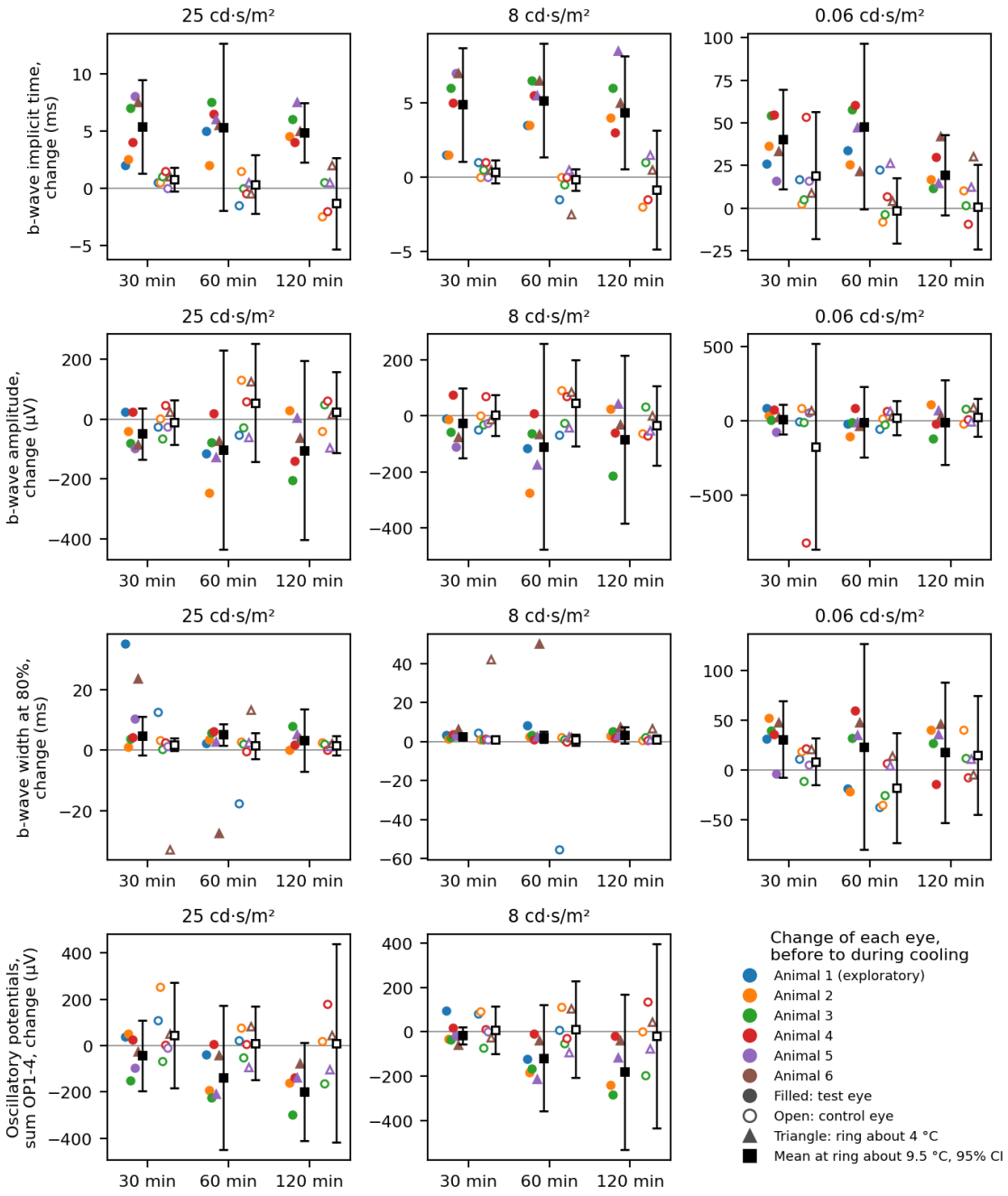

FIGURE 4.8. Change of each eye from its own recording before cooling to that during cooling: test eye (filled) and control eye (open), for each animal, and the mean with its 95% confidence interval (squares) for the sessions with the ring at about 9.5 °C; triangles, ring at about 4 °C. By cooling duration: b-wave implicit time, amplitude, and width at 80% of the amplitude, and summed amplitude of oscillatory potentials 1 to 4 (bottom), at the three flash strengths (oscillatory potentials at the two stronger). Jet electrode in both recordings.

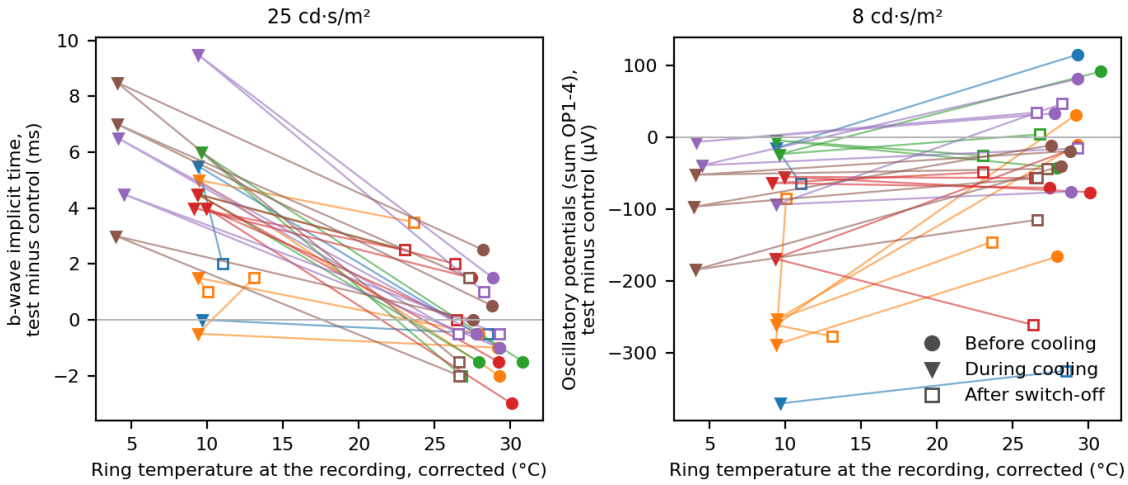

FIGURE 4.9. Test-minus-control b-wave implicit time at 25 cd·s/m^2 (left) and summed oscillatory potentials at 8 cd·s/m^2 (right) against the ring temperature at each Jet recording: before cooling (circles), during cooling (triangles), and after switch-off (squares), one line per session, coloured by animal. Ring temperatures corrected for the calibration error. The squares near 10–13 °C are the three recordings made within 2 min of switch-off.

With the measured variability, Table 4.3 gives the power of the three b-wave measures: for the test eye against itself, for the control eye against itself, and for the two changes compared. The change assumed is 30% for the amplitude, as planned, and the measured change of the test eye for the implicit time and the width.

4.3.4. Intraocular Pressure. Over the session, from before placement of the devices to after their removal, the reading of the control eye rose in 8 of the 9 sessions of animals 2 to 4, by up to 16.0 mmHg, and that of the test eye rose in 4 and fell in 5 (Figure 4.10). The test eye rose more than the control eye in 2 of the 9 sessions, both in animal 2 (sessions 1 and 2, by 18.0 and 7.5 mmHg), and less in the other 7. In animal 1 the control eye rose in all three sessions, by up to 26.5 mmHg, and the test eye rose less than the control eye in all three. Table 4.4 gives each eye's change and the two compared. The confidence interval of the comparison excluded zero only after 120 min, when the mean was −10.7 mmHg. By the upper limits of the 90% confidence intervals, a rise of the test eye exceeding that of the control eye by more

TABLE 4.3. Power of the paired t test (two-sided α of 0.05, noncentral t distribution) for the change from before to during cooling, Jet electrode, 25 cd s m^{-2}, from the ten sessions with the ring at about 9.5 °C. SD, standard deviation of the change over the sessions. Units as in the first column.

Measure	**Comparison**	**Mean change**	**SD**	**Change assumed**	**Power, n=4**	**Power, n=6**	**n for 80%**
b-wave implicit time (ms)	Test vs test	+5.2	2.1	5.2	89%	>99%	4
	Control vs control	+0.0	1.4	5.2	>99%	>99%	3
	Test vs control	+5.2	2.6	5.2	76%	97%	5
b-wave implicit time (%)	Test vs test	+15.8	6.0	15.8	92%	>99%	4
	Control vs control	+0.1	4.0	15.8	>99%	>99%	3
	Test vs control	+15.7	7.5	15.8	79%	98%	5
b-wave amplitude (%)	Test vs test	−19.6	23.1	30.0	43%	72%	7
	Control vs control	+4.8	15.5	30.0	73%	96%	5
	Test vs control	−24.4	29.5	30.0	30%	52%	10
b-wave width at 80% (ms)	Test vs test	+4.4	3.2	4.4	48%	77%	7
	Control vs control	+1.6	1.3	4.4	>99%	>99%	3
	Test vs control	+2.8	3.8	4.4	37%	64%	8
b-wave width at 80% (%)	Test vs test	+38.9	27.0	38.9	50%	80%	6
	Control vs control	+13.1	10.9	38.9	>99%	>99%	3
	Test vs control	+25.7	32.0	38.9	39%	67%	8

than 13.8 and 13.7 mmHg after 30 and 60 min is unlikely, and after 120 min the test eye rose less than the control eye (upper limit −4.8 mmHg). In animals 5 and 6 the test eye rose less than the control eye in all six sessions, by 7 to 22 mmHg; in session 1 of animal 6 both rose, by 19 and 34 mmHg. Over the 15 sessions of animals 2 to 6 the reading of the control eye rose in 14, by a mean of 11.3 mmHg, and that of the test eye by a mean of 2.7 mmHg; the test eye rose less than the control eye in 13 of the 15 (mean difference −8.5 mmHg over the five animals, 95% confidence interval −18.6 to +1.5). At the start of the later sessions the reading in animals 2 to 6 differed from

that at the start of session 1 by −6.0 to +11.0 mmHg in the test eye and by −14.5 to +13.3 mmHg in the control eye.

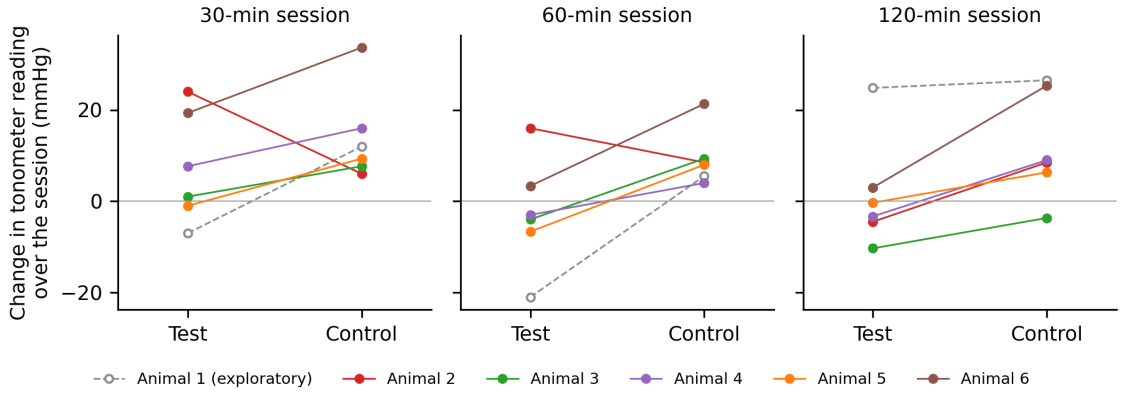

FIGURE 4.10. Change in the tonometer reading of the test and control eyes over each cooling session, from before placement of the devices to after their removal, for each animal; animal 1, exploratory, dashed grey line with open circles.

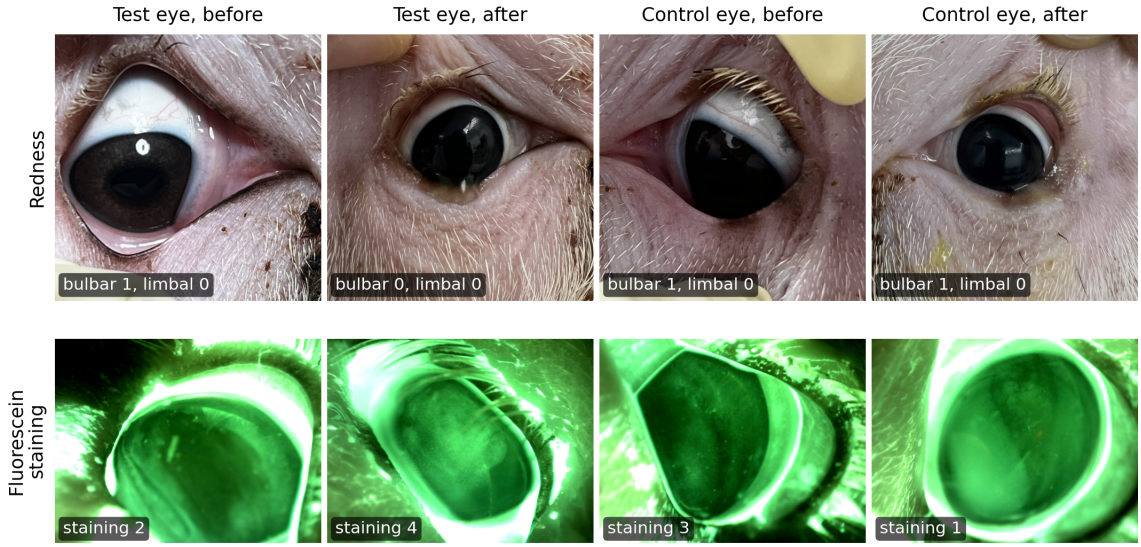

FIGURE 4.11. Representative photographs of the test and control eyes before and after a cooling session (animal 2, 120 min): conjunctival redness (top) and fluorescein staining (bottom), with the grades of record given to each photograph (0 to 4; bulbar and limbal redness, corneal staining).

4.3.5. Ocular Surface. Before the first session, before either eye had carried a device, bulbar redness was grade 1 or 2 in all six eyes of animals 2 to 4 (representative

TABLE 4.4. Change of each eye over the session, and the two changes compared: mean, with its 95% confidence interval in parentheses, and the percentage of the eye's own earlier value. ERG: b-wave and oscillatory potentials, change from the Jet recording before cooling to that during cooling, 25 cd·s/m^2; at about 9.5 °C four, three, and three sessions at 30, 60, and 120 min. Intraocular pressure: change in the tonometer reading from before placement of the devices to after their removal; the instrument is calibrated for the rodent eye, so these are relative values. p, paired t test of the test eye's change against the control eye's, given for the primary measures, the b-wave implicit time and amplitude; p_{Holm}, corrected across the three cooling durations within each measure.

Measure	**Min**	**Test eye**	**Control eye**	**Test against control**	p	p_{Holm}
Ring about 9.5 °C						
Implicit time (ms)	30	+5.4 (+1.3, +9.5); +16%	+0.8 (−0.3, +1.8); +2%	+4.6 (+0.1, +9.2); +14%	0.048	0.097
	60	+5.3 (−1.9, +12.6); +17%	+0.3 (−2.3, +2.9); +1%	+5.0 (−4.7, +14.7); +15%	0.157	0.157
	120	+4.8 (+2.2, +7.4); +15%	−1.3 (−5.3, +2.7); −4%	+6.2 (+4.3, +8.1); +19%	0.005	0.015
Amplitude (μV)	30	−48 (−134, +38); −13%	−10 (−84, +64); −1%	−38 (−79, +3); −12%	0.060	0.179
	60	−101 (−434, +232); −26%	+55 (−142, +252); +11%	−156 (−634, +321); −38%	0.294	0.589
	120	−104 (−404, +197); −22%	+24 (−112, +159); +6%	−127 (−557, +302); −28%	0.330	0.589
Width at 80% (ms)	30	+4.8 (−1.5, +11.2); +45%	+1.9 (−0.2, +3.9); +17%	+3.0 (−4.7, +10.6); +29%		
	60	+5.1 (+1.5, +8.8); +43%	+1.5 (−2.8, +5.7); +10%	+3.7 (−4.0, +11.3); +33%		
	120	+3.2 (−7.0, +13.5); +26%	+1.6 (−1.5, +4.6); +11%	+1.7 (−8.5, +11.9); +15%		
Oscillatory potentials (μV)	30	−43 (−196, +110); −17%	+45 (−182, +272); +8%	−88 (−234, +58); −25%		
	60	−137 (−448, +174); −42%	+11 (−149, +171); +2%	−148 (−485, +188); −45%		
	120	−199 (−411, +14); −59%	+11 (−416, +438); +10%	−210 (−450, +30); −69%		
Intraocular pressure (mmHg)	30	+7.9 (−10.1, +26.0)	+9.7 (+2.8, +16.7)	−1.8 (−23.0, +19.3)		
	60	+3.0 (−25.0, +31.0)	+7.3 (+0.1, +14.4)	−4.3 (−30.8, +22.3)		
	120	−6.1 (−15.4, +3.3)	+4.6 (−13.2, +22.4)	−10.7 (−19.3, −2.0)		
Ring about 4 °C, one or two sessions at each duration						
Implicit time (ms)	30	+7.5; +24%	+1.0; +3%	+6.5; +21%		
	60	+5.8 (+2.6, +8.9); +17%	+0.0 (−6.4, +6.4); −0%	+5.8 (+2.6, +8.9); +17%		
	120	+6.2 (−9.6, +22.1); +19%	+1.2 (−8.3, +10.8); +4%	+5.0 (−20.4, +30.4); +15%		
Amplitude (μV)	30	−85; −24%	+24; +7%	−108; −32%		
	60	−190 (−445,	+33 (−1151,	−133 (−973,		

photographs in Figure 4.11, all selected photographs in Appendix K), limbal redness was grade 0 or 1, and corneal staining was grade 3 in three of the six eyes, all three of them the eyes later cooled. In animal 1 redness was grade 0 or 1 and staining grade 1 in both eyes. Over a session, the grade of the test eye changed by at least one step more, in the direction of harm, than that of the control eye in 1 of the 9 sessions of animals 2 to 4 for bulbar redness, 2 for limbal redness, and 1 for corneal staining. These were a 30-min session for bulbar redness, a 30-min and a 60-min session for limbal redness, and a 120-min session for staining. In animal 1 it did so in one of its three sessions for bulbar redness, one for limbal redness, and two for staining. In most of these sessions the grade of the test eye did not rise: both eyes improved or stayed the same, the test eye by less. The redness of the test eye rose only in session 1 of animal 4, limbal, from 0 to 1. In the six sessions of animals 5 and 6 the grades of the test eye did not rise, except staining from 2 to 3 in session 1 of animal 5, where the control eye rose equally, and in session 2 of animal 6, where the control eye rose from 1 to 3.

The corneal staining of the test eye rose by two steps in each of the three sessions in which it was worse: from 2 to 4 in session 3 of animal 2 and from 1 to 3 in sessions 1 and 2 of animal 1. In animal 2 it was still grade 4 in the terminal session, against grade 2 in the control eye; in animal 1 it was back to grade 1 and 2 by the start of the next session.

An independent reader graded nine photographs of animal 2. All three staining photographs were graded 4, and on two of them the reader wrote that dye pooled on the surface could not be told from staining. On the six redness photographs, the reader's bulbar grades were 0 to 3 steps above the author's grades of that time and the limbal grades within one step of them.

4.3.6. Systemic Course and Adverse Events. No session was paused or ended for a physiological reason (Figure 4.12). Across all sessions of animals 2 to 6 oxygen saturation stayed at or above 93%, heart rate between 74 and 153 beats per

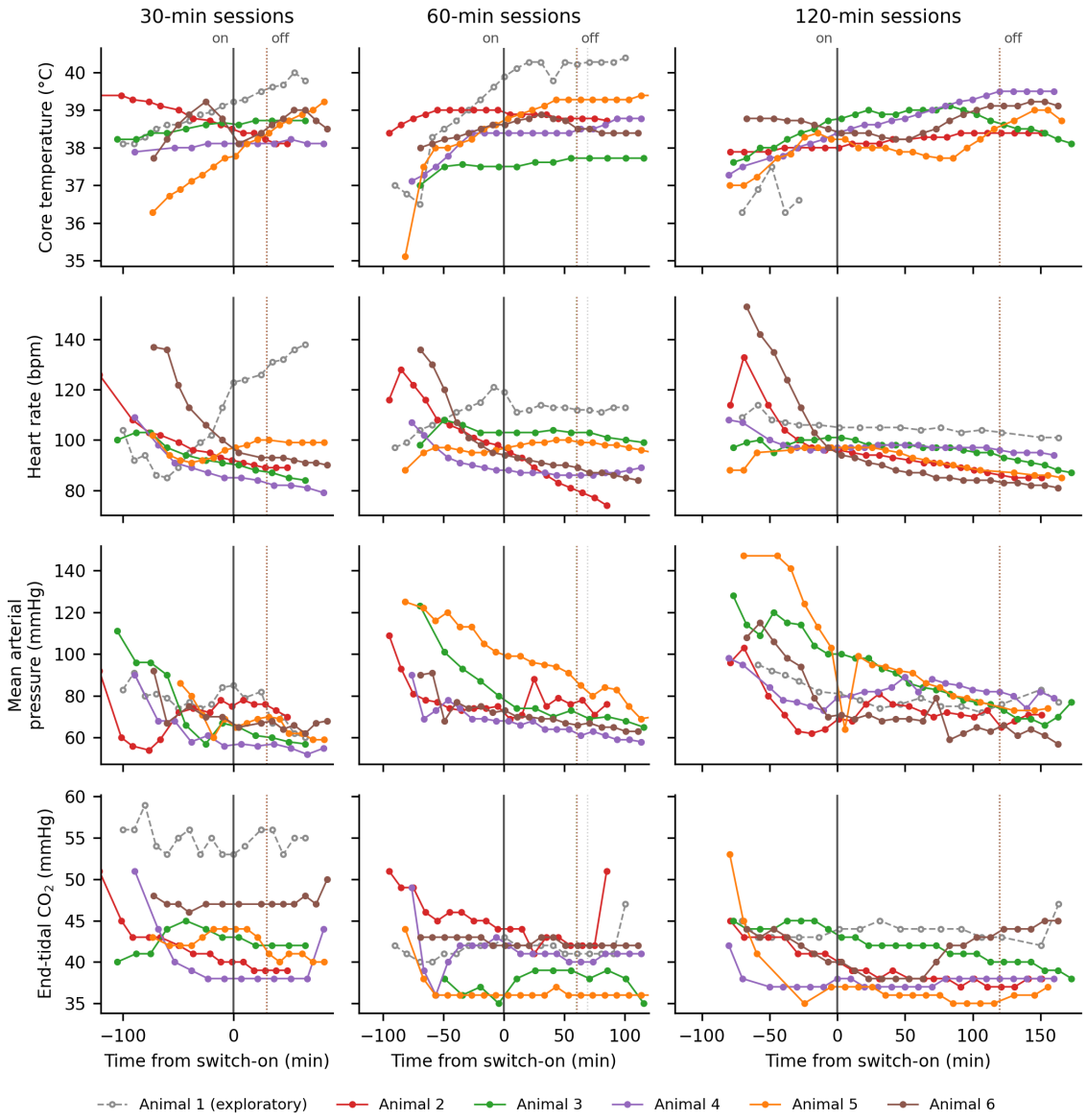

FIGURE 4.12. Core temperature, heart rate, mean arterial pressure, and end-tidal carbon dioxide during each cooling session, against time from switch-on, from the anesthesia record (one reading about every 10 min). Solid vertical line, switch-on; dotted lines, switch-off of each session. Animal 1, exploratory, dashed grey line with open circles. Mean arterial pressure was not recorded in session 2 of animal 1, nor core temperature after switch-on in its session 3.

minute, end-tidal carbon dioxide between 35 and 53 mmHg, and core temperature at or above 35.1 °C, its lowest in session 2 of animal 5. In animal 1 oxygen saturation was 100% throughout, heart rate 85 to 138 beats per minute, end-tidal carbon dioxide

35 to 59 mmHg, and core temperature 36.2 to 40.4 °C. All values were within the limits of Section 4.2.3.

On the day after session 1 of animal 2 the left eyelid, on the control side, was moderately swollen, closing the eye, and the animal scratched at it; the right eyelid was only very mildly swollen, and the right eye appeared normal. On the following day both eyes were equally open and the mild inflammation around them was resolving. After session 2 animal 3 had expiratory wheezing and tracheal irritation, with blood on the endotracheal tube, which may have been too large; it was treated with meloxicam (0.1 mg/kg by mouth) and was much improved the next day. After session 3 it arrested during recovery and was euthanized by the BRL staff, at about 14:30, with the same agent (Fatal-Plus, 10 mL intravenously). Necropsy found laryngeal necrosis and cardiac hypertrophy. The BRL veterinarian and veterinary pathologist attributed the necrosis most likely to a traumatic intubation a few days earlier, in session 2, and judged the hypertrophy incidental and not the cause of death; neither was attributed to the device. Both eyes were collected after death.

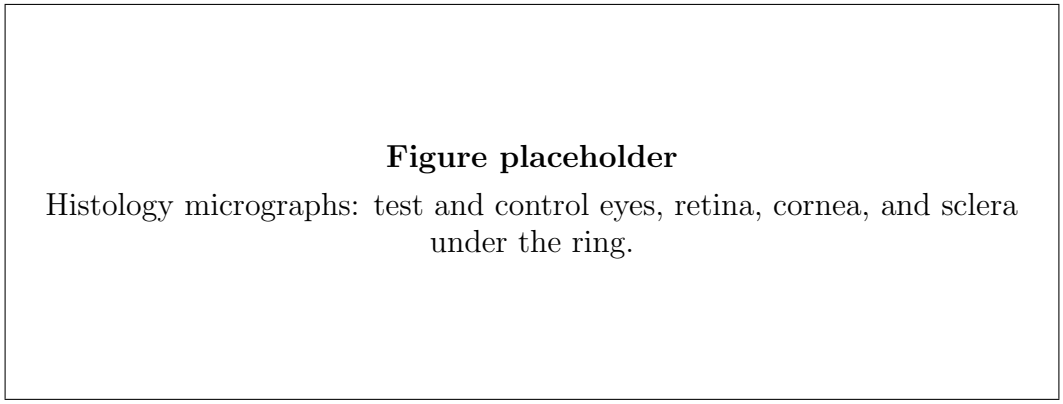

FIGURE 4.13. Representative sections of the retina, the cornea, and the sclera under the ring in the test and control eyes.

4.3.7. Histology. Both eyes of each animal were enucleated and fixed, those of animal 3 after its death, when the interval from death to fixation differed from that of the other animals. The sections of all animals are to be imaged together (Section 4.2.8, Figure 4.13) and had not been read at the time of writing. A first set

of scans of the sections of animal 1, fixed in formalin, showed the retina detached in both eyes, which led to the change of fixative from animal 3 onward. That set was not graded and is not used here.

4.4. Discussion

Apart from the relay interruptions, the device held the ring near its set point for up to 2 h against the heat brought by the living eye and the lids. The relay of the Gen 3 controller was the weak element, and the MOSFET module that replaced it ran without an interruption (Section 4.3.2). Only this Gen 3 device, fitted with the reduced-bulk aluminum ring, was used on an animal (Section 4.2.4); the rest of the Gen 4 design remains to be tested in vivo (Section 5.4).

During cooling the b-wave of the cooled eye became slower than before cooling in every session, and wider in all but one, while that of the control eye did not slow (Section 4.3.3, Table 4.4). A longer implicit time is the response expected of a cooler retina [**73, 74, 122, 76**]. The control eye, which carried an identical ring through the same anesthesia, core temperature, and time, shows that the change did not arise from these. The control eye also widened, by less, so part of the widening is shared; the larger widening of the cooled eye fits a slower and possibly uneven response of its retina. The slowing diminished after switch-off, while the ring was rewarming, which also points to temperature rather than to a lasting effect of the contact. The five sessions with the ring at about 4 °C slowed within the range of the others and are too few to show a relation to the ring temperature. Ischemia caused by the cold and a difference in the pressure applied by the two rings cannot be excluded as explanations other than a cooler retina.

The implicit time of the control eye, which did not slow, not the amplitude, is the better evidence that the slowing of the test eye arises from its temperature. The relation of the delay to the ring temperature (Figure 4.9) rests on the contrast between the cold recordings and the warm ones, and it does not show how the delay

varies within cooling. The b-wave amplitude varied widely between recordings, and no confidence interval of the fall of the summed oscillatory potentials excluded zero (Table 4.4). The slowing of the b-wave, about 5 ms, is several times the variability of its timing between sessions on this device, which is under 1 ms in healthy eyes under light adaptation [**146**]. The fall of 20% in the b-wave amplitude of the test eye, by contrast, is within the normal difference between the two eyes of a subject, whose 95th percentile is 27 to 36% [**143**], so the amplitude is weak evidence of cooling on its own. Anesthesia also lowers the amplitude more than it delays the b-wave. In dogs under isoflurane the b-wave amplitude was 54% lower than in the same dogs awake, whereas its implicit time did not differ significantly (37.6 against 33.8 ms) [**147**]. In anesthetized pigs the amplitude of the multifocal ERG fell over the 150 min after induction, in all its components under propofol and in the global-flash components under isoflurane [**148**]. A decline of the amplitude in either eye over a long session is therefore expected.

The hypothesis that the ERG returns to its baseline after rewarming was tested only in part, because the Jet recording after switch-off was made before the ring had recovered fully (Section 4.2.9). The b-wave of the test eye returned to its own baseline in all recovery sessions but one, session 3 of animal 3, in which the ERG of both eyes changed together, which points to a systemic cause.

The safety endpoints showed no harm attributable to the device within the measurements made. The pressure readings followed a consistent pattern. The reading of the control eye, which carried an identical ring that was not cooled, rose over almost every session, so the placement of the device, together with the anesthesia [**149, 150**] and fluids, raised it. The reading of the cooled eye rose less than that of the control eye, or fell, in 13 of the 15 sessions of animals 2 to 6 and in all three of animal 1 (Section 4.3.4), so the cooling reduced or reversed the rise that the device produced on its own. Intraocular pressure under anesthesia moves by several mmHg with induction,

intubation, and the agents given, and in anesthetized rats it falls with body temperature when no heat support is given [**151**]. The readings before and after a session were taken at different stages of the anesthetic, but both eyes shared the anesthetic and the body temperature, so these effects cancel in the difference between the eyes.

That local cooling lowers the pressure is established in humans; its mechanism is not. A stream of cold air on one eye of 11 subjects lowered the pressure from 14.1 to 12.6 mmHg and the episcleral venous pressure from 8.4 to 6.2 mmHg, with no change in aqueous flow or outflow facility, so the fall was attributed to the venous pressure [**152**]. In a randomized trial in 108 healthy adults, a mask at about 5 °C worn for 10 min lowered the pressure by 2.0 mmHg, a mask at room temperature left it unchanged, and a heated mask raised it by 2.1 mmHg [**153**]. The authors attributed the fall to less aqueous production, through vasoconstriction and slower metabolism of the ciliary body, but did not measure it. A coil on the rabbit sclera left the pressure unchanged [**74**]. No study has tested whether cooling offsets a rise produced by a device on the eye. The episcleral veins offer one route by which it could: the rise under a scleral lens is attributed to their compression [**22**], the ring rests on the same region of the sclera, and cold lowers their pressure [**152**]. Neither the episcleral venous pressure nor the aqueous flow was measured here, so this explanation is a hypothesis. Cooling also thickens the cornea, which alters applanation readings [**154**] and may alter a rebound reading, and corneal thickness was not measured, so the present readings cannot separate these effects. Under a scleral lens, which also rests over the sclera, the pressure rises within minutes and is back at baseline 2 to 3 min after removal [**155**], and studies that measured only after removal find no change [**156**].

Redness and staining were worse in the cooled eye in only a few sessions, of every duration (Section 4.3.5), and neither points to harm from the cooling.

Bulbar redness fell over a session in both eyes. In the nine sessions of animals 2 to 4 the bulbar redness of the cooled eye changed less in the direction of harm than

that of the control eye in three, equally in five, and more in one; limbal redness showed no pattern (two, five, and two). Cold is known to reduce conjunctival redness: in a masked trial, a cold compress applied for 5 min reduced bulbar hyperemia faster than no treatment [**157**], and cold air on the eye lowered the pressure of the episcleral veins, which drain the same vascular bed [**152**]. A lower bulbar redness in the cooled eye is therefore plausible, but these counts are too few, and the grades too coarse, to show it. It is a question for the masked grading (below).

Corneal staining rose over a session in the control eye as often as in the cooled eye, in five and three of the nine sessions of animals 2 to 4. Most of it therefore arises from what the two eyes shared: the ring, the corneal ERG electrode, the topical anesthetic, tonometry, and hours of exposure under anesthesia. Corneal ERG electrodes alone cause transient epithelial changes, seen with fluorescein in 62% of normal subjects and associated with the topical anesthetic [**158**]. Because both eyes carried all of these, the data cannot say which of them caused the staining, only that the cooling added nothing that could be measured. The rises mostly resolved by the next session. The exception, grade 4 that persisted to the terminal session in the cooled eye of animal 2, is the observation the histology of the cornea has to address. The staining grades are weak evidence in either direction: three of the six eyes of animals 2 to 4 were graded 3 before any device had touched them, and the independent reader could not separate pooled dye from staining.

The one death in the study followed a complication of intubation, and the eyelid swelling of animal 2 was on the control side.

Three practical problems affected the earlier sessions. The device did not always fit well under the eyelid, until the lid was drawn over it with tweezers. The ring thermistor sometimes came out of the device, which gave poor readings or no recording, until its insertion point was fixed with silicone. The channel was first calibrated with a second thermistor in the well that did not touch the walls, which made the ring read about 5.5 °C low until a custom piece put the thermistor in contact with the

aluminum (Section 4.2.6). With that correction, most of the exposure was at about 9.5 °C, not at the 4 °C intended.

The in-vivo study is based on the sessions of the six planned animals completed so far, and on three animals at 120 min with the ring at about 9.5 °C for the ERG (animals 2 to 4: animal 1 is exploratory, and animals 5 and 6 reached that session with the ring at about 4 °C). With the measured variability the paired comparison has a power of about 90% for the implicit time with four animals, but about 43% for a 30% change in amplitude and 48% for the width (Table 4.3). The fixed order of the three durations confounds a trend with duration with repeated anesthesia and cumulative exposure. The test eye was always the right eye, fitted and recorded first, so the control eye was slightly longer dark-adapted. The same order applied to the pre-cooling recordings, and in the porcine ERG the b-wave implicit times at strong flashes do not differ significantly between 15 and 25 min of dark adaptation [**159**]. The recovery recording was made before the eye had rewarmed, so whether the difference between the eyes closes completely was not measured. The tonometer was not validated for the porcine eye and gives relative values only, and it was not used with the devices on the eyes, the period in which a device resting on the sclera can raise the pressure [**155, 22**]. Redness and staining were graded by the author, who knew which eye had been cooled, on one photograph per eye and time point that the author also chose. The grades are whole steps of a 0 to 4 scale, so most sessions tie and one step is the smallest change that can be seen, and the independent reader's bulbar grades were up to three steps above the author's. Staining was confounded by dye pooled on the surface, which cannot be told from staining on a photograph. From the second session of animal 4 onward, the laptop screen used to preview the photographs was reflected in the cornea of the test eye in the staining photographs, as a pale square that can read as staining; it biases the grades against the cooled eye. A masked grading of all the photographs by three graders, with the images coded, partly mirrored, cleared of these marks, and in part repeated, is under way. The histology

has not been read, and it will be read by the author without masking. It covers the cornea, sclera, conjunctiva, peripheral retina, and optic nerve head, not the ciliary body, pars plana, limbus, lens, or posterior retina, and there was no slit-lamp, fundus, or corneal endothelial examination. The core temperature of the anesthetized animals should be read against the normal porcine range of 38.5 to 39.5 °C. No temperature was measured inside the eye, and the relation between b-wave timing and retinal temperature has not been established in this preparation, so the size of the delay cannot be converted into a retinal temperature.

CHAPTER 5

General Discussion and Conclusions

5.1. Synthesis Across Aims

The three aims approach one question from three sides: how much of the cooling applied to the anterior sclera reaches the back of the eye. Aim 1 measured it directly, in eyes without a circulation. Aim 2 compared ring designs in the same preparation and used a computational model to estimate what a circulation does to it. Aim 3 looked for its functional trace, a slowing of the electroretinogram (ERG), in the living eye, where no temperature could be measured.

In the isolated eye a ring held at 4 °C produced a steep gradient from front to back (Figure 5.1A). The anterior sclera cooled the most and the proximal optic nerve the least, by a few degrees, and at that site the two species cooled by nearly the same amount, with no difference detected (Table 2.7).

The refinements of Aim 2 left this deep cooling largely unchanged (Chapter 3). In the same donor eyes, the reduced-bulk ring, with about half the volume, and the ceramic ring both gave the proximal optic nerve about the same cooling as the original ring. The reduced-bulk ring cooled it slightly less, by less than the spread between eyes. The other elements of the Gen 4 design were examined only in the model or on the bench.

The isolated eye has no blood flow, and the choroid of the living eye carries heat to the tissue the device is trying to cool. The model of the perfused eye therefore gives less cooling than the bath experiments at the sites away from the ring (Figure 5.1B). This gap is not a measurement of the effect of perfusion: the model has not been compared with a temperature measured in a perfused eye, and most of the gap reflects the difference between the two conditions (Section 3.3.6). Across the model runs, body

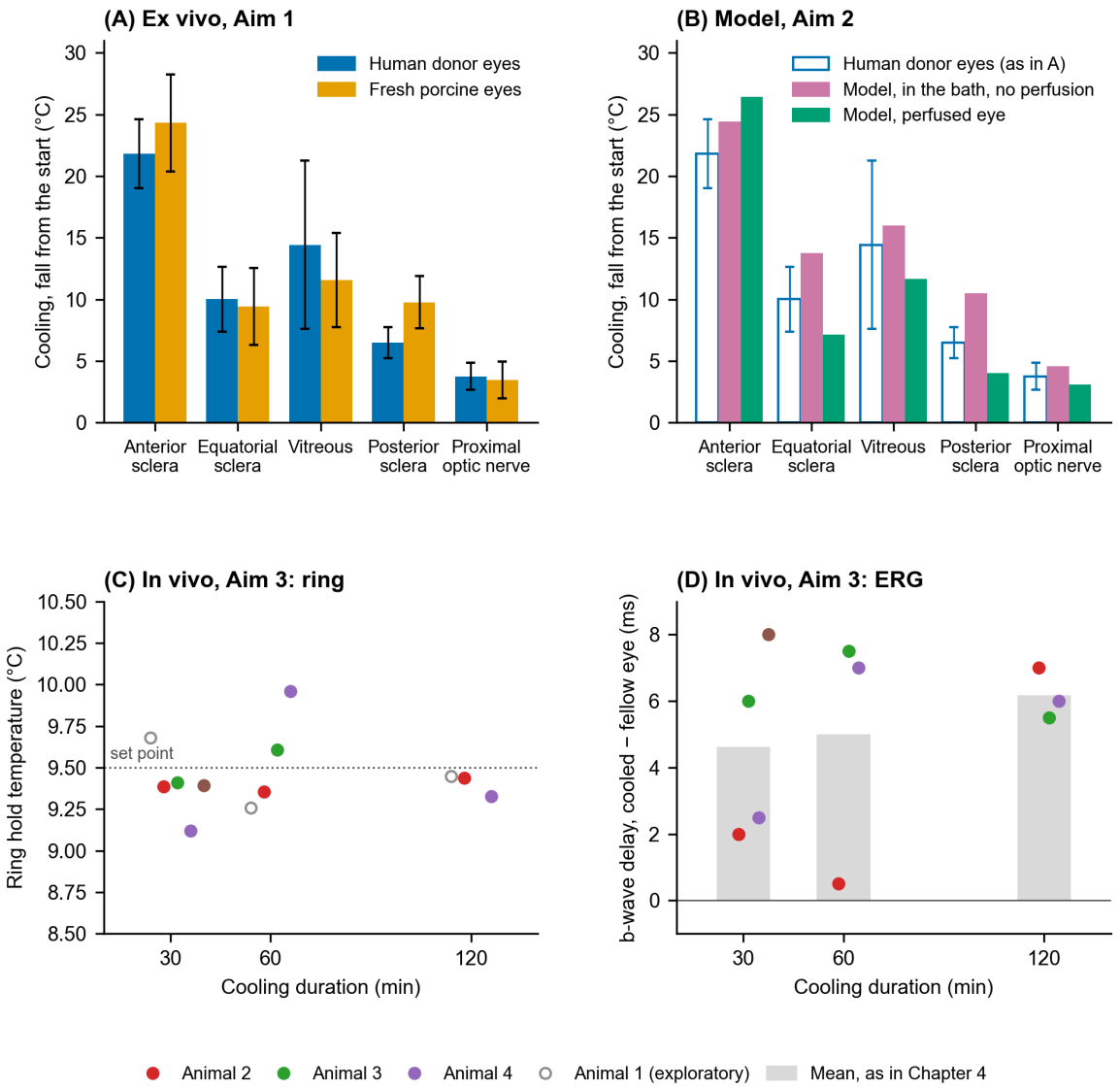

FIGURE 5.1. Cooling at the sites shared by the three aims. (A) Fall in temperature from the start of cooling, over the 20-min record, in human donor eyes and fresh porcine eyes with the original ring (mean and SD; the values of Tables 2.3 and 2.4). (B) The same fall in the human donor eyes (open bars, as in A), in the model of the enucleated eye in the oil bath without perfusion, with an oil coefficient chosen before the comparison, and in the model of the perfused eye in the orbit (Section 3.3.6). The perfused model is read after 35 min and the measurements over minutes 16 to 20 (Section 3.2.3). (C) Mean ring temperature held in each in-vivo session, by cooling duration. (D) Delay of the b-wave implicit time of the cooled eye relative to the fellow eye during cooling, at the primary flash; the bars are the means with the ring at about 9.5 °C, corrected (Section 4.3.3). Animal 1, exploratory, open grey circles.

and ring temperature set the temperature at the back of the eye far more than the design of the ring did.

In vivo, the cooling applied at the anterior sclera reached the retina closely enough to change its function, but the ERG does not show by how much the retina cooled. No temperature was measured in vivo, so Chapter 4 addresses the question only through the ERG, whose timing slows as the retina cools [**73, 74, 76**]. The ring was held close to its set point for up to 2 h, and during cooling the b-wave of the cooled eye slowed from its own baseline in every session, and that of the fellow eye did not (Figure 5.1C and D). The relation of the ERG to retinal temperature has not been established in the pig under these conditions, so the delay cannot yet be set against the cooling that the model predicts. No adverse event was attributed to the device, and the histology has not yet been read.

The isolated eyes fix the size and the pattern of the cooling that the ring produces when no circulation opposes it, and show that it is much the same in human and porcine eyes and with the reduced and ceramic rings. The model places that cooling against a circulation and identifies body temperature and ring temperature as what sets the temperature reached at depth. The in-vivo study shows that the same ring, at the same temperature, on a perfused eye, changes retinal function in the direction expected of cooling, in sessions of up to 2 h that the eye and the animal tolerated as far as was measured. What links the three, a temperature measured at the back of a perfused eye, is still missing.

5.2. Comparison with Prior Work

The approaches reviewed in Section 1.4 fall into two groups. The first cools the inside of the eye directly and reaches the retina or the nerve, but each method requires surgical access [**73, 79, 80**]. The second acts at the surface without surgery [**85, 87, 89**], and for none of these devices has a temperature at the back of the eye been reported. Research preparations in living animals had shown that cold applied

outside the eye lowers posterior temperatures [**73, 74, 64**], and the one study with feedback control delivered its fluid into the anterior chamber [**78**].

The SCEC belongs to the second group by its access and aims at the target of the first. It is placed without surgery, it holds the sclera at a set temperature for tens of minutes to hours instead of seconds, and its effect has been measured at located sites inside the eye, up to the optic nerve. The fresh-porcine part of this work, published as [**1**], established that effect in porcine eyes. This dissertation extends it to human donor eyes and to two further ring designs, and applies the device to the living animal. To the author's knowledge it is the first scleral-contact cooler whose temperature field has been measured in human donor eyes at sites verified by micro-computed tomography. It also differs from the other contact coolers that come closest. The ETX-4143 scleral devices cool for 4 to 10 min from a reservoir frozen before use and are aimed at ocular surface pain [**91, 90**]. Lachapelle's coil cooled the retina but had to be glued to the sclera with the eyelids removed [**74**]. Kataoka's circulating-water contact lens protected the cornea and was not tested at the back of the eye [**84**].

The SCEC trades depth of cooling for the absence of surgery, and the comparison shows this limit of the approach. Direct methods bring cold fluid to the retina or the nerve itself, whereas a ring on the anterior sclera cooled the proximal optic nerve of an isolated eye by a few degrees. In a Pennes model a circulation reduces this further, whereas a multiphase blood model predicts deeper cooling [**54**].

5.3. Clinical Translation

The two clinical settings that motivate the device (Section 1.1) ask different things of it: speed of application in retinal artery occlusion, and tolerance of long exposures in perioperative visual loss. In retinal artery occlusion the injury has already begun when the patient is seen, usually hours after onset [**2, 14, 15, 16**], so cooling could only follow the insult. In perioperative visual loss, of which supine cardiac surgery is

the natural target because direct pressure on the eye is to be avoided in prone surgery [**21**], the period of risk is known in advance, and cooling could begin before it and continue through it. The model places the reach of the device at the retina and the optic nerve head, not at the nerve 1 cm behind the globe (Section 3.4), whereas most ischemic optic neuropathy after spine surgery is posterior [**19**].

The effect is best described as a temperature reduction of a few degrees at the back of the eye, not as the attainment of a target temperature. What the device delivers has to be set against what the laboratory evidence suggests is needed. The measured effect at the proximal optic nerve of the isolated eye was a fall of 3 to 4 °C. Protection of the retina has been reported for temperature reductions of this size (Section 1.3), including the 3 °C step used here as a benchmark [**62**]. Whether a perfused human eye cools by as much is not known. The model suggests somewhat less. With fever the reduction is about the same, but the temperature reached is higher, because it follows body temperature.

Several steps separate the present device from clinical use. The temperature reached at the posterior pole of a perfused eye has to be measured. The exposures tested in vivo, up to 120 min, are shorter than the operations in which perioperative visual loss occurs (a mean anesthetic duration of 9.8 h in the registry cases after spine surgery [**19**]). The tolerance of the anterior sclera, cornea, and lens to longer contact at low temperature is not established. The intraocular pressure with the device on the eye has not been measured. No limit for the cold side has been based on tissue tolerance. The board stops below 0 °C only to prevent freezing, and the controller used in vivo has no cold-side limit at all. The limit of 43 °C used for the warm surfaces is taken from the standard for medical electrical equipment [**129**] and was examined in the model only. Biocompatibility of the materials in contact with the eye, sterility, electrical safety, and the software of the controller have not been addressed.

The in-vivo safety evidence covers three exposures of 30, 60, and 120 min with the ring near 9.5 °C, and near 4 °C in the latest sessions, given over about ten days

to an anesthetized pig with an uninjured eye. Within that envelope, in the animals studied, the cooled eye showed no larger rise in intraocular pressure than its fellow once the device was removed. Redness and corneal staining were rated worse in the cooled eye in only a few sessions, without a relation to duration. No adverse event was attributed to the device, and the b-wave timing was restored after switch-off in twelve of the thirteen recovery sessions (Section 4.4). The histology, which is the test of injury to the cornea, sclera, and retina, has not yet been read. The evidence does not extend to longer exposures, to repeated daily use, to awake patients, or to eyes that are already ischemic.

5.4. Limitations

Each chapter gives the limitations of its own work (Sections 2.4, 3.4, and 4.4). Four cross the aims.

No temperature was measured in a perfused eye. The bath is not a model of perfusion, the model has not been validated against such a measurement and depends on perfusion rates taken from the literature, and the ERG is indirect evidence of retinal cooling, which pressure on the globe or cold-induced ischemia can also mimic.

The groups were small: nine human eyes from five donors, five fresh and five preserved porcine eyes, and four animals. The porcine experiments of 2022 were run with the current adjusted by hand and the human experiments under the comparator controller. Conclusions from the eyes rest on excursions, which are better determined than absolute temperatures (Appendix D).

Several analysis settings were set or revised after the data had been seen: the benchmark of 3 °C, the vitreous position limit of 75%, the 3.3-mm spatial screen, the 4-min final window, and the visual exclusions.

SCEC Gen 4 was not assembled as a whole. The in-vivo device was Gen 3 with the reduced-bulk ring (Section 4.2.4). The ceramic ring was tested ex vivo and in the model, the insulating layer, the corneal lens of v3, and the smaller heat exchangers

only in the model, and the integrated board only on the bench, where the fault of its reference voltage remains open.

5.5. Future Work

The first need is a direct measurement of temperature at the back of a perfused eye during cooling, for example with a single fine probe in a terminal experiment. It would test the model, fix the factor by which a circulation reduces the deep cooling, and turn the ERG from the only evidence of retinal cooling into a measure that can be calibrated against temperature.

The model should also be run over the plausible range of choroidal blood flow and then compared with that measurement. The control board needs a revision in which the reference voltage cannot disturb the current limits, a sensor on the warm side, a cold-side limit based on tissue tolerance, and a test of each safety cutout on the hardware. The thermoelectric module can also be developed further, with a higher density of thermoelectric posts and a module printed directly on the ring. The question that motivates the device, whether this degree of cooling protects the retina or the optic nerve, requires a study in an injury model, with cooling begun at a clinically realistic time relative to the insult.

The computational work would be extended in two steps. A complete model of the set-up would couple the eye and orbit to the ring, the thermoelectric module with its Peltier, Joule, and conduction terms, the heat exchanger, and the coolant loop under the control law. It would thus predict tissue temperatures from the drive current instead of from an imposed ring temperature. An optimization on that model would then replace the comparison of a few chosen designs. It would range over the geometry and material of the ring, the module, and the heat exchanger, constrained by the 43 °C limit for warm surfaces, the power of a portable supply, and the size of the device. Both should be run with more than one representation of perfusion,

because Pennes and multiphase models predict opposite effects of the circulation on posterior cooling [**54**].

Several additions to the device itself were conceived in this work and remain to be built or evaluated (Table 3.1). A contact-pressure sensor in the ring would report the force applied to the eye, which bears on the intraocular pressure during treatment. Under scleral lenses the rise in pressure is attributed to compression of the episcleral veins, whose pressure is about 7 to 11 mmHg, and a lens exerts its full compression only after more than 4 h of wear [**22**]. The intraocular pressure would also be measured during application and over exposures of several hours, as it has been during scleral-lens wear [**155**]. The present ring leaves no access for a rebound tonometer. An electrode in the ring would allow the electroretinogram to be recorded through the device, without a separate corneal electrode. Other tools developed in the same laboratory could turn the ERG into a spatial measure of cooling. The contact lens electrode array (CLEAr) records the ERG at many sites on the cornea at once, and the differences between sites reflect where on the retina the response arises [**160, 161, 162**]. The hemispherical stimulus domes of the laboratory stimulate selected regions of the retina. PeriStim presents patterns to the mid and far peripheral retina [**163, 164**], and its LED successor, HoneyDome, delivers full-field and partial-field flashes [**165, 166, 167**]. Used together during cooling, they could show where the retina cools and how far the cooling extends.

Thermistors built into both ceramic plates of the thermoelectric module would measure its cold and its warm side directly, which the 43 °C limit for the warm surfaces and a future cold-side limit based on tissue tolerance both require. A ring that detaches from the module could be replaced or sterilized between patients, and an injection port through the ring would give access to the eye without removing the device. Finally, the silicone potting between the plates of the module, present in every device used here, has not been evaluated on its own for electrical isolation or for protection against moisture.

The reverse polarity used for experimental equilibration could also give the device a therapeutic heating mode, which would need its own validation of temperature control, safety, and efficacy. Existing ocular heating applications, including cyclophotocoagulation, act through different mechanisms [**168**] and do not validate moderate heating by this device.

Thermal analgesia is another possible application. Cooling affects corneal sensory-nerve activity [**169**]. Whether the SCEC relieves pain would have to be tested with a pain endpoint and an appropriate exposure protocol.

Other uses of controlled ocular temperature could follow from the same platform, none of them tested here. Cooling could protect adjacent tissue during laser procedures, as a cooled contact lens did in the rabbit [**84**], or support hypothermic vitreoretinal surgery. Each would need its own temperature target and its own evidence.

Part of this work has been published, and the rest is to be submitted as a series of papers. The fresh-porcine experiments of Aim 1 were published with Y. Nakamura [**1**]. Two manuscripts on the ex-vivo work are in preparation: a methods paper on the ex-vivo system, and the comparison of human donor and porcine eyes. The computational model and the ring designs of Aim 2 will be the subject of a computational manuscript. The in-vivo safety and ERG study of Aim 3 will be submitted for publication once all six animals have been studied, and is also planned for presentation at the ARVO Annual Meeting.

5.6. Conclusions

A thermoelectric ring held at 4 °C on the anterior sclera cooled the proximal optic nerve of isolated human and porcine eyes by a few degrees within the record. The first hypothesis held for the occurrence and timing of this cooling but not for its size. Every eye cooled at that site and approached its final temperature within 15 min, but not every eye reached the 3 °C benchmark. No difference was detected at that

site between the species, or between preserved and fresh porcine eyes. The second hypothesis was supported at the proximal optic nerve, where the reduced-bulk ring and the ceramic ring each cooled it about as much as the original ring. A model of the perfused eye indicates that blood flow reduces the deep cooling, by a factor that remains to be measured, and that body temperature, more than the design of the ring, sets the temperature reached.

In anesthetized pigs the device, fitted with the reduced-bulk ring, held the ring near its set point for up to 2 h. The b-wave of the cooled eye slowed and widened from its own baseline while that of the fellow eye did not slow, a change consistent with retinal cooling, and its summed oscillatory potentials fell. The first part of the third hypothesis was supported, as a longer implicit time and a smaller b-wave amplitude were found together in 11 of the 15 sessions at the primary flash. After switch-off the b-wave timing returned to its own baseline in all but one recovery session, in which both eyes changed together, and the oscillatory potentials in 11 of 13. Since these recordings were made before rewarming was complete, the second part was tested only in part. In the animals studied the intraocular pressure after removal of the device, the ocular surface, and the systemic course showed no harm attributable to the device. The histology is still to be read.

The contributions of this dissertation are the following.

(1) The temperature field produced by a scleral-contact cooler was measured in human donor eyes, at sites inside the eye located by micro-computed tomography.

(2) Human donor and fresh porcine eyes were compared with the same ring, and they cooled in the same spatial pattern and by a similar amount at the proximal optic nerve.

(3) A computational model showed that an alumina ring cools almost as well as an aluminum one, that body and ring temperature matter more at depth than the design of the ring, and that blood flow reduces the deep cooling.

(4) An integrated control board for the device was designed and tested on the bench.

(5) The device was applied to living eyes for the first time, in anesthetized pigs, where it held the ring near its set point, slowed the ERG of the cooled eye, and caused no harm attributable to it in the measures read so far.

(6) The analysis of the ex-vivo experiments and of the model is documented as code that regenerates every table and figure from the raw data (Appendix E).

Before clinical use, the temperature at the back of a perfused eye must be measured, longer exposures must be shown to be tolerated, and protection of the retina or the optic nerve must be tested in a model of ischemic injury.

Appendices

APPENDIX A

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Figure 2.1C	Nakamura et al. 2026, *Ann. Biomed. Eng.*, Fig. 2A [**1**]	CC BY 4.0 (open access)	Not required; attributed in the caption

APPENDIX B

Tissue Properties, Boundary Conditions, and Meshes of the Computational Model

The eye measures 24.8 mm from the corneal apex to the choroid at the posterior pole and 25.0 mm across at the outer surface of the sclera. The cornea is 11.5 mm across and 0.55 mm thick at its center, the anterior chamber 3.0 mm deep, the lens 9.0 mm across and 4.5 mm thick, and the sclera 0.42 mm thick on average.

The thermal properties of the eye model (Table 3.3) were compiled from published eye models, tissue databases, and physiological measurements. Published values differ widely and most come from animals [**52**], so each property was estimated from the composition of the tissue, averaged over published values, or taken from the IT'IS database [**170**]; the tear film and the ring materials came from the COMSOL library. Regional variation was not represented. The mesh and boundary figures, like the schematics of Appendix H, are at full resolution in the electronic copy.

B.1. Estimates from Tissue Composition

Most ocular tissues were treated as water containing a dispersed solid phase. Their conductivity followed the Hashin–Shtrikman bound with water as the continuous phase [**171**],

$$k = k_w + \frac{1-\phi_w}{\dfrac{1}{k_s - k_w} + \dfrac{\phi_w}{3k_w}}, \tag{B.6}$$

where k is the thermal conductivity of the tissue, ϕ_w its water fraction, $k_w = 0.6\ \mathrm{W\,m^{-1}\,K^{-1}}$ the conductivity of water, and k_s that of the solids, k_s 0.41 (cornea), 0.33 (vitreous), 0.23 (ciliary body), 0.20 (retina), 0.22 (choroid), and

0.43 $\mathrm{W\,m^{-1}\,K^{-1}}$ (sclera). The specific heat and density followed from the same fractions, $c_p = [\phi_w + 0.4\,(1-\phi_w)] \times 4184\ \mathrm{J\,kg^{-1}\,K^{-1}}$ and $\rho = [\phi_w + (1-\phi_w)\,\rho_s] \times 1000\ \mathrm{kg\,m^{-3}}$, where ρ_s is the density of the solids, 1.37, 1.48, 1.35, 1.16, 1.37, and 1.34 $\mathrm{g\,cm^{-3}}$ in the same order (1.34 for the lens), 4184 $\mathrm{J\,kg^{-1}\,K^{-1}}$ the specific heat of water, 0.4 that of the solids relative to water, and 1000 $\mathrm{kg\,m^{-3}}$ the density of water.

The solid-phase values were checked against published compositions [**172, 28, 173, 174, 175, 176, 177, 178, 179, 180, 170**] and component properties [**181, 182, 183, 184**]. These reproduce the conductivities used within 0.01 $\mathrm{W\,m^{-1}\,K^{-1}}$, except for the cornea and the sclera, 0.51 and 0.47 against 0.56 and 0.545 used; the values used lie closer to the measured means [**170**].

B.2. Perfusion and Metabolic Heat

The perfusion rate of each tissue followed from its measured blood flow and the volume of its domain, $\omega = Q_{\mathrm{blood}}/(60\,V)$, where ω is the perfusion rate in $\mathrm{s^{-1}}$, Q_{blood} the blood flow in $\mu\mathrm{L\,min^{-1}}$, V the volume of the domain in $\mathrm{mm^3}$, and 60 converts minutes to seconds. The choroid took the mean whole-eye flow measured with microspheres in monkeys, 917.5 $\mu\mathrm{L\,min^{-1}}$ [**40**], over 165 $\mathrm{mm^3}$ (0.0927 $\mathrm{s^{-1}}$); indirect estimates for the human eye are of the same order [**185, 41, 111**]. The retina took the mean of six laser Doppler studies in humans, 54.5 $\mu\mathrm{L\,min^{-1}}$ [**29, 30, 31, 33, 32, 186**], over 236 $\mathrm{mm^3}$ (0.0039 $\mathrm{s^{-1}}$). The extraocular muscles, eyelids, and orbital fat followed from published flows per unit mass [**187, 188**], converted with the density of the tissue (for the eyelids no measured value was found and the rate is an assumption), and the ciliary body received 133 $\mu\mathrm{L\,min^{-1}}$ [**189**] over 348.25 $\mathrm{mm^3}$.

The metabolic heat was the specific metabolic rate of each tissue [**190**], converted to a volumetric source, $Q_{\mathrm{met}} = \dot{m}\,\rho \times 4184/86\,400$, where Q_{met} is the metabolic heat in $\mathrm{W\,m^{-3}}$, $\dot{m}$ the metabolic rate in $\mathrm{kcal\,kg^{-1}\,day^{-1}}$, ρ the density in $\mathrm{kg\,m^{-3}}$, 4184 the number of joules in a kilocalorie, and 86 400 the number of seconds in a day. The rate of the retina was derived from its oxygen consumption [**191**], which gives

3443 $\mathrm{W\,m^{-3}}$. The blood properties, $\rho_b = 1068\ \mathrm{kg\,m^{-3}}$ and $c_b = 3682\ \mathrm{J\,kg^{-1}\,K^{-1}}$, were composition estimates close to the IT'IS values [**170**].

B.3. Values Used

Table B.1 lists the values used, how they were obtained, and the published values they were compared with. The exposed ocular surface lost 12 $\mathrm{W\,m^{-2}}$ by tear evaporation [**192, 193**], and the tear film had the properties of water. The rings of aluminum, aluminum oxide (alpha, polycrystalline), and fused quartz had emissivities of 0.05, 0.95, and 0.90 (the COMSOL models used 0.05 for every ring).

B.4. Fluid Domains and Boundary Conditions

This section describes the COMSOL models, which the replication followed. The corrected runs differ as stated in Section 3.2.2.1.

The aqueous humor, the vitreous humor, and the air were laminar, non-isothermal fluids under gravity. The vitreous had a viscosity of 0.7 Pa s, aqueous humor entered and left at $1.23 \times 10^{-6}\ \mathrm{m\,s^{-1}}$, and the intraocular pressure was 2000 Pa (15 mmHg). The optic nerve had an outer sheath with the properties of the sclera and a core with those of the retina. The outer shell of the orbit was held at the body temperature and that of the air domain at the ambient temperature; where the two meet on the eyelid, COMSOL and the Python program impose their mean on 227 nodes. The cornea (v1, v2) lost heat by tear evaporation, and surfaces exposed to air radiated to the surroundings with emissivities of 0.98 for tissue and 0.05 for aluminum. The ring was in perfect contact with the eye and the eyelids; a 5-μm film in the Python program raised the anterior sclera by at most 0.14 °C.

The boundary conditions of the corrected runs are drawn on the model in Figure B.1.

B.5. Boundary Conditions and Meshes of the Heat Exchanger

Figure B.2 shows the boundary conditions of each heat exchanger (Section 3.2.2.3).

TABLE B.1. Thermal properties of the eye model, their basis, and the published values they were compared with (k in $\mathrm{W\,m^{-1}\,K^{-1}}$, ρ in $\mathrm{kg\,m^{-3}}$, c_p in $\mathrm{J\,kg^{-1}\,K^{-1}}$, ω in $\mathrm{s^{-1}}$).

Domain	**Used (k; ρ; c_p; ω)**	**Basis**	**Published values compared**
Cornea	0.56; 1070; 3707; –	Composition estimate (81% water [**170**])	k 0.35–0.58 [**194, 52, 170, 195**]
Aqueous humor	0.593; 1005; 4159; –	k the mean of four published values [**194, 196, 197, 198**]; ρ from [**197**]; c_p composition estimate	k 0.58 [**194, 170**]
Lens	0.43; 1126; 3255; –	k from its water content [**198**]; ρ, c_p composition estimate (63% water, the nuclear value [**176**])	k 0.40–0.43 [**199, 170**]
Vitreous humor	0.59; 1010; 4134; –	Composition estimate (98% water); viscosity 0.7 Pa s, within the 0.2–1 Pa s range of the liquid phase [**177**]	k 0.58–0.60 [**52, 199, 194, 170**]
Ciliary body	0.49; 1088; 3556; 0.0064	Composition estimate; perfusion from ciliary blood flow [**189**]	k 0.49–0.50 [**170**]
Retina	0.55; 1014; 3958; 0.0039	Composition estimate (91% water [**178**]); perfusion from retinal blood flow [**29, 30, 31, 33, 32, 186**]	k 0.55–0.57 [**200, 170**]
Choroid	0.51; 1074; 3682; 0.0927	Composition estimate (80% water; the IT'IS database, which gives the choroid the properties of blood, lists 80.4% [**170**]); perfusion from choroidal blood flow [**40**]	k 0.52–0.53 [**200, 170**]
Sclera	0.545; 1102; 3431; –	k, ρ, c_p composition estimate (70% water [**172**])	k 0.58–1.0 [**201, 200, 194, 170**]; the value of 1.0 traces to a single measurement and exceeds the conductivity of water
Extraocular muscles	0.49; 1090; 3421; 0.0045	IT'IS database (muscle) [**170**]; perfusion from extraocular muscle blood flow in primates [**187**]	k 0.57 [**53**]
Eyelids	0.34; 1070; 3660; 0.0011	Eyelid dermis [**202**]; perfusion from skin blood flow	k 0.37 (skin) [**170**]
Orbital fat	0.2245; 920; 2500; 0.0004	k the mean of published values; ρ, c_p [**200**]; perfusion from adipose blood flow [**188**]	k 0.20–0.25 (subcutaneous) [**197, 200**]; IT'IS 0.21 [**170**]
Tear film	water	COMSOL library (water); evaporation 12 $\mathrm{W\,m^{-2}}$	12.7–20.5 $\mathrm{g\,m^{-2}\,h^{-1}}$ [**192, 193**]

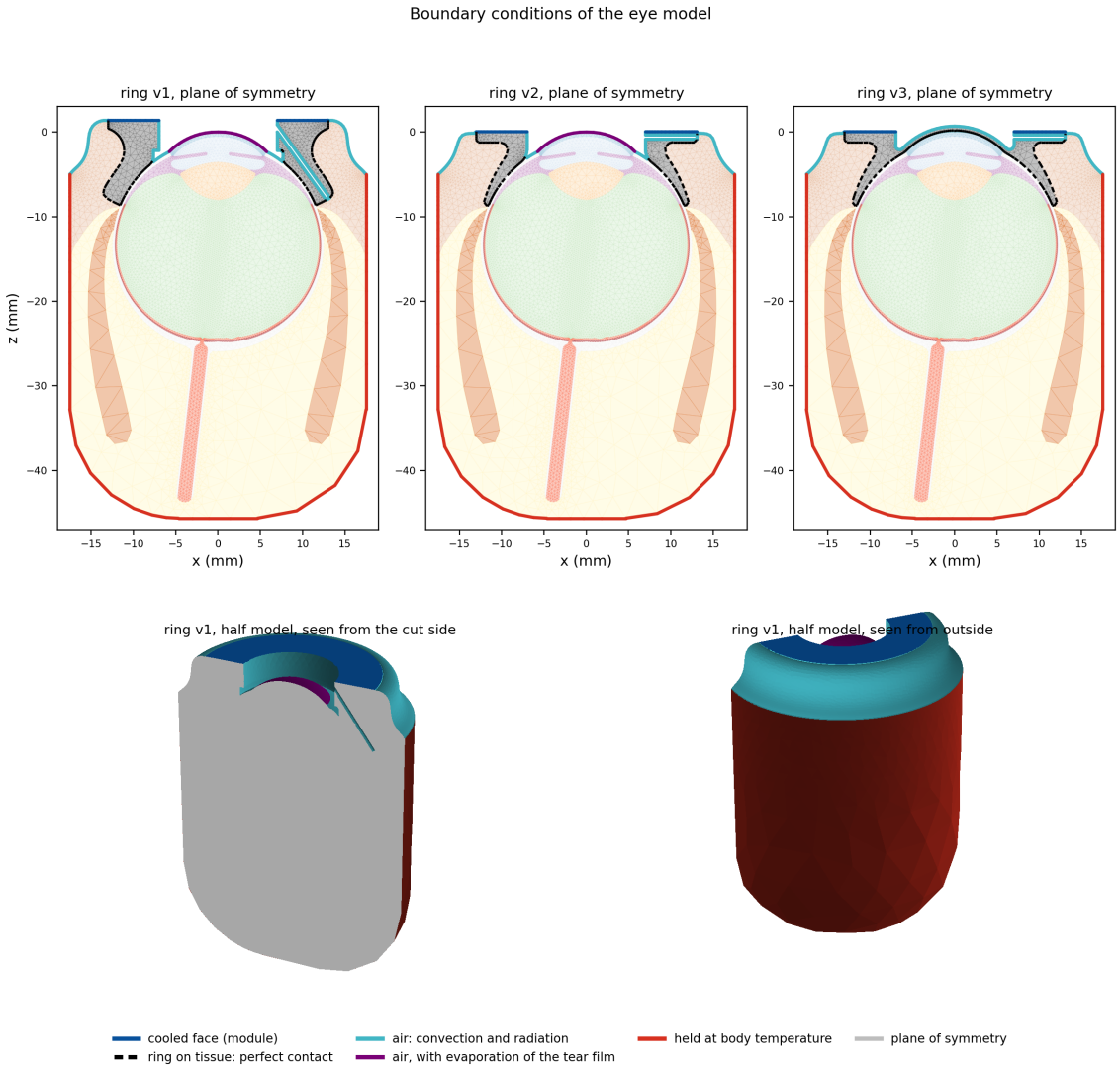

FIGURE B.1. Boundary conditions of the eye model in the corrected runs, in the plane of symmetry for the three rings and in three dimensions for v1. The face of the ring that met the module is cooled. Exposed surfaces lose heat to the room by convection and radiation, and the cornea also by evaporation of the tear film; in v3 the lens covers it. The outer boundary of the orbit is held at body temperature, and the ring is in perfect thermal contact with the tissue.

The heated face rose above the coolant by 0.24, 0.12, and 0.07 °C per watt in v1 at 0.10, 0.25, and 0.50 L min^{-1}, by 0.15, 0.07, and 0.04 in v2, and by 0.12, 0.05, and 0.03 in v3, the means over the three inlet temperatures on the intermediate mesh. These values changed by up to 12% from one mesh to the next, and the order of the designs was the same on every mesh. They are lower bounds: on meshes with

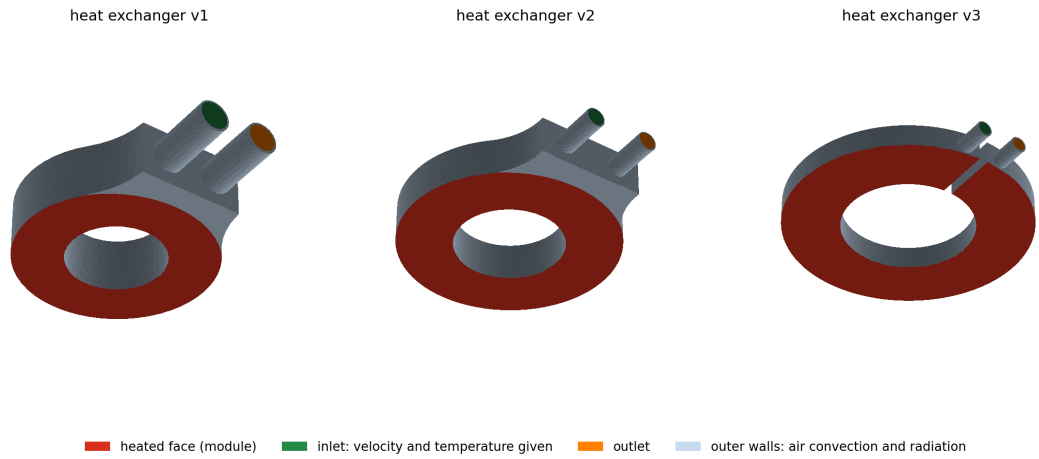

FIGURE B.2. Boundary conditions of the heat exchangers, seen from the side of the module: the heated face (red), the inlet, where the velocity and the temperature of the coolant are given (green), the outlet (orange), and the outer walls, which exchange heat with the ambient air by natural convection and radiation (gray-blue).

resolved wall layers the rise of v1 converges to about 0.32 and 0.18 °C per watt at 0.10 and 0.25 L min^{-1}, and the values here lie 24 to 37% below that limit.

The Reynolds number at the narrowest section of the channels stayed between 170 and 1240 in v1, 360 and 2690 in v2, and 510 and 3800 in v3, laminar in v1 and laminar to transitional in v2 and v3.

B.6. Meshes and Mesh Sensitivity

Figure B.3 shows the meshes of the eye model.

In COMSOL the energy balance of the ring closed to within 0.2% after the first 5 min. From the intermediate to the finest mesh the final temperature rose by 0.01 to 0.04 °C at the optic nerve and the posterior sclera, by 0.04 to 0.08 °C at the equatorial sclera and the vitreous, and by 0.08 to 0.11 °C in the average over the retina (Table B.2). At the anterior sclera the two steps had opposite signs, so no trend can be read there.

This change (bars in Figure B.4) is the threshold below which differences between designs are not interpreted. It is not the error of the finest mesh, which reads low: by 0.02, 0.06 to 0.08, and 0.36 to 0.62 °C at the proximal optic nerve, the vitreous,

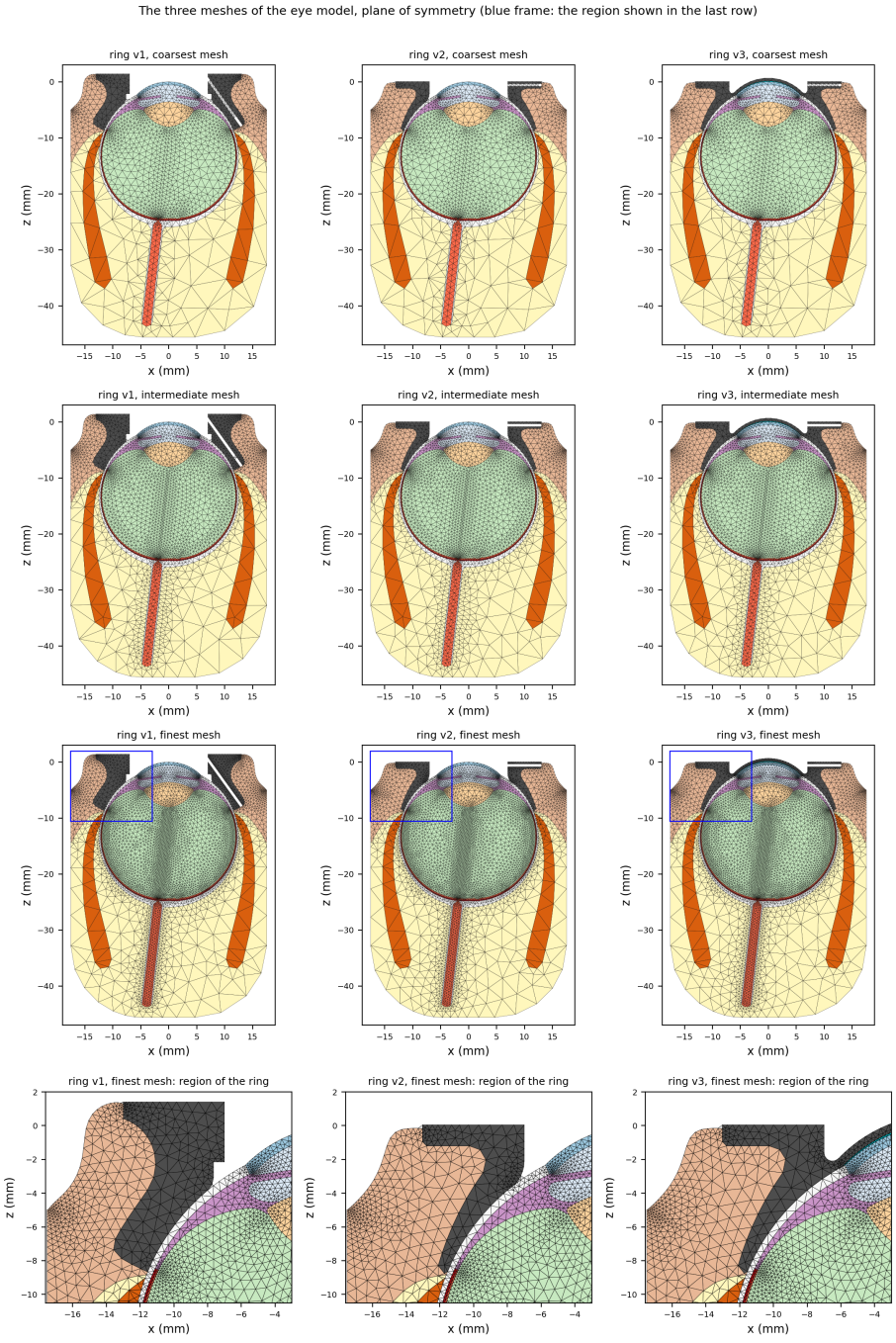

FIGURE B.3. The three COMSOL meshes of the eye model for each ring, with the air and the module layer removed as in the corrected runs, in the plane of symmetry, from the coarsest (top row) to the finest (third row). The bottom row enlarges the region of the ring on the finest mesh (blue frame in the row above). Results were computed on the finest mesh.

and the anterior sclera against the same mesh with every element divided into eight, and by 0.03 to 0.04, 0.10 to 0.14, and 0.75 to 1.28 °C against the extrapolated value, so temperatures at the anterior sclera are lower bounds. Table B.2 and Figure B.4 come from the runs of the Python program on these meshes (`mesh_steps.csv` in `figures/aim2/sources/data/python`).

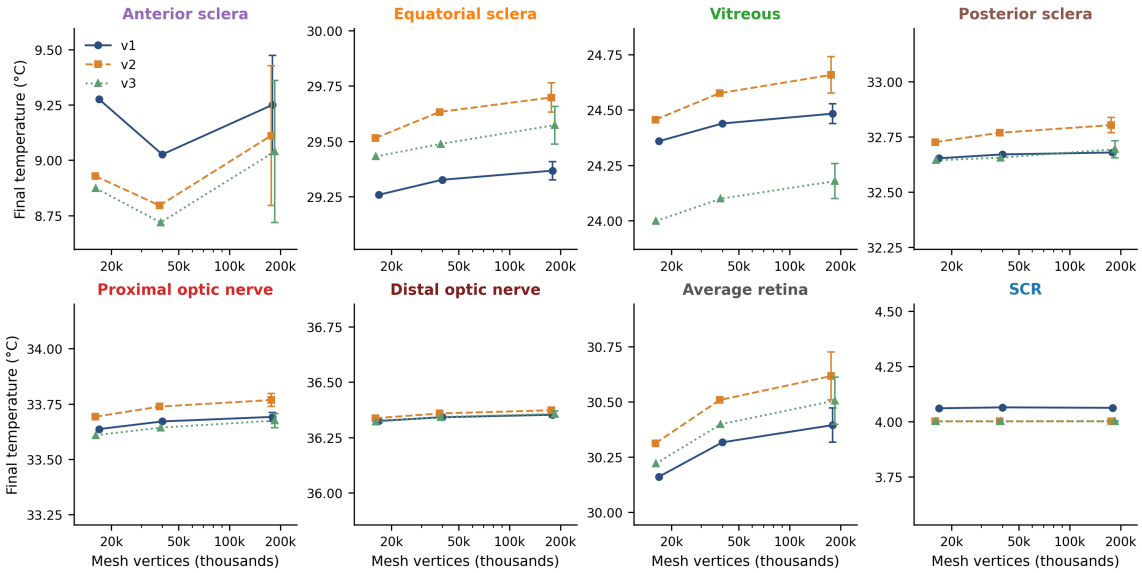

FIGURE B.4. Final temperature at each site of the eye model on the three meshes (vertex counts), for each ring. Every panel spans 1 °C, centered on its own values, so that changes can be compared between sites. The bar on the finest mesh spans plus and minus the change from the intermediate to the finest mesh (Table B.2), a step between meshes, not the error of the finest mesh, which reads low (see text). At the anterior sclera the temperature does not change in one direction from mesh to mesh.

From the intermediate to the finest mesh the laminar COMSOL solution of the heat-exchanger model changed by 0.03 °C at the module face and by 0.02 to 0.03 °C for the average temperature of the blocks. The Python program on the same meshes agreed within 0.04 °C at the outlet but only within 0.6 °C at the module face, whose temperature is therefore known to about 1 °C.

TABLE B.2. Change of the final temperature at each site of the eye model from one mesh to the next finer one, in °C, at the baseline condition, for each ring: (a) from the coarsest to the intermediate mesh and (b) from the intermediate to the finest mesh (about 17 000, 40 000, and 179 000 vertices).

Site	**v1 (a)**	**v1 (b)**	**v2 (a)**	**v2 (b)**	**v3 (a)**	**v3 (b)**
Anterior sclera	−0.25	+0.22	−0.13	+0.32	−0.15	+0.32
Equatorial sclera	+0.07	+0.04	+0.12	+0.07	+0.06	+0.09
Posterior sclera	+0.02	+0.01	+0.04	+0.03	+0.01	+0.04
Vitreous	+0.08	+0.04	+0.12	+0.08	+0.10	+0.08
Proximal optic nerve	+0.03	+0.02	+0.05	+0.03	+0.03	+0.03
Distal optic nerve	+0.02	+0.01	+0.02	+0.01	+0.02	+0.01
Average retina	+0.16	+0.08	+0.20	+0.11	+0.18	+0.11
SCR	0.00	0.00	0.00	0.00	0.00	0.00

APPENDIX C

Reconciliation of Published and Reprocessed Fresh-Porcine Values

The fresh-porcine recordings of Nakamura et al. [**1**] were reprocessed for Chapter 2 with the pipeline of Section 2.2.6, which retained all 30 channel traces, so its gates change no value. The initial temperatures agree to 0.1 °C at every site, and the differences come from two definitions. The published final temperature is that of the averaged curve at 1200 s and the reprocessed one the mean over minutes 16–20 of each trace, which gives the uniform −0.1 °C in ΔT. The published t_{90} is read from the averaged curve and the reprocessed one is the mean over the traces, so the largest difference, −27 s at the proximal optic nerve, is mostly one of definition. Tables C.1 and C.2 compare the two sets; no difference in ΔT or final temperature exceeds 0.1 °C, and none changes a conclusion of the published study.

TABLE C.1. Fresh-porcine cooling excursion, ΔT, as published [**1**] and as reprocessed for Chapter 2. Published values are the differences between the published cohort-mean initial and final temperatures. Difference is reprocessed minus published. All values in °C. A conclusion is judged unchanged (last column) when the difference is smaller than the differential measurement uncertainty at the proximal optic nerve (±0.40 °C, Appendix D); every difference meets that criterion.

Location	**Published**	**Reprocessed**	**Difference**	**Unchanged?**
Anterior sclera	24.4	24.3	−0.1	Yes
Equatorial sclera	9.5	9.4	−0.1	Yes
Posterior sclera	9.9	9.8	−0.1	Yes
Vitreous	11.7	11.6	−0.1	Yes
Proximal optic nerve	3.6	3.5	−0.1	Yes

TABLE C.2. Fresh-porcine final temperature and time to 90% of the cooling excursion (t_{90}), as published [**1**] and as reprocessed for Chapter 2.

	Final temperature (°C)		t_{90} **(s)**	
Location	**Published**	**Reprocessed**	**Published**	**Reprocessed**
Anterior sclera	10.7	10.8	194	200
Equatorial sclera	25.0	25.1	338	340
Posterior sclera	24.4	24.5	532	519
Vitreous	22.3	22.4	569	566
Proximal optic nerve	30.2	30.3	703	676

APPENDIX D

Temperature Measurement Uncertainty

This appendix gives the budget behind Section 2.2.4: an expanded uncertainty ($k = 2$) of ±1.8 °C for a single reading and, for a within-trace excursion ΔT in the human eyes, ±0.40 °C at the proximal optic nerve and ±0.85 °C at the ring. It follows the Guide to the Expression of Uncertainty in Measurement [**203**]: standard uncertainties u_i are combined in quadrature, and Type B contributions bounded by a half-width a are rectangular ($u_i = a/\sqrt{3}$) unless stated otherwise.

D.1. Measurement Chain

Each thermistor (NTC, $R_{25} = 10$ kΩ, $\beta = 3976$ K) sat in a linearized divider and was read through an amplifier by a 16-bit data-acquisition system with a 20 V input range (slope 2.06 °C V^{-1} between the 37 and 20.5 °C plateaus). Human-donor channels were converted with a per-channel least-squares line through plateaus at 4.0, 20.5, and 37.0 °C in a dry-block calibrator, and porcine channels with a fixed linear transfer function trimmed at 20.5 and 37.0 °C (Section 2.2.4). The human lines miss their own plateaus by up to 0.57 °C, more than the 0.34 °C measured on the porcine chain in 2022, so the linearity terms come from them. The tables below are those of the human eyes. For the porcine eyes the slope term doubles, because the two set points are 16.5 rather than 33 °C apart: the expanded uncertainty of an excursion is ±0.33 °C at the proximal optic nerve and ±1.1 °C at the ring, and that of a single reading ±1.8 °C.

The ring channel of the in-vivo study (Section 4.2.6) is not linearized: a straight line misses its plateaus by about 1 °C. Its readings were converted with a model of

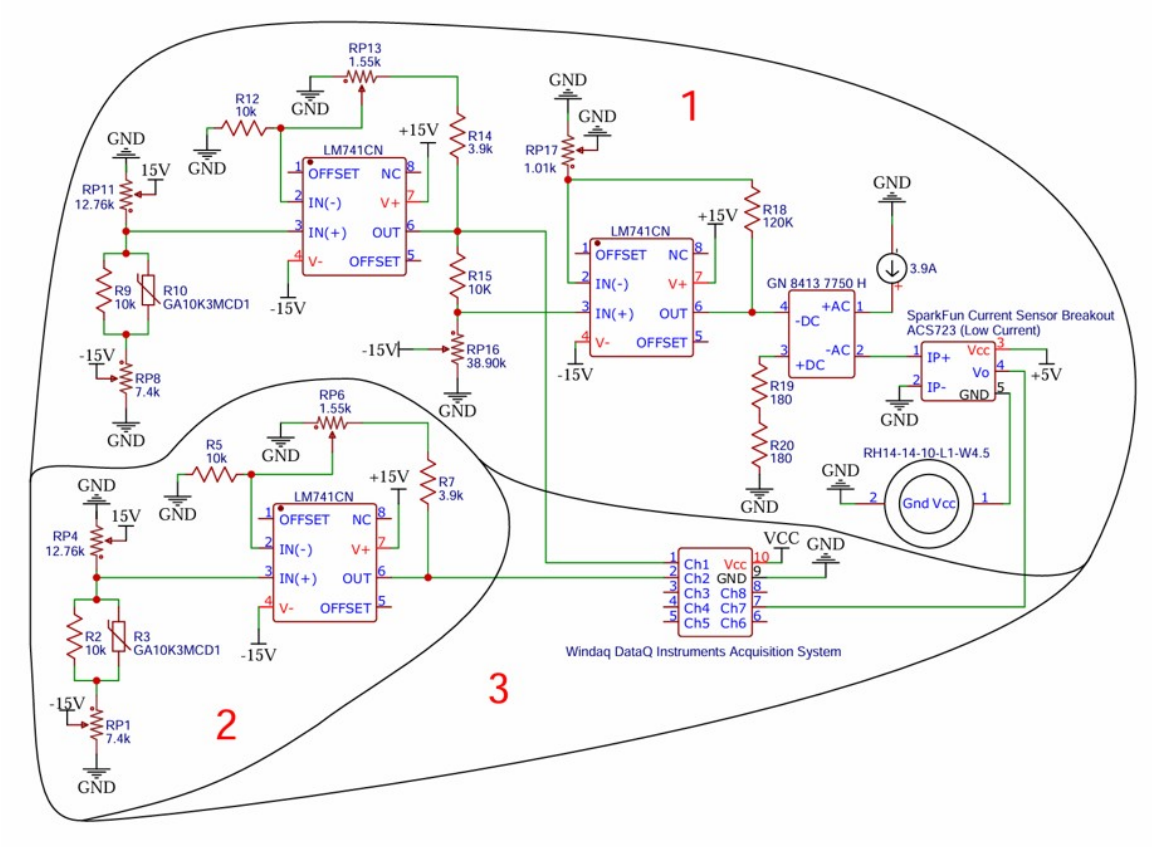

FIGURE D.1. Signal-conditioning and acquisition circuit. Area 1, the channel of the SCR feedback thermistor with the automated supply circuit and the current-sensing board; area 2, one of the five identical channels of the implanted sensors; area 3, the data acquisition unit.

the circuit,

$$V = \frac{aR + b}{R + g}, \tag{D.7}$$

fitted to the plateaus recorded in each session, where V is the output voltage of the channel, R the resistance of the thermistor, a and b the gain and offset of the amplifier, and g the effective resistance of the divider in which the thermistor sits. The resistance follows the Steinhart–Hart law of the thermistor type (10K3A),

$$\frac{1}{T} = A + B \ln R + C(\ln R)^3, \tag{D.8}$$

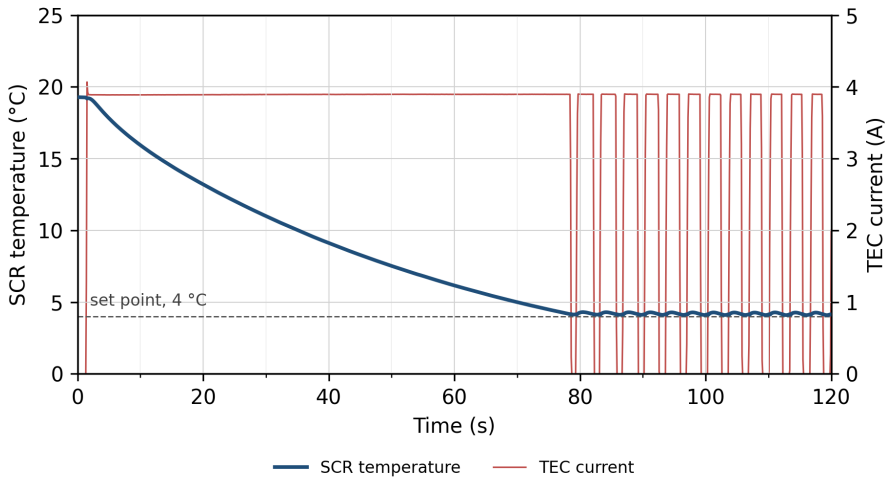

FIGURE D.2. Automatic feedback control of the SCR. The TEC drive current (right axis) is held at its maximum until the SCR temperature measured by the feedback thermistor (left axis) reaches the 4 °C set point; the comparator then switches the drive on and off to hold the ring at the set point. Bench recording on a preserved porcine eye (2023), not one of the analyzed runs.

where T is the absolute temperature and $A = 1.129241 \times 10^{-3}$, $B = 2.341077 \times 10^{-4}$, and $C = 8.775468 \times 10^{-8}$ are the manufacturer's coefficients for R in ohms. Only a, b, and g were fitted; with five plateaus the residuals were at most 0.16 °C.

D.2. Absolute Uncertainty

Table D.1 lists the contributions to the uncertainty of a single reading. One term, sensor self-heating, accounts for 80% of the variance; without it the expanded uncertainty would be ±0.81 °C.

Self-heating. The current through a thermistor warms it above the tissue it measures. With the sensors in the calibrator, raising the excitation from 3 V to the ±15 V operating supply raised the reading by 4.6 °C on average (3.6 to 6.0 °C by channel). The calibration was made at the operating supply, and heat leaves the sensor more readily in tissue and oil than in the well, so a reading can only be too low. This underreading was bounded from the study's own data, since at steady state no submerged tissue can be warmer than the oil: in the eleven runs with a recorded oil temperature

TABLE D.1. Uncertainty budget for a single absolute temperature reading. u_i is the standard uncertainty; the share is u_i^2/u_c^2.

Source	**Basis**	u_i **(°C)**	**Share**
Self-heating	one-sided bound 1.43 °C, not corrected (see below)	0.826	80%
Linear conversion	largest miss of a human calibration line at its own plateaus, 0.57 °C	0.329	13%
Calibrator accuracy	dry-block specification ±0.25 °C [**126**]	0.144	2%
Calibrator block uniformity	between wells of the aluminum insert, ±0.2 °C (the manufacturer's figure for the 9100S; none is given for the 9102S with a custom insert)	0.115	2%
Sensor interchangeability	thermistor tolerance ±0.2 °C [**125**]	0.115	2%
Dynamic lag	time constant 0.2 s [**125**] at the fastest cooling rate measured, 0.54 °C s^{-1}	0.062	<1%
Drift after calibration	0.05 V recorded after calibration	0.060	<1%
Calibrator stability	±0.05 °C [**126**]	0.029	<1%
Reading noise (Type A)	standard deviation on the settled calibration plateaus	0.021	<1%
Lead resistance	9.4 Ω in series with the sensor, computed from the length and gauge of the leads and cable	0.022	<1%
DAQ accuracy	±5 mV	0.006	<1%
Combined standard uncertainty u_c		0.920	
Expanded uncertainty U ($k = 2$)		1.84	

(36.9 to 37.5 °C) the oil exceeded the starting reading by at least 1.3 °C at the proximal optic nerve and 1.2 °C at the posterior sclera. A bound of 1.43 °C was used for every channel; the vitreous channel, which showed the largest rise in the calibrator, may exceed it. No correction was applied, so the term is rectangular between zero and the bound, $u = a/\sqrt{3}$ with $a = 1.43$ °C. The reported absolute temperatures can therefore be up to about 1.4 °C lower than the true tissue temperature, more likely

near that limit than near zero, and not higher. The underreading largely cancels in ΔT; only its change with temperature remains (Table D.2).

D.3. Differential Uncertainty

A cooling excursion is the difference between two readings of one sensor, so the calibrator offset, sensor interchangeability, lead resistance, and most of the self-heating cancel. Three terms grow with the excursion: the slope of the calibration line, the curvature that a straight line ignores, and the change of self-heating between the two temperatures. Table D.2 gives the budget for a small excursion, the proximal optic nerve (34.6 to 30.8 °C), and a large one, the ring (29.7 to 4.1 °C).

TABLE D.2. Uncertainty budget for a within-trace cooling excursion ΔT: standard uncertainty u_i, in °C, for an excursion at the proximal optic nerve and at the ring.

Source	**Basis**	**Optic nerve**	**Ring**
Bath stability	±0.2 °C	0.115	0.115
Drift during the run	as Table D.1	0.060	0.060
Linear conversion, on a difference	human calibration lines, 0.24 and 0.54 °C	0.139	0.311
Reading scatter	standard deviation of single samples about the filtered trace over the final window, median over the included traces	0.054	0.054
Slope of the calibration line	calibrator accuracy and uniformity at two set points, over the excursion	0.030	0.203
Change of self-heating with temperature	bound of 1.43 °C scaled by the change in dissipated power (source resistance 10.8 kΩ, the median estimated from 36 calibration channel-sessions)	0.026	0.140
Thermistor β tolerance	±0.5% [**125**]; the implanted beads are not the calibration set	0.012	0.070
Combined standard uncertainty u_c		0.202	0.427
Expanded uncertainty U ($k = 2$)		0.40	0.85

The differential value at the proximal optic nerve is the uncertainty of a single excursion at the primary site; differences between groups are judged by their confidence intervals (Section 2.2.6).

APPENDIX E

Analysis Code

This appendix documents the code used to analyze the experiments of Chapter 2 and the computational model of Chapter 3. Each analysis is a Python package that runs from the raw data to every table and figure with a single command, holds its parameters in one configuration file, and records the software versions and file checksums with its results.

E.1. Ex-Vivo Analysis Pipeline

The ex-vivo package reproduces the study's analysis notebook (version 4.28) exactly when the settings added after that version are switched off (Figure E.1).

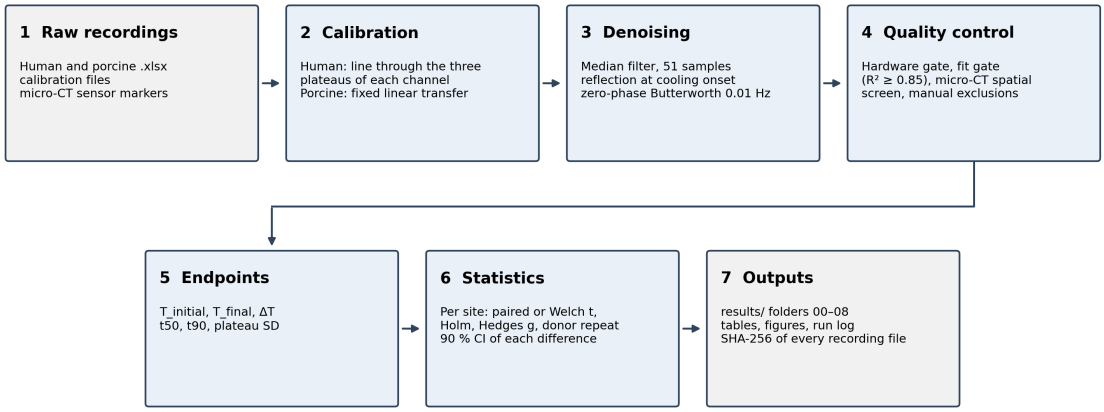

FIGURE E.1. Stages of the ex-vivo analysis, from the raw recordings to the tables and figures of Chapter 2.

E.2. Analysis Parameters

Table E.1 lists the parameters of the method (Section 2.2.6).

Table E.1: Parameters of the ex-vivo analysis.

Stage	Parameter and value
Acquisition	sampling interval 0.1 s
Calibration	human: per-channel least-squares line through the plateaus at 4.0, 20.5, and 37.0 °C, each plateau located by k-means clustering of the voltages (three clusters, seed 42) and read as the median of its last 3 min before the next ramp (last 10 s excluded), or of its longest stretch of 10-s medians within ±0.01 V where those 3 min drifted by more than 5 mV per minute, or of the whole recorded plateau where less than 3 min was recorded; porcine: $T = -2.0656\,V + 20.439$ °C for all channels, with V the recorded channel voltage in volts and T in °C; readings outside 0–43 °C set to missing
Denoising	median filter 51 samples (5.1 s), edge-value padding; point-reflected extension of 300 s before the cooling onset; second-order zero-phase Butterworth low-pass, 0.01 Hz
Channel corrections	human eye 4: equatorial and anterior channels exchanged (the anterior sensor was recorded on the equatorial channel); eye 6: the anterior and posterior workbook columns exchanged, each converted with the other column's calibration
Hardware gate	at least 100 samples; flat line if the raw-voltage SD is below 10^{-4} V; stuck if 90% of samples sit at the median value; railed if 75% of samples lie beyond ±9.9 V; and by hand, the failing anterior-sclera sensor of human eye 9 (original ring)
Fit gate	single-exponential fit $T = A\,e^{-kt} + C$ (bounds $0 \le A \le 50$, $0 \le k \le 1$ s^{-1}, $0 \le C \le 40$ °C), $R^2 \ge 0.85$, at least 20 samples
Spatial screen	sensors identified by their marker labels; the four sensors fixed to tissue (anterior, equatorial, and posterior sclera, proximal optic nerve) of each eye rigidly aligned (rotation and translation) to the cohort mean; limit on the offset from the mean position of one thermistor length (3.3 mm); vitreous sensor excluded if more than 75% of the distance from the center of the globe to its wall on micro-CT; a spatial breach takes precedence over an earlier reason
Manual exclusions	four traces: three in the original-ring data of Chapter 2 (Section 2.2.6) and one, the same human vitreous sensor under the reduced-bulk ring, in the ring comparisons of Chapter 3
Endpoints	final temperature and plateau SD over minutes 16–20 (240 s); t_{50}, t_{90} as the first times the trace has fallen by 50% and 90% of ΔT
Statistics	paired t test when the two groups share at least two eyes, otherwise Welch's t test; Hedges' g; $\alpha = 0.05$; Holm correction over the five tissue sites; 90% confidence interval of each difference (its limits give the largest difference allowed); species comparison repeated on donor means

The spatial screen aligned the four tissue-fixed sensors of each eye to the mean set of the cohort. The offset of sensor s in eye e was

$$r_{e,s} = \left\| \mathbf{R}_e \, \mathbf{x}_{e,s} + \mathbf{t}_e - \bar{\mathbf{x}}_s \right\| , \tag{E.9}$$

where $\mathbf{x}_{e,s}$ is the micro-CT position of sensor s in eye e, $\mathbf{R}_e$ and $\mathbf{t}_e$ the rotation and translation that best map the sensor set of eye e onto the mean set in the least-squares sense, and $\bar{\mathbf{x}}_s$ the mean aligned position of sensor s over the eyes of the cohort. The alignment removes the position and orientation of each eye in the scanner.

E.3. ERG Processing

The width of the b-wave and the number of its peaks (Section 4.2.9) are computed by `erg_cohort.py` from the same filtered trace as the picks. The analysis used the raw record of each flash step, sampled every 0.512 ms, not the waveform reported by the instrument. Drift was removed with a 0.3 Hz high-pass filter (second-order Butterworth, forward and backward) [**139**]. Sweeps from 20 ms before to 200 ms after each flash were referred to the mean of the 20 ms before the flash and interpolated onto a 0.5-ms grid. A sweep was rejected when the noise of the second before its flash was more than three times the median of the sweeps of the recording, or when its b-wave amplitude differed from their median by more than half.

The accepted sweeps were averaged and low-pass filtered at 75 Hz (fourth-order Butterworth, forward and backward) for the a-wave and b-wave. The a-wave was the minimum between 2 and 26 ms after the flash, and the b-wave the maximum after that trough, searched from 8 ms and up to 50 ms (2 to 35 ms, and from 12 up to 110 ms, at the weakest flash). The a-wave amplitude was measured from the pre-stimulus baseline and the b-wave amplitude from the a-wave trough [**139**].

For the oscillatory potentials, the accepted sweeps of each flash step except the first were averaged without the 75 Hz low-pass and band-pass filtered from 75 to 300 Hz (fourth-order Butterworth, forward and backward) [**139**]. They were the

positive peaks on the rising limb of the b-wave, from the a-wave trough (not earlier than 10 ms after the flash) to 5 ms after the b-wave peak, each measured from the trough before it. The measure is the sum of the first four, with the root-mean-square over the same window reported alongside, at 8 and 25 cd·s/m^2.

Replacing the drift removal by a straight line fitted to the 1 s before each flash changed the b-wave amplitude by a median of 1 μV and the implicit time by a median of 0 ms (at most 77 μV and 1.5 ms) over the 186 recordings at each of the two stronger flashes. Compared with the waveform the instrument reported, the median correlation was 0.996, and 371 of the 372 b-wave amplitudes at the two stronger flashes agreed within 15%. The oscillatory-potential sums measured on the instrument's waveform correlated with those of this analysis at 0.99, with a median difference of 2%. A recording was flagged when its residual drift before the flash exceeded three times the median of all recordings (67 of the 558 traces of animals 1 to 6) or when a pick fell on the edge of its search window, and flagged picks were checked against the waveform. A latency corrected by hand is kept in a separate file with the detector's value, and the phase of each recording was assigned by hand from the session log.

E.4. Globe Outline and Position of the Vitreous Sensor

For imaging, the thermistor wires were cut and the eye on its holder was sealed in a container (Figure E.4), filled with oil for most porcine eyes and for one scan of human eye 2; the other scans were made in air. MRI (Agilent, Santa Clara, CA, USA) used a multislice gradient-echo sequence with $0.078 \times 0.078 \times 0.8$ mm voxels (repetition time 550 ms, echo time 7.5 ms, flip angle 20°, 36 contiguous slices, 16 averages) and a 72-mm volume coil. The oil is displaced in these images relative to the water-bearing tissue, by about 1.1 mm along the readout direction, and the eye itself is imaged in place. Micro-CT was performed on North Star Imaging systems (Rogers, MN, USA) at 70 kV and 1200 μA (60 kV and 550 μA for human eyes 5 to 9), reconstructed at isotropic voxels of 32 to 79 μm.

The outline of each globe on micro-CT was found automatically: a smooth closed surface fitted to the tissue edges along rays from the center of the globe in the fresh eyes, and the tissue mask after a 3 mm morphological opening in the folded preserved porcine eyes. Every outline was inspected on fifteen sections and moved by hand onto the wall where it departed from it, in 18 of the 19 eyes; it then followed the drawn lines to a median distance of 0.2 mm or less. The center of the globe is the centroid of the automatic outline. The axis of human eyes 3 to 9 is the normal of the holder ring; for human eyes 1 and 2 and the porcine eyes it is found from the nerve stump and the lens. The code is archived with the ex-vivo data (folder `CT Segmentation/code`), and the results enter the package as `config/vitreous_position_ct.csv` and `config/vitreous_depth_ct.csv`.

The position of the vitreous sensor was the ratio r/R, where r is the distance from the center of the outline to the sensor and R the distance from the center to the outline in the same direction, so that 0 is the center of the globe and 1 its wall. A vitreous sensor was excluded when r/R exceeded 0.75 (Figures E.2 and E.3), as in human eyes 5 and 8 (0.82 and 0.81).

E.5. Reproducing the Analysis

The package was run with Python 3.13, NumPy 2.2.6, pandas 2.3.2, SciPy 1.16.1, statsmodels 0.14.6, scikit-learn 1.9.0, Matplotlib 3.10.6, and openpyxl 3.1.5, pinned in its requirements file, because later releases no longer reproduce the stored references exactly. The analysis is run with

```
pip install -r requirements.txt
python run_analysis.py --data <recordings folder> --out results
```

and the agreement with the notebook is checked with

```
python tests/test_reproduce.py <recordings folder>
```

with the later settings switched off (per-site statistics to a relative tolerance of 10^{-9}). `tests/test_default.py` is a regression test of the current settings. The version used

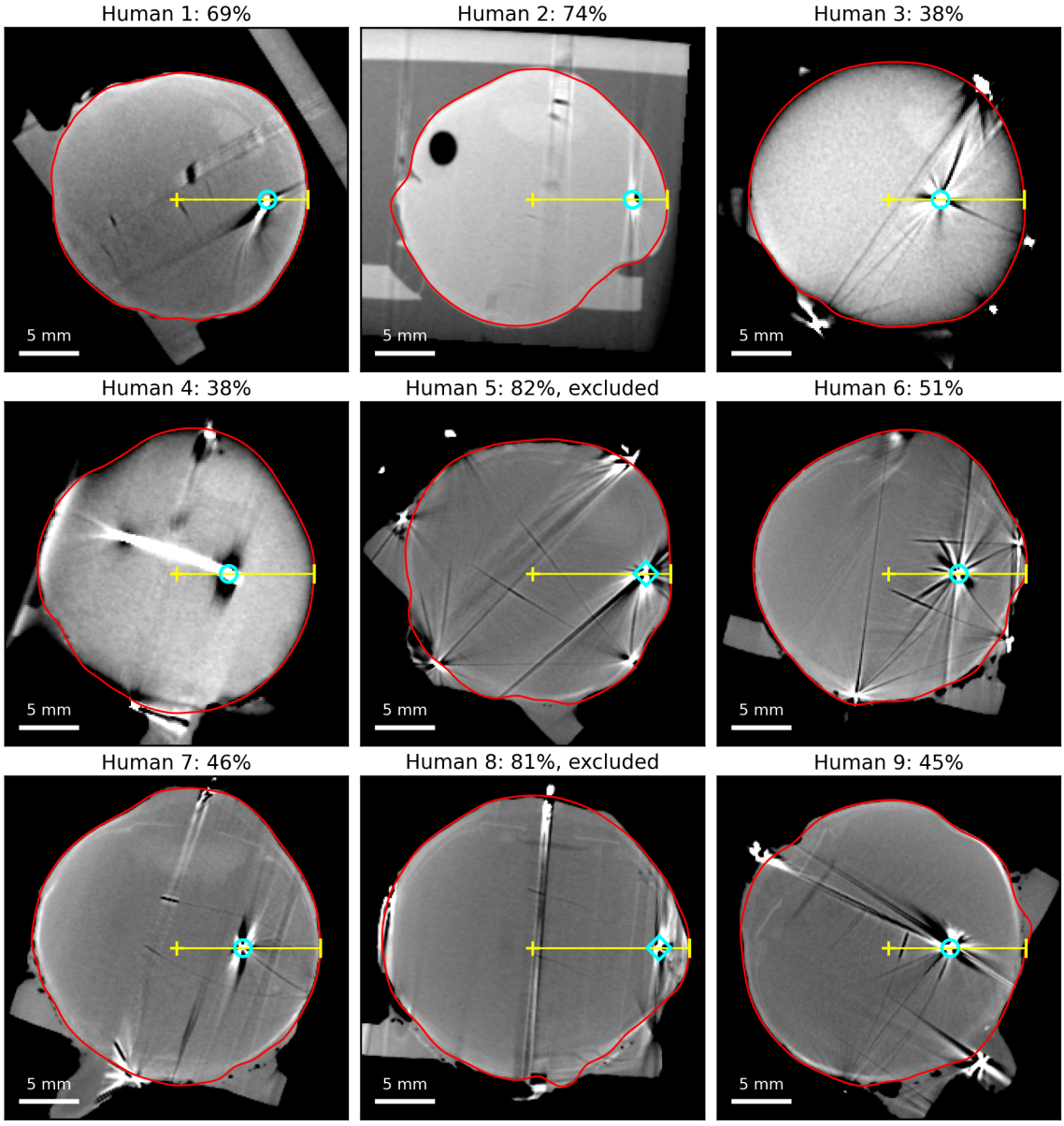

FIGURE E.2. Position of the vitreous sensor in the nine human eyes. Each panel is the micro-CT section through the center of the globe outline (cross) and the vitreous sensor (cyan marker), with the outline in red and the line from the center to the wall in yellow. The number is the ratio r/R of Section E.4; a diamond marks a sensor beyond the limit of 0.75, which was excluded.

in this dissertation, on which both tests pass, is in folder `code/ch2-exvivo-analysis` of the code repository (Section E.7).

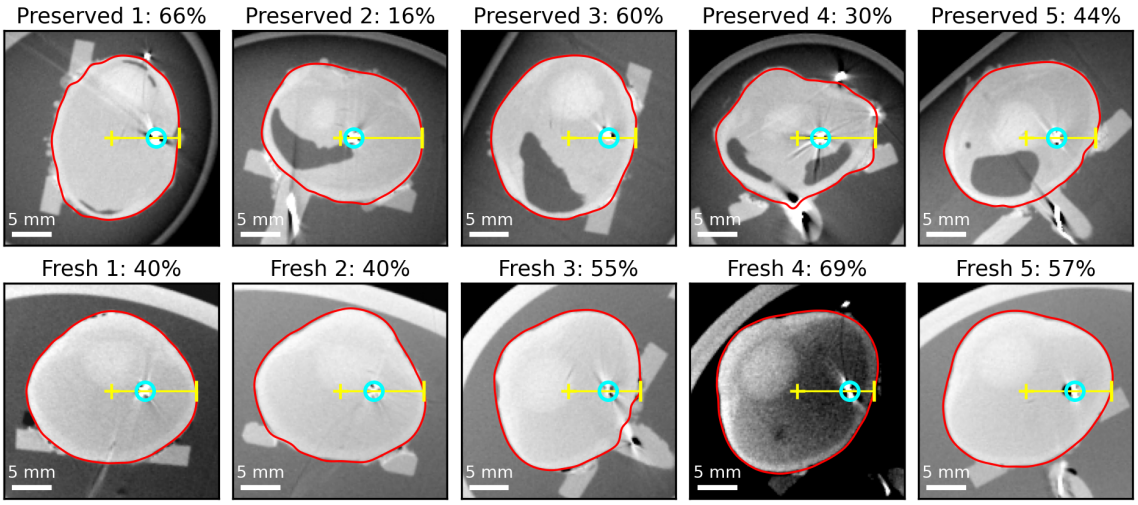

FIGURE E.3. Position of the vitreous sensor in the five preserved (top) and five fresh (bottom) porcine eyes, drawn as in Figure E.2. No porcine vitreous sensor lay beyond the limit of 0.75.

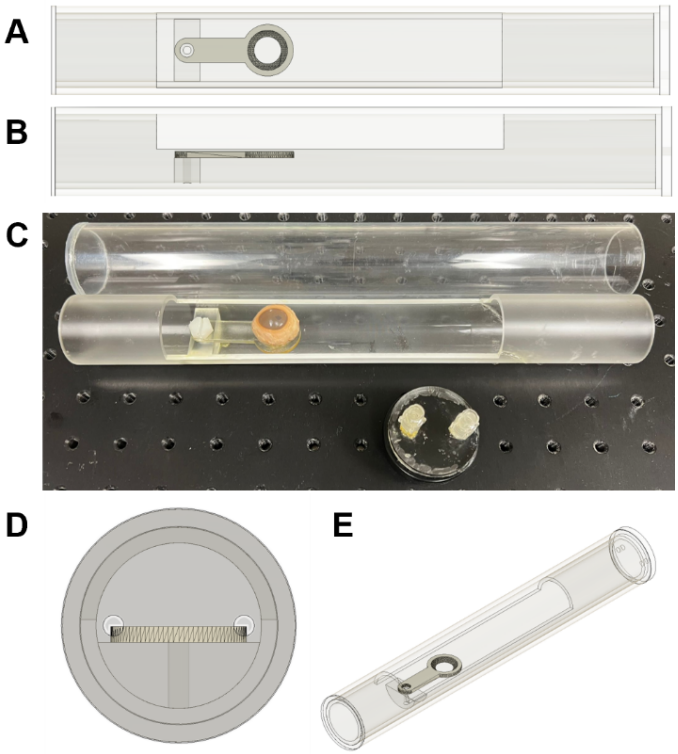

FIGURE E.4. Sealed, oil-filled container used for MRI and micro-CT. Top: top and side views of the CAD model. Middle: the container with an eye on its holder, and the cap with the two valves used to evacuate air. Bottom: front view of the cap and panoramic view of the container (E).

E.6. Computational Model Analysis

The COMSOL exports behind Chapter 3 were analyzed with a second package of the same design (Figure E.5; folder `code/ch3-model-analysis` of the same repository). It reproduces the model-analysis notebook (version 3.8.0) exactly when its corrections are switched off (183 195 checks), and eleven corrections are applied by default and documented in the package, among them the doubling of the heat exported for the face of the module and the division of the heat among eye, eyelids, and air. It reads the ex-vivo results, so the ex-vivo package is run first.

The figures of Chapter 3 are produced by the scripts in `code/figures` of the code repository, and each is listed with its inputs and their checksums in the manifest of its folder. The eye-model temperatures and heat values come from the corrected runs, the validation from the replication runs, and the heat-exchanger results from the sweep of `scr-heat-replica`.

Mesh sensitivity. In COMSOL, temperature, velocity, and pressure were interpolated with linear elements, the stationary initial state was solved to a relative tolerance of 10^{-3}, and the transient was integrated with a variable-step backward differentiation formula of order 1 to 2 at a relative tolerance of 0.005. The eye model was solved for each ring on three meshes, from COMSOL's *Extra coarse*, *Coarser*, and *Coarse* presets with local refinement of the retina, choroid, and tear film: about 17 000, 40 000, and 179 000 vertices. The heat exchangers were solved on three meshes in the same way. The grid convergence index [**204**] is stored with the results but not reported, because the refinement was not strictly systematic and the solutions did not converge monotonically at every site.

Python finite-element program. The program reads the meshes, the material definitions, and the boundary conditions from the COMSOL model files and solves the heat equation with linear tetrahedral elements and a second-order backward differentiation formula, with fixed steps of 6 s for the first 5 min, 15 s to 15 min, and 30 s afterwards, the first step by backward Euler. In the heat exchangers the coolant

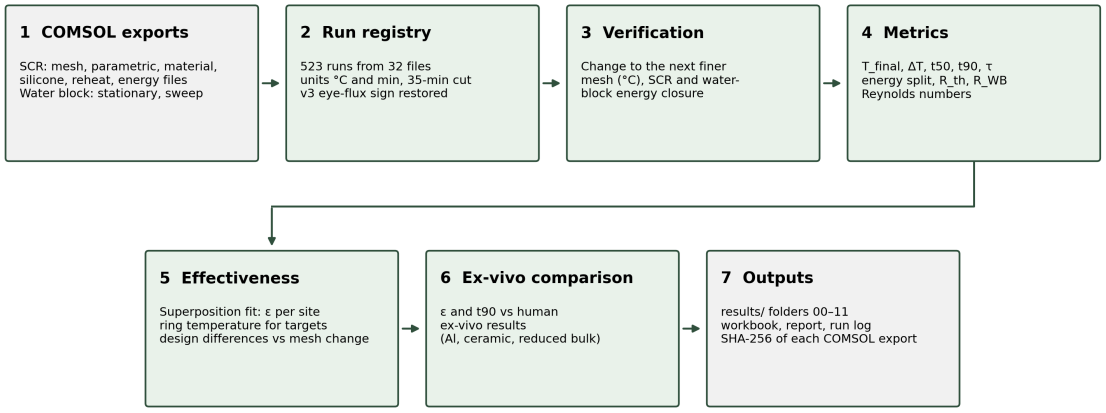

FIGURE E.5. Stages of the computational model analysis, from the COMSOL exports to the results of Chapter 3.

velocity was a fixed quadratic profile, without buoyancy. The corrected and extension cases follow Section 3.2.2.1. The flow solver and the reading of the model files are verified only by the agreement with COMSOL. The program is in folder `code/ch3-heat-replica` of the code repository (Section E.7); its inputs and the list of cases are available from the author.

E.7. Code Availability

The code is kept in a public repository rather than reproduced here: `https://github.com/GigiosGros/eyecooler-thesis`. Folder `code/ch2-exvivo-analysis` holds the ex-vivo analysis of Chapter 2, `code/ch2-bath-vs-perfusion` the conduction estimate of the oil bath, `code/ch3-model-analysis` the analysis of the COMSOL exports and `code/ch3-heat-replica` the finite-element program of Chapter 3, and `code/ch4-live-pigs` the processing of the in-vivo recordings (temperature conversion, ERG extraction, and the cohort statistics of Chapter 4). The scripts that draw the figures are in `code/figures`, the figures in `figures`, and this dissertation in `thesis`. The raw recordings and the COMSOL model files are available from the author on request.

APPENDIX F

Selection of the Water-Block Coolant

The heat exchanger of SCEC Gen 3 was run with a 50:50 mixture of propylene glycol and distilled water in the porcine experiments of Chapter 2, and with distilled water alone in the human-donor experiments, in every in-vivo session, and in the water-block model of Chapter 3.

Propylene glycol lowers the freezing point, which was not needed here, since the loop never approached freezing, and every thermal property that governs the loop is less favorable with it. Relative to water, pure propylene glycol has about one third of the thermal conductivity, 60% of the specific heat, and about 55 times the viscosity at 25 °C [**170, 205**], and water–glycol mixtures lie between the two.

The Lynch laboratory at the University of Illinois Chicago allowed the heat flow of each fluid to be measured by differential scanning calorimetry (Figure F.1). Per gram, the 50% mixture absorbed 0.79 and propylene glycol 0.57 times as much heat as distilled water at 10 °C, close to the ratio of the tabulated specific heats, 0.60. Distilled water was therefore used from the human-donor experiments onward.

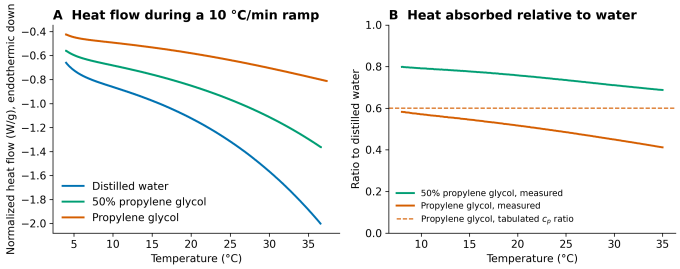

FIGURE F.1. Differential scanning calorimetry of the candidate coolants. (A) Normalized heat flow. (B) Heat flow relative to distilled water (solid) and tabulated ratio for propylene glycol (dashed) [**170, 205**]. Only the comparison between samples is interpreted.

APPENDIX G

Use of Generative Artificial Intelligence

A generative artificial intelligence tool (Claude, Anthropic, accessed via Claude Code) was used during the preparation of this dissertation. Its use, as described below, was approved by all five members of the dissertation committee by e-mail between October 23 and November 4, 2026.

Writing. The tool assisted with literature search and organization, verification of cited sources against their original publications, drafting and revision of prose, and formatting of the LaTeX source. All AI-assisted text was reviewed, checked against the primary literature, and edited by the author.

Code. Under the author's direction, the tool helped write, revise, test, and run the analysis code of Chapters 2 and 3 and the scripts that generate their figures and tables, and helped transfer the results into the text. It wrote the Python finite-element program of Chapter 3 and its test scripts, and it reviewed the code and results of that program, which was accepted only after it had reproduced the solutions of the commercial software and exact solutions (Section 3.3.1). Every analytical decision was made by the author.

The tool was not used to design experiments or to collect data. The author takes full responsibility for the accuracy and content of this dissertation.

APPENDIX H

The Integrated Control Board

H.1. Hardware and Firmware

An STM32L476RG microcontroller [**206**] sets the drive of an ADN8831 thermoelectric-module controller (Analog Devices) [**207**], which powers the module through an H-bridge of four MOSFETs and senses its current across a 10 mΩ resistor. Trimmers set hard limits on the module current and voltage, independently of the firmware, and the module is off by default. Six thermistor channels read the ring and five ocular sites from a buffered 2.5 V reference; only the ring channel is used for control. The board also measures the module current and voltage, reads two coolant-flow sensors (SLF3S-4000B, Sensirion) [**208**] and an ambient temperature and humidity sensor (AHT20, Aosong) [**209**], has inputs for two coolant-pressure transducers, and drives the pump and a fan (Figure H.1). As designed, the ring signal fed an analog loop in the module controller. On the board as used that connection was cut, and the control loop runs in the microcontroller, which sets the drive through its digital-to-analog converter.

The ring is regulated by a digital proportional–integral–derivative controller at 5 Hz. A safety routine running every 50 ms stops the module and latches the fault when the ring thermistor reads open or outside 0–45 °C, when the 2.5 V reference leaves its band, when the module current exceeds 5 A for 500 ms or, in closed loop, is absent, or when the link to the operator falls silent for more than 2 s over the binary protocol. A hardware watchdog resets the microcontroller if its program stops. Faults on the five measurement channels are reported without stopping cooling. After a fault the pump and fan run at full speed for 30 s (3 s in console runs) to clear the stored

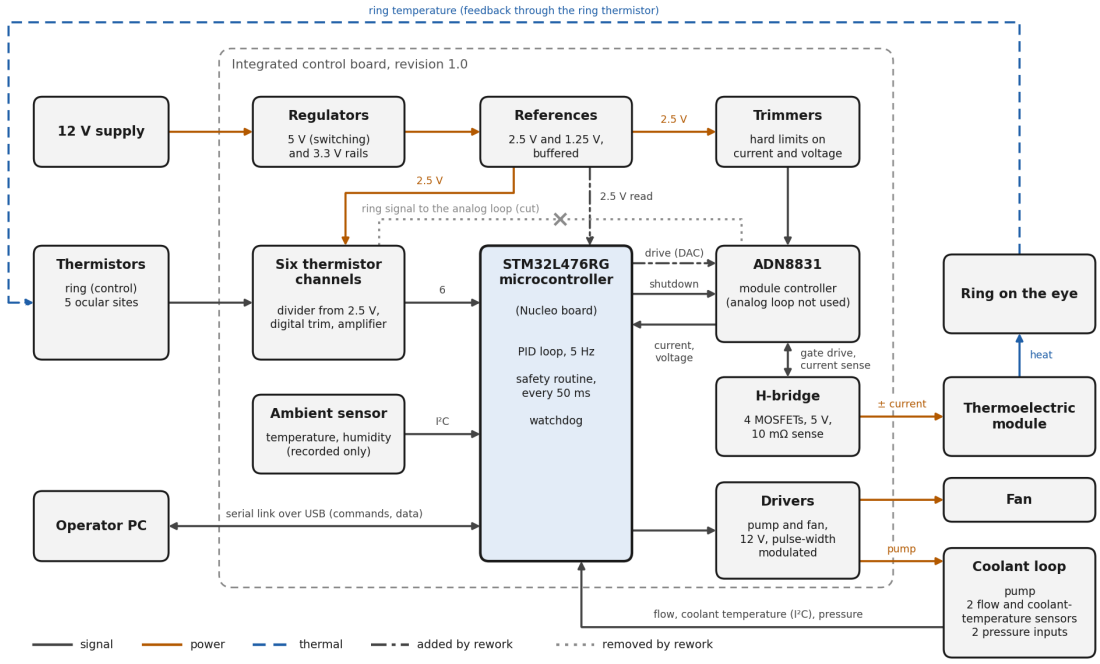

FIGURE H.1. Block diagram of the integrated control board (revision 1.0) as used. Solid gray lines are signals, orange lines power, and the dashed blue line the thermal path that closes the loop through the ring thermistor. Dash-dot lines are connections added by rework: the drive set directly by the microcontroller and the reading of the 2.5 V reference. The dotted line is the connection removed by rework, the ring signal to the analog loop of the module controller.

heat. The firmware logic was tested against a simulated hardware layer, in a suite of 310 checks.

The module voltage is five times the departure of the drive signal from a 1.25 V center, cooling below it and heating above, with one leg of the H-bridge linear and the other switched at 500 kHz. The board runs from an external 12 V supply, through a buck converter to 5 V and a linear regulator to 3.3 V. Each thermistor forms a divider with a digital potentiometer, followed by a buffer amplifier, and is converted to temperature with a Steinhart–Hart model.

H.2. Bench Verification

Before it drove a ring, the board was verified in stages: the bridge transistors, power-up, the reference, operation into dummy loads to confirm the hard limits, the

thermal response, the polarity of the drive, and closed-loop regulation. The last stage was a soak at 4 °C with the device on a preserved porcine eye in the bath, which ended with the ring thermistor disconnected. In a later session the module was disconnected and the program stalled (the latter with the module off).

In a second closed-loop run, with chilled coolant and the ring out of the bath, the ring went from 30 to 4.5 °C in 352 s and held 3.98 °C on 1.65 A. The firmware held the drive at 2.9 A, so the pull-down was slower than that of the SCEC Gen 3 ring ex vivo, which completed 90% of its fall in about 2 min.

The other functions of the board worked, with limits. All six thermistor channels read, and in room air five agreed within 3 mV. Both flow sensors read about 0.1 mL min^{-1} with the pump off and 570 to 800 mL min^{-1} with it on, the upper values beyond their range. In later sessions they dropped off their bus during the reference fault of Section 3.3.8 and returned only when restarted, so that a flow reading accompanied 28% of the samples of one session. The board switched the pump and the fan, but neither could be graded: the pump gave no flow below 65% PWM duty, and the fan turned only at full duty. The ambient sensor read 25.3 to 25.9 °C over 12 min and was not compared with a reference. One of the two pressure inputs was connected to a transducer and worked.

H.3. Schematics and Drawings

Figures H.2 to H.6 reproduce the schematics of the board of Section 3.2.4 (design PCB_Project_LM_260407, revision 1.0) as designed and fabricated; the reworks are not drawn (CS4 moved from PA4 to PH1, PA4 wired to the 2.5 V reference, and the ring signal to IN1N cut). Figures H.7 and H.8 are the fabrication and assembly drawings. The board has four copper layers, a total thickness of 1.61 mm, and 741 plated holes.

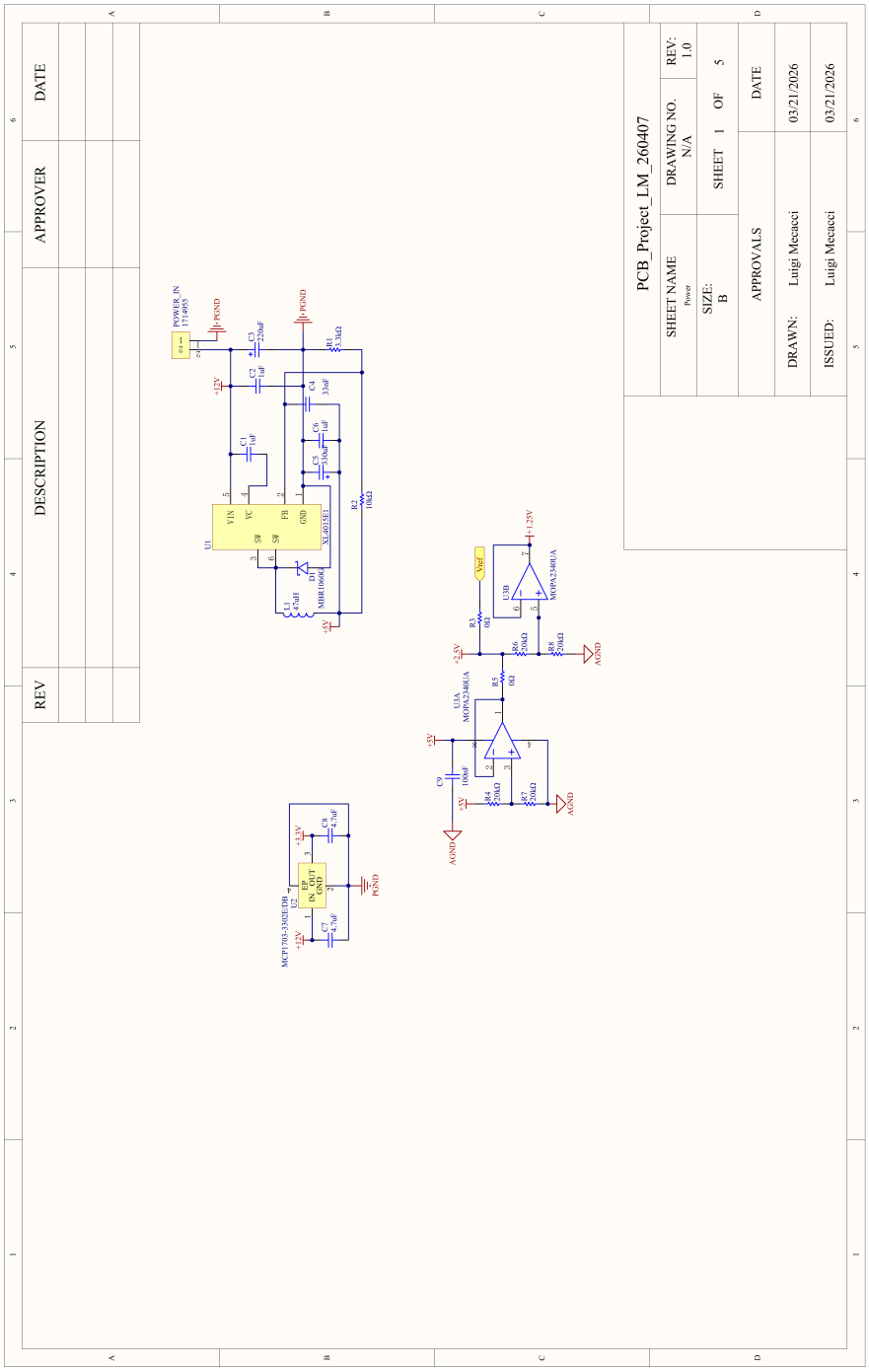

FIGURE H.2. Schematic of the integrated control board, sheet 1 of 5. Power: the 12 V input, the buck converter from 12 to 5 V (XL4015), the linear regulator from 12 to 3.3 V (MCP1703), and the buffered 2.5 V and 1.25 V references.

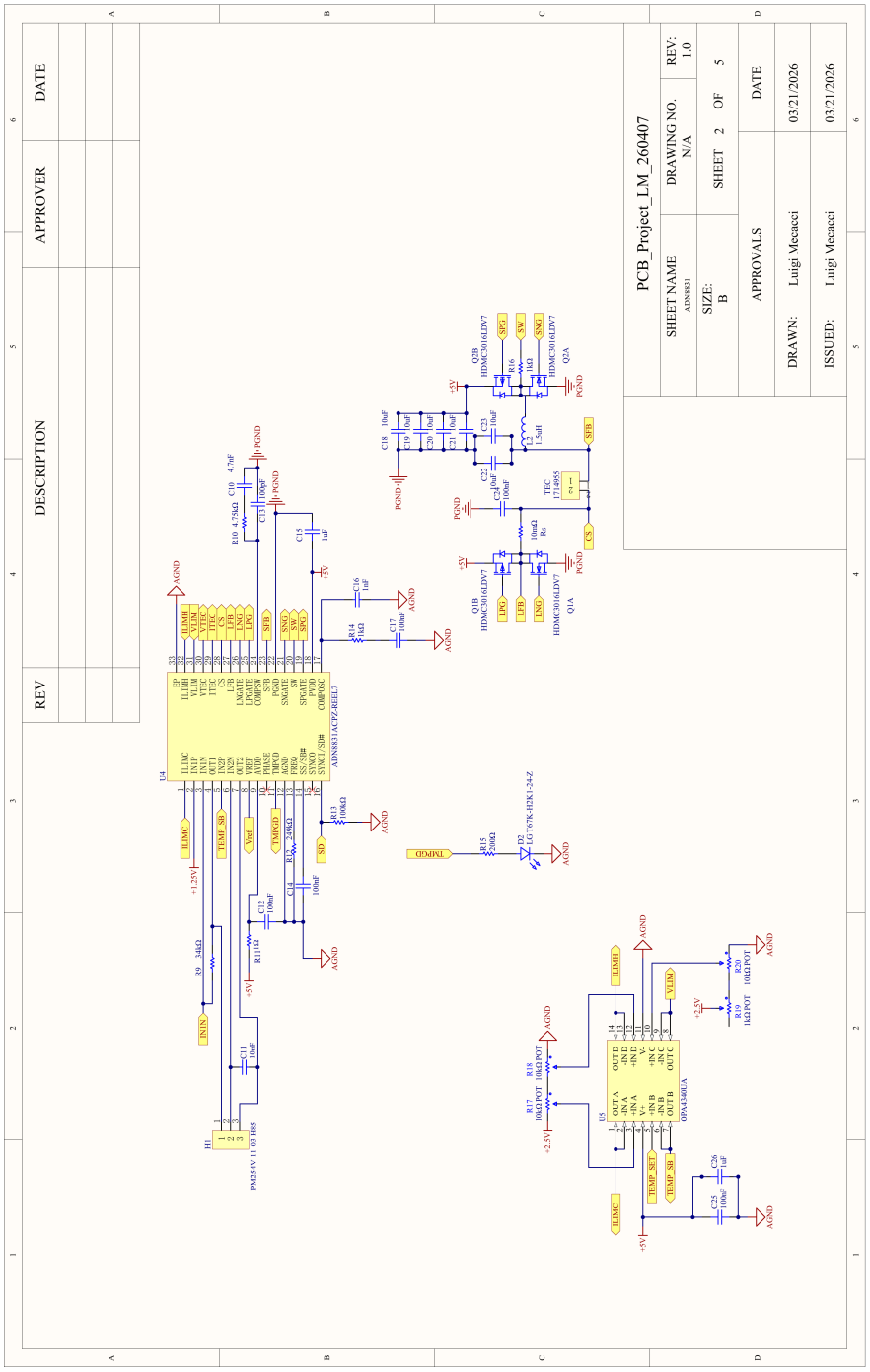

FIGURE H.3. Schematic of the integrated control board, sheet 2 of 5. Thermoelectric-module controller (ADN8831), its output stage and H-bridge of four MOSFETs with the current-sense resistor and filter inductor, and the trimmers that set the current and voltage limits.

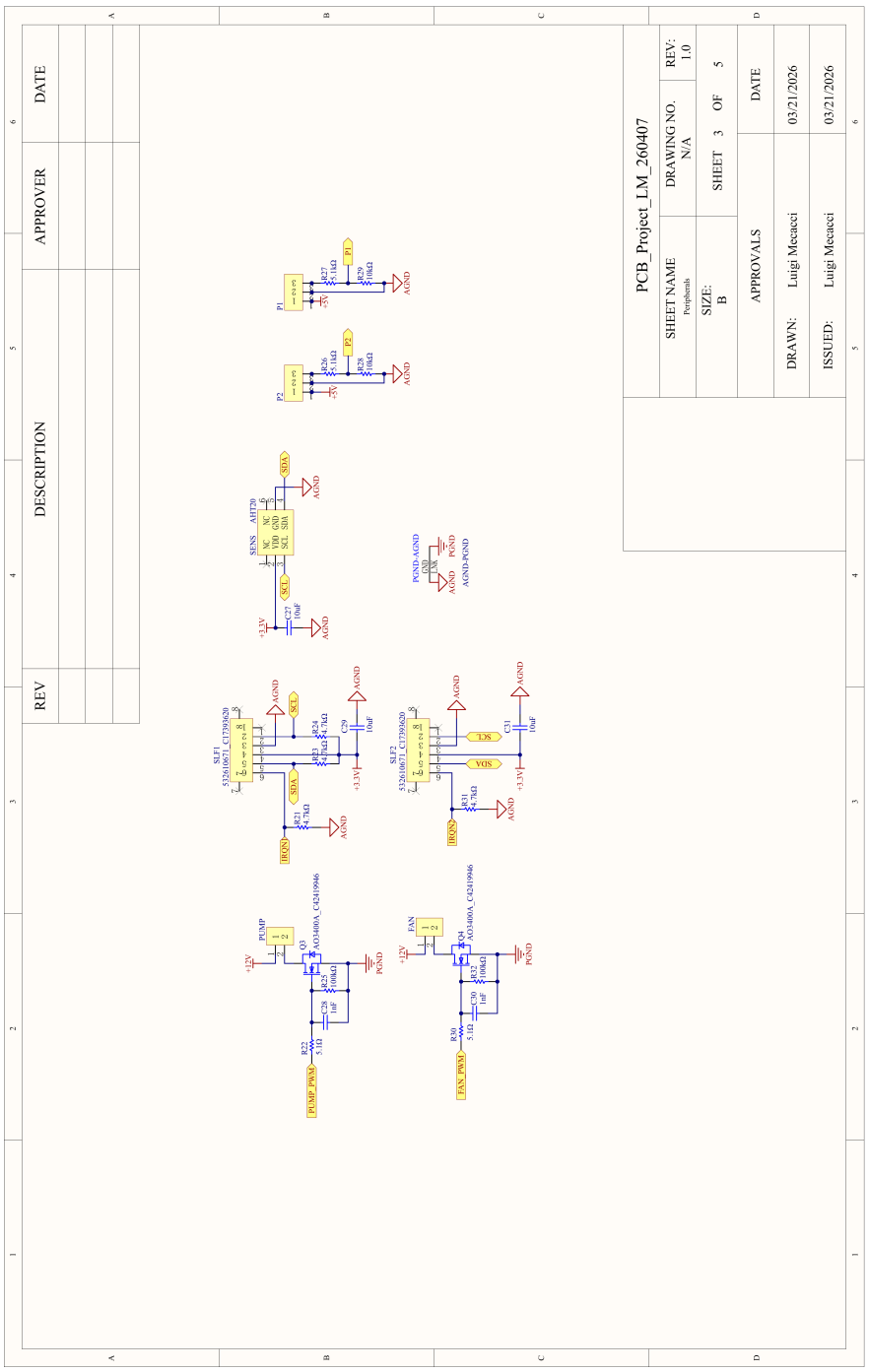

FIGURE H.4. Schematic of the integrated control board, sheet 3 of 5. Peripherals: the low-side drivers of the pump and fan, the two coolant-flow sensors, the ambient temperature and humidity sensor, and the inputs of the two pressure transducers.

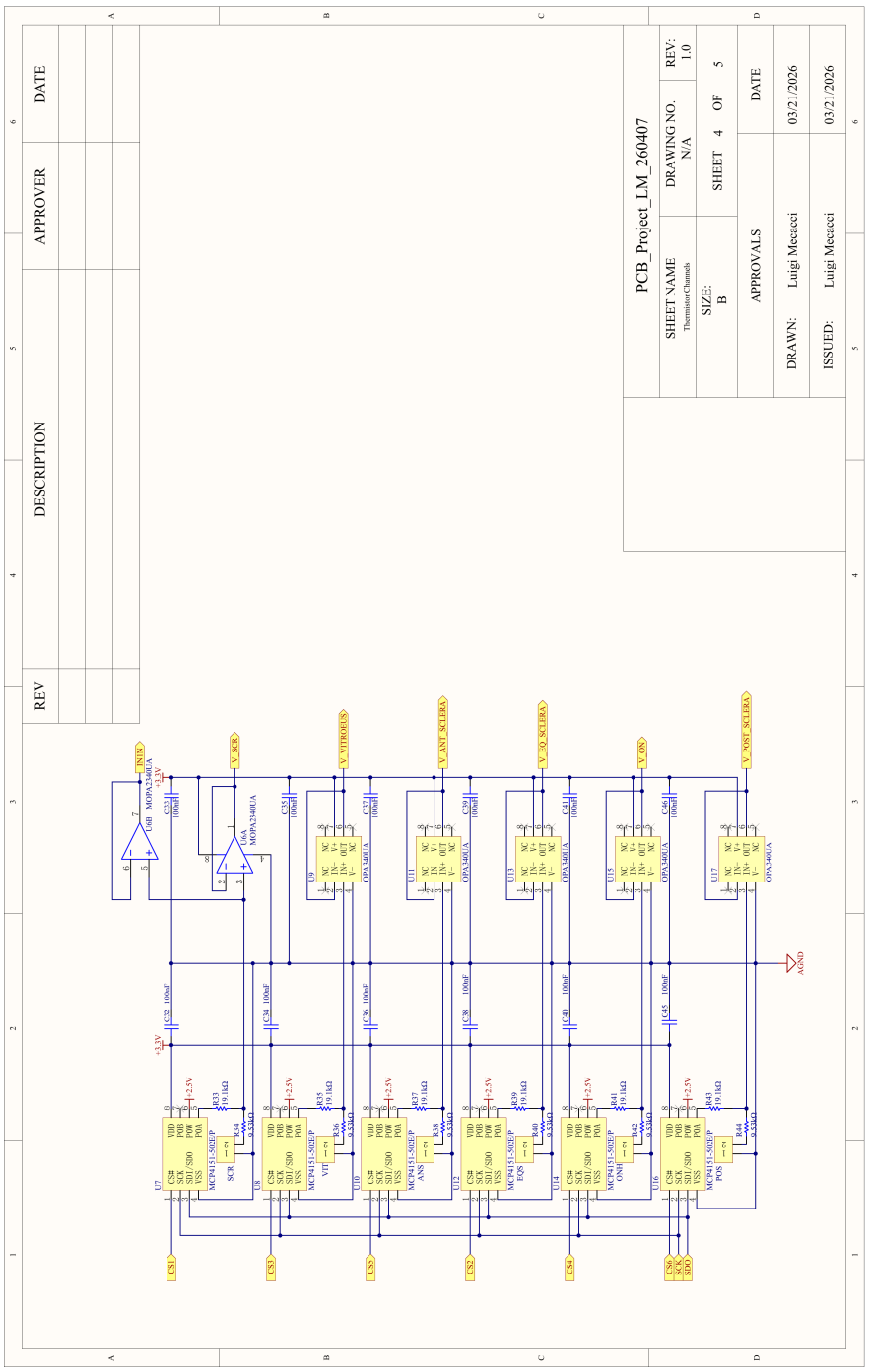

FIGURE H.5. Schematic of the integrated control board, sheet 4 of 5. Thermistor channels: for each of the six thermistors, a digital potentiometer forming the divider from the 2.5 V reference and a buffer amplifier feeding the converter of the microcontroller.

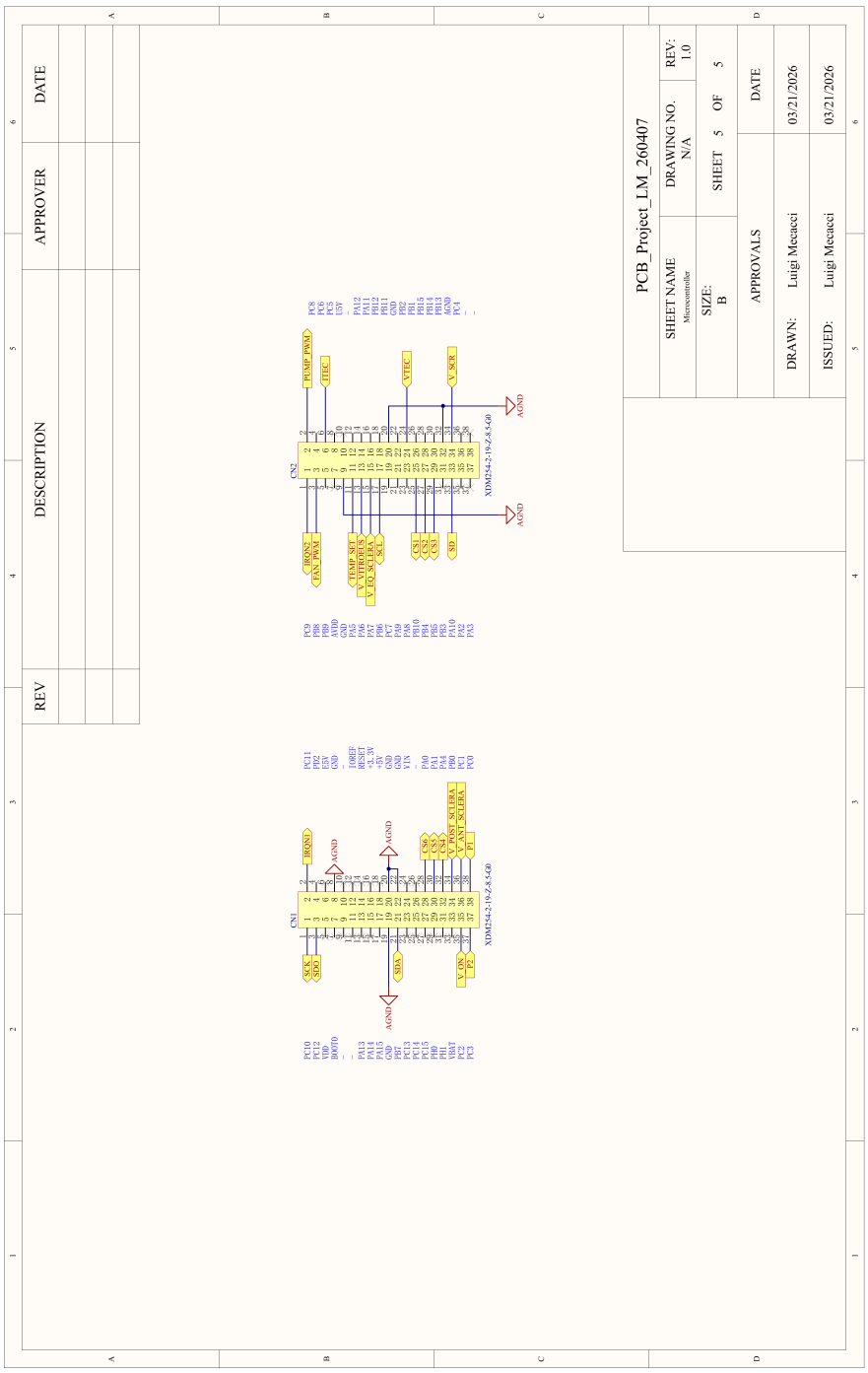

FIGURE H.6. Schematic of the integrated control board, sheet 5 of 5. Microcontroller interface: the connections of the board to the microcontroller development board through its expansion headers.

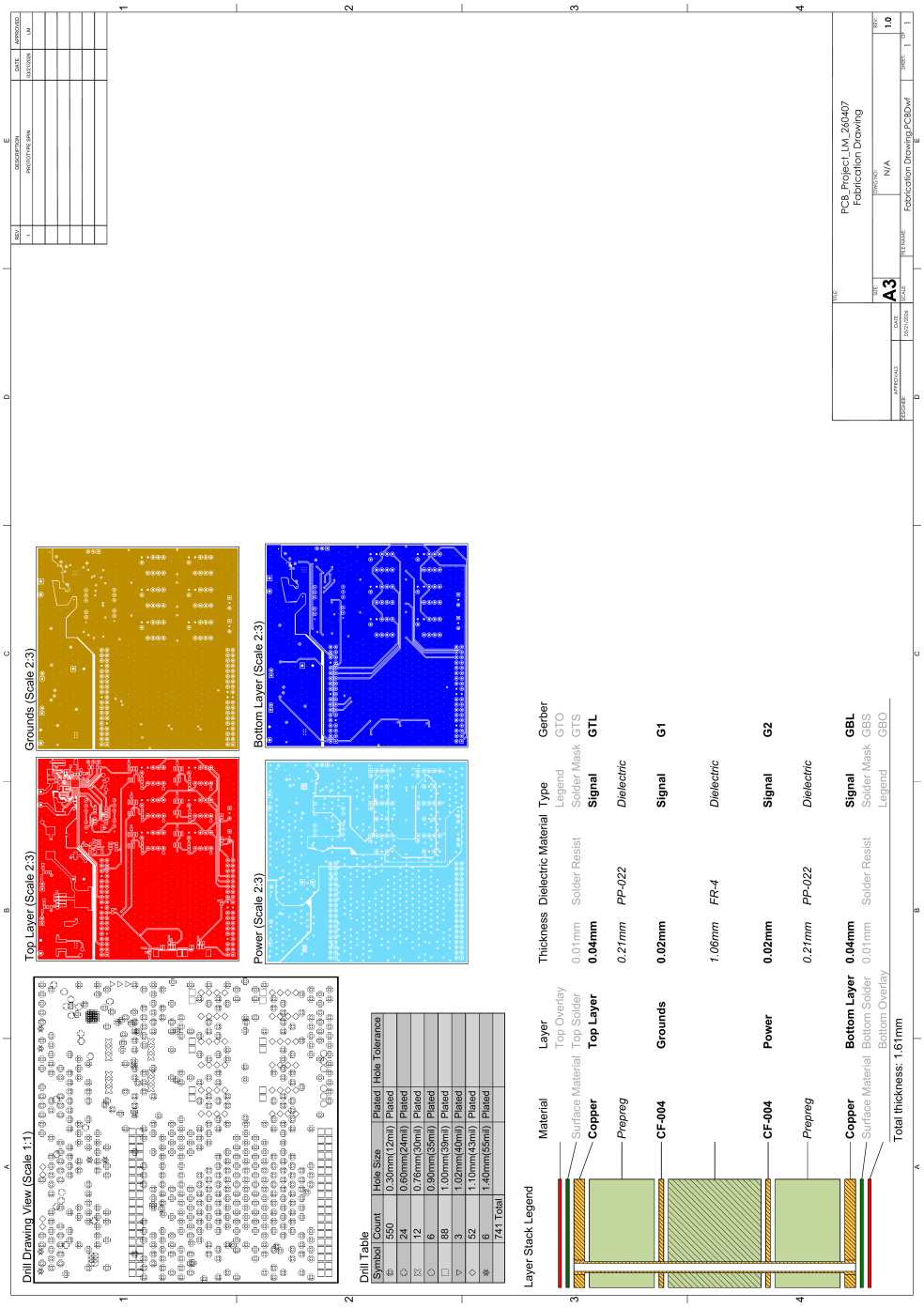

FIGURE H.7. Fabrication drawing of the integrated control board: drill drawing and drill table, the four copper layers (top, ground, power, bottom), and the layer stack, 1.61 mm in total.

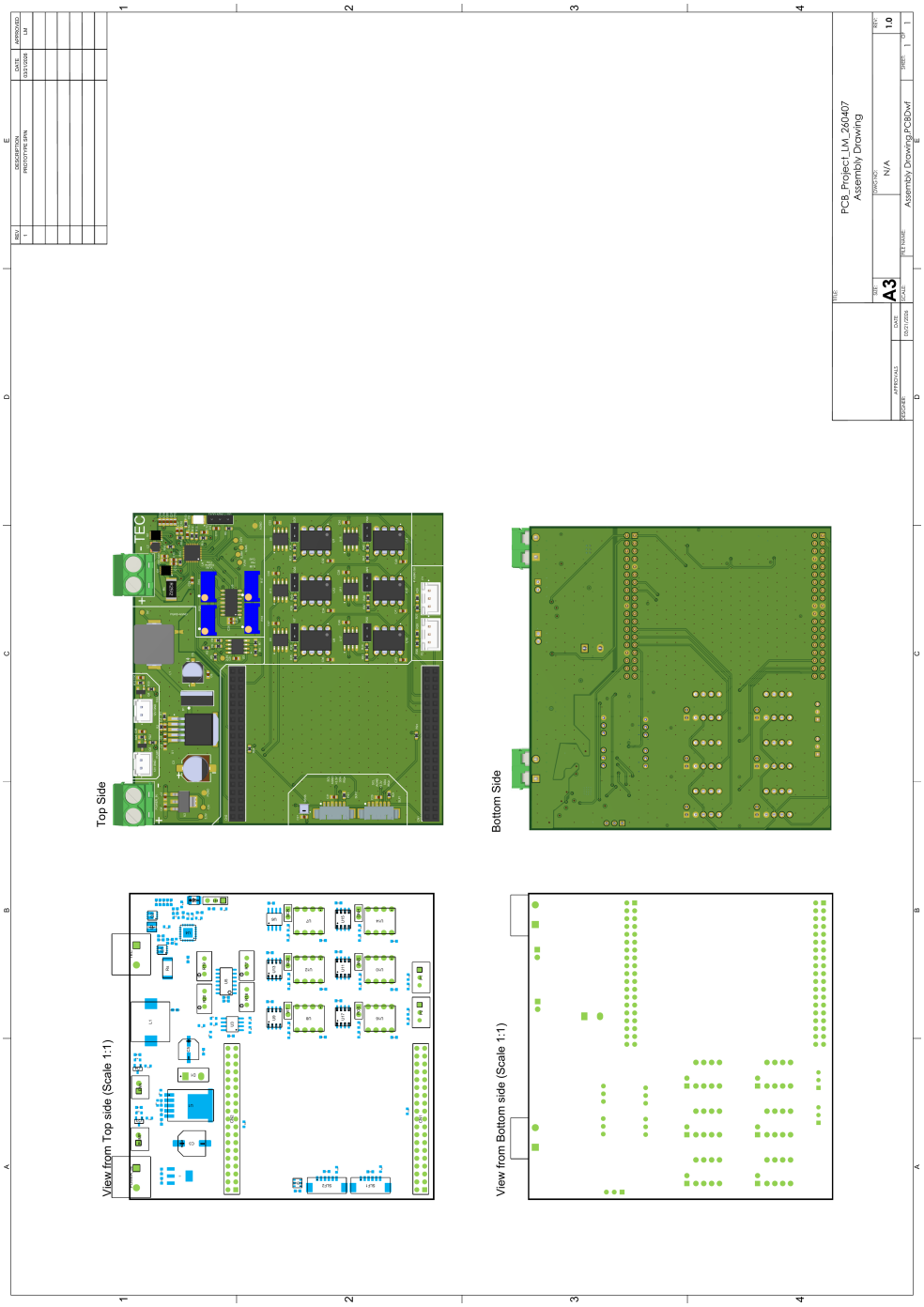

FIGURE H.8. Assembly drawing of the integrated control board: component placement on the top and bottom sides at full scale (left) and rendered views of the populated board (right).

APPENDIX I

Supplementary Material

I.1. Paired Differences between Rings in the Donor Eyes

Table I.1 gives the values drawn in Figure 3.7 (Sections 3.3.2 and 3.3.3).

TABLE I.1. Paired differences in cooling excursion between rings in human donor eyes, in °C: mean difference and 90% confidence interval. Eyes, number of eyes with a valid pair.

		Aluminum − ceramic		**Original − reduced-bulk**	
Site	**Eyes**	**Mean**	**90% interval**	**Mean**	**90% interval**
SCR	9	+0.16	−0.41 to +0.73	+0.49	−0.35 to +1.34
Anterior sclera	8	−1.49	−3.54 to +0.56	+2.26	−0.66 to +5.18
Equatorial sclera	8	−0.90	−2.47 to +0.68	+1.39	+0.23 to +2.54
Vitreous	4	−1.71	−4.34 to +0.93	−0.46	−2.91 to +1.98
Posterior sclera	8	−1.26	−2.40 to −0.11	+0.68	−0.21 to +1.58
Proximal optic nerve	8	−0.20	−0.82 to +0.43	+0.56	−0.09 to +1.20

APPENDIX J

Anesthesia Records

Table J.1 gives the drugs and doses of the BRL anesthesia record of each session (Section 4.2.3). After rocuronium the animal was weaned from the ventilator until spontaneous breathing resumed, and normal respiration and oxygen saturation were confirmed before it was returned to its housing.

TABLE J.1. Drugs given in the sessions, from the BRL anesthesia records. A, animal; S, session; IM, intramuscular; IV, intravenous.

Drug	Dose	Route	Given in
Ketamine and medetomidine (20 mg/mL)	Protocol 10 and 0.08 mg/kg; recorded 5 and 0.2 mg/kg in A2; up to 13.6 and 0.32 mg/kg in A1 S3	IM	All others
Tiletamine–zolazepam, xylazine, butorphanol (100, 40, 6 mg/mL)	0.8 mL, 4.4 mg/kg of the combined drugs	IM	A1 S1
Tiletamine–zolazepam with xylazine (300 mg/mL combined)	1.4 mL (14.7 mg/kg) and 0.7 mL (5.8 mg/kg)	IM	S4 of A1 and A4
Isoflurane	1–2%	Inhaled	All
Rocuronium (10 mg/mL)	20 mg, then 10 mg about every 30 min; every 20 min in A2 S4, up to 60 min in A2 S2	IV	From A2 onward
Sugammadex	2 mg/kg (0.7 mL in A2 S1, S2 and A4 S1); none recorded in the other sessions	–	Session end
Flunixin	Protocol 2.2 mg/kg; 3.7 mg/kg recorded in A2 S1	IM, IV	All
Propofol	1.5 and about 1.8 mg/kg, when premedication did not allow intubation	IV	A1 S1, S3
Hydromorphone	Two doses	–	A1 S1
Lidocaine and ketamine infusion	Adjunct to isoflurane	IV	A2 S3, S4; all of A3 and A4

APPENDIX K

Selected Ocular Surface Photographs

This appendix gives, for every session, the photographs selected for grading: redness (top) and fluorescein staining (bottom), of the test and control eyes, before and after the session, with the grades given to each, on the Efron scale from 0 to 4 (Section 4.2.8). All grades are the author's. Animal 1 is exploratory.

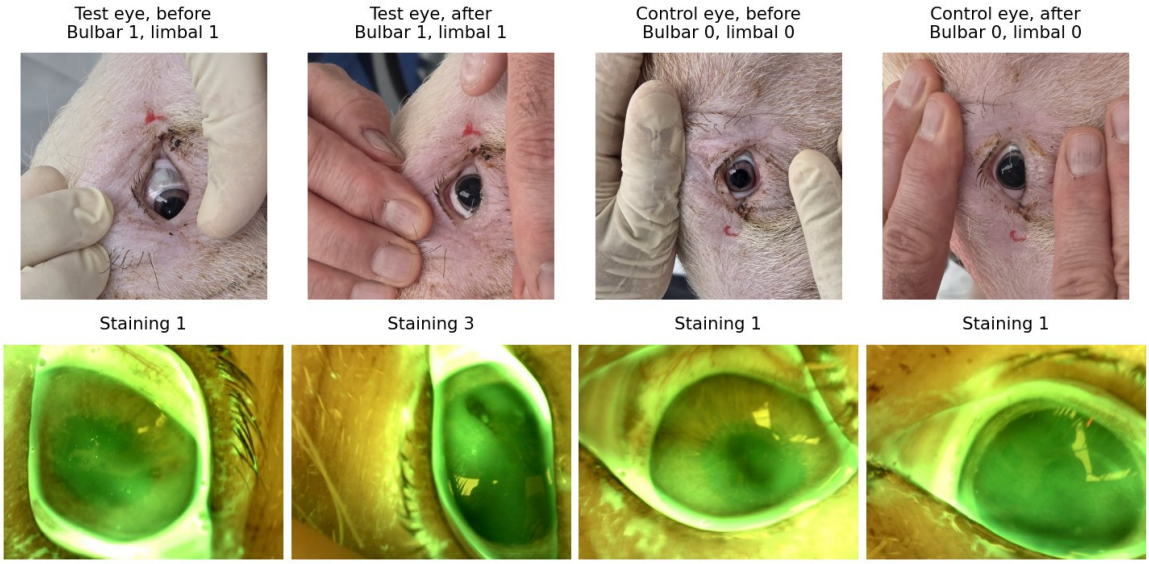

FIGURE K.1. Selected photographs of animal 1, session 1 (30-min session), with the grades of record.

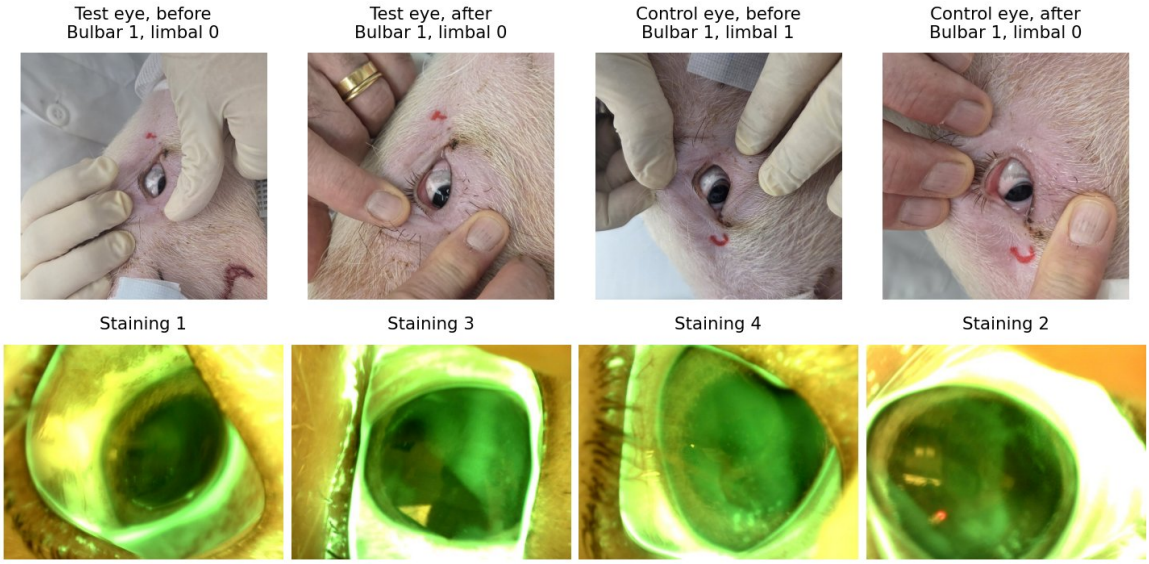

FIGURE K.2. Selected photographs of animal 1, session 2 (60-min session), with the grades of record.

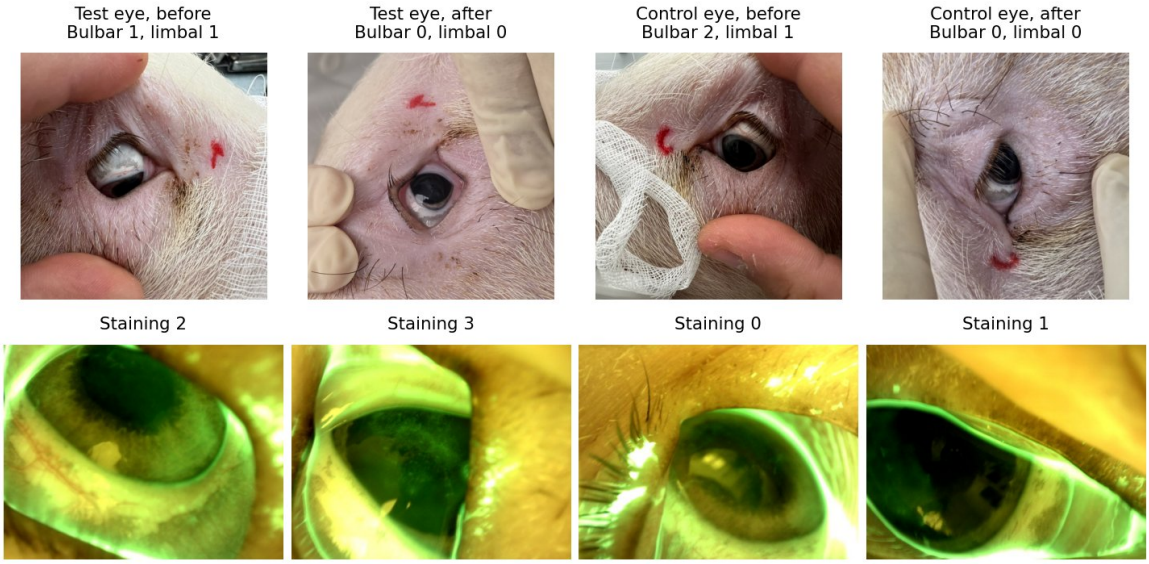

FIGURE K.3. Selected photographs of animal 1, session 3 (120-min session), with the grades of record.

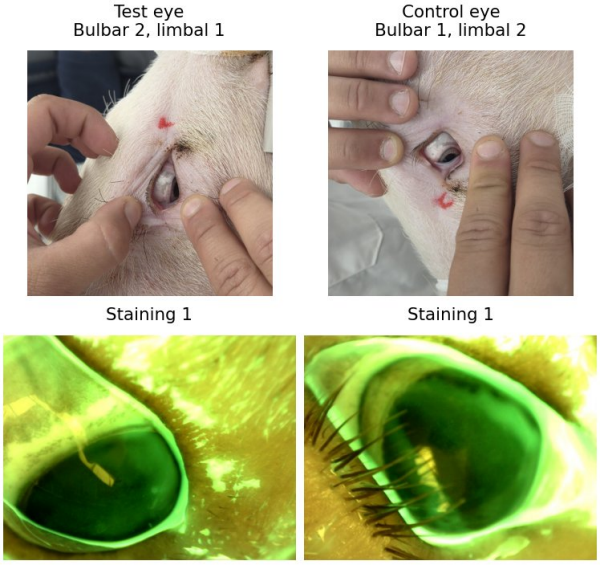

FIGURE K.4. Selected photographs of animal 1, session 4 (end-of-life session), with the grades of record.

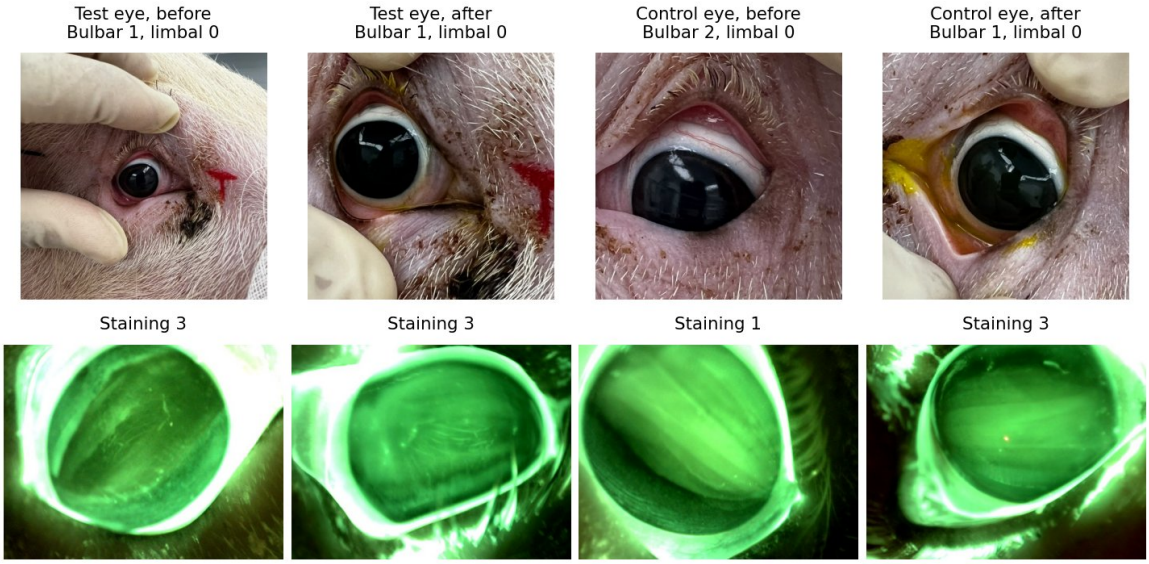

FIGURE K.5. Selected photographs of animal 2, session 1 (30-min session), with the grades of record.

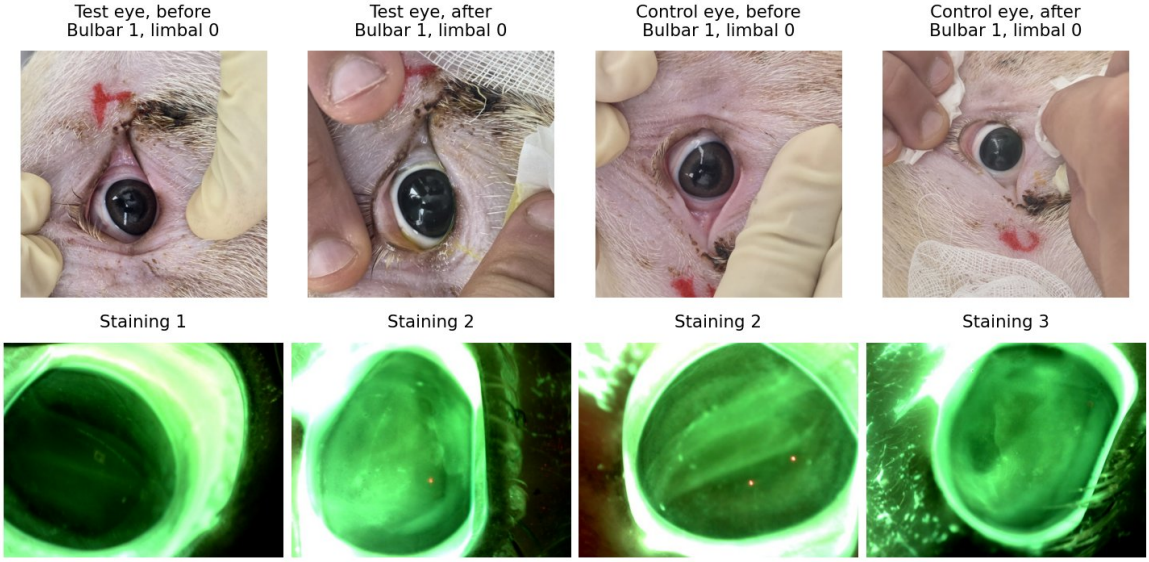

FIGURE K.6. Selected photographs of animal 2, session 2 (60-min session), with the grades of record.

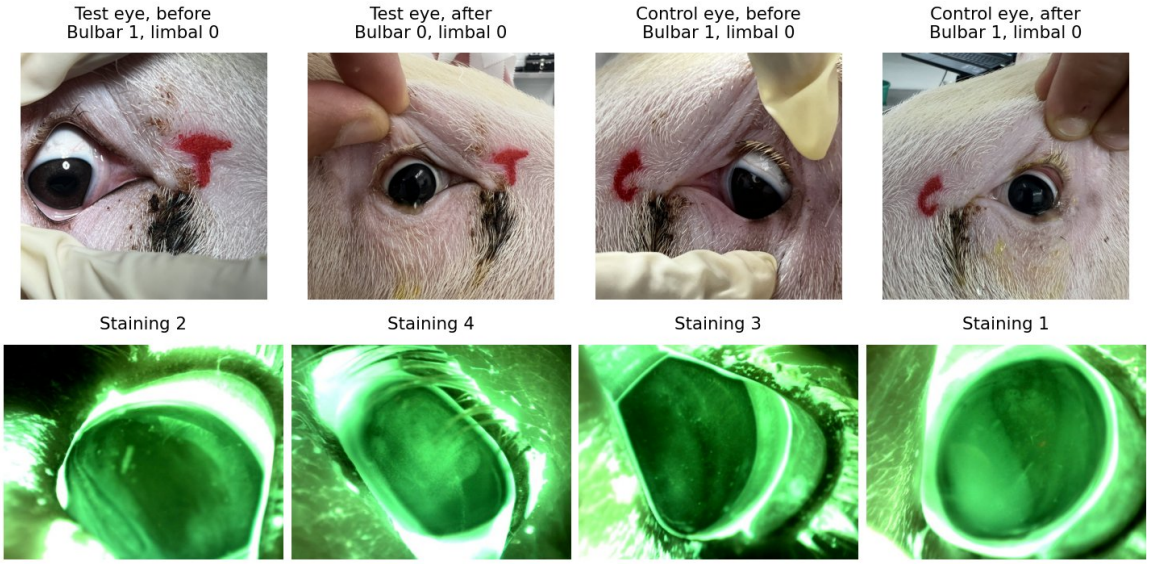

FIGURE K.7. Selected photographs of animal 2, session 3 (120-min session), with the grades of record.

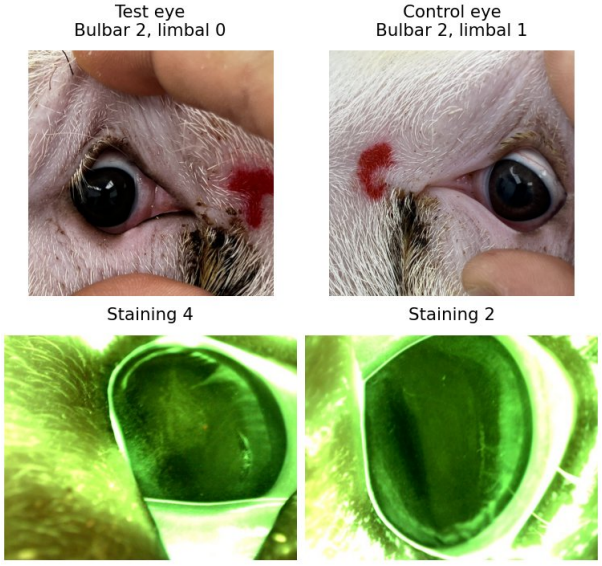

FIGURE K.8. Selected photographs of animal 2, session 4 (end-of-life session), with the grades of record.

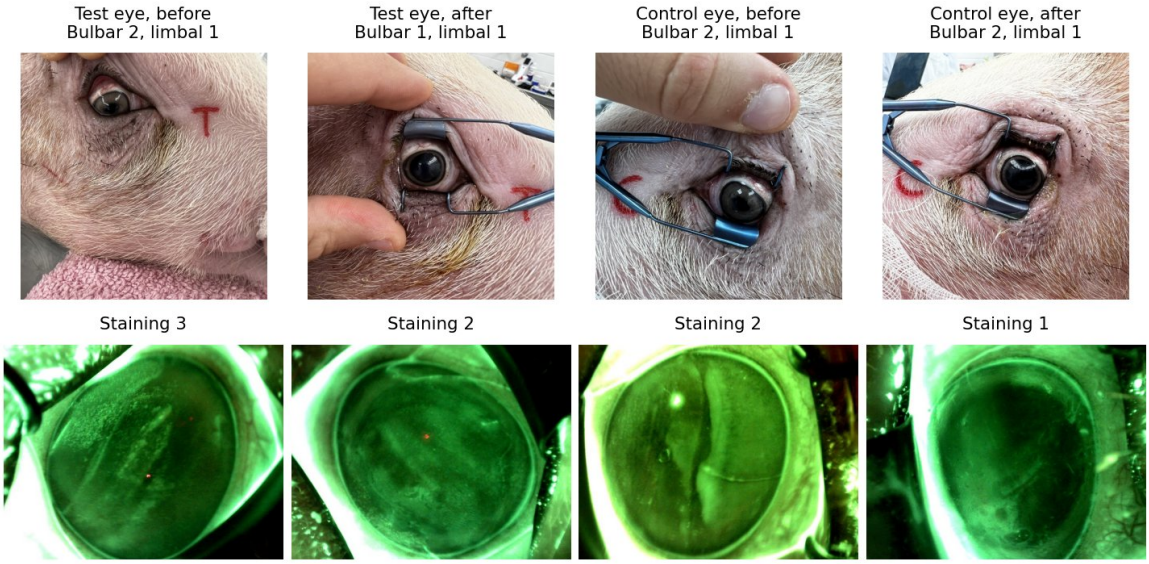

FIGURE K.9. Selected photographs of animal 3, session 1 (30-min session), with the grades of record.

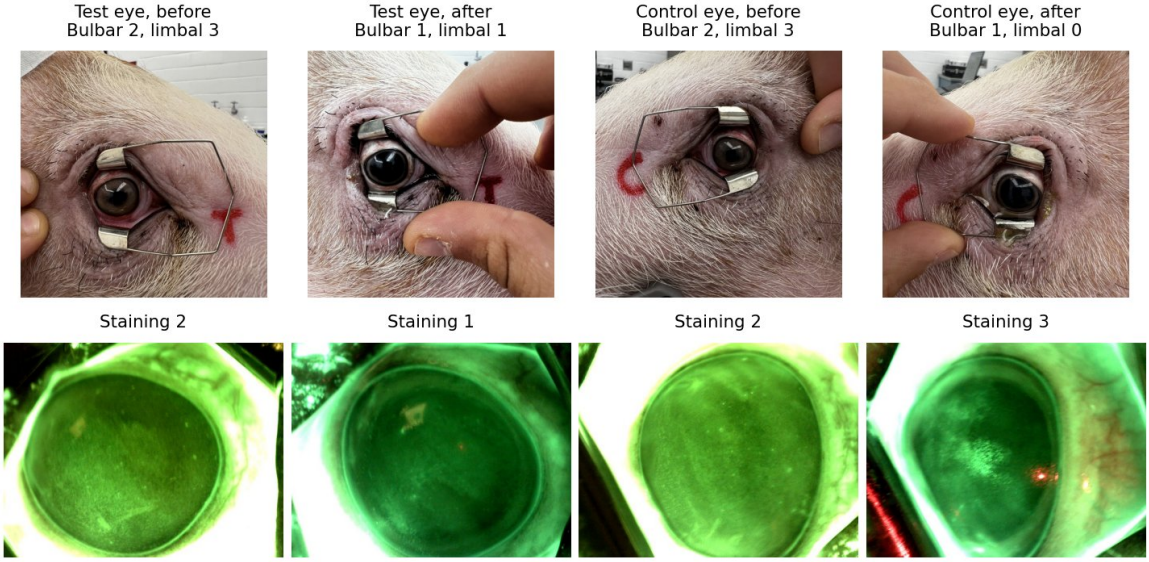

FIGURE K.10. Selected photographs of animal 3, session 2 (60-min session), with the grades of record.

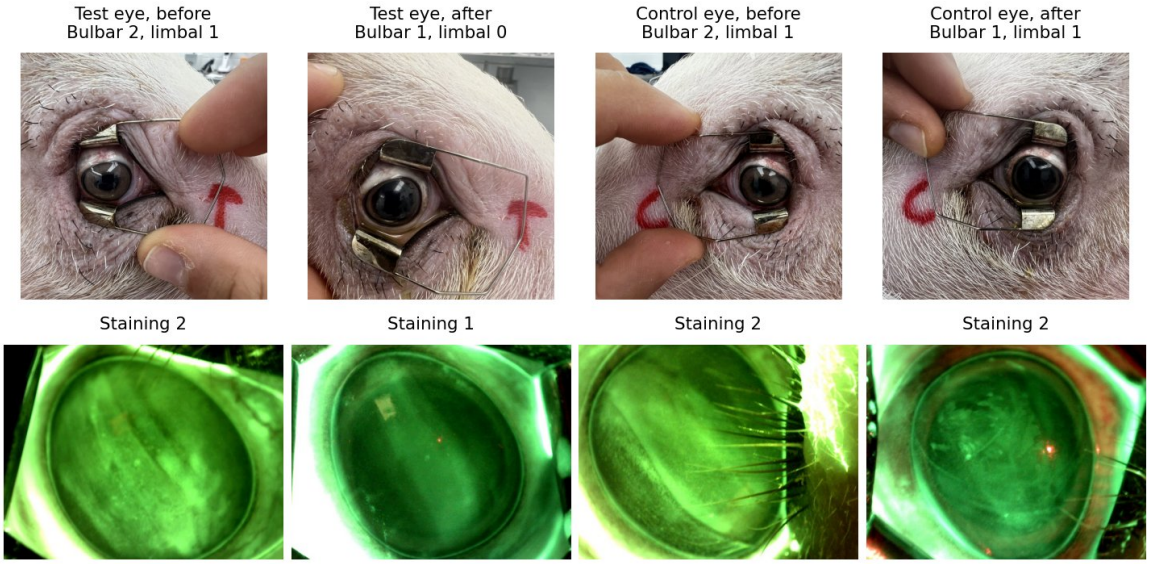

FIGURE K.11. Selected photographs of animal 3, session 3 (120-min session), with the grades of record.

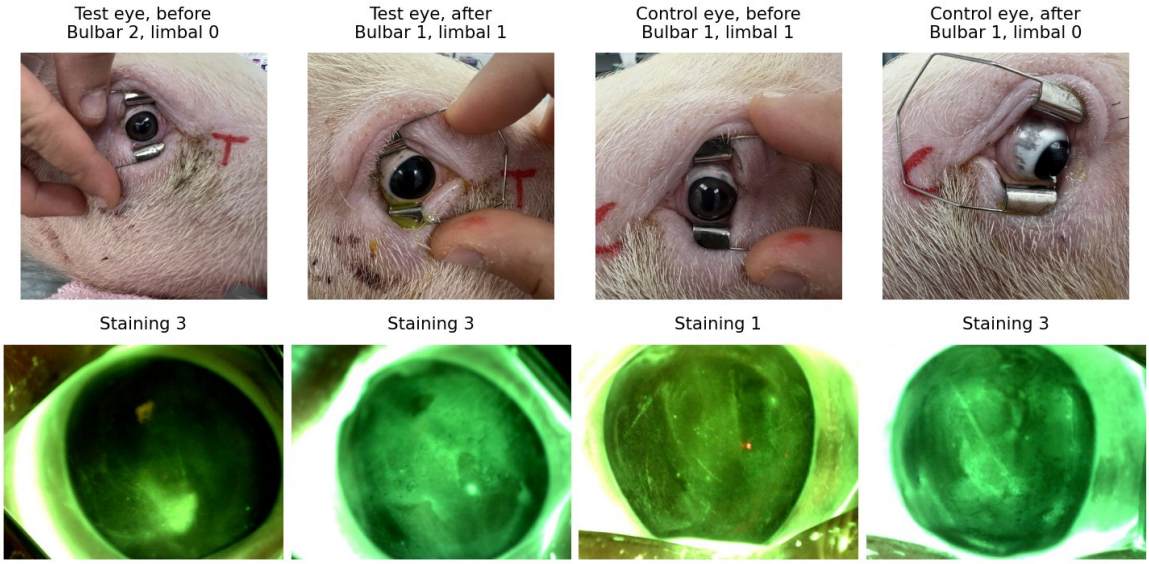

FIGURE K.12. Selected photographs of animal 4, session 1 (30-min session), with the grades of record.

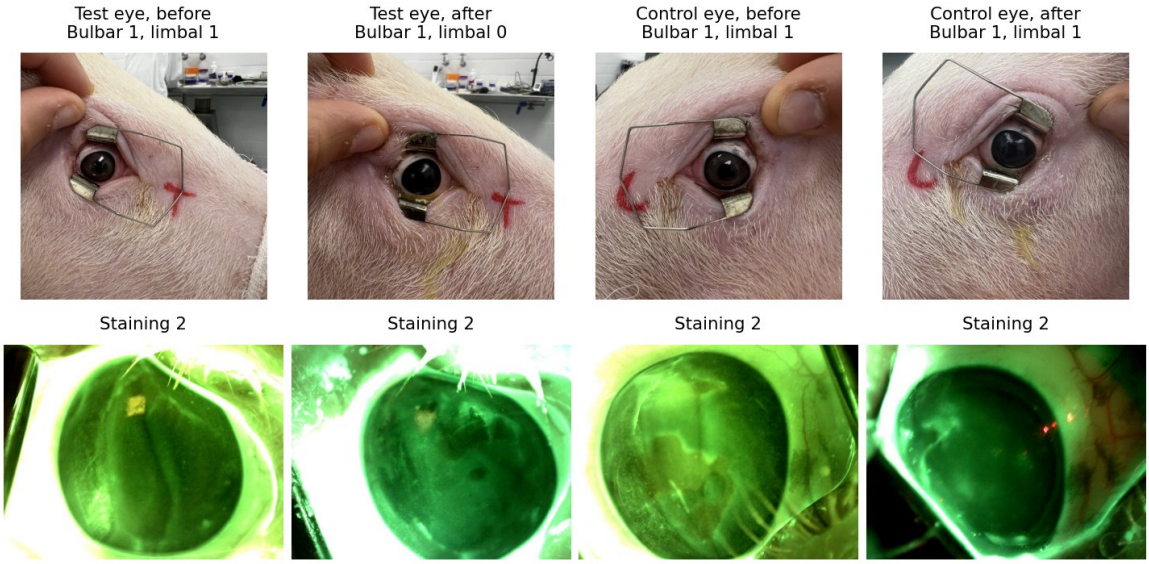

FIGURE K.13. Selected photographs of animal 4, session 2 (60-min session), with the grades of record.

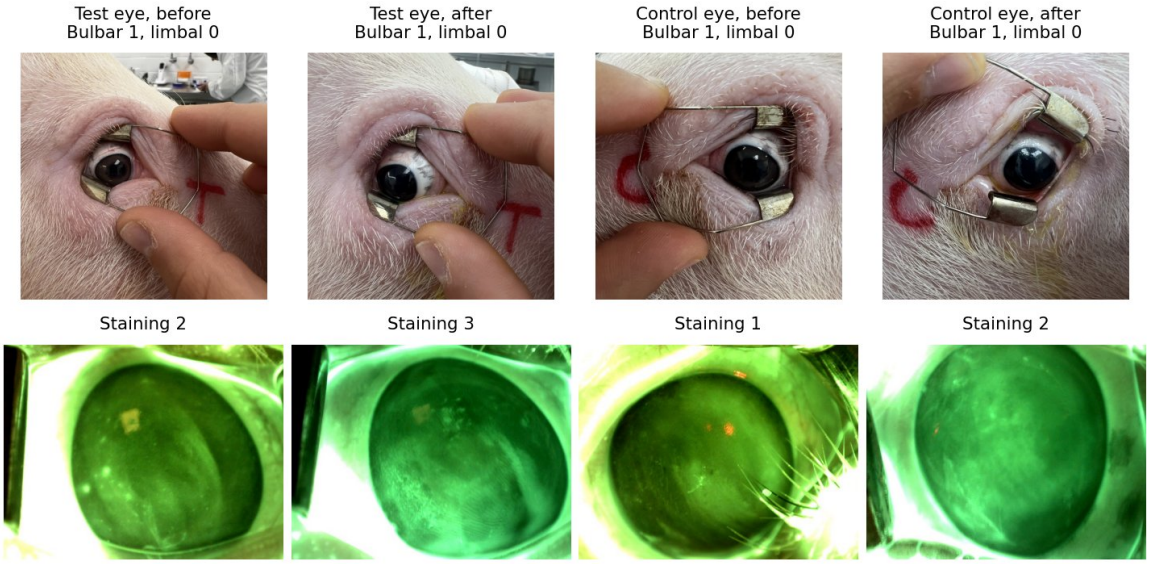

FIGURE K.14. Selected photographs of animal 4, session 3 (120-min session), with the grades of record.

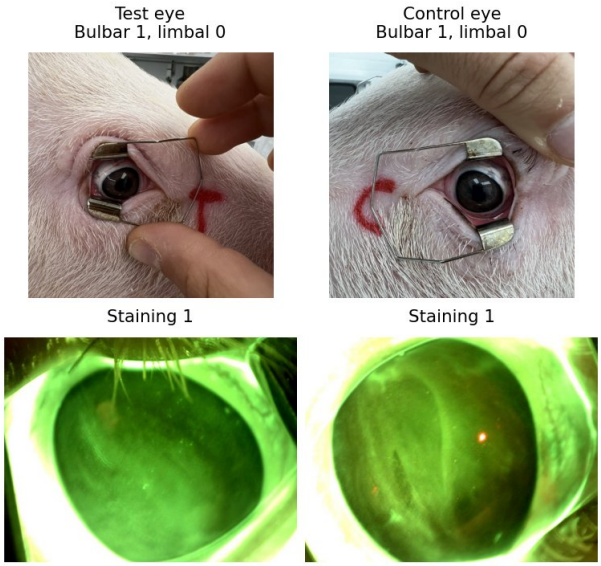

FIGURE K.15. Selected photographs of animal 4, session 4 (end-of-life session), with the grades of record.

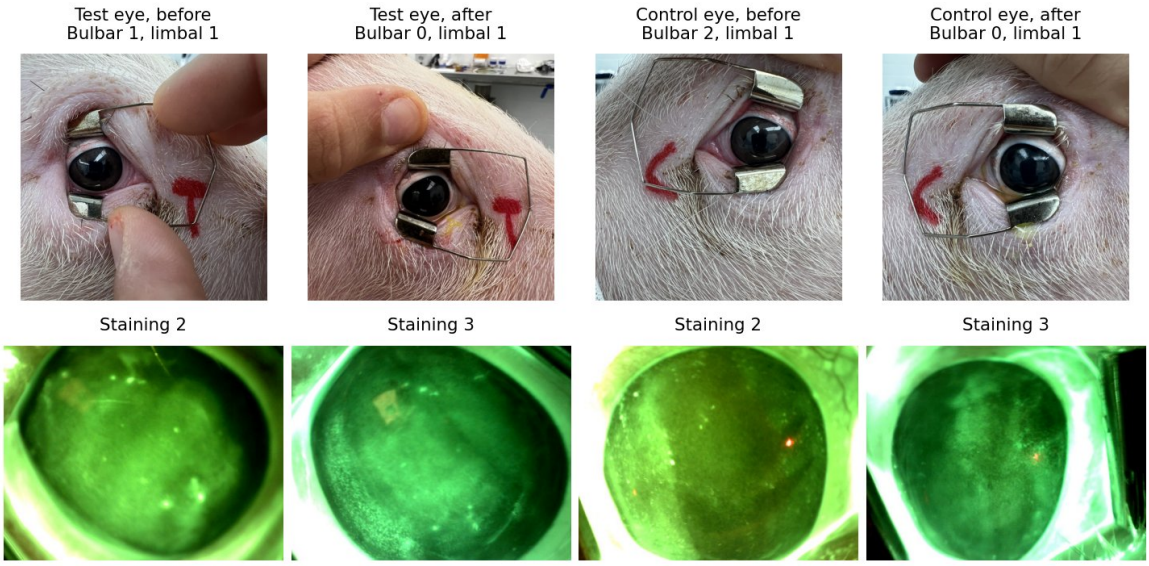

FIGURE K.16. Selected photographs of animal 5, session 1 (30-min session), with the grades of record.

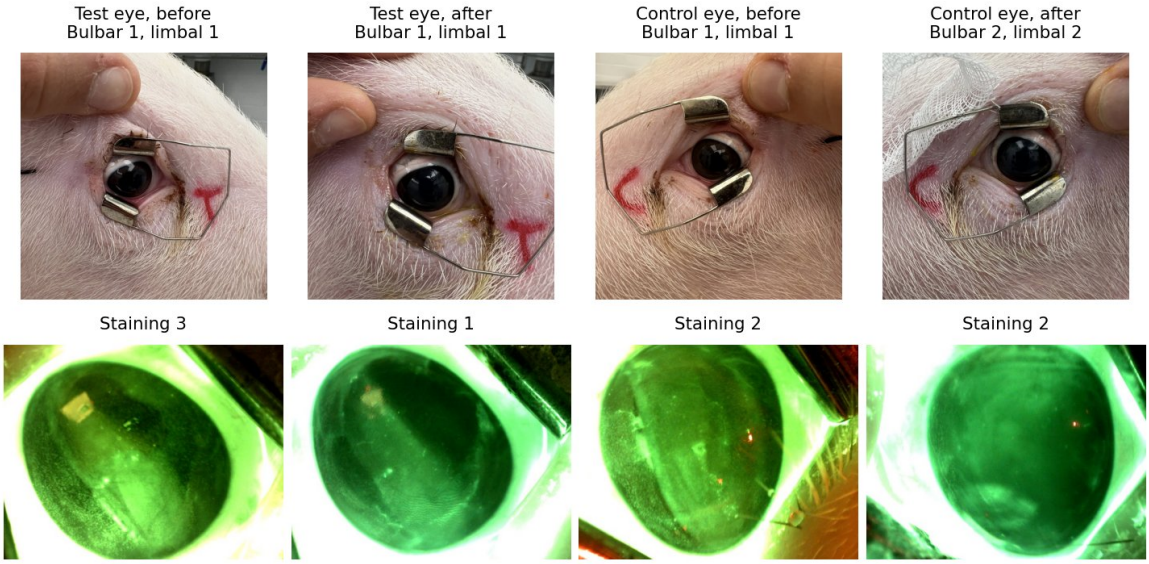

FIGURE K.17. Selected photographs of animal 5, session 2 (60-min session), with the grades of record.

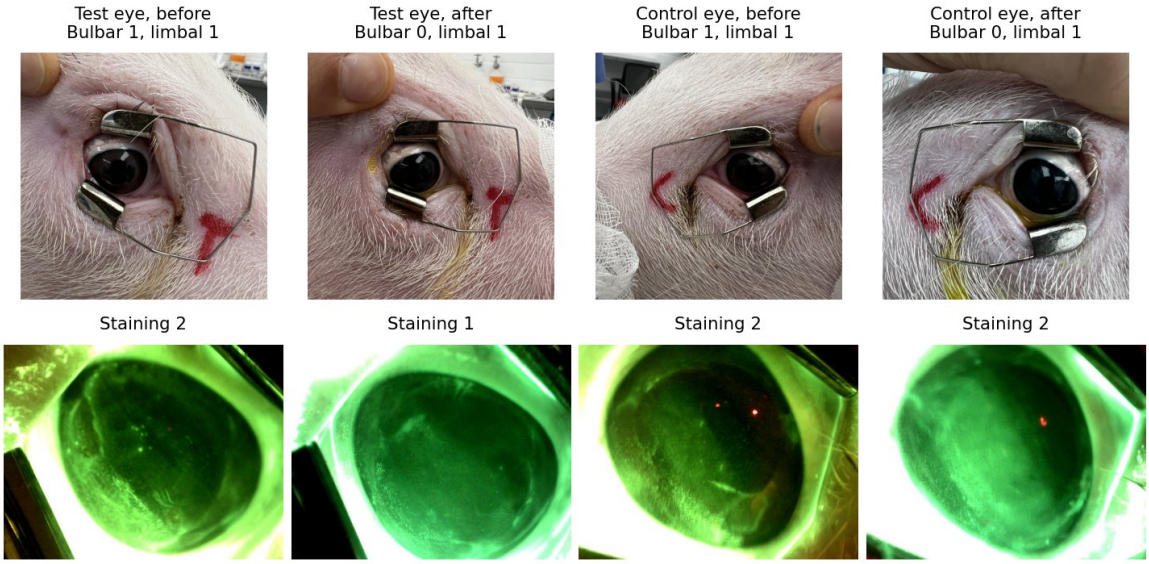

FIGURE K.18. Selected photographs of animal 5, session 3 (120-min session), with the grades of record.

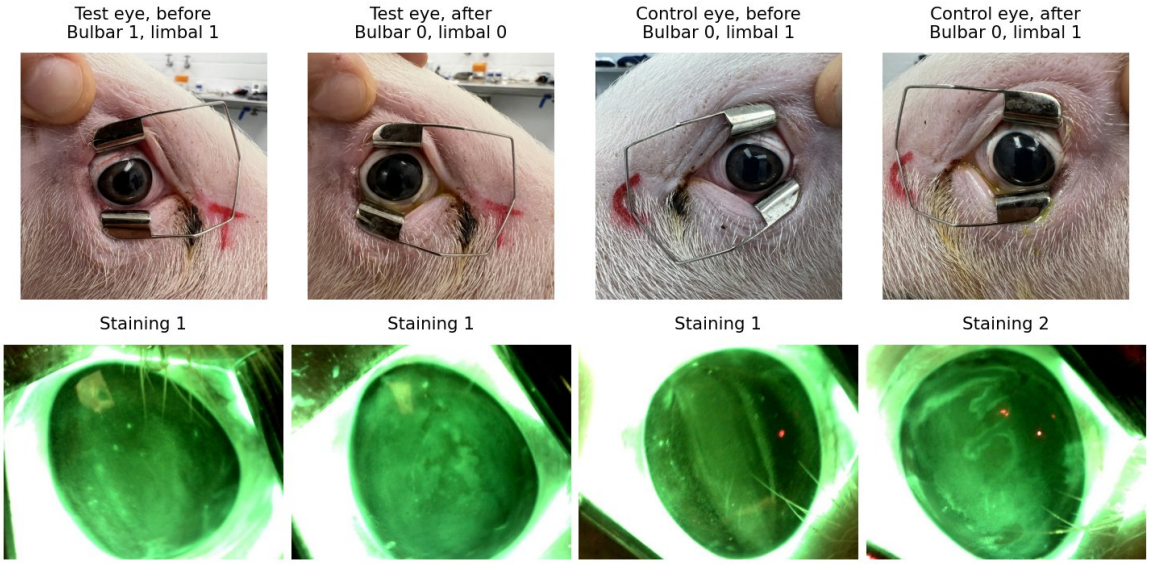

FIGURE K.19. Selected photographs of animal 6, session 1 (30-min session), with the grades of record.

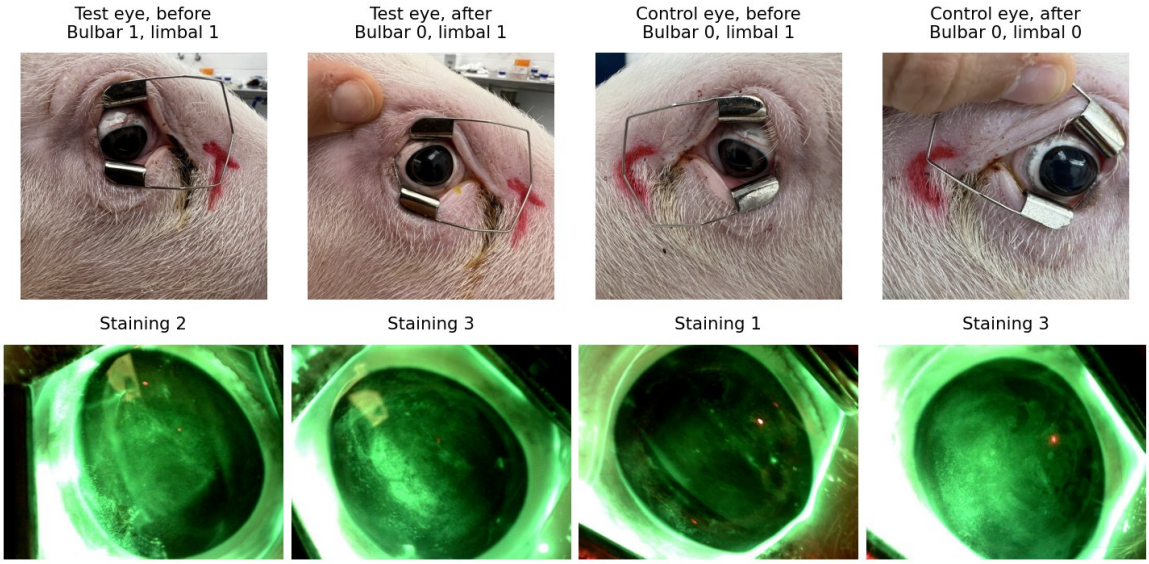

FIGURE K.20. Selected photographs of animal 6, session 2 (60-min session), with the grades of record.

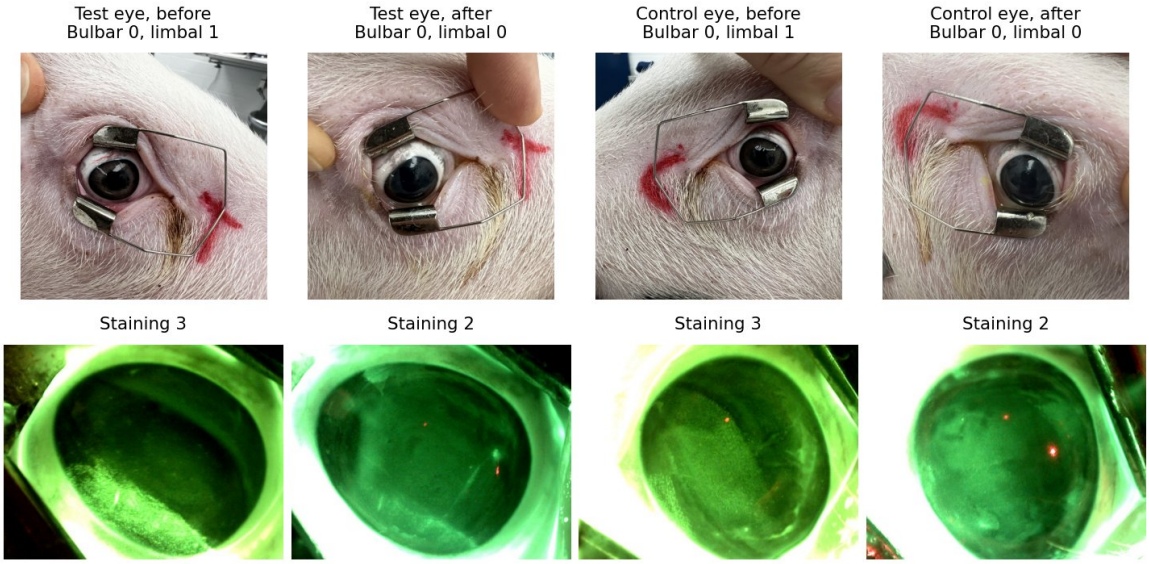

FIGURE K.21. Selected photographs of animal 6, session 3 (120-min session), with the grades of record.

Cited Literature

[1] Y. Nakamura, L. Mecacci, and J. R. Hetling, "Therapeutic levels of hypothermia achieved in isolated porcine eyes using a scleral contact interface," *Annals of Biomedical Engineering*, vol. 54, no. 2, pp. 570–579, 2026 (published online 19 Nov. 2025). doi:10.1007/s10439-025-03905-w.

[2] B. Mac Grory, M. Schrag, V. Biousse, K. L. Furie, M. Gerhard-Herman, P. J. Lavin, L. Sobrin, S. I. Tjoumakaris, C. M. Weyand, and S. Yaghi, "Management of central retinal artery occlusion: A scientific statement from the American Heart Association," *Stroke*, vol. 52, no. 6, pp. e282–e294, 2021. doi:10.1161/STR.0000000000000366.

[3] J. A. Leavitt, T. A. Larson, D. O. Hodge, and R. E. Gullerud, "The incidence of central retinal artery occlusion in Olmsted County, Minnesota," *American Journal of Ophthalmology*, vol. 152, no. 5, pp. 820–823.e2, 2011. doi:10.1016/j.ajo.2011.05.005.

[4] J. Kim, S. J. Byun, S. J. Woo, K. H. Park, and S. J. Park, "Assessment of trends in the incidence rates of central retinal artery occlusion in Korea from 2002 to 2015," *JAMA Ophthalmology*, vol. 139, no. 4, pp. 399–405, 2021. doi:10.1001/jamaophthalmol.2020.6860.

[5] D. D. Hwang, K. E. Lee, Y. Kim, M. S. Kim, T. H. Rim, M. Kim, H. Kim, D. S. Kyoung, and J. I. Park, "Incidence of retinal artery occlusion and related mortality in Korea, 2005 to 2018," *JAMA Network Open*, vol. 6, no. 3, e233068, 2023. doi:10.1001/jamanetworkopen.2023.3068.

[6] S. S. Hayreh and M. B. Zimmerman, "Central retinal artery occlusion: Visual outcome," *American Journal of Ophthalmology*, vol. 140, no. 3, pp. 376–391, 2005. doi:10.1016/j.ajo.2005.03.038.

[7] M. Schrag, T. Youn, J. Schindler, H. Kirshner, and D. Greer, "Intravenous fibrinolytic therapy in central retinal artery occlusion: A patient-level meta-analysis," *JAMA Neurology*, vol. 72, no. 10, pp. 1148–1154, 2015. doi:10.1001/jamaneurol.2015.1578.

[8] S. Prabhakaran, N. R. Gonzalez, K. S. Zachrison, et al., "2026 guideline for the early management of patients with acute ischemic stroke: A guideline from the American Heart Association/American Stroke Association," *Stroke*, vol. 57, pp. e316–e436, 2026. doi:10.1161/STR.0000000000000513.

[9] M. Schumacher *et al.* (EAGLE Study Group), "Central retinal artery occlusion: Local intra-arterial fibrinolysis versus conservative treatment, a multicenter randomized trial," *Ophthalmology*, vol. 117, no. 7, pp. 1367–1375.e1, 2010. doi:10.1016/j.ophtha.2010.03.061.

[10] C. Préterre *et al.* (THEIA collaborators), "Intravenous alteplase versus oral aspirin for acute central retinal artery occlusion within 4.5 h of severe vision loss (THEIA): A multicentre, double-dummy, patient-blinded and assessor-blinded, randomised, controlled, phase 3 trial," *The Lancet Neurology*, vol. 24, no. 11, pp. 909–919, 2025. doi:10.1016/S1474-4422(25)00308-4.

[11] S. J. Ryan *et al.* (TenCRAOS Investigators), "A randomized trial of tenecteplase in acute central retinal artery occlusion," *New England Journal of Medicine*, vol. 394, no. 5, pp. 442–450, 2026. doi:10.1056/NEJMoa2508515.

[12] ClinicalTrials.gov, "Early reperfusion therapy with intravenous thrombolysis for recovery of VISION in acute central retinal artery occlusion (REVISION)," registry entry NCT04965038, status Terminated (stopped after prespecified interim analysis due to futility), last updated May 18, 2026, `https://clinicaltrials.gov/study/NCT04965038`, accessed Oct. 28, 2026.

[13] B. Mac Grory, C. Preterre, P. Lebranchu, S. J. Ryan, Ø. Kalsnes Jørstad, M. C. Moe, et al., "Intravenous thrombolysis for acute central retinal artery occlusion: Protocol for a systematic review and individual participant data meta-analysis of randomized controlled trials," *PLoS One*, vol. 21, no. 3, e0342739, 2026. doi:10.1371/journal.pone.0342739.

[14] E. Ardila Jurado *et al.* (Swiss Stroke Registry Investigators), "Central retinal artery occlusion: Current practice, awareness and prehospital delays in Switzerland," *Frontiers in Neurology*, vol. 13, 888456, 2022. doi:10.3389/fneur.2022.888456.

[15] C. Stretz, J. E. Paddock, T. M. Burton, T. Bakaeva, M. Freeman, A. Choudhury, S. Yaghi, K. L. Furie, M. Schrag, and B. Mac Grory, "IV thrombolysis for central retinal artery occlusion: Real-world experience from a comprehensive stroke center," *Journal of Stroke and Cerebrovascular Diseases*, vol. 33, no. 4, 107610, 2024. doi:10.1016/j.jstrokecerebrovasdis.2024.107610.

[16] A. K. Rudkin, A. W. Lee, and C. S. Chen, "Central retinal artery occlusion: Timing and mode of presentation," *European Journal of Neurology*, vol. 16, no. 6, pp. 674–677, 2009. doi:10.1111/j.1468-1331.2009.02616.x.

[17] Y. Shen, M. Drum, and S. Roth, "The prevalence of perioperative visual loss in the United States: A 10-year study from 1996 to 2005 of spinal, orthopedic, cardiac, and general surgery," *Anesthesia & Analgesia*, vol. 109, no. 5, pp. 1534–1545, 2009. doi:10.1213/ane.0b013e3181b0500b.

[18] T. Calway, D. S. Rubin, H. E. Moss, C. E. Joslin, K. Beckmann, and S. Roth, "Perioperative retinal artery occlusion: Risk factors in cardiac surgery from the United States National Inpatient Sample 1998–2013," *Ophthalmology*, vol. 124, no. 2, pp. 189–196, 2017. doi:10.1016/j.ophtha.2016.10.025.

[19] L. A. Lee, S. Roth, K. L. Posner, F. W. Cheney, R. A. Caplan, N. J. Newman, and K. B. Domino, "The American Society of Anesthesiologists Postoperative Visual Loss Registry: Analysis of 93 spine surgery cases with postoperative visual loss," *Anesthesiology*, vol. 105, no. 4, pp. 652–659, 2006. doi:10.1097/00000542-200610000-00007.

[20] Postoperative Visual Loss Study Group, "Risk factors associated with ischemic optic neuropathy after spinal fusion surgery," *Anesthesiology*, vol. 116, no. 1, pp. 15–24, 2012. doi:10.1097/ALN.0b013e31823d012a.

[21] American Society of Anesthesiologists Task Force on Perioperative Visual Loss, North American Neuro-Ophthalmology Society, and Society for Neuroscience in Anesthesiology and Critical Care, "Practice advisory for perioperative visual loss associated with spine surgery 2019," *Anesthesiology*, vol. 130, no. 1, pp. 12–30, 2019. doi:10.1097/ALN.0000000000002503.

[22] L. Michaud, "The impact of scleral lenses on intraocular pressure," *Journal of Clinical Medicine*, vol. 15, no. 4, art. 1635, 2026. doi:10.3390/jcm15041635.

[23] I. Bekerman, P. Gottlieb, and M. Vaiman, "Variations in eyeball diameters of the healthy adults," *Journal of Ophthalmology*, vol. 2014, art. 503645, 2014.

[24] R. C. Augusteyn, D. Nankivil, A. Mohamed, B. Maceo, F. Pierre, and J.-M. Parel, "Human ocular biometry," *Experimental Eye Research*, vol. 102, pp. 70–75, 2012.

[25] L. A. Remington, *Clinical Anatomy and Physiology of the Visual System*, 3rd ed. St. Louis, MO: Elsevier Butterworth-Heinemann, 2012. ISBN 978-1-4377-1926-0.

[26] J. Kur, E. A. Newman, and T. Chan-Ling, "Cellular and physiological mechanisms underlying blood flow regulation in the retina and choroid in health and disease," *Progress in Retinal and Eye Research*, vol. 31, no. 5, pp. 377–406, 2012. doi:10.1016/j.preteyeres.2012.04.004.

[27] M. Ferrara, G. Lugano, M. T. Sandinha, V. R. Kearns, B. Geraghty, and D. H. W. Steel, "Biomechanical properties of retina and choroid: a comprehensive review of techniques and translational relevance," *Eye*, vol. 35, pp. 1818–1832, 2021. doi:10.1038/s41433-021-01437-w.

[28] P. G. Watson and R. D. Young, "Scleral structure, organisation and disease. A review," *Experimental Eye Research*, vol. 78, no. 3, pp. 609–623, 2004. doi:10.1016/S0014-4835(03)00212-4.

[29] C. E. Riva, J. E. Grunwald, S. H. Sinclair, and B. L. Petrig, "Blood velocity and volumetric flow rate in human retinal vessels," *Investigative Ophthalmology & Visual Science*, vol. 26, no. 8, pp. 1124–1132, 1985.

[30] G. T. Feke, H. Tagawa, D. M. Deupree, D. G. Goger, J. Sebag, and J. J. Weiter, "Blood flow in the normal human retina," *Investigative Ophthalmology & Visual Science*, vol. 30, no. 1, pp. 58–65, 1989.

[31] J. E. Grunwald, C. E. Riva, J. Baine, and A. J. Brucker, "Total retinal volumetric blood flow rate in diabetic patients with poor glycemic control," *Investigative Ophthalmology & Visual Science*, vol. 33, no. 2, pp. 356–363, 1992.

[32] J. P. S. Garcia Jr., P. T. Garcia, and R. B. Rosen, "Retinal blood flow in the normal human eye using the Canon laser blood flowmeter," *Ophthalmic Research*, vol. 34, no. 5, pp. 295–299, 2002. doi:10.1159/000065600.

[33] A. Yoshida, G. T. Feke, H. Ogasawara, D. G. Goger, and J. W. McMeel, "Retinal hemodynamics in middle-aged normal subjects," *Ophthalmic Research*, vol. 28, no. 6, pp. 343–350, 1996. doi:10.1159/000267926.

[34] S. Szegedi, N. Hommer, M. Kallab, S. Puchner, D. Schmidl, R. M. Werkmeister, G. Garhöfer, and L. Schmetterer, "Repeatability and reproducibility of total retinal blood flow measurements using bi-directional Doppler OCT," *Translational Vision Science & Technology*, vol. 9, no. 7, art. 34, 2020.

[35] R. F. Spaide, "Choroidal blood flow: review and potential explanation for the choroidal venous anatomy including the vortex vein system," *Retina*, vol. 40, no. 10, pp. 1851–1864, 2020.

[36] A. Bill and G. O. Sperber, "Control of retinal and choroidal blood flow," *Eye*, vol. 4, no. 2, pp. 319–325, 1990. doi:10.1038/eye.1990.43.

[37] L. M. Parver, "Temperature modulating action of choroidal blood flow," *Eye*, vol. 5, no. 2, pp. 181–185, 1991.

[38] L. M. Parver, C. R. Auker, and D. O. Carpenter, "The stabilizing effect of the choroidal circulation on the temperature environment of the macula," *Retina*, vol. 2, no. 2, pp. 117–120, 1982.

[39] A. Alm and A. Bill, "Ocular and optic nerve blood flow at normal and increased intraocular pressures in monkeys (*Macaca irus*): A study with radioactively labelled microspheres including flow determinations in brain and some other tissues," *Experimental Eye Research*, vol. 15, no. 1, pp. 15–29, 1973. doi:10.1016/0014-4835(73)90185-1.

[40] T. M. Nork, C. B. Y. Kim, D. Shanmuganayagam, M. S. Van Lysel, J. N. Ver Hoeve, and J. D. Folts, "Measurement of regional choroidal blood flow in rabbits and monkeys using fluorescent microspheres," *Archives of Ophthalmology*, vol. 124, no. 6, pp. 860–868, 2006.

[41] Y. Zhang, O. San Emeterio Nateras, Q. Peng, C. A. Rosende, and T. Q. Duong, "Blood flow MRI of the human retina/choroid during rest and isometric exercise," *Investigative Ophthalmology & Visual Science*, vol. 53, no. 7, pp. 4299–4305, 2012.

[42] C. J. Pournaras, E. Rungger-Brändle, C. E. Riva, S. H. Hardarson, and E. Stefansson, "Regulation of retinal blood flow in health and disease," *Progress in Retinal and Eye Research*, vol. 27, no. 3, pp. 284–330, 2008. doi:10.1016/j.preteyeres.2008.02.002.

[43] S. S. Hayreh, "Blood flow in the optic nerve head and factors that may influence it," *Progress in Retinal and Eye Research*, vol. 20, no. 5, pp. 595–624, 2001. doi:10.1016/S1350-9462(01)00005-2.

[44] C. E. Riva, P. Titze, M. Hero, A. Movaffaghy, and B. L. Petrig, "Choroidal blood flow during isometric exercises," *Investigative Ophthalmology & Visual Science*, vol. 38, no. 11, pp. 2338–2343, 1997.

[45] S. S. Hayreh, M. B. Zimmerman, A. Kimura, and A. Sanon, "Central retinal artery occlusion. Retinal survival time," *Experimental Eye Research*, vol. 78, no. 3, pp. 723–736, 2004. doi:10.1016/S0014-4835(03)00214-8.

[46] S. Tobalem, J. S. Schutz, and A. Chronopoulos, "Central retinal artery occlusion – rethinking retinal survival time," *BMC Ophthalmology*, vol. 18, art. 101, 2018. doi:10.1186/s12886-018-0768-4.

[47] K. Shinoda, K. Yagura, S. Matsumoto, G. Terauchi, A. Mizota, and Y. Miyake, "Intraocular temperature at different sites in eye measured at the beginning of vitreous surgery," *Journal of Clinical Medicine*, vol. 10, no. 15, art. 3412, 2021. doi:10.3390/jcm10153412.

[48] R. Kobylianskyi, K. Przystupa, V. Lysko, J. Majewski, L. Vikhor, V. Boichuk, O. Zadorozhnyy, O. Kochan, M. Umanets, and N. Pasyechnikova, "Thermoelectric measuring equipment for perioperative monitoring of temperature and heat flux density of the human eye in vitreoretinal surgery," *Sensors*, vol. 25, no. 4, art. 999, 2025. doi:10.3390/s25040999.

[49] G. J. Ríos-Nequis, J. E. Covarrubias-Avilés, A. Levine-Berebichez, A. C. Esquivel-Muciño, and J. A. Ramírez-Estudillo, "Intraocular thermodynamics during vitreoretinal surgery,"

Journal of VitreoRetinal Diseases, 2026 (published online Oct. 22, 2026, ahead of print). doi:10.1177/24741264261496265.

[50] O. S. Zadorozhnyy, A. R. Korol, V. O. Naumenko, N. V. Pasyechnikova, and L. L. Butenko, "Heat exchange in the human eye: a review," *Journal of Ophthalmology (Ukraine)*, no. 6, pp. 50–58, 2022. doi:10.31288/oftalmolzh202265058.

[51] E. H. Ooi, W. T. Ang, and E. Y. K. Ng, "Bioheat transfer in the human eye: a boundary element approach," *Engineering Analysis with Boundary Elements*, vol. 31, no. 6, pp. 494–500, 2007. doi:10.1016/j.enganabound.2006.09.011.

[52] J. A. Scott, "A finite element model of heat transport in the human eye," *Physics in Medicine & Biology*, vol. 33, no. 2, pp. 227–241, 1988. doi:10.1088/0031-9155/33/2/003.

[53] V. M. M. Flyckt, B. W. Raaymakers, and J. J. W. Lagendijk, "Modelling the impact of blood flow on the temperature distribution in the human eye and the orbit: fixed heat transfer coefficients versus the Pennes bioheat model versus discrete blood vessels," *Physics in Medicine & Biology*, vol. 51, no. 19, pp. 5007–5021, 2006. doi:10.1088/0031-9155/51/19/018.

[54] D. Gongal, J. R. Hetling, and C. D. Foster, "Hypothermia treatment for ocular ischemia: A multiphase porous thermal model of human eye," *Journal of Thermal Biology*, vol. 131, p. 104139, 2025. doi:10.1016/j.jtherbio.2025.104139.

[55] M. H. Geiser, M. Bonvin, and O. Quibel, "Corneal and retinal temperatures under various ambient conditions: a model and experimental approach," *Klinische Monatsblätter für Augenheilkunde*, vol. 221, no. 5, pp. 311–314, 2004. doi:10.1055/s-2004-812878.

[56] D. Gongal et al., "Thermal finite element analysis of localized hypothermia treatment of the human eye," *Medical Engineering & Physics*, vol. 111, p. 103928, 2023. doi:10.1016/j.medengphy.2022.103928.

[57] K. H. Polderman, "Mechanisms of action, physiological effects, and complications of hypothermia," *Critical Care Medicine*, vol. 37, no. 7 (Suppl.), pp. S186–S202, 2009. doi:10.1097/CCM.0b013e3181aa5241.

[58] N. Nielsen, J. Wetterslev, T. Cronberg, et al., "Targeted temperature management at 33°C versus 36°C after cardiac arrest," *New England Journal of Medicine*, vol. 369, no. 23, pp. 2197–2206, 2013. doi:10.1056/NEJMoa1310519.

[59] J. Dankiewicz, T. Cronberg, G. Lilja, et al., "Hypothermia versus normothermia after out-of-hospital cardiac arrest," *New England Journal of Medicine*, vol. 384, no. 24, pp. 2283–2294, 2021. doi:10.1056/NEJMoa2100591.

[60] A. Lavinio, K. M. Busl, J. P. Coles, K. Donadello, R. Helbok, M. S. Sekhon, M. B. Skrifvars, F. S. Taccone, S. Wahlster, and C. Robba, "Temperature control in acute brain injury," *Intensive Care Medicine*, vol. 52, pp. 714–728, 2026. doi:10.1007/s00134-026-08367-9.

[61] M. A. Yenari and H. S. Han, "Neuroprotective mechanisms of hypothermia in brain ischaemia," *Nature Reviews Neuroscience*, vol. 13, no. 4, pp. 267–278, 2012. doi:10.1038/nrn3174.

[62] A. Quiñones-Hinojosa, J. Y. Malek, A. Ames III, C. S. Ogilvy, and K. I. Maynard, "Metabolic effects of hypothermia and its neuroprotective effects on the recovery of metabolic and electrophysiological function in the ischemic retina in vitro," *Neurosurgery*, vol. 52, no. 5, pp. 1178–1187, 2003. doi:10.1227/01.neu.0000057836.64972.a0.

[63] A. M. Mueller-Buehl et al., "Oxidative stress-induced retinal damage is prevented by mild hypothermia in an ex vivo model of cultivated porcine retinas," *Clinical & Experimental Ophthalmology*, vol. 48, no. 5, pp. 666–681, 2020. doi:10.1111/ceo.13731.

[64] E. M. Salido, D. Dorfman, M. Bordone, M. Chianelli, M. F. González Fleitas, and R. E. Rosenstein, "Global and ocular hypothermic preconditioning protect the rat retina from ischemic damage," *PLoS ONE*, vol. 8, no. 4, art. e61656, 2013. doi:10.1371/journal.pone.0061656.

[65] Hypothermia after Cardiac Arrest Study Group, "Mild therapeutic hypothermia to improve the neurologic outcome after cardiac arrest," *New England Journal of Medicine*, vol. 346, no. 8, pp. 549–556, 2002. doi:10.1056/NEJMoa012689.

[66] S. A. Bernard, T. W. Gray, M. D. Buist, B. M. Jones, W. Silvester, G. Gutteridge, and K. Smith, "Treatment of comatose survivors of out-of-hospital cardiac arrest with induced hypothermia," *New England Journal of Medicine*, vol. 346, no. 8, pp. 557–563, 2002. doi:10.1056/NEJMoa003289.

[67] J. P. Nolan, C. Sandroni, A. Cariou, T. Cronberg, S. D'Arrigo, K. Haywood, A. Hoedemaekers, G. Lilja, N. Nikolaou, T. M. Olasveengen, C. Robba, M. B. Skrifvars, P. Swindell, and J. Soar, "European Resuscitation Council and European Society of Intensive Care Medicine guidelines 2025: post-resuscitation care," *Resuscitation*, vol. 215, suppl. 1, art. 110809, 2025. doi:10.1016/j.resuscitation.2025.110809.

[68] K. S. Alotaibi, N. Saleem, T. A. Chandos Snow, P. Arina, A. Dyson, and M. Singer, "Does time to achieve a targeted body temperature matter for survivors of cardiac arrest? A systematic review and meta-analysis," *Critical Care Medicine*, vol. 54, no. 2, pp. 312–323, 2026. doi:10.1097/CCM.0000000000006964.

[69] T. Kass-Hout, C. Horn, C.-H. J. Sun, A. Yoo, O. O. Zaidat, et al., "Adjuvant systemic hypothermia during mechanical thrombectomy (ReCCLAIM II): A multicenter, randomized, controlled trial," *Stroke*, 2026 (published online Oct. 9, 2026, ahead of print). doi:10.1161/STROKEAHA.126.056651.

[70] S. Becker, Z. L'Ecuyer Morison, J. Allen, S. Saeid, L. Sturgis, A. Adderley, A. Koskelainen, and F. Vinberg, "Healing of ischemic injury in the retina," *Science Advances*, vol. 12, eadx7204, 2026. doi:10.1126/sciadv.adx7204.

[71] P. Klemm, J. Hurst, M. Dias Blak, T. Herrmann, M. Melchinger, K. U. Bartz-Schmidt, G. Zeck, M. Schultheiss, M. S. Spitzer, and S. Schnichels, "Hypothermia protects retinal ganglion cells against hypoxia-induced cell death in a retina organ culture model," *Clinical & Experimental Ophthalmology*, vol. 47, no. 8, pp. 1043–1054, 2019. doi:10.1111/ceo.13565.

[72] M. Coassin, K. G. Duncan, K. R. Bailey, A. Singh, and D. M. Schwartz, "Hypothermia reduces secretion of vascular endothelial growth factor by cultured retinal pigment epithelial cells," *British Journal of Ophthalmology*, vol. 94, no. 12, pp. 1678–1683, 2010. doi:10.1136/bjo.2009.168864.

[73] L. R. Wolin and L. C. Massopust Jr., "Selective cooling of the eye: effects on evoked potentials of the visual system," *Archives of Ophthalmology*, vol. 76, no. 5, pp. 723–728, 1966. doi:10.1001/archopht.1966.03850010725020.

[74] P. Lachapelle, J. Benoit, and P. Guité, "The effect of in vivo retinal cooling on the electroretinogram of the rabbit," *Vision Research*, vol. 36, no. 3, pp. 339–344, 1996. doi:10.1016/0042-6989(95)00126-3.

[75] M. Pitkänen, O. Kaikkonen, and A. Koskelainen, "A novel method for mouse retinal temperature determination based on ERG photoresponses," *Annals of Biomedical Engineering*, vol. 45, no. 10, pp. 2360–2372, 2017. doi:10.1007/s10439-017-1872-y.

[76] O. Kaikkonen, T. T. Turunen, A. Meller, J. Ahlgren, and A. Koskelainen, "Retinal temperature determination based on photopic porcine electroretinogram," *IEEE Transactions on Biomedical Engineering*, vol. 69, no. 2, pp. 991–1002, 2022. doi:10.1109/TBME.2021.3111533.

[77] M. Amirkavei, O. Kaikkonen, T. Turunen, A. Meller, J. Åhlgren, A. Kvanta, H. André, and A. Koskelainen, "Non-damaging laser treatment with electroretinography-based thermal dosimetry activates hormetic heat response in pig retinal pigment epithelium," *Nature Communications*, vol. 16, art. 9533, 2025. doi:10.1038/s41467-025-64095-6.

[78] D. R. May, R. J. Freedland, S. Charles, C. Wang, and J. Bakos, "Ocular hypothermia: anterior chamber perfusion," *British Journal of Ophthalmology*, vol. 67, no. 12, pp. 808–813, 1983. doi:10.1136/bjo.67.12.808.

[79] K. Tamai, E. Toumoto, and A. Majima, "Local hypothermia protects the retina from ischaemic injury in vitrectomy," *British Journal of Ophthalmology*, vol. 81, no. 9, pp. 789–794, 1997. doi:10.1136/bjo.81.9.789.

[80] Y. Zhang, M. Li, B. Yu, S. Lu, L. Zhang, S. Zhu, Z. Yu, T. Xia, et al., "Cold protection allows local cryotherapy in a clinical-relevant model of traumatic optic neuropathy," *eLife*, vol. 11, art. e75070, 2022. doi:10.7554/eLife.75070.

[81] B. S. Kim, Y. J. Kim, S. W. Seo, et al., "Long-term results from cyclocryotherapy applied to the 3-o'clock and 9-o'clock positions in blind refractory glaucoma patients," *Korean Journal of Ophthalmology*, vol. 29, no. 1, pp. 47–52, 2015. doi:10.3341/kjo.2015.29.1.47.

[82] N. Faberowski, E. Stefansson, and R. C. Davidson, "Local hypothermia protects the retina from ischemia: a quantitative study in the rat," *Investigative Ophthalmology & Visual Science*, vol. 30, no. 11, pp. 2309–2313, 1989.

[83] T. Yahalomi, I. Hecht, O. Lagstein, A. Nemet, L. Pe'er, F. Hadad, et al., "Reduction of postintravitreal injection pain using ice: an open-label interventional randomized controlled trial," *Retina*, vol. 40, no. 7, pp. 1434–1438, 2020. doi:10.1097/IAE.0000000000002608.

[84] T. Kataoka, M. Zako, M. Takeyama, A. Ohno-Jinno, et al., "Cooling prevents induction of corneal damage by argon laser peripheral iridotomy," *Japanese Journal of Ophthalmology*, vol. 51, no. 5, pp. 317–324, 2007. doi:10.1007/s10384-007-0460-3.

[85] L. I. Anatychuk et al., "Thermoelectric device for contact cooling of the human eye," *Physics and Chemistry of Solid State*, vol. 21, no. 1, pp. 140–145, 2020. doi:10.15330/pcss.21.1.140-145.

[86] R. R. Kobylianskyi, V. V. Lysko, O. S. Zadorozhnyy, and N. V. Pasechnikova, "Complex of thermoelectric equipment for diagnostics and treatment of ophthalmological diseases," *Journal of Thermoelectricity*, no. 3, pp. 101–116, 2025. doi:10.63527/1607-8829-2025-3-101-116.

[87] C. G. Besirli, S. J. Smith, D. N. Zacks, et al., "Randomized safety and feasibility trial of ultra-rapid cooling anesthesia for intravitreal injections," *Ophthalmology Retina*, vol. 4, no. 10, pp. 979–986, 2020. doi:10.1016/j.oret.2020.04.001.

[88] D. L. Chao, N. T. Rinella, A. M. Khanani, C. C. Wykoff, and G.-H. Kim, "Cooling anesthesia for intravitreal injection: Results of the prospective open-label, dose-ranging COOL-1 trial," *Clinical Ophthalmology*, vol. 15, pp. 4659–4666, 2021. doi:10.2147/OPTH.S336653.

[89] U.S. Food and Drug Administration, "De Novo classification request for OcuCool," De Novo request DEN230011, sponsor RecensMedical, Inc., received Feb. 16, 2023, granted Sept. 30, 2024. Established the generic device type "ophthalmic cooling anesthesia device" (21 CFR 886.4190, Class II, product code QZQ).

[90] A. Galor et al., "A randomized double-masked study evaluating safety and efficacy of a novel topical cooling device for chronic ocular surface pain," *Ophthalmology and Therapy*, vol. 15, pp. 2033–2050, 2026. doi:10.1007/s40123-026-01392-3.

[91] A. Galor, B. Henderson, C. Ifantides, J. A. Stefater, and T. P. Stryjewski, "The safety and tolerability of a single scleral treatment with a cooling device in rabbits," *Translational Vision Science & Technology*, vol. 14, no. 10, art. 2, 2025. doi:10.1167/tvst.14.10.2.

[92] J. C. B. Chiang, M. Wu, J.-H. Lee, A. Curkpatrick, C. Ifantides, T. Nguyen, A. Galor, H. Wu, R. Lindstrom, V. Thompson, and L. E. Downie, "Impact of an ophthalmic cooling device on corneal immune and sensory nerve features in chronic ocular surface pain: A prospective, feasibility study," *Ophthalmology and Therapy*, vol. 15, pp. 2979–2996, 2026. doi:10.1007/s40123-026-01454-6.

[93] C. Ifantides et al., "Improved corneal sensitivity after treatment with a novel topical ophthalmic cooling device in patients with chronic ocular surface pain," *Investigative Ophthalmology & Visual Science*, vol. 67, p. 3458, 2026 (ARVO Annual Meeting abstract).

[94] F. J. DiSalvo, "Thermoelectric cooling and power generation," *Science*, vol. 285, no. 5428, pp. 703–706, 1999. doi:10.1126/science.285.5428.703.

[95] X.-L. Shi, J. Zou, and Z.-G. Chen, "Advanced thermoelectric design: from materials and structures to devices," *Chemical Reviews*, vol. 120, no. 15, pp. 7399–7515, 2020.

[96] Tark Thermal Solutions, Inc., "Annular RH series RH14-14-10-L1-W4.5 thermoelectric cooler (MFG part 71062-514)," product datasheet, Rev. 00, dated June 1, 2022, print date Aug. 11, 2025.

[97] S. H. Zaferani, M. W. Sams, R. Ghomashchi, and Z.-G. Chen, "Thermoelectric coolers as thermal management systems for medical applications: design, optimization, and advancement," *Nano Energy*, vol. 90, part A, art. 106572, 2021.

[98] B. Hu, X.-L. Shi, J. Zou, and Z.-G. Chen, "Thermoelectrics for medical applications: progress, challenges, and perspectives," *Chemical Engineering Journal*, vol. 437, art. 135268, 2022.

[99] M. Zhang, Z. Li, Q. Wang, Y. Xu, P. Hu, and X. Zhang, "Performance investigation of a portable liquid cooling garment using thermoelectric cooling," *Applied Thermal Engineering*, vol. 214, art. 118830, 2022.

[100] M. A. McCall, "Pig models in retinal research and retinal disease," *Cold Spring Harbor Perspectives in Medicine*, vol. 14, no. 4, art. a041296, 2024. doi:10.1101/cshperspect.a041296.

[101] I. Sánchez, R. Martín, F. Ussa, and I. Fernández-Bueno, "The parameters of the porcine eyeball," *Graefe's Archive for Clinical and Experimental Ophthalmology*, vol. 249, no. 4, pp. 475–482, 2011.

[102] T. W. Olsen, S. Sanderson, X. Feng, and W. C. Hubbard, "Porcine sclera: thickness and surface area," *Investigative Ophthalmology & Visual Science*, vol. 43, no. 8, pp. 2529–2532, 2002.

[103] S. Nicoli, G. Ferrari, M. Quarta, C. Macaluso, P. Govoni, D. Dallatana, and P. Santi, "Porcine sclera as a model of human sclera for in vitro transport experiments: histology, SEM, and comparative permeability," *Molecular Vision*, vol. 15, pp. 259–266, 2009.

[104] F. Lorget, A. Parenteau, M. Carrier, et al., "Characterization of the pH and temperature in the rabbit, pig, and monkey eye: key parameters for the development of long-acting delivery ocular strategies," *Molecular Pharmaceutics*, vol. 13, no. 9, pp. 2891–2896, 2016. doi:10.1021/acs.molpharmaceut.5b00731.

[105] J. Rootman, "Vascular system of the optic nerve head and retina in the pig," *British Journal of Ophthalmology*, vol. 55, no. 12, pp. 808–819, 1971.

[106] S. Fouquet, O. Vacca, F. Sennlaub, and M. Paques, "The 3D retinal capillary circulation in pigs reveals a predominant serial organization," *Investigative Ophthalmology & Visual Science*, vol. 58, no. 13, pp. 5754–5763, 2017.

[107] S. Middleton, "Porcine ophthalmology," *Veterinary Clinics of North America: Food Animal Practice*, vol. 26, no. 3, pp. 557–572, 2010. doi:10.1016/j.cvfa.2010.09.002.

[108] M. L. Beauchemin, "The fine structure of the pig's retina," *Albrecht von Graefes Archiv für Klinische und Experimentelle Ophthalmologie*, vol. 190, no. 1, pp. 27–45, 1974. doi:10.1007/BF00414333.

[109] A. Hendrickson and D. Hicks, "Distribution and density of medium- and short-wavelength selective cones in the domestic pig retina," *Experimental Eye Research*, vol. 74, no. 4, pp. 435–444, 2002. doi:10.1006/exer.2002.1181.

[110] P. Törnquist, A. Alm, and A. Bill, "Studies on ocular blood flow and retinal capillary permeability to sodium in pigs," *Acta Physiologica Scandinavica*, vol. 106, pp. 343–350, 1979.

[111] Y. Zhang, J. M. Harrison, O. S. Nateras, S. Chalfin, and T. Q. Duong, "Decreased retinal-choroidal blood flow in retinitis pigmentosa as measured by MRI," *Documenta Ophthalmologica*, vol. 126, no. 3, pp. 187–197, 2013. doi:10.1007/s10633-013-9374-1.

[112] O. S. Emeterio Nateras *et al.*, "Choroidal blood flow decreases with age: An MRI study," *Current Eye Research*, vol. 39, no. 10, pp. 1059–1067, 2014. doi:10.3109/02713683.2014.892997.

[113] S. Vurgese, S. Panda-Jonas, and J. B. Jonas, "Scleral thickness in human eyes," *PLoS ONE*, vol. 7, no. 1, art. e29692, 2012.

[114] J. Ruiz-Ederra, M. García, M. Hernández, H. Urcola, E. Hernández-Barbáchano, J. Araiz, and E. Vecino, "The pig eye as a novel model of glaucoma," *Experimental Eye Research*, vol. 81, no. 5, pp. 561–569, 2005.

[115] H. Morén, P. Undrén, B. Gesslein, G. K. Olivecrona, S. Andreasson, and M. Malmsjö, "The porcine retinal vasculature accessed using an endovascular approach: a new experimental model for retinal ischemia," *Investigative Ophthalmology & Visual Science*, vol. 50, no. 11, pp. 5504–5510, 2009.

[116] S. G. Michel, G. M. LaMuraglia II, M. L. L. Madariaga, and L. M. Anderson, "Innovative cold storage of donor organs using the Paragonix Sherpa Pak devices," *Heart, Lung and Vessels*, vol. 7, no. 3, pp. 246–255, 2015.

[117] S. J. Dreyer, "Design, fabricate, and test an in vitro system to investigate therapeutic hypothermia in porcine eyes," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2017.

[118] L. Mecacci, "Development and validation of a novel device to induce localized therapeutic hypothermia," M.S. thesis, University of Illinois Chicago, Chicago, IL, 2022.

[119] L. Mecacci, Y. Nakamura, and J. R. Hetling, "Ocular hypothermia delivery system: *in vitro* validation using a non-perfused pig eye model," ARVO Annual Meeting, 2023 (poster; abstract control ID 3879496).

[120] L. Mecacci, Y. Nakamura, and J. R. Hetling, "Ocular hypothermia, in-vitro validation of a novel therapeutic apparatus using non perfused porcine eye model," Summer Biomechanics, Bioengineering and Biotransport Conference (SB3C), Lake Geneva, WI, June 11–14, 2024.

[121] L. Mecacci and J. R. Hetling, "Ocular hypothermia delivery system: *in vitro* validation using non-perfused human donor eyes," ARVO Annual Meeting, 2026 (poster; abstract control ID 4485539).

[122] J. Kong and P. Gouras, "The effect of body temperature on the murine electroretinogram," *Documenta Ophthalmologica*, vol. 106, no. 3, pp. 239–242, 2003. doi:10.1023/A:1022988332578.

[123] R. L. Lindstrom *et al.*, "Optisol corneal storage medium," *American Journal of Ophthalmology*, vol. 114, no. 3, pp. 345–356, 1992. doi:10.1016/S0002-9394(14)71803-3.

[124] A. V. Shakurov, A. V. Pushkarev, V. A. Pushkarev, and D. I. Tsiganov, "Prerequisites for developing new generation cryosurgical devices (review)," *Sovremennye Tehnologii v Medicine*, vol. 9, no. 2, pp. 178–187, 2017. doi:10.17691/stm2017.9.2.23.

[125] TE Connectivity, "GA10K3MCD1 Micro BetaCHIP (MCD) thermistor probes," datasheet, Feb. 2018.

[126] Fluke Calibration, "9102S Handheld Dry-Well," product specifications, `https://www.fluke.com/en-gb/product/calibration-tools/temperature-calibrators/fluke-calibration-9102s`, accessed Oct. 20, 2026.

[127] S. Zhang, Z. You, B. Rosner, and G.-S. Ying, "Evaluation of statistical methods for clustered eye data with skewed distribution," *Ophthalmology Science*, vol. 6, no. 2, art. 101020, 2026. doi:10.1016/j.xops.2025.101020.

[128] L. I. Los, R. J. van der Worp, M. J. A. van Luyn, and J. M. M. Hooymans, "Age-related liquefaction of the human vitreous body: LM and TEM evaluation of the role of proteoglycans and collagen," *Investigative Ophthalmology & Visual Science*, vol. 44, no. 7, pp. 2828–2833, 2003. doi:10.1167/iovs.02-0588.

[129] International Electrotechnical Commission, *IEC 60601-1: Medical electrical equipment – Part 1: General requirements for basic safety and essential performance*, consolidated ed. 3.2 (IEC 60601-1:2005+AMD1:2012+AMD2:2020), Subclause 11.1.2.2 and Table 24 (allowable maximum temperatures of applied parts). Geneva: IEC, Aug. 20, 2020.

[130] A. Krell, P. Blank, H. Ma, T. Hutzler, M. P. B. van Bruggen, and R. Apetz, "Transparent sintered corundum with high hardness and strength," *Journal of the American Ceramic Society*, vol. 86, no. 1, pp. 12–18, 2003. doi:10.1111/j.1151-2916.2003.tb03270.x.

[131] R. Apetz and M. P. B. van Bruggen, "Transparent alumina: a light-scattering model," *Journal of the American Ceramic Society*, vol. 86, no. 3, pp. 480–486, 2003. doi:10.1111/j.1151-2916.2003.tb03325.x.

[132] F. Kotz, K. Plewa, W. Bauer, et al., "Liquid glass: a facile soft replication method for structuring glass," *Advanced Materials*, vol. 28, no. 23, pp. 4646–4650, 2016. doi:10.1002/adma.201506089.

[133] F. Kotz, K. Arnold, W. Bauer, et al., "Three-dimensional printing of transparent fused silica glass," *Nature*, vol. 544, no. 7650, pp. 337–339, 2017. doi:10.1038/nature22061.

[134] H. H. Pennes, "Analysis of tissue and arterial blood temperatures in the resting human forearm," *Journal of Applied Physiology*, vol. 1, no. 2, pp. 93–122, 1948. doi:10.1152/jappl.1948.1.2.93.

[135] C. Kiel and A. J. E. Foss, "Steady-state thermal homeostasis model of heat generated and dissipated in the human retina," *Physical Biology*, vol. 23, art. 046008, 2026. doi:10.1088/1478-3975/ae9665.

[136] Tark Thermal Solutions, Inc., "Annular RH series RH14-14-06-L1-W4.5 thermoelectric cooler (MFG part 71063-505)," product datasheet, web page, accessed Oct. 31, 2026.

[137] K. Heil, R. Thomas, G. Robertson, A. Porter, R. Milner, and A. Wood, "Freezing and non-freezing cold weather injuries: A systematic review," *British Medical Bulletin*, vol. 117, no. 1, pp. 79–93, 2016. doi:10.1093/bmb/ldw001.

[138] C. Yap, A. M. Wong, J. Naor, and D. S. Rootman, "Corneal temperature reversal after storage in Chen medium compared with Optisol GS," *Cornea*, vol. 20, no. 5, pp. 501–504, 2001. doi:10.1097/00003226-200107000-00012.

[139] A. G. Robson, L. J. Frishman, J. Grigg, R. Hamilton, B. G. Jeffrey, M. Kondo, S. Li, and D. L. McCulloch, "ISCEV Standard for full-field clinical electroretinography (2022 update)," *Documenta Ophthalmologica*, vol. 144, pp. 165–177, 2022. doi:10.1007/s10633-022-09872-0.

[140] Johnson & Johnson Vision, *Clinical Assessment Guide*, Johnson & Johnson Middle East Inc., 2025 (document 2025PP11584). Grading scales adapted from N. Efron, *Contact Lens Practice*, 2nd ed., Butterworth-Heinemann, 2010.

[141] N. Efron, *Efron Grading Scales for Contact Lens Complications*, Millennium ed. Oxford, U.K.: Butterworth-Heinemann, 2000.

[142] American Veterinary Medical Association, *AVMA Guidelines for the Euthanasia of Animals: 2020 Edition*. Schaumburg, IL, USA: American Veterinary Medical Association, 2020. ISBN 978-1-882691-09-8.

[143] Y. Rotenstreich, G. A. Fishman, R. J. Anderson, and D. G. Birch, "Interocular amplitude differences of the full field electroretinogram in normal subjects," *British Journal of Ophthalmology*, vol. 87, no. 10, pp. 1268–1271, 2003. doi:10.1136/bjo.87.10.1268.

[144] K.-E. Choi, V. T. Q. Anh, J.-H. Oh, C. Yun, and S.-W. Kim, "Normative data of axial length, retinal thickness measurements, visual evoked potentials, and full-field electroretinography in female, wild-type minipigs," *Translational Vision Science & Technology*, vol. 10, no. 12, art. 3, 2021. doi:10.1167/tvst.10.12.3.

[145] G. A. Fishman, A. V. Chappelow, R. J. Anderson, Y. Rotenstreich, and D. J. Derlacki, "Short-term intervisit variability of ERG amplitudes in normal subjects and patients with retinitis pigmentosa," *Retina*, vol. 25, no. 8, pp. 1014–1021, 2005. doi:10.1097/00006982-200512000-00010.

[146] C. Sommer, F. Turgut, A. Saad, M. D. Becker, M. Al-Sheikh, C. Q. Davis, U. Rahn, M. T. S. Barboni, D. C. DeBuc, and G. M. Somfai, "The reproducibility of handheld ERGs recorded with skin electrode and natural pupils," *Documenta Ophthalmologica*, 2025. doi:10.1007/s10633-025-10065-8.

[147] K. S. Freeman, K. L. Good, P. H. Kass, S. A. Park, B. Nestorowicz, and R. Ofri, "Effects of chemical restraint on electroretinograms recorded sequentially in awake, sedated, and anesthetized dogs," *American Journal of Veterinary Research*, vol. 74, no. 7, pp. 1036–1042, 2013. doi:10.2460/ajvr.74.7.1036.

[148] N. B. Sørensen, A. T. Christiansen, T. W. Kjær, K. Klemp, M. la Cour, and J. F. Kiilgaard, "Time-dependent decline in multifocal electroretinogram requires faster recording procedures in anesthetized pigs," *Translational Vision Science & Technology*, vol. 6, no. 2, art. 6, 2017. doi:10.1167/tvst.6.2.6.

[149] M. Gencer and M. Yalvaç, "Comparison of the effects of spinal versus general anesthesia on the perioperative intraocular pressure: A prospective, randomized clinical trial," *Saudi Medical Journal*, vol. 47, pp. 1549–1557, 2026. doi:10.15537/1658-3175.8815.

[150] M. Mohammadzadeh, R. Azargoun, and R. Mohammadi, "Sedative effects of dexmedetomidine and ketamine or dexmedetomidine and midazolam on tear production, ocular perfusion pressure and pupil diameter in rabbits," *Veterinary Medicine and Science*, vol. 12, e71017, 2026. doi:10.1002/vms3.71017.

[151] C. M. Nicou, A. Pillai, and C. L. Passaglia, "Effects of acute stress, general anesthetics, tonometry, and temperature on intraocular pressure in rats," *Experimental Eye Research*, vol. 210, art. 108727, 2021. doi:10.1016/j.exer.2021.108727.

[152] G. J. Ortiz, D. J. Cook, M. E. Yablonski, H. Masonson, and G. Harmon, "Effect of cold air on aqueous humor dynamics in humans," *Investigative Ophthalmology & Visual Science*, vol. 29, no. 1, pp. 138–140, 1988.

[153] T. C. B. de Oliveira, J. L. M. Ferreira, H. T. de Lima, C. O. A. Bezerra Filho, and J. C. Ribeiro, "Intraocular pressure modulation with thermal stimuli," *Arquivos Brasileiros de Oftalmologia*, vol. 88, no. 1, art. e2023-0083, 2025. doi:10.5935/0004-2749.2023-0083.

[154] S. Orgül, J. Flammer, D. Stümpfig, and P. Hendrickson, "Intraocular pressure decrease after local ocular cooling is underestimated by applanation tonometry," *International Ophthalmology*, vol. 19, no. 2, pp. 95–99, 1995. doi:10.1007/BF00133179.

[155] P. Dhungel, M. K. Alanazi, P. Caroline, L. Yudcovitch, and M. Liu, "Short-term impact of scleral lens wear on intraocular pressure and retinal nerve fiber layer thickness," *Life*, vol. 16, no. 7, art. 1094, 2026. doi:10.3390/life16071094.

[156] C. Martinez-Perez, M. C. Sánchez-González, and J.-M. Sánchez-González, "Corneal and intraocular pressure responses to scleral lens wear: A meta-analysis," *Ophthalmic and Physiological Optics*, vol. 46, pp. 765–778, 2026. doi:10.1007/s44402-026-00110-7.

[157] P. S. Bilkhu, J. S. Wolffsohn, S. A. Naroo, L. Robertson, and R. Kennedy, "Effectiveness of nonpharmacologic treatments for acute seasonal allergic conjunctivitis," *Ophthalmology*, vol. 121, no. 1, pp. 72–78, 2014. doi:10.1016/j.ophtha.2013.08.007.

[158] G. W. Aylward, K. A. McClellan, R. Thomas, and F. A. Billson, "Transient corneal changes associated with the use of gold foil electrodes," *British Journal of Ophthalmology*, vol. 73, no. 12, pp. 980–984, 1989. doi:10.1136/bjo.73.12.980.

[159] B. C. Ackermann, L. Skrzypczyk, L. Geisweid, N. Junker, S. Wohlfart, P. Uhl, G. U. Auffarth, V. A. Augustin, and M. Hammer, "Increasing dark adaption times and ERG response in landrace pigs – An intraindividual comparison in 20 animals," *Experimental Eye Research*, vol. 262, art. 110733, 2026. doi:10.1016/j.exer.2025.110733.

[160] T. Ban, "Development of the multi-electrode electroretinogram: Technology, application and commercialization," Ph.D. dissertation, University of Illinois at Chicago, Chicago, IL, 2011.

[161] Y. Krakova, "Development of a multi-electrode electroretinography system and characterization of meERG signals in rat," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2012.

[162] G. Balasubramanian, "Development of multielectrode electroretinography (meERG) for human clinical use," Ph.D. dissertation, University of Illinois Chicago, Chicago, IL, 2025.

[163] S. Patangay, Z. Derafshi, T. S. Vajaranant, J. C. Park, E. Ghahari, J. J. McAnany, and J. R. Hetling, "Three dimensional stimulus source for pattern electroretinography in mid- and far-peripheral retina," *Translational Vision Science & Technology*, vol. 7, no. 1, art. 8, 2018. doi:10.1167/tvst.7.1.8.

[164] P. Roy, "Peripheral pattern electroretinography: System upgrade, novel protocol development, & clinical validation," Ph.D. dissertation, University of Illinois Chicago, Chicago, IL, 2025.

[165] L. Bentivoglio, "Design and fabrication of a synchronously updated 3-D source for retinal electrodiagnostic testing," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2021.

[166] E. Puddu, "Design, fabrication and testing of a novel stimulus source for visual electrophysiology," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2022.

[167] S. Salameh, "First working prototype of a configurable full-field stimulus source for electrodiagnostic vision testing," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2024.

[168] S. Regal, J. Troughton, R. Delattre, T. Djenizian, and M. Ramuz, "Changes in temperature inside an optomechanical model of the human eye during emulated transscleral cyclophotocoagulation," *Biomedical Optics Express*, vol. 11, no. 8, pp. 4548–4559, 2020. doi:10.1364/BOE.385016.

[169] R. W. Carr, S. Pianova, J. Fernandez, J. B. Fallon, C. Belmonte, and J. A. Brock, "Effects of heating and cooling on nerve terminal impulses recorded from cold-sensitive receptors in the guinea-pig cornea," *The Journal of General Physiology*, vol. 121, no. 5, pp. 427–439, 2003. doi:10.1085/jgp.200308814.

[170] IT'IS Foundation, *Tissue Properties Database*, version 5.0 (released August 21, 2025). Zurich: IT'IS Foundation. doi:10.13099/VIP21000-05-0; `https://itis.swiss/database`. Values compared: blood density 1049.75 $\mathrm{kg\,m^{-3}}$ and heat capacity 3617 $\mathrm{J\,kg^{-1}\,K^{-1}}$; water heat capacity 4178 $\mathrm{J\,kg^{-1}\,K^{-1}}$ and thermal conductivity 0.6045 $\mathrm{W\,m^{-1}\,K^{-1}}$.

[171] Z. Hashin and S. Shtrikman, "A variational approach to the theory of the effective magnetic permeability of multiphase materials," *Journal of Applied Physics*, vol. 33, no. 10, pp. 3125–3131, 1962. doi:10.1063/1.1728579.

[172] K. Trier, "The sclera," in *The Biology of the Eye*, ser. Advances in Organ Biology, vol. 10. Amsterdam: Elsevier, 2005, pp. 353–373. doi:10.1016/S1569-2590(05)10013-5.

[173] P. Asbell and D. Brocks, "Cornea overview," in *Encyclopedia of the Eye*, D. A. Dartt, Ed. Oxford: Academic Press, 2010, vol. 1, pp. 522–531. doi:10.1016/B978-0-12-374203-2.00058-0.

[174] S. Hayes, T. White, C. Boote, C. S. Kamma-Lorger, J. Bell, T. Sorenson, N. Terrill, O. Shebanova, and K. M. Meek, "The structural response of the cornea to changes in stromal hydration," *Journal of the Royal Society Interface*, vol. 14, no. 131, art. 20170062, 2017. doi:10.1098/rsif.2017.0062.

[175] Y. M. Michelacci, "Collagens and proteoglycans of the corneal extracellular matrix," *Brazilian Journal of Medical and Biological Research*, vol. 36, no. 8, pp. 1037–1046, 2003. doi:10.1590/S0100-879X2003000800009.

[176] R. F. Fisher and B. E. Pettet, "Presbyopia and the water content of the human crystalline lens," *The Journal of Physiology*, vol. 234, no. 2, pp. 443–447, 1973. doi:10.1113/jphysiol.1973.sp010353.

[177] N. K. Tram and K. E. Swindle-Reilly, "Rheological properties and age-related changes of the human vitreous humor," *Frontiers in Bioengineering and Biotechnology*, vol. 6, art. 199, 2018. doi:10.3389/fbioe.2018.00199.

[178] O. İnam, H. J. Kaplan, and T. H. Tezel, "Retinal hydration assessment with optical coherence tomography: Unraveling its significance in retinal fluid dynamics, macular edema and cell viability," *Translational Vision Science & Technology*, vol. 12, no. 8, art. 4, 2023. doi:10.1167/tvst.12.8.4.

[179] R. M. Forbes, A. R. Cooper, and H. H. Mitchell, "The composition of the adult human body as determined by chemical analysis," *Journal of Biological Chemistry*, vol. 203, no. 1, pp. 359–366, 1953. doi:10.1016/S0021-9258(19)52646-1.

[180] H. H. Mitchell, T. S. Hamilton, F. R. Steggerda, and H. W. Bean, "The chemical composition of the adult human body and its bearing on the biochemistry of growth," *Journal of Biological Chemistry*, vol. 158, no. 3, pp. 625–637, 1945. doi:10.1016/S0021-9258(19)51339-4.

[181] Y. Choi and M. R. Okos, "Effects of temperature and composition on the thermal properties of foods," in *Food Engineering and Process Applications*, vol. 1, M. Le Maguer and P. Jelen, Eds. London: Elsevier Applied Science, 1986, pp. 93–101.

[182] ASHRAE, "Thermal properties of foods," in *2002 ASHRAE Handbook—Refrigeration* (SI ed.). Atlanta, GA: American Society of Heating, Refrigerating and Air-Conditioning Engineers, 2002, ch. 8.

[183] E. E. García Rojas, J. S. R. Coimbra, and J. Telis-Romero, "Thermophysical properties of cotton, canola, sunflower and soybean oils as a function of temperature," *International Journal of Food Properties*, vol. 16, no. 7, pp. 1620–1629, 2013. doi:10.1080/10942912.2011.604889.

[184] H. Fischer, I. Polikarpov, and A. F. Craievich, "Average protein density is a molecular-weight-dependent function," *Protein Science*, vol. 13, no. 10, pp. 2825–2828, 2004. doi:10.1110/ps.04688204.

[185] M. E. Langham, R. A. Farrell, V. O'Brien, D. M. Silver, and P. Schilder, "Blood flow in the human eye," *Acta Ophthalmologica*, vol. 67, suppl. 191, pp. 9–13, 1989. doi:10.1111/j.1755-3768.1989.tb07080.x.

[186] G. T. Dorner, E. Polska, G. Garhöfer, C. Zawinka, B. Frank, and L. Schmetterer, "Calculation of the diameter of the central retinal artery from noninvasive measurements in humans," *Current Eye Research*, vol. 25, no. 6, pp. 341–345, 2002. doi:10.1076/ceyr.25.6.341.14231.

[187] L. M. Wilcox Jr., E. M. Keough, R. J. Connolly, and C. E. Hotte, "Comparative extraocular muscle blood flow," *Journal of Experimental Zoology*, vol. 215, no. 1, pp. 87–90, 1981. doi:10.1002/jez.1402150110.

[188] O. A. Larsen, N. A. Lassen, and F. Quaade, "Blood flow through human adipose tissue determined with radioactive xenon," *Acta Physiologica Scandinavica*, vol. 66, no. 3, pp. 337–345, 1966. doi:10.1111/j.1748-1716.1966.tb03208.x.

[189] J. W. Kiel, M. Hollingsworth, R. Rao, M. Chen, and H. A. Reitsamer, "Ciliary blood flow and aqueous humor production," *Progress in Retinal and Eye Research*, vol. 30, no. 1, pp. 1–17, 2011. doi:10.1016/j.preteyeres.2010.08.001.

[190] Z. Wang, Z. Ying, A. Bosy-Westphal, J. Zhang, B. Schautz, W. Later, S. B. Heymsfield, and M. J. Müller, "Specific metabolic rates of major organs and tissues across adulthood: Evaluation by mechanistic model of resting energy expenditure," *The American Journal of Clinical Nutrition*, vol. 92, no. 6, pp. 1369–1377, 2010. doi:10.3945/ajcn.2010.29885.

[191] R. M. Werkmeister, D. Schmidl, G. Aschinger, V. Doblhoff-Dier, S. Palkovits, M. Wirth, G. Garhöfer, R. A. Linsenmeier, R. A. Leitgeb, and L. Schmetterer, "Retinal oxygen extraction in humans," *Scientific Reports*, vol. 5, art. 15763, 2015. doi:10.1038/srep15763.

[192] A. A. Abusharha, A. Al Yami, K. Alsreea, R. Fagehi, A. Alsaqr, S. Alanazi, and A. Masmali, "Repeatability and reproducibility of tear film evaporation rate measurement using a new closed-chamber evaporimeter," *The Open Ophthalmology Journal*, vol. 15, pp. 117–121, 2021. doi:10.2174/1874364102115010117.

[193] R. Fagehi, G. A. El-Hiti, M. H. Alsubaie, A. Abusharha, M. A. Alanazi, A. M. Masmali, and T. Almubrad, "Measurements of tear evaporation rate in subjects with refractive errors using a portable evaporimeter," *Healthcare*, vol. 10, no. 2, art. 405, 2022. doi:10.3390/healthcare10020405.

[194] U. Cicekli, "Computational model for heat transfer in the human eye using the finite element method," M.S. thesis, Louisiana State University, Baton Rouge, LA, 2003. LSU Master's Theses 2762.

[195] E. Y. K. Ng and E. H. Ooi, "FEM simulation of the eye structure with bioheat analysis," *Computer Methods and Programs in Biomedicine*, vol. 82, no. 3, pp. 268–276, 2006. doi:10.1016/j.cmpb.2006.04.001.

[196] A. F. Emery, P. Kramar, A. W. Guy, and J. C. Lin, "Microwave induced temperature rises in rabbit eyes in cataract research," *Journal of Heat Transfer*, vol. 97, no. 1, pp. 123–128, 1975. doi:10.1115/1.3450259.

[197] F. A. Duck, "Thermal properties of tissue," in *Physical Properties of Tissues: A Comprehensive Reference Book*. London: Academic Press, 1990, ch. 2, pp. 9–42. doi:10.1016/B978-0-12-222800-1.50006-1.

[198] T. Okuno, "Thermal effect of infra-red radiation on the eye: A study based on a model," *The Annals of Occupational Hygiene*, vol. 35, no. 1, pp. 1–12, 1991. doi:10.1093/annhyg/35.1.1.

[199] J. J. W. Lagendijk, "A mathematical model to calculate temperature distributions in human and rabbit eyes during hyperthermic treatment," *Physics in Medicine and Biology*, vol. 27, no. 11, pp. 1301–1311, 1982. doi:10.1088/0031-9155/27/11/001.

[200] S. C. DeMarco, G. Lazzi, W. Liu, J. D. Weiland, and M. S. Humayun, "Computed SAR and thermal elevation in a 0.25-mm 2-D model of the human eye and head in response to an implanted retinal stimulator—Part I: Models and methods," *IEEE Transactions on Antennas and Propagation*, vol. 51, no. 9, pp. 2274–2285, 2003. doi:10.1109/TAP.2003.816395.

[201] P. Bernardi, M. Cavagnaro, S. Pisa, and E. Piuzzi, "SAR distribution and temperature increase in an anatomical model of the human eye exposed to the field radiated by the user antenna in a wireless LAN," *IEEE Transactions on Microwave Theory and Techniques*, vol. 46, no. 12, pp. 2074–2082, 1998. doi:10.1109/22.739285.

[202] K. C. Gokul, D. B. Gurung, and P. R. Adhikary, "Thermal effects of eyelid in human eye temperature model," *Journal of Applied Mathematics & Informatics*, vol. 32, no. 5–6, pp. 649–663, 2014. doi:10.14317/jami.2014.649.

[203] Joint Committee for Guides in Metrology, *Evaluation of Measurement Data — Guide to the Expression of Uncertainty in Measurement*, JCGM 100:2008 (GUM 1995 with minor corrections), 1st ed. Sèvres: BIPM, 2008.

[204] I. B. Celik, U. Ghia, P. J. Roache, C. J. Freitas, H. Coleman, and P. E. Raad, "Procedure for estimation and reporting of uncertainty due to discretization in CFD applications," *Journal of Fluids Engineering*, vol. 130, no. 7, art. 078001, 2008. doi:10.1115/1.2960953.

[205] Coolants Plus, Inc., "Propylene Glycol, Industrial Grade (Starfire)," product data sheet, Form No. 117-01540-0604X-AAD. Typical properties at 25 °C: thermal conductivity 0.2061 $\mathrm{W\,m^{-1}\,K^{-1}}$, specific heat 2.51 $\mathrm{J\,g^{-1}\,K^{-1}}$, viscosity 48.6 mPa s.

[206] STMicroelectronics, "STM32L476xx: Ultra-low-power Arm Cortex-M4 32-bit MCU+FPU, 100DMIPS, up to 1MB flash, 128 KB SRAM, USB OTG FS, LCD, ext. SMPS," datasheet DS10198, Rev. 11, 2024.

[207] Analog Devices, "ADN8831: Thermoelectric cooler (TEC) controller," data sheet D04663, Rev. C, 2019.

[208] Sensirion, "SLF3S-4000B liquid flow sensor," data sheet, version 1.2, November 2022.

[209] Aosong Electronics, "AHT20 humidity and temperature sensor," data sheet, version 1.0, May 2021.

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Ph.D., Biomedical Engineering, University of Illinois Chicago, Chicago, Illinois, 2027

RESEARCH AND PROFESSIONAL EXPERIENCE

- Graduate Researcher, Neural Engineering Vision Laboratory (NEVL), University of Illinois Chicago, 2021–present. Design and validation of a scleral-contact thermoelectric ocular cooling device for optic-neural tissue protection.
- Engineering Intern, R&D, RetMap Inc., Chicago, Illinois, Summers 2023, 2024, and 2025.
- Graduate Researcher, Innovation Center, University of Illinois Chicago, 2021–2022.

TEACHING

Teaching Assistant, Department of Biomedical Engineering, University of Illinois Chicago, 2023–2025.

PUBLICATIONS

- Y. Nakamura, L. Mecacci, and J. R. Hetling, "Therapeutic levels of hypothermia achieved in isolated porcine eyes using a scleral contact interface," *Annals of Biomedical Engineering*, vol. 54, 2026 (published online November 2025).
- L. Mecacci, Y. Nakamura, K. Fong, N. Shirdade, and J. R. Hetling, "An ex vivo system to study clinically feasible induction of therapeutic hypothermia in porcine eyes," *Journal of Neuroscience Methods* (manuscript pending submission).
- L. Mecacci, Y. Nakamura, and J. R. Hetling, "Ocular hypothermia delivery system: in vitro validation using a non-perfused pig eye model," ARVO Annual Meeting, 2023 (poster).
- L. Mecacci, Y. Nakamura, and J. R. Hetling, "Ocular hypothermia, in-vitro validation of a novel therapeutic apparatus using non perfused porcine eye

model," Summer Biomechanics, Bioengineering and Biotransport Conference (SB3C), Lake Geneva, WI, 2024.

- L. Mecacci and J. R. Hetling, "Ocular hypothermia delivery system: in vitro validation using non-perfused human donor eyes," ARVO Annual Meeting, 2026 (poster).

PATENTS AND GRANTS

- Co-inventor, U.S. patent application "Ocular Cooling Devices and Associated Methods" (filed November 2024) and PCT application (EU/UK), building on a November 2023 provisional; technology exclusively licensed to Precision Ocular Cooling, Inc. for commercial development.
- ARVO EyeFind Research Grant, 2023.

SERVICE

Secretary, Biomedical Engineering Association, Politecnico di Milano, 2018–2020.

LANGUAGES

Italian (native); English (professional working proficiency; IELTS 8.0/9.0); Spanish (limited working).